

HYPOCHONDRIA

IN SICKNESS AND IN STORY

SUSANNAH B. MINTZ

HYPOCHONDRIA



OceanofPDF.com

HYPOCHONDRIA

In Sickness and in Story

Susannah B. Mintz

REAKTION BOOKS

OceanofPDF.com

Published by
REAKTION BOOKS LTD
2–4 Sebastian Street
London EC1V 0HE, UK
www.reaktionbooks.co.uk

First published 2026
Copyright © Susannah B. Mintz 2026

All rights reserved

EU GPSR Authorised Representative
Logos Europe, 9 rue Nicolas Poussin, 17000, La Rochelle, France
email: contact@logoseurope.eu

No part of this publication may be reproduced, stored in a retrieval system or transmitted, in any form or by any means, electronic, mechanical, photocopying, recording or otherwise, without the prior permission of the publishers. No part of this publication may be used or reproduced in any manner for the purpose of training artificial intelligence technologies or systems.

Page references in the Index match the printed edition of this book.

Printed and bound in Great Britain by Bell & Bain, Glasgow

A catalogue record for this book is available from the British Library

eISBN 978 1 83639 158 6

OceanofPDF.com

Contents

Introduction:

Hypochondria Exists

1 Pleading the Case:

Hypochondria Speaks

2 Bodies in Time:

Hypochondria Stalls

3 Hyp-istemology:

Hypochondria Knows

4 Bodies of Matter:

Hypochondria Consolidates

5 Illness on the Doorstep:

Hypochondria Precedes

6 The Wisdom of Worry:

Hypochondria Ages

Afterword:

Hypochondria Cares

REFERENCES

SELECT BIBLIOGRAPHY

ACKNOWLEDGEMENTS

INDEX

OceanofPDF.com

Introduction:

Hypochondria Exists

‘Physical medicine is a country with boundaries that are artificially maintained in order to limit the doctor’s obligation.’

D. W. WINNICOTT, ‘The Psycho-Somatic Field’^{[1](#)}

As a schoolgirl in Philadelphia in the 1970s, studying William Penn and the early Quakers, I became convinced that I had yellow fever. I remember distinctly the creeping sensation of panic as I sat in the library with a textbook in my lap and every symptom of the awful disease glaringly obvious in my own body: the achiness, the nausea, the spots on my skin that I was sure were jaundice. When I was sent to the school nurse, feeling faint and flushed (now fully submerged in panic), I held my tongue for fear of being proved right; and in the car driving home I could only hesitantly admit to my mother what was happening to me, so certain was I of the agony to come. This was the era of my brain tumour, too; and a series of not-really-broken digits that I cradled in my lap at the dinner table; and the heart attack when my throat tightened with sadness over the wrenching novel version of *West Side Story* (that tragic scene where Maria in a white dress runs towards Tony, who has been shot), and I locked myself in the bathroom, chest pounding, until my father came to knock on the door asking me what was wrong. Each of these instances was characterized by an overwhelming certitude that the worst I could think of was the only reasonable explanation for the sensations I felt and a fear that naming the problem out loud would not only confirm my demise but burden my parents with an unendurable grief – to say nothing of the hassle of carting me off to hospital.

The yellow fever episode, now encoded in family lore as a source of affectionate amusement, exemplifies one etiology of hypochondriacal

anxiety. I learned early that I was a ‘suggestible’ child who, like many frightened readers who appear in stories of hypochondria, both literary and scientific, would catch whatever I encountered in books. As Catherine Belling puts it in her 2012 book *A Condition of Doubt*, hypochondria frequently begins ‘with subjects who read bodies and books onto, into and against each other. For this reason, certain childhood reading experiences are often associated with the onset of hypochondria.’² Later I was cautioned that majoring in psychology in college would expose me to a host of psychiatric conditions sure to activate my gullible imagination (‘medical students’ disease’ dates back centuries);³ I should have known that becoming an English major instead was hardly a defence against this brand of fear. These days the internet exposes many more of us than health sciences students to the perils of catching the latest disease we encounter in the news, and ‘cyberchondria’ refers to the fact that trawling sites like WebMD doesn’t just inform but can actually *create* an experience of sickness.⁴ It took a few years, but I did eventually learn to place my own direst and absolute certainty in some ‘proper’ category of vigilance, since I was rarely proven right in the form of verifiable disease. Many of us who call ourselves hypochondriacal grow to understand that the more frequently our worst fears don’t pan out, the less likely they are to be true the next time. But of course even hypochondriacs get cancer, as the inside joke goes, and then a novel virus arrived at our doors. Never is the interface between personal fears about being ill and the definition of appropriate, salutary health precautions more charged than in the context of a pandemic.

It was during the first phase of COVID-19 that I began to think about the puzzling collision of hypochondriacal worry and a life-threatening disease sweeping through our neighbourhoods. (As Judy Z. Segal wrote in 2005 of SARS, ‘A cough that would have been unremarkable only days before becomes alarming’ once a virus hits ‘the hypochondriac’s radar’.)⁵ Was it hysteria that made me feel toxically contaminated after a thirty-second encounter with a delivery person outside my house, or a reasonable acknowledgement that an unmasked, as-yet-unvaccinated interchange might expose me to peril? A public health crisis in which the possibility of illness becomes unignorable will heighten a sense of vulnerability even for those who do not identify as hypochondriacs. At the start of the pandemic some quickly adopted masking and social distancing as necessary and

uncomplicated safety measures; for others, these actions seemed to capitulate to an uninformed, even infantile, fear of sickness and death. My interest had less to do with which of these responses to a communicable disease was medically or bioethically correct and more to do with the conundrum of scale. At what point does prudence tip into hypervigilance or confidence become recklessness? And why do we laugh at one but not the other? Hypochondriacs are typically described as ridiculous fantasists, after all (certainly in literature), rather than sufferers of legitimate psychosomatic phenomena: they are made fun of, not cared for. Now with the acute phases of the pandemic behind us, the symptoms of long- or post-COVID – which are not easily aggregated or treated as a singular syndrome – have given rise to new accusations of irrational fear. Whatever cannot be verified through clinical observation or testing is easily underplayed as anxiety and depression, a phenomenon that both discounts the possibility that an organic cause may yet be discovered and perpetuates the stigma of psychological distress as somehow weak and even unreal.

The COVID-19 era has also brought into high relief an especially vexed aspect of hypochondria: its simultaneous distrust of and appeal to healthcare expertise. Until the most recent edition of the American Psychiatric Association's *Diagnostic and Statistical Manual of Mental Disorders* (*DSM*), the diagnostic criteria for hypochondriasis included resistance to medical reassurance as one of its defining features. While some hypochondriacs are distinguished by their avoidance of doctors as a defence against finding out the worst, the condition is primarily characterized by the opposite behaviour, an excessive reliance on the healthcare system – repeated trips to the doctor or to A&E, requests for diagnostic testing or referrals to specialists, insistence on nosological specificity – even as the results of such measures are repeatedly disbelieved. The frustration of this relationship extends in both directions. Providers may feel they're contending with a garrulous deluge of factoids and theories from a person who autocratically demands to be affirmed as correct in the absence of proof – repeat patients seem to refuse what is sought, a negative diagnosis – all the while anticipating the mountain of insurance paperwork awaiting them on their desk or quelling the urge to assert superior evidence of likelihoods and probabilities.⁶ But the patient has an equivalent surety, an embodied one, that something is amiss as well as a politicized conviction of rights to informed consent and treatment

options. In the United States, certainly, the segmented nature of the healthcare system, privatized and siloed by specialty, can exacerbate a frustrated sense that no one individual could ever adequately or accurately detect the problem. This in turn can lead to protracted diagnostic odysseys as parties on either side of the divide square off over a struggle for final adjudication. And even if they end in a compelling diagnosis, such situations tend to stoke a sense that for all the vaunted prestige of medicine and our hope for definitive answers, we are in fact distressingly variable, even inscrutable, beings.

Carlton A. Hornung and Michael Massagli wrote some years ago that professional impatience towards hypochondria abounds in the front-line encounter of general practice, typically a patient's 'initial contact with the health care system' and so prone to exacerbating disagreements over competence, clout and one's appropriate role in a clinical setting.⁷ But an enduring problematic in the history of hypochondria, as I will detail in subsequent chapters, is where it properly belongs in the first place: in the emergency room, a department of psychiatry, a therapist's office, a pharmacy – or a history textbook, a literature course, a philosophical treatise. At various points in hypochondria's lengthy genealogy, all of these locations have been pertinent to how the condition is understood. Even in its earliest manifestation in the Hellenic period as a disorder of the internal organs – *hypokhondria* refers to the area of the body 'under the cartilage of the breastbone', the upper abdomen – hypochondria was bound up with ideas about how to be a good citizen of a healthy, orderly *polis* (Plato's *Republic* is discussed in [Chapter One](#)). In later centuries it has become entangled in questions of religious devotion, national identity, gendered and racial disempowerment and academic debates about mind, the unconscious and the self. Scholars have argued that *all* disease has social history and meaning, but hypochondria has been an exceptionally contested condition in its millennia of existence. That it is now a phenomenon primarily *enacted* in clinical contexts, *defined* in psychiatric terms and *symbolized* in popular media suggests hypochondria's unique mutability. For what *is* hypochondria if not a contradiction – a condition all in the head, whose content is the body; a condition defined by the fact that it isn't (organic disease), indeed that becomes justified, reified, only at the moment of its own disproving. To be 'really' ill, after all – presumably the hypochondriac's quest – is to be legitimized as *not* a hypochondriac.

A fundamental proposition of this study is that hypochondria is an identity position as much as a medical or psychiatric circumstance. Who are we *being* when we are *behaving* hypochondriacally? Suggestibility as one precondition for hypochondria places responsibility for the condition within the individual, whether psychologically or physiologically, but what would change in our thinking about hypochondria if we instead emphasized *proximity* – the encounter with possibility that derives from fictional characters, a textbook of gut-churning terminology, televised statistics of hospitalizations and deaths, loved ones becoming ill or a single experience of illness in oneself, surely an indication that more might come? In each of these circumstances, *knowledge* is a powerful if unexpected trigger for subsequent fear: it is not that we ought to know better than to feel the shivers of concern at the merest anomaly or discomfort, but that knowing something of sickness, witnesses to the vagaries of embodiment as we humans inevitably are, we are also susceptible and suspicious. The indeterminacy of the fact that everything could be a symptom of *something* raises many more questions than just diagnostic ones, engaging a field of interpretive potential and clash of answers, well beyond the clinical setting. For who knows best what bodymind sensation really means?⁸ To whose authority should we appeal for answers to the urgent questions of our mortality – who gets to decide? To what extent can we maintain autonomy, dignity, self-determination where health and illness are concerned – who gets to intervene? And to what extent is hypochondria a collective phenomenon? Scholars have suggested that hypochondria tends to circulate not only in families, whether it is inherited or learned, but within communities. This relates to large-scale epidemics as well as to the faddishness of both health scrutiny and particular conditions – early modern melancholy, for example, or the Victorian era’s neurasthenia. But it also implies that hypochondria might encourage connection, empathy, understanding. What do we convey to, and learn from, each other through illness worry that might have more to do with personhood than with sickness?

This is precisely the question animating Harriet Martineau’s 1844 essay collection *Life in the Sick-Room*. In a piece titled ‘Life to the Invalid’, journalist and social theorist Martineau declares that she ‘could fill a volume’ with the ‘many truths’ made apparent to her from her bedridden years.⁹ Of this period in Martineau’s life, Maria Frawley notes that the

author ‘provides little overt explanation of what accounts for her “sequestration”’,¹⁰ but overwork and anxiety seem to have combined in a protracted state of exhausted debility that resembles the similarly indistinct but strongly felt distress of other self-professed hypochondriacs to be discussed throughout this study. And like many others, Martineau renders sickness an opportunity for heightened intellectual activity – not despite but because of an illness that, vague as it might be in strict diagnostic terms, afforded her time and focus. It’s worth ‘remind[ing] the healthy and busy’, Martineau announces, that ‘there may be influences in the life of the meditative invalid which may render his views more comprehensive, and his judgments more, rather than less, sound than heretofore.’¹¹ Several points emerge here that will be central to my discussion of hypochondria, especially as a condition that highlights the arbitrariness of ‘legitimate’ sickness, that redefines productivity and what it means to ‘do’ and that boasts creative, generative potential as a form of attentiveness. Thus Martineau writes that ‘when [she] think[s] of what [she has] seen with [her] own eyes from one back window’, she could ‘analyze life in its entirety’. The hypochondriacal state produces a certain vibrancy of mental work, the sick perspective seeming to distil and concentrate ‘meaning’ to a kind of essential form: ‘there is much . . . in our condition, which it will be wiser . . . to observe than to judge of.’¹² Of course Martineau does *suffer* as an invalid – a word that recurs throughout the ten essays in this volume – but so too in sickness does she become an eloquent version of herself. Maria Frawley puts it succinctly in her introduction to *Life in the Sick-Room*: ‘Sickness was . . . the linchpin of [Martineau’s] self-understanding.’¹³

As an early sociologist, Martineau believed firmly in health as an important indicator of what she called the ‘morals and manners’ of a society.¹⁴ If, as *Sick-Room* articulates, we become ourselves in illness (itself a counterpoint to the more popular idea that illness alienates us from normalcy, so that we must recover to ‘feel like myself’ again), so too do we find out something important by observing how we *respond* to illness as a people. The import of disease is not just figurative in Martineau’s work. It is instead a real consequence of social inequities and hardships that primarily affect women, children and the aged, often construed as a direct result of patriarchal subjugation – as when girls are destined ‘to the languid lot of invalidism’ because they are deprived of sufficient physical activity.¹⁵ Such

arguments disrupt a centuries-old equation of femaleness with biological weakness in the very moment of restoring corporeality to a critique of social systems. Embodiment is foundational to sociological knowledge, in other words, but not because our physical characteristics limit in a deterministic way who we are or may become. This means that when a woman takes to her sickbed, she is far from performing an essential femininity; to the contrary, she registers the quality of the world around her, both its demands (as of work or domestic life) and its affordances. As for several of the hypochondriacs to be discussed below, illness served Martineau as a room of her own – a space in which to divest, if temporarily, from normative social roles and obligations. But *Life in the Sick-Room* goes further too: as Ellen Annandale explains, the book was ‘an attempt to overturn the idea that the sickroom was for those who had opted out of life and to instate it instead as a platform for direct political intervention’. Annandale also writes that Martineau’s work exemplifies ‘the crucial recognition that . . . the sick can still *be* even though they cannot *do*’,¹⁶ but this strikes me as an odd contradiction of the earlier point in perpetuating the equation of illness with stasis and estrangement. Martineau’s own writing powerfully argues otherwise: that illness *is itself* a form of doing.

Martineau certainly had experience of both the travails of corporeality and different methods of accommodation and treatment. She had been partially deaf since childhood and was a strong advocate of assistive technologies like the hearing trumpet,¹⁷ and she eventually sought mesmeric treatment for an ovarian cyst and uterine prolapse. But the precise nature of the illness that impelled *Life in the Sick-Room* is ‘difficult to discern’, as Annandale says, in part because Martineau believed that ‘invalidism is constituted by what happens within the sickroom rather than what specifically has brought one there.’¹⁸ This construction is intriguing, since it ignores diagnosis and posits a dynamic identity forged within relationships of embodiment, location, activity and anyone else who enters the arena of illness. To be ‘sick’ is in this sense simply to declare that one is – to demarcate one’s sphere of experience as such and then carry on in a self-determined domain that bustles with the work of rigorous observation and analysis. It is agential, therefore, and Martineau did insist on the primacy of the patient’s own knowledge: ‘Doctors’, in fact, ‘were subject to her sickroom challenge to the authority of those who thought that they,

rather than the patient, knew best.’¹⁹ Thus an unspecified illness requires no confirming sanction from professional expertise; nor does it delimit the individual from participating in, and contributing to, social and intellectual life. A hypochondriac in the style of Harriet Martineau dominates the space of sickness ‘as a seat of authority’, to quote Frawley once more – asserting the troubled bodymind as the very foundation of momentous, transformative ideas.²⁰ Sickness, as will be evident throughout this book, is no divestiture.

SOURCES HAVE BEEN consistent in their estimates of how many people experience health-related anxiety. In the early 2000s the fourth edition of the *DSM* placed the prevalence of clinically significant hypochondria at 1–5 per cent of the general population, a range that has not dramatically altered; 4–6 per cent of Americans was noted by *UCLA Health* in 2021, and 4 per cent by *Time* magazine in the autumn of 2022.²¹ Yet these numbers reflect only diagnosed cases of health anxiety and do not tell the whole story of this uniquely human tendency to spin elaborate imaginings of the very worst that might occur to us. John R. Walker and Patricia Furer, authors of the 2006 chapter ‘Health Anxiety: Hypochondriasis and Somatization’, describe the incidence somewhat differently. ‘Anxiety concerning [one’s] own health or the health of loved ones’ is felt by ‘many people’, they write, arguing that while it has historically been ‘viewed as . . . resistant to treatment’, health anxiety in fact responds favourably to certain forms of cognitive behavioural therapy. ‘Steadily expanding knowledge’, Walker and Furer conclude, promises new understanding of and responses to this ‘common problem’.²² Such an account anticipates more recent reports of the generalized prevalence of health anxiety – by some estimates, as many as 50 per cent of primary care visits involve ‘medically unexplained symptoms’, or MUS – but without the condescension that has long been attached to hypochondria, with its accompanying whiff of malingering.²³

At the same time, a 2020 article in the *British Journal of General Practice* states that a vaguely defined diagnosis of ‘worried well’ is ‘being increasingly used, particularly by policymakers and now by some clinicians’, to refer to people ‘perceived as using health services inappropriately or disproportionately’.²⁴ The authors argue that the

emphasis on ‘well’ in characterizing this group of patients refuses to legitimize emotional distress, as if only ‘pathologically based disease’ merits serious attention from doctors. Patients’ own experience of their discomfort is discounted in this scenario as a valid piece of information that might encourage further enquiry into the cause of symptoms or determine a course of treatment. The implication is that anything that cannot be verified through standard clinical testing and so eludes a physician’s capacity to identify will be dismissed as ‘psychosomatic’ – at once perpetuating an ‘old-fashioned’ stigma against mental suffering and missing possible signs of minor illness or organic disease. ‘Worried well’, then, however much it might sound like a cheerful assuagement of the anxious, is but ‘hypochondria’ in new clothes: a disparaging term that denies respect, patronizes patients as childish fabricators and undermines confidence in the healthcare relationship.

The language of wellness underscores a basic assumption that successful identity demands both fitness and contentment, from which anything else is a pathological deviation. Hypochondria is defined, of course, in relation to cultural attitudes about what constitutes a ‘normal’ relationship to health – including the primacy of healthiness as an unquestioned individual and social goal – since it is through some governing authority that hypochondriacal fears are officially validated or allayed. But as I suggest above, hypochondria lies at a provocative and curious crossroads, inviting the input of medical professionals and diagnostic testing but ultimately failing to be assuaged by expert examination. In this context, hypochondria defies the logic of ableism’s progress narrative: that we will get better, that we *want* to recover, that no one would *intentionally* seek to be sick. Indeed, given the considerable social pressure towards wellness, hypochondria will inevitably seem like a strange courting of the very identity position shunned by the health-obsessed as damaged and degraded. When Carla Cantor writes in *Phantom Illness* that ‘though our society is generally tolerant of the ill, hypochondriacs are sneered at, joked about, and ignored,’ she touches on this enigma.²⁵ However much we might ‘tolerate’ it, we really don’t *want* to be sick (or to be thought of as sick), and it’s the discredited social role of illness as much as the suspicion of falsehood that inspires the repugnance with which hypochondria is met.

Consider the effect of popular synonyms for the hypochondriac. Words such as ‘bellyacher’, ‘crank’, ‘malingerer’, ‘pessimist’, ‘defeatist’,

‘whiner’, ‘handwringer’ and ‘grouch’ suggest overt impatience not just with the supposed speciousness of hypochondria but with the unseemly abdication of social obligation that attaches to illness, especially to psychological suffering. Anxiety about sickness connotes giving up, a failure to struggle valiantly back to healthfulness; if sickness connotes loss of hope, then hypochondria becomes an act of courting a terrible future, since nothing can be imagined as worse than limitation in our mortal fleshliness. The fact that some near synonyms given for hypochondria are real ailments – such as carsickness or lameness – involves a different slippage, between what can be verified and quantified and what seems to exist, somehow dangerously, only in the mind of a suspect storyteller. To worry about being sick in these idioms – to appear to fabricate the already compromised subject position of ill or impaired person in the absence of trustworthy evidence – is to incur an ethical or moral taint associated with eschewing responsibility, productivity, emotional self-restraint and maturity. Illness in this discourse is coded as a threatening disruption of rationality, empiricism and the forward motion of life’s progressive arc, so that to seem to *fake* it is not just tainted by deviousness. It’s radically anarchic.

My own exploration of hypochondria is grounded in the idea that the act of leaning *towards* a disgraced position can reveal constraints and pressures associated with prevailing norms of subjectivity, and in turn can have salient import for how we might rethink our embodied, situated selves, apart from whether or not we are diagnostically ill or impaired. I consider hypochondria as an orientation, a *mode* more than a *condition*, with surprisingly revisionary meaning. The very fact of hypochondria’s various cultural resonances reminds us that the status of ‘invalid’ has also shifted over time, treated not always with antipathy, which means in turn that hypochondria’s putative inventions – beheld in some quarters as indecent pursuit of conditions that are always, already and ‘actually’ bad – might encourage us in our own moment to interrogate the obviousness that health is preferable to unhealth. Given hypochondria’s entanglement with medicine, it will be counter-intuitive though important to this inquiry to resist a deeply ingrained biomedical paradigm (which restricts analysis of disease to individual biology) or even a biopsychosocial model (which widens the conceptual framework to include patients’ personal and environmental circumstances but nonetheless persists in endorsing health as an individual responsibility).²⁶ To understand hypochondria’s cultural

valence, we must consider what precisely the hypochondriac fears – or conversely just what is longed for – as well as why both fear and sickness are scorned by the healthy and sane; we examine the many ways in which ‘Health with a capital “H”’, to quote Richard Klein, has been ideologically constructed.²⁷ In what follows, I frame hypochondria as a potent if paradoxical dismantling of that construct. For even as they seem to endorse medicine’s palliative command, hypochondriacs are brazenly, vulgarly and incessantly ill.

It matters to my purposes here that the institution of medicine has its antecedents in intellectual practices defined by what Shakespeare’s Othello calls *ocular proof*, because so many claims easily disputed as hypochondriacal entail sensation that cannot be visually detected, such as pain, or do not appear on standard tests and so cannot be verified by an expert trained to ascertain illness clinically rather than relying on the individual’s subjective account. As French philosopher Michel Foucault wrote some years ago in his influential *The Birth of the Clinic* (first published in 1963), the professional authority of the modern medical setting concretizes around a ‘pure Gaze’ that derived from shifts in the practice of natural history in the late seventeenth century that endeavoured to map the natural world, including the human body, according to visible characteristics. ‘To describe’, Foucault argues, ‘is to see and to know at the same time, because by saying what one sees, one integrates it spontaneously into knowledge.’ In this synonymy of seeing and knowing, power is both symbolized and literally enacted by looking, so that the doctor’s ‘speaking eye’ becomes what Foucault calls the ‘master of truth’.²⁸ We thus arrive at a historical moment in which the hypochondriac, unable to produce evidence recognizable within the institution to which they’ve turned for reassurance and confirmation, is prone to being deemed an unreliable source of information, even though hypochondria’s affective and philosophical resonances remain an important aspect of how humans experience being sick. By raising the possibility that something that cannot be detected through standard scientific mechanisms is nonetheless *there*, the hypochondriac explodes the mastery of objective knowing, at least in the sense of having legitimate information to impart about the many and peculiar qualities of being alive.

It's worth asking whether hypochondria mounts a reaction to the slightest twinge because environments girded by fear of mortality provide so few resources to help us manage the unpredictability and fragility of embodiment with something like equanimity. In this sense hypochondria has nothing to do with manipulation and deceit and instead simply calls our collective attention to an uncomfortable imminence our culture goes to great financial and psychic lengths to deny: that we will all, at some point, cross the thresholds of illness, impairment, aging and death. No one, in this respect, is ever actually an imaginary invalid (to invoke the title of Molière's 1673 play *Le malade imaginaire*, discussed in [Chapter Three](#)), since we are all always in some measure ill in the sense that we are always already dying. What but a suspicion of one's own frailty would make us insist that someone else's rash, cramps, angst or indigestion is so plainly *not* a harbinger of worse? Surely the tendency to claim that something can't possibly be as bad as the sufferer thinks it is derives from a desire to ward off the spectre of our own vulnerability and impermanence. When health is at a premium and we're compelled to scrutinize and monitor ourselves – to say nothing of our digital lives, in which we're inundated by medical language (if not always reliable medical science) – the claim to know pits us in contests of meaning about bodymind complaints, and dismissing the hypochondriac's concerns can maintain a comfortable fiction that we are not so vulnerable as *that*. 'A person who calls someone else a hypochondriac', as Belling puts it, 'tends to be invested in the unlikeliness of the feared event.'²⁹ To demand of the hypochondriac that they demonstrate the validity of their complaints betrays a set of prejudices that deserve forestalling – for an elusive empirical truth, for health as a moral imperative, for relief from an awkward recognition of our fundamental humanness.

But what would happen if Molière's protagonist Argan, a flagrant hypochondriac, were met not with pity, derision, conniving and fatuous capitalizing on his nervousness but instead recognition, curiosity and care? What new forms of embodied cognition, relational intimacy, familial organization or professional identity might be set in motion? What alternative structures of thought, communication, personal and collective boundaries, attitudes towards work and time, and approaches to aging might be discovered? Hypochondria raises such questions, with myriad possibilities to offer.

IN THE CHAPTERS that follow I delve into literary, historical, scientific and popular material that details hypochondria's clinical and common manifestations and most crucially its philosophical import. I explore hypochondria as it is diagnosed, satirized, tended to, endured and finally celebrated and redefined in treatises, texts and the social world we inhabit as always potentially ill and injured people. I want to take hypochondria seriously, not because it is a condition that causes distress and deserves to be treated – though I believe that to be true – but because it poses an intriguing epistemological challenge, bringing us to the limits of what we can claim to be sure of about our embodied experience and in turn provokes us to tolerate, at least to investigate, ambiguity and interpretive disagreement. Perhaps hypochondriacal ailments demonstrate morbid preoccupation with sickness, maybe an uninformed sense of how it actually occurs; perhaps, in a different way, hypochondria serves a protective function, compelling us towards usefully hygienic routines. In either case, it focuses our attention on vulnerability and risk, on how we make meaning from the unknown, and on where we draw boundaries between ourselves and our collectives. I suggest in this study that hypochondria's fiercely contested battle of interpretation raises myriad questions that are not just about disease. I write as a scholar of disability rights and representations but also as a nervous relative of a family of physicians, which is to say, someone well versed in how the medical model has often ill served us but also as an anxious person who has benefited immeasurably over a lifetime from the gentle reassurances of those to whom I text, in the wee hours, the litany of my fears.

[Chapter One](#), 'Pleading the Case: Hypochondria Speaks', begins this book by reading hypochondria against the grain as a positively communicative act. In clinical settings, hypochondria tends to be thought of as befuddling, an obfuscation of biological reality – or a wilful refusal to accept information (whether from patient or practitioner) that is delivered in good faith. A frequently enumerated psychological approach holds that hypochondriacs use a focus on physical suffering to avoid more difficult psychological admissions and even to control interpersonal dynamics; the hypochondriac, in stereotypical but also academic renderings, is a master manipulator. In this scenario, hypochondria actively stymies 'healthy' communication – not only between people but within individuals who fail to apprehend, or cannot admit, the true nature of their feelings.

Hypochondria is also often construed as an unintended consequence of both public health messaging and advertising – its scrutiny an inevitably exaggerated form of the self-surveillance we are encouraged to practise by the juggernaut of the health, beauty and fitness industries. Here hypochondria exemplifies communicative failure, missing the point of where and how to direct our self-improving efforts.

But what do we convey in articulating our symptoms, or more importantly our *concern* about our symptoms, if not vital ideas about what it means to be human and about the kind of citizenry we hope to be a part of? Contrary to a prevailing dismissiveness about hypochondria's recalcitrant stubbornness and misdirection, its emotional sleight-of-hand, I argue that hypochondriacal utterance merits our attention rather than our exasperation for what it has had to say, through its long history, about mind-body interconnection, intimate relationships and social organization, civic responsibility and personhood. Klein writes that 'we have become strangely divorced from our bodies', incited to monitor 'our numbers' and to behold our wayward physical selves with stringent distaste.³⁰ But the hypochondriac's heightened awareness of bodymind sensation conveys a phenomenology of aliveness, enacting – if not always precisely honouring – our origins in the flesh, compelling interlocutors to participate in moments of embodiment that refuse to be curtailed by common explanations. Thus hypochondria is profoundly companionable, despite the solitary hours of doom scrolling and frantic self-diagnosing; it reaches towards others in a plea to be recognized, maybe refuted, surely *reconstituted*, even if in an inherent mysteriousness that it is vital to acknowledge we share. Indeed hypochondria can stimulate mutual knowing, often across thresholds of otherwise dramatic differences. Hypochondria demands too that we accept our own as well as our collective vulnerability, because disease is at the door; and yet it keeps on being sick, even when well, defying the stability of these categories as mutually exclusive instruments of social order. It announces us, finally, in the urgency of a discomfort that is thoroughly ours and yet entwined with the groups of which we are part, whose caretaking we might choose, because we will also inevitably require it, to foster.

[Chapter Two](#) takes up a centuries-old critique of hypochondria as time-wasting idleness to ask what *else* we might be doing when we're anxiously anticipating the worst. The very word 'idle' appears in countless treatises of nervous affliction that – even when sympathetic to the anguish of

hypochondria – explicitly define acceptable ‘doing’ as *work* and link such work with health. Before modern psychological or pharmaceutical interventions, it was *activity* that purported to dispel the miasmatic listlessness of hypochondria, and the so-tormented were counselled towards an industrious busyness in some territory other than their own appalled imaginations. Still today, in the context of more thoroughly neurological explanations for somatic disorders, hypochondria continues to be framed in terms of waste, not just of time or mental effort but even of *self*, figured in its hypochondriacal state as useless – as if the suffering person achieves nothing, offers nothing and deserves nothing. Our common language of sickness, freighted by idioms of departure, interruption, estrangement and also cost, burden and loss, specifically fashions ill health as a rupture of workaday rhythms; even mundane advertising for such over-the-counter remedies as aspirin positions healthiness in a comfortably nonthreatening *here* of conformity and illness as an alienated margin from which the sick person violates the commandment to earn. If, as Lauren Berlant argues, ‘under capitalism sickness is defined as the inability to work,’³¹ then a *pseudo-sickness* becomes the epitome of obstinate failure to contribute to the system, to be a fully vested, economically productive member of society.

In this chapter, however, I explore writers and scientists whose hypochondria facilitated their work – often acting as a necessary counterpoint to the mental as well as physical efforts of significant research or composition – or in fact *was* the work, as in Martineau’s claims for her sick years as ones of robust intellectual action. Here I also differentiate between the apparent stasis of hypochondria and a generative *stalling* in which unconventional forms of expression or experience can occur. If hypochondria is deemed an obstruction, after all, it’s worth interrogating what precisely is presumed to be delayed: timelines of work, trajectories of normative family life, the progression of recovery and cure. By suspending, certainly recalibrating, narratives of viable selfhood that restrict us to a familiar movement through time, hypochondria poses an important challenge to what gets legitimized as accomplishment and reorientates an expectation that success is governed by future reward. I thus propose ‘hyp-time’ as an alternative to the stuck recursiveness of hypochondria. Following a notion of *crip* time – which refers to how disability experience takes shape both within the rhythms of but also in frank opposition to what

has been called *chrononormativity* – hyp-time becomes not a wasted intermission in the onwardness of a life we ought to return to, but the real time of embodiment in all its precarious, inquisitive, proliferative disorderliness. Hypochondria's alternative time-scape defies linearity, and in this sense hews closer to the natural loopings of consciousness – of memory, of creativity – than to the temporal strictures of regularized social life.

If hypochondria communicates, and produces, what is it that hypochondria knows? In [Chapter Three](#) I trace an epistemological conundrum. The contest of hypochondria pits patient against practitioner in a struggle over domains of knowledge. Resistance to reassurance – to professional interpretation of symptoms – was a diagnostic criterion of *hypochondriasis* well into the twenty-first century. It will seem unremarkable that a hierarchy of proficiency is inherent in the clinical encounter, and of course it's in the nature of hypochondria for a patient to have recourse to a more knowledgeable counterpart. Yet the very fact of the hypochondriac's ongoing symptomatology (setting aside whatever competing theories about etiology or treatment a patient might bring to the office) casts doubt on that authority – even as it also calls into some question what can be known of the state of the body through our subjective experience of it. Thus an additional challenge emerges from within the individual whose bodymind is the site of signs that refuse to be stabilized, either diagnostically or sensorially. In this alarming circumstance of pains and lesions that readily attach themselves to a vast database of possible answers, what can be known in any absolute way of our bodies and minds? And what damage might be done – what knowledge might in fact be lost – by pretending to a degree of mastery that disparages hypochondria's own investigative intensities?

Hypochondria foregrounds a debate about the nature of proof – who knows best what 'it' is and in turn who is best positioned to allay resurgent sensations and fears when the absence of definitive explanation threatens to overwhelm patient and practitioner alike. The conflict has been a disciplinary one, a roiling disagreement about whether hypochondria is rightfully a medical or a psychological disorder – or, even if exclusively (some would argue obviously) the latter, what its criteria are and whether the trouble is best assuaged through medication, behavioural therapy or excavation of repressed traumatic experience. This chapter traces

hypochondria's progress from psychoanalytic into modern psychiatric realms, but its primary focus is what can be known *by* the hypochondriac rather than *of* hypochondria as a disorder. I hope to shift the emphasis of hypochondria's crisis of (not) knowing, reading the condition as a mechanism of knowledge that paradoxically gives the lie to professional omniscience by repeatedly defying scientific explanation. Coining the term *hyp-istemology*, I argue for hypochondria as a way of making sense of embodiment's ever-receding truths – one that, in constantly eluding the disciplinary paradigms that would corral it but also impelling us towards others, encourages more holistic, interpersonal methods of care and grants the elusive, open-ended quality of our human experience. Hypochondria's habit of searching, I suggest, presents not an embarrassing nothingness – the disease that isn't – but rather the virtues of testing alternatives, seeking anew and bearing the sting of contradiction: of cultivating a restless impatience, we might say, at being diagnostically, ideologically or politically confined.

Yet hypochondria works in the opposite way, too. For all its capacity to provoke reconsideration, to reveal the fissures in our fantasies of self-containment by bringing bodymind unruliness inexorably back to consciousness, hypochondria also substantiates, coalesces. [Chapter Four](#) explores the unifying manoeuvres of hypochondria at the individual as well as group level. From a malaise styled as national identity in the eighteenth century to an after-effect of geopoliticized sickness, for example, hypochondria can work to shore up national and geographic as well as corporeal borders; since the boundaries of the body have long been crafted as an analogue for those of the state, disease may be feared in a specifically xenophobic sense, as Daniel Defoe's 1722 *Journal of the Plague Year* makes clear, no less than the global politics of the COVID-19 era. But as a process of sensing and reasoning, hypochondria also pulls us together in more salutary ways, particularly when solidifying oneself in a story of illness overtly refutes the idea that in sickness we become diminished, other to ourselves, or that the healthy, normatively functional bodymind is the only kind that counts.

Hypochondria is no fanciful disorder in the context of empire, when privileged subjectivities are maintained through a discourse of contagion that conflates fear of disease with that of other bodies, leveraging nationalist, racist, religionist anxiety to mobilize attitudes and behaviour in

support of colonial power. At the same time, the essential factitiousness of geopolitical borders can be rendered in terms of hypochondria's alert cognizance of porousness. An interconnected world is, among other vulnerabilities, an epidemiologically exposed one; this serves in some literature to articulate the disastrous consequences of expansion and elsewhere to register anxieties about gender and race in complex multicultural milieus. The hypochondriacal body also manifests the friction of assimilation, in which racial and cultural difference may be felt indistinctly as unhealthiness, even as it proves that dominant culture, hypochondriacally guarding its boundaries, is built on exploitation and disgust. Hypochondria protests too against the dehumanizing effects of the workplace; a body that does not stay put in medically recognizable ways disrupts the organization (in both senses of the word) but then amalgamates itself in explicit defiance of commercial interests that depend on regularity and sameness. In these various scenarios hypochondria reveals us in perplexing unfamiliarity – but it holds out a provocation too that we might restyle the ideas of wholeness on which ableism is constructed to allow for all the eventualities our fragile, ever-on-the-brink forms are capable of.

Integral to these swirling vectors of meaning is a question of respect. Will hypochondria's outcry be heeded, its busyness credited as an intellectually valid form of doing, its particular brand of scrutiny understood as a means of self-knowledge and composure? In arguing *for* hypochondria in these ways, I hope to interrupt both popular and professional tendencies to reject hypochondriacal thinking as obnoxiously irrelevant and disorganized, and to make a case for what we can learn from hypochondria's uniquely corporeal signification. But in [Chapter Five](#) I turn to a different set of clinical circumstances, ones that are not rightfully about hypochondria at all, except that hypochondria provides such a convenient deflection from structural – and of course deeply individual – prejudices. Here I argue that in addition to masking practitioners' own doubts about cause, their insufficient training or knowledge, hypochondria is also thickly entangled with the mechanisms of gender, racial, class, fatness and sexuality bias that have direct and often dire consequences for access to and quality of healthcare. Hypochondria certainly inspires scepticism, as we have seen, and it may be that health worriers are always subject to a degree of negative feeling on the part of their physicians. But some patients are

scorned as hypochondriacal – declared unbelievable, non-existent – even before they’ve crossed the threshold of care.

There is ample evidence, for example, of enormous disparities in the treatment of both acute and chronic pain for women and people of colour, owing in part to deep-seated stereotypes about Black and female bodies as simultaneously better able to withstand pain and more prone to exaggerate their suffering – and despite the fact that opioid abuse is statistically higher among whites. Girls and non-white children are similarly more likely to be denied pain relief, even after surgery or for broken bones. Homophobia too mediates quality of care, as when patient anxiety about an AIDS diagnosis is deemed hypochondriacal, not reasonable, thus not worth addressing. Another factor influencing claims of hypochondria is that because so much less medical research takes a female, nonwhite body as its representative subject, marginalized groups are more likely to experience misdiagnosis. When symptoms don’t align with the body type practitioners expect of particular diseases, those symptoms can be readily discounted as hypochondriacal and proper diagnosis may be seriously delayed; heart disease, for instance, is notoriously misinterpreted in women as mental illness. Indifferent medical response to the new and amorphous syndrome of long-COVID, loudly decried by some as a form of gaslighting that distorts reality, is but a recent instance in a long history of ignoring or misreading some individuals’ symptoms – and, worse, of embedding diagnoses in systems of oppression that justify themselves through a discourse of biological inferiority. At stake in these and other situations to be explored are forms of cognitive error whereby practitioners disregard certain accounts of embodied experience in ways that discredit not just *interpretation* of sensation but the very phenomenon of feeling at all – an existential denial that many patients report as a profoundly dislocating kind of erasure. Might hypochondria, in the face of pre-existing disbelief, be cultivated, counterintuitively, for its insistence on alternative possibilities, its alternate truths? For its protective, assertive qualities? Hypochondria mounts a challenge to standard ways of knowing – to culturally accepted *knowers* – but it also rebuts the *ontological* hierarchies of medical encounter, insisting that we *are*, and on the wisdom, inherent in the fact, of our being.

When I began the final chapter I assumed the veracity of a common stereotype that the elderly are preoccupied by the deterioration of their

health. Surely the aged are more prone to hypochondria, I thought, since cognitive and physical change can be difficult to withstand – ‘aging is a bitch,’ as writer Thelma Adams declares in her novel *The Last Woman Standing* – and because death itself looms as an ever-more-imminent reality. Indeed, its alleged inertia notwithstanding, hypochondria implies the future: the arrival of the dreaded diagnosis, the worsening symptoms, the inevitable, final, decline. Yet research shows that aging populations are not statistically more hypochondriacal than younger ones. To the contrary, in fact, aging seems to usher in a newfound contentment and resilience. And if older people *are* hypochondriacal, then either they already were as children or teens or somatic preoccupation *makes sense* as adaptive and functional, given that most adults over the age of 65 have multiple, chronic health conditions. What sounds like hypochondria in an eighty-year-old, in other words, may just be appropriate self-assessment. But a complication arises in some efforts to uncouple aging from a hypochondriacal cliché. The so-called positive aging movement purports to retrieve our inevitable advance in years from pitiable narratives of decline, loneliness and social irrelevance, but, as critics have argued, only by reinforcing an ideal of perpetual *youth*: an ableist fantasy, in fact, in which old people do not get sick and suffer no diminishment of capability. This version of thriving elderly folk, many protest – golfing, cruising and MasterClass-ing away their over-scheduled hours of retirement – is but ageist reviling repackaged, exactly as ‘worried well’ doesn’t go that far to dispel the horror of an infantilized, even demonized hypochondriasis.

In this chapter, then, I present an alternative version of aging that embraces what Scott Herring has called ‘later-life floundering’ in explicit rejection of the pressure to hold fast to youth – which is to say, to an ableist fiction of cognitive and corporeal normalcy, where to be ‘fit’ in the sense of ‘valid’ presupposes full functionality.³² Instead, we might choose to celebrate the mistakes, the goofing up, the *repetitiveness* of aging – even the unfiltered narration of its ailments – as evidence of life’s ongoingness, compelling modes of discovery and storytelling. We might think of advancing years not as an inexorable slide into the burden of helplessness but rather as hyp-time, where we do the work of finding out who we are *now*, even as we daily transform. We might honour the inwardness of age as hyp-istemological self-reliance, a subtle process of acclimation to surprise. We might value, in other words, precisely those qualities of aging that enact

hypochondria's own chaotic flourishing, even celebrating its querulousness as a hard-won sneer at social decorum. The older hypochondriac might be a fretful and testy one and is likely an actually ailing one, but contentment, as the work in this chapter demonstrates, need not correlate to healthiness. To consider aging *through* hypochondria is to disentangle static binaries – of illness obsession against recaptured youth, of doddering cantankerousness versus emotional calm borne of wisdom – and helps us take stock of the damage done by splitting off the messy, unpredictable, disorganized and finally mortal facts of ourselves.

A POPULAR VIEW beholds hypochondriacs just as hypochondriacs regard their own bodies: with profound suspicion, as a threat – even an adversary. But there is opportunity for curiosity here to focalize our awareness of having and being a body and so to complicate what Steven Connor has described as ‘the ignorant or indifferent condition that is health’.³³ It's interesting to note the way in which hypochondria's long etiological history travels through the body, from a disorder of the brain to the vibrating actions of the nerves, from the roiling turmoil of the guts to palpitations of the heart, from rising vapours to a wandering uterus, along with its hallmark cramps, weakness, weariness, numbness, sweats, headaches and nausea, and of course its accompanying emotional distress, its panic, depression, anger and ennui – as if hypochondria were a form of embodied cartography, mapping the wiliness of sickness even as other diseases have stabilized over time through ever-more-precise readings of origin. Hypochondria is a condition haunted by proof yet epitomizing elusiveness. It acts out, even as it strives to disprove, the uncomfortable imprecision of cause – which is another way of saying that it also disproves the guarantees of health.

My childhood hypochondria began not by catching a virus but by reading a book. Writing a book about hypochondriacs has made me both less convinced that I am one and more intrigued by hypochondria's complexly interactional, philosophical, revisionary nature. Whether transient or sustained, a preoccupation with worry about bodymind sensation engages urgent but not always readily unravelled puzzles about who, why, how we are in our most vulnerable human guises. A plethora of symptoms will generate an equal number of stories. The importance of

hypochondria, as I hope to show in what follows, is just as much about the effect of those tales – who’s listening, how we respond, what we’re saying to one another about what it feels like to be alive – as it is a question of pathologic or curative study.

This book records what I’ve learned.

OceanofPDF.com

1

Pleading the Case: Hypochondria Speaks

‘Hypochondria knows he’s never taken her
symptoms seriously.’

EMILY PAIGE WILSON, ‘Hypochondria and Her Estranged Half-Brother Sisyphus’^{[1](#)}

Reading Daniel Defoe’s *A Journal of the Plague Year* in the aftermath of COVID-19, as I did, was an uncanny experience. Published in 1722, the *Journal* is a novel posing as an eyewitness account of London’s 1665 Great Plague, and its descriptions of an eerily quiet city coming to ‘a full stop’ will be familiar to any reader sensitized by our recent pandemic to the challenges of social distancing and the fear of infection.^{[2](#)}

Defoe’s narrator (an unnamed saddler) writes of Londoners nervously covering their faces, crossing the street ‘to shun meeting’ others ‘for fear of the plague’ and ‘tak[ing] no change’ from a shopkeeper’s hand in contactless payment. He recounts one elaborate scene in which three men attempt to retrieve a dropped leather coin purse using gunpowder, a bucket of water and ‘red hot [tongs]’; they lacked only kitchen roll and disinfectant. He details the legally enforced practice of ‘shutting [up] of houses’ and the hardship of isolation and confinement with ‘no way to converse with any of [one’s] friends but out at [one’s] windows’; he attributes the worsening of people’s ‘apprehensions’ to a kind of pre-Internet doom scrolling, claiming that printed ‘predictions and prognostications . . . frightened [readers] terribly’. He emphasizes the particularly egregious situation of poorer Londoners who lack the means to

escape the city to the countryside and are thus exposed to the houses of the sick, to goods presumed to be infectious and to the newly dead as they '[go] about their employment with a sort of brutal courage' – his era's first responders and front-line workers. Later Defoe lingers over the anguish of childbearing during an epidemic, when midwives refused to enter the houses of the ill and mothers died with newborns in their arms. Even the 'foolish humor of the people in running after quacks and mountebanks, wizards and fortune-tellers' might remind us of early, misguided theories about how to combat a disease made fearsome by its novelty and speed.³

Scholars have surmised that Defoe wrote *Journal* to justify British shipping embargoes (to curry favour with the government, that is) as the plague swept west across Europe in late 1721, and it's easy to see how this horror story of death, of the madness brought about by desperation and pain, of divine wrath and economic devastation, might persuade a public sceptical of its ministry's decisions. The book is, indeed, dominated by fear. The narrator observes both nonchalance and terror in his compatriots in the early months of the epidemic. Some are emboldened by the initially gradual spread of pestilence and so caught off guard when the city goes into lockdown and provisions are hard to come by; others are so alarmed 'by the force of their own imagination' that comets passing over the capital are taken as omens of death, and visions of angels and ghosts rouse crowds to a pitch of hysterical apprehension about what's to come. 'The imagination of the people was really turned wayward and possessed,' remarks our narrator, who tries but fails to see what his compatriots in the streets are staring at. As the months drag on, however, and the death toll rises by thousands a day, he begins to name fear as a danger nearly as potent as bubonic plague itself, speaking with empathy of people 'frighted to death', just as he does of those who succumb to disease.⁴

If there is ambivalence in the narrative's tone regarding fear, it seems located at the junction of realism and superstition. It is one thing to be afraid of God's judgement or the sight of bodies being carted off to communal graves and another to place one's trust in 'astrological conjurations, dreams and old wives' tales'. Twice in quick succession the narrator refers to the latter as forms of 'fancy', and he defines the credulous acquiescence to such mystical signs as a kind of *hypochondria*. He thus differentiates between a legitimately piteous but frenzied dread of catching

‘real’ pestilence and the mockable fear of unsubstantiated portents driven by mass hysteria. Given Defoe’s stake in cautioning an overconfident public about the risks of plague, it’s curious that the novel begins by lambasting such nonsense as imaginary voices and ineffectual ‘Antipestilential pills’, but ultimately this seems precisely the point: an *actual* epidemic is no mere fanciful stargazing but would be a materially catastrophic event against which the city must guard its borders. The difference is certitude. We cannot know for sure what angels in the sky mean (or whether there *is* an angel in the sky in the first place), but we can know absolutely that people die from plague, and that the strains of another epidemic would catapult the city into ‘dreadful calamity’. The novel’s many tables showing the numbers of deaths graphically demonstrate as much.⁵

But what does it mean to have so-called hypochondriacal fear in the midst of an epidemic decimating your neighbourhood? Defoe frames hypochondria as a problem of faulty imagination (seeing or hearing what isn’t verifiably there) rather than of interpretation (inaccurately diagnosing what is felt, whether bodily or psychological). Yet the fact of a raging plague sets in relief the way in which hypochondria might always be said to involve what is there rather than fabricating something that isn’t. As our own experience of COVID-19 has made obvious, worrying over a sensation (scratchy throat) can hardly be deemed excessive when such a symptom, though likely attributable to a range of less deadly conditions (allergies, for example), is also a common early sign of serious disease. In Defoe’s *Journal* hypochondria lies squarely in the realm of religious dread and is framed as a process of becoming afraid of metaphysical portents. But the goal of the book is not to soothe or dismiss such fear at all, even in the absence of plague in the body of any individual reader. Hypochondriacs do not appear in this text because there is no threat of disease; to the contrary, their wild imaginings are leveraged to help the *Journal*’s narrator stoke its readers’ fears that illness is deadly, costly and imminent. The urgency of his message depends not on readers’ capacity to set aside childish belief in dreams and fairy tales – or even, finally, on a rational awareness of the socioeconomic consequences of plague – but rather on their deepest worries about embodied fragility, our basic human susceptibility to getting sick.

In this chapter I will explore several related but not synonymous ways in which hypochondria participates in the exchange of information. Our modern sense of the hypochondriac as someone who both craves and

refuses medical intelligence distracts us from what else is going on in the communicative scenarios in which hypochondria takes place, from the most interiorized, deep attendance to the sensations and sounds of embodiment to the broadest public scale of advertising, medicine and ableism's inducements towards health. Catherine Belling states in *A Condition of Doubt* that 'to exist at all, hypochondria requires a physician',⁶ but I will argue that hypochondria frequently happens in locations and between people with no connection to medicine at all – precisely because hypochondria, for all its apparent focus on sickness, entails forms of listening, sharing and appeal that engage embodiment in ways not strictly clinical. To be a hypochondriac in Defoe's *Journal*, for instance, is to be positioned as one of society's foremost bellwethers; if they appear initially as subjects of the narrator's mocking contempt, hypochondriacs also augur that same narrator's eventual recognition of his city's dire circumstance. In this communal and interpersonal sense, hypochondria is not a condition of no-disease but foregrounds the fact of disease: its probability, its proximity.

In what follows, then, I consider hypochondria in various locations of communication – between body and mind, as a mode of deep investigation of ourselves; between people, as a method of learning and bonding; in social conversation, to register distress and concern, foster empathy and perhaps coerce civic responsibility; on the page, in writing to others about melancholic anxiety; in broader mechanisms of ideological persuasion, where the prejudices of ableism are conveyed through advertising and public messaging; and finally as an utterance of selfhood, a way of announcing who we are. Each of these is of course bound up with the others, as we interpret our innermost sensations according to what we've learned about health and sickness and bring our group identities to conversations about public health no less than to our personal essays. In each case, too, important forms of interchange take place, ones that instruct us in how to comprehend our embodied lives. James Boswell wrote in 1777 that 'the malady known by the denomination of melancholy, hypochondria, spleen, or vapours, has been long supposed almost universal.'⁷ He took readers' understanding of (or at least their patience for reading about) a certain intensity of mood for granted, and he used the viewpoint of hypochondria as a platform from which to disseminate his philosophizing. Given all the ways in which it stimulates exchange, then, hypochondria

cannot be simply regarded as a failure of argument or convincing, grievous though the condition may feel in the event of its worryings.

SETTING ASIDE THE religious framework in which Defoe understands the plague (his narrator says explicitly that the scourge is sent from a displeased God), the heightened pitch of fear he renders as hypochondriacal evokes the correspondence between hypochondria and melancholia that would dominate explanations of health worry for hundreds of years. This entanglement of physical and psychical problems – so central to the operations of hypochondria right up to a contemporary neuropsychiatric understanding – comes fully into view in Robert Burton's 1621 *The Anatomy of Melancholy*. This extremely popular tome includes subsections on the causes, symptoms and cures of 'Hypochondriacal, or Windy Melancholy', which Burton attributes to various disturbances of the liver, spleen, stomach, womb, diaphragm, pylorus and abdominal veins. A 'Midriff distempered by heat', a 'cold liver', a 'cold stomach & over-hot liver' – the *Anatomy* is far from concise or consistent – can produce 'crudities, obstructions, wind, rumbling, griping, &c' and in turn 'fear, sorrow, dullness, heaviness, [and] many terrible conceits and chimaeras' as if '*a black and thick Cloud covers the Sun, and intercepts his beams and light*' (italics in original).⁸ Equally significantly, Burton offers 'fear, grief, and some sudden commotion or perturbation of the mind' as causes of the very bodily issues he initially names as the culprits: 'some grievous trouble, dislike, passion, or discontent' can distress the viscera just as the other way around.⁹

A modern habit of separating and securing illness in discrete categories of mind or body would not have made sense to Burton's many readers, for whom what they were was quite explicitly a matter of what they ate – too much tart or sweet, for instance – or what their mothers saw and dreamt while pregnant, or the relative moisture of their brains. While the 'hypochondriacal' of Burton's headings typically refers to the same bodily cavity that Hippocrates mapped in the fifth century BCE, what happens there is psychosomatic in the literal sense: at once emotional and physiological in a way that might be irrelevant to uncouple. In the same section describing blockages of the lower intestines, for instance, Burton cites two examples of

hypochondriacal fantasy that to us will seem evidently psychiatric: one a woman who ‘imagined she had swallowed an eel’ and the other a man who ‘began to suspect that he had . . . swallowed frogs’ spawn’; nothing could convince him otherwise: ‘it was no wind, but real frogs.’¹⁰ This psychosomatic conception persists in the century after Burton, when physician and proto-psychiatrist George Cheyne diagnosed the ‘English Malady’ as ‘Glandular’, citing the offending stomach as ‘the first or principal Organ’ and declaring that ‘Of all the Miseries that afflict Human Life, Nervous Disorders, in their extream and last Degrees, are the most deplorable, and, beyond all comparison, the worst.’¹¹ And practitioners through the eighteenth and nineteenth centuries believed that hypochondria involved a kind of ‘facilitated power of apperception . . . of the physical organs’, as the German psychiatrist Richard von Krafft-Ebing wrote in 1893, which could in turn produce ‘absurd interpretations of sensations that are actually experienced’.¹²

Though psychosomatic illnesses originating in the viscera were often mysteriously undiagnosable, the spleen was a common malefactor. Hysteria had of course been associated since the classical period with the uterus (from the Greek *hystéra*), and one theory of early modern medical development holds that since men and women were equally afflicted by nervous conditions with similar symptomology, then either hysteria needed to be uncoupled from the supposedly guilty uterus or some other ‘similarly troublesome organ’ found to explain the suffering of men.¹³ Towards the end of the seventeenth century, though, the physician Thomas Sydenham collapsed the distinction between hysteria and splenetic hypochondria, writing that the latter ‘is as like hysteria as one egg is like another’.¹⁴ Sydenham endorsed an explanation for the mutually reinforcing psychosoma of hypochondria consistent with humoral theory, which taught that the four humours of the body – black and yellow bile, blood and phlegm – corresponded to four distinct temperaments: melancholic, sanguine, choleric and phlegmatic. Disease caused by an imbalance of the humours was treated by restoring their movement or removing blockages (as through purging or bloodletting). This is why Burton recommends such therapies as ‘decoctions of Wormwood’ and ‘Pennyroyal’ or drinking ‘the water of a Smith’s Forge’ – to ‘forcibly’ propel fluids and cleanse the

organs, including ‘Liver, Spleen, Stomack, Hypochondries’ and ultimately also ‘heart and brain’.¹⁵

Whatever the offending organ, these were vexing conditions, so ‘protean’ in affecting the body, as Sydenham claimed, that they might ‘[resemble] almost all the diseases poor mortals are inclinable to’,¹⁶ and characterized by a psychological assault that it would be difficult to assign definitively as symptom or cause. For Sydenham as for Burton, body and mind functioned as a systemically integrated whole, and hypochondriacal melancholy was in effect just what it sounds like to a twenty-first-century reader: low spirits accompanied by profound and resistant belief that something is terribly awry in the body.¹⁷ Burton mentions the specific fear of illness we now associate almost exclusively with hypochondria in a subsection on ‘Symptoms or Signs in the Mind’. Here he writes that

Some are afraid that they shall have every fearful disease they see others have, hear or read of, and dare not therefore hear or read of any such subject, no not of melancholy itself, lest, by applying to themselves that which they hear or read, they should aggravate and increase it.¹⁸

It might surprise a modern reader to come upon such a description, so familiar will it sound to anyone who has experienced this kind of nervous impressionability and/or avoidance. Burton’s explicit link between fear of disease and melancholy also resonates with a correlation now codified in psychological research between health anxiety and depression.

The complete original title of Burton’s text is *The Anatomy of Melancholy, What It Is: With all the Kinds, Causes, Symptomes, Prognostickes, and Several Cures of it. In Three Maine Partitions with Their Several Sections, Members, and Subsections. Philosophically, Medicinally, Historically, Opened and Cut Up*. While its length and granularity are not atypical for the era, this title does also suggest that melancholy, far from being a simple emotion – or a singular deviation from normative human temperament – is in fact a lens on *everything*, as editor Floyd Dell suggests: the basis from which Burton explores meteorology, magic, geography, religion, the law and love.¹⁹ Hypochondria is embedded

in this voluminous discourse as a bodily manifestation of complex ‘perturbations’ – assisting Burton’s effort, we might say, to help readers understand themselves. In this sense hypochondria as it was understood in the early modern era mattered integrally to how a society might recognize, treat and assuage the most challenging and mystifying of their life experiences; to articulate one’s intense if fanciful interpretation of what is happening within was to trust that someone else would be able to receive the oddest or most piteous of complaints. The *Anatomy* is extraordinary for the breadth of its erudition and the lively voice that collates its myriad medical and philosophical examples – but it is also an appealingly sympathetic account. Burton takes the emotional pain of health fear seriously, even when he recounts travails as bizarre as a belly full of frogs. The fact that he revised and reissued his lengthy treatise five times in his own lifetime demonstrates its relevance, and its popularity, as a disseminator of both information and care.

MODERN NEUROLOGISTS MIGHT look to medial lobe activity or neurotransmitter imbalance for the cause of such delusion as gastric amphibians, but other contemporary research seeking explanation for hypochondriacal anxiety in corporeal phenomena demonstrates an intriguing continuity with earlier theories of heightened ‘apperception’ rather than psychosis. Some evidence suggests, for example, that the obscure ailments of the hypochondriac might be caused by misfires of the immune system’s proteins, called cytokines, which help to generate the body’s immune response to disease. A so-called cytokine storm can produce the very kind of unspecific symptoms – intense fatigue, diffuse pain, even emotional malaise – that might lead a hypochondriac to fear serious afflictions. In this sense the apparent irrationality of hypochondria is anything but, since something *is*, physiologically speaking, happening, even if it has a more benign etiology than the sufferer might know or believe. Arthur J. Barksy has argued more generally that ‘hypochondriacal patients’ tend to be ‘flooded with a wide range of “background noise” that they are unable to damp down, ignore or “gate out”’.²⁰ Such figurations regard hypochondria as precisely the psychosomatic condition Burton’s *Anatomy* so multifariously details, with its many sufferers of dyspepsia, migraines,

lethargy, constipation, irritability, flatulence and every imaginable symptom of a panic attack, from palpitations to sweating to lightheadedness.

Take the case of James Boswell (1740–1795), best known as the biographer of Samuel Johnson, but also a lawyer, diarist and notorious melancholic, who penned a column for *London Magazine* from 1777 to 1783, under the pseudonym ‘The Hypochondriack’. As Theodore Dalrymple explains of Boswell in the *British Medical Journal*, the man who titled himself thusly was ‘not the man who is consumed by fear of illnesses he does not have but the one who suffers from melancholy, spleen, or the vapours’ and goes on to suggest that if Boswell were alive today, he might be called ‘The Depressive’.²¹ Certainly that sobriquet seems apt for the ‘brood[ing]’ Boswell who wrote in 1763 that he was ‘seized’ by a ‘deep melancholy’, that he felt ‘old and wretched and forlorn’ (he was but 22 at the time), that ‘despair’ slid inexorably to ‘darkness and woe’ and that he feared he was going ‘mad’.²² Boswell’s letters and journals from this period are full of such lamentation – descriptions of ‘the most shocking and most tormenting’ unhappiness, a state of mind tortured by ‘the blackest ideas’ and ‘no inclination for anything’, of a ‘wretchedness’ that cannot be imagined by ‘one whose mind is sound’.²³

But ‘Depressive’ doesn’t quite capture Boswell’s ‘Demon of Despondency’, which also activated serious terror about the state of his health.²⁴ ‘I dreaded the worst,’ he writes on 3 February 1763, upon feeling himself ‘not so well’, and the next day reports a bad night and ‘dire apprehension’ about a swollen testicle.²⁵ Cases like Boswell’s epitomize the conundrum of origins at the heart of hypochondria, the difficulty of teasing apart which comes first: a predisposition to pay acute attention to the signals of the body (‘I was seized with a severe cold. My nerves were relaxed, my blood was thickened’),²⁶ or a body so activated by sensation that it becomes impossible to ignore (28 December 1779: ‘I dreamt that I saw the cause of my toe being so painful’; 3 January 1780: ‘Had dreamt again of my toe’).²⁷ We seek such distinctions, differentiating between how, what and where we feel. But at what point, and in what contexts, does the difference matter? For Boswell, the *fact* of suffering and the *fear* of suffering were inextricably connected as manifestations of what he called his ‘hippishness’ – a slang variant of ‘hyp’, from hypochondriacal – and his

experience of that suffering entangled mood and corporeality in a repeated and agonized anticipation of death.

Did Boswell have a panic disorder, illness anxiety disorder, somatic symptom disorder, conversion disorder, a mood disorder like cyclomythia, or bipolar III? Allan Beveridge rejects any effort to assign psychiatric labels to Boswell's infamous and lifelong suffering, writing in the *Journal of the Royal Society of Medicine* in 2000 that 'the best guide to Boswell is still Boswell himself, in the form of his extensive journal.'²⁸ Jess Keiser takes a different approach, calling Boswell a 'neuroscientist of the self' and arguing that Boswell's 'relationship to the world, and to himself, [was] mediated by (what he [took] to be) the workings of his nervous system'; Keiser suggests that Boswell's 'nervousness concerning the health of his mind and body' derived from a sense of being able to perceive his own corporeal interiority.²⁹ Boswell's many assessments of himself demonstrate a mind thoroughly involved in a programme of self-scrutiny meant to uncover the nature, the quality, of his suffering. A hereditary etiology is frequently invoked,³⁰ but more often Boswell cites his animal spirits and the vibrations of his nerves. Bolstered by that plethora of data, Keiser tells us, Boswell was inclined to theorize about the causes and cures for hypochondria even though he recognized the ultimate futility of that endeavour. Too much remained elusive about the relationship between mind and body, and Boswell was cognizant of how often medical learning crafted itself along metaphorical, *rhetorical* lines.

Was Boswell's hypochondria also a function of societal pressure not to be ill? It's clear that, whatever he believed to be the cause, bouts of 'spleen' and 'distemper' brought Boswell great shame and mortification; for all his intensively philosophical introspection, his susceptibility to illness felt like something of a failure. 'The pain which this affair will give my worthy father shocks me in the most severe degree,' he wrote of an especially bad bout to his friend William Johnson Temple in August of 1763, and the next year to John Johnston, 'Oh, how I was galled! Oh, how did I despise myself!'³¹ Boswell and friends alike – including fellow melancholic Samuel Johnson – urged him towards a regimen of strict industry to counteract the psychophysiological morass of hypochondria. Speaking to himself in his 'Inviolable Plan' of October 1763 ('To be read over frequently') Boswell adjures himself to 'rational and prudent conduct', to

have ‘a sense of piety ever on your mind’, to leave off ‘frivolous’ and ‘ludicrous’ habits and to ‘remember that idleness renders you quite unhappy.’³² The plan reiterates the word ‘firm’ several times, because hypochondria seemed to be both characterized and caused by loose, dissipated behaviour. Here Boswell seeks to assuage his struggle with recourse to the sense of ideological duty befitting a man of his class: ‘Be steady to the Church of England,’ he advised himself; ‘Be firm to religion . . . like a decent gentleman.’³³

And yet other energies emerge from the many references to hypochondria in Boswell’s work, ones that subvert the pressure to resist ‘vacuity, gloom, and fretfulness’ as violations of upright social behaviour.³⁴ Not least of these are the buoying effects of friendship. Boswell seems to take it for granted that his interlocutors will grasp not just the extent but the legitimacy of his worry. Indeed, in a note titled ‘Memorandum of Topics to Discuss with Rousseau’ – Boswell had a series of visits with the great philosopher in late 1764 – ‘hypochondria’ is listed second only to suicide.³⁵ He assumes the subject is of broad social, *intellectual* interest, in other words, and that others will have something to offer to, and to gain from, the conversation. This may account for how many of the essays printed under the moniker ‘The Hypochondriack’ don’t concern health anxiety – or melancholy or depression – at all (but instead cookery, drinking songs and the use of quotations). Far from a mockable, disgraced subject, the ‘hypochondriac’ announces himself as a person of authority from whom wisdom of all sorts might be disseminated to a widespread audience of erudite and socially engaged readers. When Boswell does venture a more intimate, revelatory persona, in column no. 39 from November 1780, writing in a ‘state of very dismal depression’, he speaks to readers whom he ‘considers . . . as his friends’ and assumes their understanding. ‘I have a mind to try what I can write in so wretched a frame of mind’, he says, ‘as there may perhaps be some of my unhappy brethren just as ill as myself, to whom it may be soothing to know that I now write at all.’³⁶

The powerful fear of getting sick or the sensations of a bodymind indeterminately off-kilter propel people forwards: towards reassurance, contact, utterance, alleviation, knowledge and, as a mechanism of immediate comfort, *writing* – as when Boswell asserts at the end of the aforementioned issue of ‘The Hypochondriack’, ‘While writing this paper, I

have by some gracious influence been insensibly relieved from the distress under which I laboured when I began it.’³⁷ Hypochondria in such instances operates as a mode of discovery, as a dialogue between bodies and selves, between the hypochondriac and anyone who will listen: a conversation about evidence, about subjectivity, about ambiguity and meaning – the very meaning of life. It might behove us, in a world unexpectedly beset by pathogens, to take the signals of the body seriously, in whatever form they present. But the opportunity for awareness raised by hypochondriacal anxiety goes beyond the local or imminent possibilities of contracting illness, because hypochondria is always already a heightened encounter with the existential dread that more of us might be susceptible to, if we stopped working, exercising, travelling, shopping, posting and otherwise distracting ourselves long enough to feel it.

In this sense, to return to Defoe’s *Journal*, hypochondriacal fanciers don’t represent naive faith in magic so much as the fact of a mortality that is making itself urgently known. The *narrator* of the novel scolds those gullible enough to see angels in the sky for misdirecting their fear, but the *text* allows us to ask why the particular trigger for fear really matters if its result is to conduct readers towards more salutary practices. The book’s inclusion of mystical portents might then have an effect not much different than a shadow on a scan or a bone-deep ache – or just the certitude that something is wrong (because something always *is*, at least in theory) – in initiating inquiry. The hypochondriac is then not simply a helpless manifestation of ideas about the dreadful consequences of being ill (or of *losing* ‘health’) but instead a reminder of our contingency, the vulnerabilities of the human condition. Again Boswell serves as a signature example. He addresses ‘The Hypochondriack’ to ‘people like [him]self . . . who are arrived at the age of serious thinking; to beings . . . who can judge rationally, yet feel keenly; who have an incessant wish for happiness, but find it difficult to have that wish gratified’.³⁸ In other words, life is strenuous, full of troubles and likely disappointments; some of us will feel its challenges acutely and in ways that are not easily assuaged. It’s up to the *hypochondriac*, Boswell intimates, to articulate these ‘serious’ matters of existence – and all who consider themselves similarly invested in thoughtful, unafraid dedication to the fullest range of emotions and experiences will pay heed.

DISCUSSIONS OF HYPOCHONDRIA in the modern age frequently make the claim that hypervigilance about one's health is an inevitable consequence of societal incitement not to be sick. Carla Cantor in her 1996 book *Phantom Illness* defines 'healthism' as 'a symptom of our time' and cites media-driven 'diseases of the month', a healthcare system financially motivated to test and treat us, the scaremongering tactics of advertising ('Is It a Mole or Melanoma?' 'The Itch to Worry About' and so on) and the paradox that generalized social emphasis on wellness has made us 'intimidated by disease' – all of which conspire to focus our attention inordinately on what is, or could be, wrong with us. Without employing the language of ableism, Cantor rightly describes an ideology of normalcy in which 'our bodies' are made 'the enemy' and 'virtue' is 'redefined . . . as health'.³⁹ In this scenario, it is incumbent upon us – a moral imperative – to hold ourselves to strict regimens of diet and exercise and to scan ourselves regularly for signs of illness. And when we *do* get sick (which, obviously, we will) it may feel like, or even be overtly construed as, our fault: a result of our reckless bad habits or repressed, mismanaged emotions. The hypochondriac is taken to be at once the logical consequence of this situation *and* its unsavoury exaggeration, helplessly responding to messages about the imminence of frailty and disease and thereby summoning the very conditions we are everywhere exhorted to forestall.

None of this, of course, is precisely new. We can trace the notion of 'communal hypochondriasis' to a 1934 article in the *British Medical Journal* in which physician Robert Hutchison argued that 'over-anxiety about health', stoked by 'lectures, talks, leaflets, and slogans', can 'affect a whole community' (an 'attitude of mind' Hutchison claimed to be 'rampant in America').⁴⁰ In the nineteenth century, the English physician Thomas Beddoes similarly laid some blame for hypochondria on opportunistic physicians and medical fads, also suggesting that illness-obsession reveals more about how societies think about health than it does any individual's pathological worry: his multi-volume collection of medical and moral essays called *Hygëia* (1802) puzzles over hypochondria as an 'indicator of something wrong in the very notion of well-being', as George C. Grinnell explains.⁴¹ Before Beddoes, accounts of a widespread 'morbid streak' among the English were so common that Sir Richard Blackmore wrote in 1725 that his countryfolk experienced 'hypocondriacal and hysterick affections' to 'a superior and distinguishing degree', and so frequent had the

complaint of ‘spleen’ become by 1733 that Cheyne would describe it as nearly a kind of mass national distemper.⁴²

The antecedents of such consternation about social health go all the way back to Plato, who wrote in the *Republic*, around 375 BCE, that medicine as an autonomous discipline had subordinated ends to means and was encouraging an ‘exaggerated concern with health’, as David Heyd explains, that separated ‘medical practice from other dimensions of life’. Hippocratic medical practice in Plato’s age had replaced magical explanations for disease (such as divine punishment) with naturalized theories of bodily disharmony that required treatment rather than purification. But while Plato respected this “‘rationalization” of medicine’, to quote Heyd again, he was nonetheless also critical of what he beheld as an excessive concentration on the body that made people ‘cling to absurdly strict diet and daily routine’ and ‘stimulated physicians to . . . regulate their patients’ lives and [apply] more aggressive medical techniques’. According to Plato, then, medicine had produced a ‘self-indulgent, even obsessive, concern’ with health and was guilty of ‘disgraceful’, ‘ingenious’ invention of ‘newfangled and monstrous’ diseases.⁴³ Isolating the body as its sole object of diagnosis and therapeutic intervention, medicine became problematically untethered from the social and spiritual dimensions of existence.

But in its effort to return extracorporeal matters of soul – or call it mind or personality – to the health of the body, Plato’s approach also entrenches a moralized conception of illness: one cannot *truly* be deemed healthy if moral status remains compromised. This is why Plato’s republic would deny care to ‘those who neglect their health or indulge in harmful ways of life (overeating, heavy drinking, lack of physical activity)’ and why the ‘restoration of health’ is explicitly understood as a matter of shared responsibility between doctor and patient to the larger civic good.⁴⁴ In a world where ‘guardians’ of the *polis* are charged with maintaining health so as not to become a burden, illness is both bad manners and bad politics: a personal failure to uphold virtuous standards, a drain on public resources and a potentially dangerous destabilization of the state. And this is not so far, ultimately, from much more recent public fears that disease will ‘corrupt’ the collective – a word Robert Burton would use in 1621, for example, and a condemnation that reappears in twentieth-century right-wing opposition to government-funded research on AIDS.⁴⁵

Plato's stern edict to align corporeality with civic responsibility is echoed in Defoe's novelized alarm bell: the 'body politic' is literal. Allowing for important differences in what (and where in the body) hypochondria is, the conceptual resonances from one era to another tell us that the fear of illness will always bear some relation to broad cultural attitudes about health and in particular to our tendency to deny the very humanness we can't take our minds off of. Perhaps we no longer vilify hypochondria as *intemperance*, as Plato did – an indulgence not so different from violations of the sacred triumvirate of food, drink and exercise in moderation – but hypochondria in both popular and academic depictions of it still bears the traces of something disreputable, an addictive self-absorption and a failure to uphold the moral obligation to safeguard the health of the collective. Does the fault lie with the individual? One way to think about hypochondria is that, regardless of whether we define it as vapours arising from the gut, a disease of the spleen, a disorder of nerves, an imitated family behaviour, a chemical imbalance, a cry for help or a rational response to the unpredictability of corporeal existence, its sufferers have always been exceptionally obedient students of a contradictory cultural message: to think constantly about illness, and to never get sick.

Judy Z. Segal explains this cycle whereby we are *made* ill as one of complex social persuasion – we are interpellated into the role of patient so that we can then become consumers of what will make us well again, the 'hypochondriacal imagination' being constantly fed by public discourse.⁴⁶ Segal argues that hypochondria 'is *essentially* a rhetorical disorder': we are invited to be ill via the persuasive power of advertising or media headlines ('What Is Avoidant Personality Disorder?', 'Giant Study Pinpoints Specific Gut Bacteria Linked to Alzheimer's', 'Four Things Your Eye Color Might Reveal About Your Health', '10 Things that Might Be Misdiagnosed as Anxiety') and then exhorted to do something about our alarming decline, often through the use of pharmaceuticals. In arguing for hypochondria as a 'dialogic' situation, Segal emphasizes the condition as 'a way that persons act on society and a way that society acts on persons':⁴⁷

The hypochondriac is addressed by mass media; the hypochondriac converses with himself or herself, making interpretations, coming up with arguments for illness, resisting them, being won over; the hypochondriac addresses the physician, arguing overall unpersuasively; the physician addresses the hypochondriac, matching the patient's unpersuasiveness with his or her own; the hypochondriac is left to contend with social life, usually in the absence of a satisfying diagnosis and subject again to media address and self-talk.⁴⁸

Hypochondriacs aren't just troublesome, unreasonable worriers who refuse to be quelled by medical expertise, then, but actors in an elaborated social relay of attempts to prevail and convince – *agents*, in fact, who have a great deal to say about 'risk', about 'intolerable ambiguity'.⁴⁹

Segal helps us to understand hypochondria as an entreaty to notice – not individual ailments (in the egotistical sense) but rather the common 'feature[s] of our humanity'.⁵⁰ Put slightly differently, Belling writes that 'public health discourse produces many of the cultural narratives that frame and inform hypochondriacs' perception of . . . susceptibility to disease': in other words, for the discourse to work, to motivate, we have to already believe we're vulnerable.⁵¹ The extraordinary social and economic resources devoted to ensuring our 'wellness' mask a paradoxical reality that, despite our pretensions otherwise, we are always in some measure cognizant of being in decline – and in this sense the hypochondriac is simply trying to get the rest of us to quit ignoring risk and denying a basic human truth about mortality. Hypochondria as a method of sounding the alarm thus entails a self–other dynamic. No one agonizes alone, even in a late-night perusal of mayoclinic.org, since worry, as an urge to discharge tension, compels us to tell. Thus we might, through both the anguish and the annoyance of hypochondria, refine how we talk to each other about what ails us, beyond the tug-of-war that is diagnosis. To narrow every interaction to the truth status of sickness is to miss an opportunity to ask other questions of hypochondria's communicative utility.

CARLA CANTOR BEGINS the Prologue of *Phantom Illness* by recounting the extremity of her own hypochondria, and she ends with the hope that the book will ‘giv[e] courage’ to other health worriers and their families and friends. Not a memoir but a thoroughly researched study, the book nonetheless has something of the flavour of a recovery narrative – a tone emphasized by the Prologue’s final sentences, in which Cantor implies that the responsibility for ‘solving’ hypochondria lies squarely with the patient: ‘Once you make the choice to be well, both in body and spirit, you’ll cherish your health. You’ll *never* want to take flight into sickness again.’⁵² Such language betrays a few uncritical ideas, I would argue, not least of which is a rigid binary between health and sickness and the singular definition of hypochondria as a means of escape, a process of somatizing emotions that the individual needs to unearth: ‘confronting hypochondria may mean uncovering unpleasant truths about yourself.’ What strikes me in this framing of hypochondria as a bad habit at best (a ‘way of life’) and a luxury of deflection at worst (‘an elaborate set of defenses’) is that, despite the empathetic notes, it betrays its capitulation to neurotypical ideas about mental health and manages to blame hypochondriacs for their attraction to physical illness. The message here is that sickness writ large is inherently bad, and there’s something even worse, dramatically so, about inviting it.

The paternalistic and ultimately stigmatizing view that hypochondria is a kind of interpersonal ‘stratagem’ – a concept that appears throughout Susan Baur’s 1988 *Hypochondria: Woeful Imaginings* – renders hypochondria’s communicative effects in terms of emotional manipulation.⁵³ Baur’s analysis, though ostensibly a survey of hypochondria’s cultural and medical history and popular ‘primer’ of the condition’s etiology, sociological incidence and treatments, betrays a fundamentally dismissive attitude, even a contempt, towards a phenomenon the book repeatedly frames in terms of intention and advantage. When Baur writes, ‘there are thousands of hypochondriacs who masterfully use their headaches and upset stomachs to manipulate others into waiting on them’ and others ‘who outgrow their preoccupation in a few months’, she makes the overarching slant of her study quite obvious. It isn’t just that a hypochondriac misinterprets symptoms or medical information. More insidiously condemnatory are claims that patients ‘adroitly . . . [exploit] the sick role’, that ‘the disorder is likely to be chosen by persons who have learned to play the oppressor *and* the oppressed’ and that hypochondriacal symptoms are ‘tactics’ – thus

‘sickness was champagne’ for Charles Darwin’s daughter Etty and ‘used’ by the writer Sara Teasdale to keep away from her husband. Hypochondria emerges in those ‘afraid to deal honestly with people . . . unable to deal honestly because of unacknowledged fear or weakness’, Baur declares, and it becomes especially malevolent in such circumstances:

Whatever the symptoms, they allow the hypochondriac to seek power covertly by engaging in a dialogue of innuendo and half-truths . . . Hypochondriacs thus establish a series of *mutually dependent relationships* . . . Not only does their illness function as a substitute for deeper, more dangerous conflicts but it also provides a way for hypochondriacs to punish their spouses and themselves (yet avoid personal blame . . .). They also sidestep onerous tasks yet avoid social blame because their actions are neither rebellious nor insane.⁵⁴

It’s difficult to imagine a reader turning to *Woeful Imaginings* with the hope of better understanding their own or a loved one’s health anxiety and not coming away from such an account with their worst impressions confirmed. I dwell on Baur’s slant because it represents the *scholarly* version of a deeply embedded cultural stereotype about hypochondria as misdirected psychical energy.⁵⁵ If hypochondria is inherently interpersonal and social, in this view, it’s only in the most unscrupulous of ways.

But what would happen if we took deceit and deflection out of the narrative, the idea that hypochondriacs use ailment to conceal emotion that cannot be uttered, thus lying to themselves and others about what’s ‘really’ going on? What if we exploded notions of ‘really’ to make space for forms of fear and longing for which there is no other form of request? What if hypochondriacs are telling an emphatic and absolute truth about ambiguity – proving through their apparently fictitious ailments the illusory nature of ‘health’ itself? What if we learned to read hypochondriacal body language, even if ‘the very complaints of the hypochondriac’, to quote Segal, have no ‘demonstrable correlatives in the body’?⁵⁶ What if there is no hypochondria at all, only systems interacting with systems, bodyminds striving to find balance with not-self, in a stumbly unchoreographed dance about how to live?

An answer to such questions can be found in Charlotte Brontë's late novel *Villette* (1853), which, though not 'about' hypochondriacs in a dominant thematic sense, nonetheless features a narrator who refers to herself as such more than once, a woman in whom the conflation of emotional sensitivity and physical complaints will be familiar from previous centuries' accounts of hypochondria and aligns too with shifts in nineteenth-century discussion of nervous complaints. Victorian physicians debated about whether the origins of hypochondria were mental or physical. James Johnson isolated the 'stomach and bowels' in his 1827 book on the 'proximate causes' of nervous conditions; Ralph Fletcher a few years later reversed the causal direction, locating the origin of physical symptoms in the mind.⁵⁷ Still later Thomas King Chambers noted in his 1873 'Lecture on Hypochondriasis' that 'the patient feels all wrong but understands all right', by which he means that an entirely 'sane' individual might nonetheless experience a 'pathological' degree of 'protracted sorrow' in combination with 'a disorder of the sensitive parts of the nervous system'.⁵⁸ Brontë would reiterate such ideas about the relationship between mind and body through a narrator – much as Boswell had a century prior – acutely sensitive to the operations of her bodymind, one who well understood that the 'inflamed', 'nameless agony' of nerves could sink a person to abject misery.⁵⁹

This narrator is Lucy Snowe, a confoundingly private persona who frustrates readers' impulse to know her, her back story and her evident trauma. We never find out why Lucy has been 'absent six months' from home, her memory is oddly faulty, and her vague mention of 'impediments' seems hardly sufficient to account for her 'Anomalous' circumstances. When she arrives on the doorstep of Madame Beck's boarding school in Belgium, Lucy can offer no reference, no letter of introduction, no context, no guarantee of her social or moral worth – cannot even speak French – and so her *body* must speak for her: 'I want your opinion,' Madame Beck says of Lucy to Paul Emanuel, another teacher: 'We know your skill in physiognomy; use it now. Read that countenance.'⁶⁰ We are accustomed to thinking of the hypochondriac's physical being as a site of contestation between self and physician, a battleground both overly legible and *il*-legible; here, Lucy's body becomes her ticket to passage, constituting in and of itself an opportunity for Lucy to say who she is. At the same time, in

refusing the kind of narrative transparency readers expect from realist fiction, hinting at but never confirming what might be troubling its heroine, the novel *itself* performs a kind of hypochondria on its readers, since we know something feels amiss in our experience of reading, our apprehension of its narrator, and yet our efforts to uncover the cause, to ‘diagnose’, will be repeatedly defied.

But hypochondria in *Villette* is not just a matter of slippage and misunderstanding. As I’ve suggested, Lucy Snowe’s body facilitates acceptance in a situation otherwise marked by potential rejection, in a scene that presages hypochondria as both a claim to autonomy and a modality of contact. ‘My art halts at the threshold of Hypochondria,’ says Dr John Bretton by way of apologizing that he can’t offer Lucy the same sort of assuagement with which he has attended even to ‘singularly interesting’, ‘rare’ and ‘critical’ diseases – but that stymying of the physician’s skill is no impediment to their friendship; indeed ‘acquaintance was better cemented’ as a result of the renewed contact after many years that Lucy’s suffering has occasioned. Later, upon receipt of a letter from Graham’s mother (Lucy’s godmother), Louisa Bretton, Lucy reflects on the near impossibility of conveying the anguish of melancholy to someone uninitiated in its ‘mystery’. ‘Speak of it!’ Lucy thinks; ‘you might almost as well stand up in an European market-place, and propound dark sayings in that language and mood wherein Nebuchadnezzar, the imperial hypochondriac, communed with his baffled Chaldeans.’⁶¹ Here Lucy ‘insist[s] on the singularity and incomprehensibility of [her] feelings’, as Rachel Ablow has argued, in a way that seems to take some pleasure in that opacity; denying us ‘full access to her interiority’, in Ablow’s words, Lucy defies a stereotype of hypochondria as staged performativity as well as what we might think of as hypochondria’s essential urgency towards outward appeal – for answers, for recognition.⁶² Instead, hypochondria is linked to an active obfuscation, as if *not* being ‘known’ or soothed could guarantee Lucy’s creative independence.

Yet in the very same moment, the reference to Nebuchadnezzar constitutes one melancholic *fully* comprehending the loneliness and fear of another; the analogy only works because Lucy can sardonically identify with the king’s struggle to communicate his troubling dreams.⁶³ This echoes an earlier scene when Lucy encounters the king of Labassecour at a concert,

and remarks that ‘at first the strong hieroglyphics’ of his face ‘puzzled and baffled’ her. Soon, however, ‘if [she] did not *know*, at least [she] *felt*, the meaning of those characters . . . There sat a silent sufferer – a nervous, melancholy man’ whose eyes ‘had long waited the comings and goings of that strangest spectre, Hypochondria’. This king, Lucy pronounces, has ‘constitutional melancholy’.⁶⁴ Lucy can claim no intimacy with the king, has never met the king; she is reading only body language, recognizing herself in – or perhaps mapping herself on to – the signs of another’s ordeal. Lucy and the monarch are separated by rank, gender, breeding – every marker of social stature that would matter in the world of the novel – even by the space of a concert hall. It is only hypochondria that establishes any sort of connection, even if only Lucy’s own imaginative identification with a stranger.

Villette thus reminds us that hypochondria does not always happen in clinical settings and that for some writers, Boswell and Brontë among them, it amounts to neither a contest over meaning nor a coded utterance but is instead a source of knowledge and connection on its own terms. Where Baur reinforces a threadbare appraisal of the condition as one of emotional immaturity and relational manoeuvring, Brontë presents it alongside a woman’s fiercely independent refusal of social politesse: what the critic D. A. Miller once referred to (in a piece about Jane Austen’s *Emma*, to which I turn in the next chapter) as ‘the tyranny of the social . . . so intractably oppressive that resistance can only take the form of an equally total and nonnegotiable negation’.⁶⁵ Lucy Snowe’s particular self-assertions as a hypochondriac do not entail the kind of unconscious neediness or self-absorption that Baur’s volume emphasizes; they are far more akin to Brontë’s own devotion as a woman writer to remaining ‘uninfluenced or swayed’ by a sense of dutifulness to her readers.⁶⁶ The hypochondriacal woman in this context is one who thinks for herself, *knows* herself, refutes convention and finds new pathways towards understanding and sympathy.

The flip side of social coercion may thus be ‘social relations’ – towards which Dr Chambers was encouraging his hypochondriacal patients as long ago as 1873.⁶⁷ In a more recent autobiographical piece called ‘The Wind Did It’ from his 1990 collection *Maps to Anywhere*, essayist Bernard Cooper writes hypochondria in just this way, straight into the heart of his relationship with his father. ‘Burning at the center of our discourse like the

sun', he says, 'is the one subject we never miss, the subject which dwarfs and outshines all the others: the state of our health.' Health issues are the substrate of a mood Cooper defines as 'camaraderie' and 'confidentiality', where father and son 'share' their 'mutual fascination with the body and the countless ailments that might besiege it', in dialogues that Cooper foregrounds for their shifting positions of worried and reassured: 'Oh, that's just a', 'How do you . . .', 'You don't think . . .', 'Believe me' and so on. In such an exchange, ailments are both themselves (there really *is* a 'red thing on my wrist') and a language, a process, that concretizes a bond.⁶⁸

They are also material for Cooper's own writing. To render this 'burgeoning relationship' with an aging (and formerly philandering lawyer) father, Cooper turns often to their bodies – to meals, to scenes of swimming, to episodes of illness.⁶⁹ Shared pleasures but also what can go 'wrong' in a body become generative material, both a mechanism whereby father and son give each other permission to be human, and Cooper's own artistic medium for conveying the possibilities of love, respect and forgiveness. So much of the language of and around hypochondria homes in on distress and deception that we forget, I think, that there is also care, interest and curiosity, even enjoyment in the travails of the body. I don't mean this in the sense of sardonic hypochondria humour – Woody Allen's biting neuroses or the broad comedy of books like Dennis DiClaudio's *The Hypochondriac's Pocket Guide to Horrible Diseases You Probably Already Have* – but rather the quiet intimacy that admissions of bodily difficulty and worry might facilitate.

Here I think of literature's renowned hypochondriac Mr Woodhouse, father of Jane Austen's Emma, and the puzzle of wedding cake. What is wedding cake, after all? Perhaps it congratulates a couple on their entrance into a union, but one that is itself available to contradictory meanings (hallowed conjoining or performative expectation? Heteronormative construct or serious, unapologetic commitment?). Perhaps it is meant to display the couple's (or their parents') wealth and aesthetic taste – or again, their rejection of prevailing norms and trends. Then again, as for Mr Woodhouse, whose 'stomach could bear nothing rich', it can become a means of calibrating relationships: 'he could never believe other people to be different from himself,' and so he 'earnestly trie[s] to dissuade' the wedding guests from eating any.⁷⁰ Some will read this as quintessential

hypochondriacal control, but is it not also a form of care, a casting outward of oneself, a curious blurring of distinction in an otherwise highly stratified social environment? For this is embodied cognition, too, a way of knowing what we can tolerate *not* knowing, and a way of threading ourselves towards our beloved others.

THE KIND OF ‘communal hypochondriasis’ that Robert Hutchison attributed in 1934 to the influence of leaflets and lectures is not just an indictment of a health-obsessed zeitgeist (though we will see throughout this study that ‘health and fitness’ is but the flip side of hypochondriacal sickness). Belling translates Hutchison’s third category of hypochondria as ‘an entire community’s shared fears for the health of its constituent members’, suggesting that ‘hypochondriacal’ attendance to disease on a widespread social scale is just another way of saying ‘mainstream attitudes to health’ in a medicalizing culture that insidiously berates us towards unrealistic standards of maintaining healthiness.⁷¹

Another way to frame a shared concern for health is to repeat the question with which this chapter began, and to consider hypochondria in the context of an immediately threatening disease – like plague or COVID-19. Hutchison’s article skewers what he calls ‘nosophobia’ – fear of a particular disease – as one iteration of ‘individual hypochondria’, which pertains to convictions of having specific serious illnesses in the absence of verifiable diagnostic evidence. In contrast to this individualized and supposedly irrational worry, anxiety about a highly contagious virus in the immediate surroundings should seem *reasonable* – it *could* happen because it *does* happen, is happening, all around us – whereas a similar degree of panic over heart disease, cancer or other noncommunicable diseases might seem fantastical (no different than Defoe’s angels in the sky). Yet cancer and heart disease *also* happen, and deaths from these illnesses far exceed the incidence of death from COVID-19 since the start of the pandemic.⁷² At the same time, for many even in the midst of the worst waves of subvariant spread, fears of catching COVID-19 were dismissed as excessive, labelled ‘coronophobia’. Hypochondria is characterized by a resistance to reassurance; with COVID-19, scepticism about medical authority, certainly in the United States, swung all the way in the opposite direction to a

dangerous refusal to admit the possibility of illness. We didn't need the death-tolling charts of a mock-novel to track COVID-19's lethality throughout 2020 and 2021, thanks to websites like CovidActNow or the daily case counts reported by the *New York Times*. But who, in this relentless contest over risk, has the jurisdiction to determine how seriously we take our culture's cautionary tales?

I return in [Chapter Three](#) to the particular struggle over *knowledge* that hypochondriacal fears manifest and to what the pandemic years might have taught us about medical messaging. Here I want to suggest that the terror of the pandemic was not just about the virus per se, legitimate as that feeling obviously was (or remains), and to consider expressions of fear as a dynamic, even hopeful form of exchange in a social environment that – to many but in different ways – felt acutely turbulent, hostile and unwelcoming. To say out loud one's fear of catching a virus in such circumstances is also to entail degrees of trust and to gesture at personal and social attitudes about family, authority, the future, friendship, even love. I refer here not to the notion of emotional 'use' but rather to a language of collective enterprise, since in voicing our anxiety about a symptom we articulate larger concerns too, appealing to one another not only about disease but about caretaking, dependability and the foundational tenets of our communities. If one fretted in the summer of 2021, say, over the implications of shortness of breath or sore throat and told someone so, such a worrier might have been seeking epidemiological reassurance about risk, which is also a way of ascertaining the quality of collective behaviour and wondering about the extent of what we can, and should, do for each other as residents, parents, travellers, students, employees, officials. To be sure, hypochondria may be defined as 'somatic over-concern' and 'morbid attention to . . . bodily functioning'.⁷³ But is it not also asking what sort of world we all want to live in, as the vibrant, variegated, fleshly beings we are?

The speaker of 'My Therapist Asks Me Why I Haven't Told My Parents Yet', from Emily Paige Wilson's 2020 chapbook of poems called *Hypochondria, Least Powerful of the Greek Gods*, asks plaintively – or helplessly – 'what would I tell my family?', implying *why?* or *what would be the point?*⁷⁴ Each poem in this slight but powerful collection offers a variation on this theme: how to explain one's experience, one's reality,

one's pain, when others have 'never taken her/ symptoms seriously' ('Hypochondria and Her Estranged Half-Brother Sisyphus').⁷⁵ Human speakers – and it seems clear, by inference, the poet herself – thus morph into the pantheon's embarrassing relation, destined (or doomed) to reiterate symptoms to doctors who 'tire' of hearing about them and lovers who think a woman 'deserve[s]/ [her] sickness'.⁷⁶

In the third of the volume's title poems, 'Hypochondria knows if the living could see what the dead look like, they'd never light a candle.'⁷⁷ Here the implication is that we obscure our mortality in the very act of seeming to honour it. Pretending to acts of ritual and dignity in relation to our humanness, we indulge a performance, comfort ourselves through the display of piety in lighting a candle whose very illumination distracts us from horrible spectacle of our mortality. And Hypochondria knows all this; she observes 'the divine freedom of [her] fearless' brethren, suffers the emptiness of being perpetually 'ignore[d]'. And yet the book ends on a different possibility entirely – one of reciprocity, empathy and acceptance – in its fourth and last title poem, in which Hypochondria speaks: 'Name your fears like children,' she says to the 'other gods'. 'They will listen when you call.'⁷⁸

2

Bodies in Time: Hypochondria Stalls

‘And someday it will come
for us’

WILLIAM GREENWAY, ‘Hypochondria’¹

‘Nothing, surely, can be more idle and absurd,’ Dr John Reid declared in his *Essays on Hypochondriacal and Other Nervous Affections* in 1817, two years after Jane Austen’s novel *Emma* was published, than to spend one’s life ‘endeavor[ing] to preserve it’ by constant vigilance over one’s health.² It is common to disregard hypochondria this way, as a waste of time, an untoward interruption of life’s appropriate forward motion. Until the condition moved more squarely into a psychological diagnostic realm and began to be treated either psychodynamically or psychotropically, a common tactic was to address the putative lassitude of health worries – which is why ‘hard work’ is so often prescribed as an antidote to the ‘worse’ fate of ‘idleness’, as did Dr Thomas King Chambers in 1873, and why James Boswell, in his ‘Inviolable Plan’ a century before, reminded himself that ‘idleness renders you quite unhappy.’³

Contemporary commentators also reveal this attitude towards hypochondria as a kind of emotional uselessness, a problematic, morally suspect digression from the kinds of efforts and activities that constitute a meaningful existence,⁴ in a way that betrays the thoroughgoing ideological link in industrial nations between selfhood and economic productivity. Susan Baur declares without qualification that ‘a sick person accepts the

status of a relatively weak and useless member of society.’⁵ ‘There’s a value in keeping busy,’ Jennifer Traig says more casually in *Well Enough Alone: A Cultural History of My Hypochondria* of the effect of a job on her ‘various compulsions’.⁶ While it might seem commonsensical that absorption in a worthwhile endeavour can assist us in tempering the intensity of depressed and anxious states of mind, the association between work and wholeness is far from an incidental one; when public health messaging emphasizes the ‘costs’ of disease or such statistics as the ‘lost workdays’ of conditions like back pain, our horror is being roused more immediately and seriously by the spectre of lost *productivity* than the actual incidence of *pain* – and with the suggestion that it’s the spur of *work* more than *wellness* that impels us to shuck the self-indulgence of our illness fears (even as we are of course always to be worried about getting ill, lest we lose more time at work). The insult of being a ‘drain’ on public resources is a familiar one in disability communities, where a different common problem – that of wanting to work in an environment often resistant to the hiring of people with disabilities or to making reasonable accommodations to facilitate it – leads to a similar end: the conflation of ontological with financial status. To skew this system through *fantastical* ailments, to disrupt a link between able-bodiedness, social value and contributions to the labour market through the weakness of a psychological tic one ought to be able to ‘overcome’, is an obvious violation of capitalism’s logic.⁷

Carla Cantor’s remark that ‘sickness is a departure from everyday life’ encapsulates a slightly different but no less problematic aspect of hypochondria’s unique relationship to the diurnal rhythms conventionally determined by work as well as the progress narrative of improvement inherent in our ideas about both sickness and life arc more generally.⁸ The metaphor of ‘departure’ (sometimes phrased as the ‘interruption metaphor’) insists on the binary, foundational to ableist ideology, that health is the norm from which illness is the deviation. This is why we say that we’re feeling ‘like ourselves’ again when we recover from sickness, as if falling ill amounts to a lesser simulacrum of real subjectivity, and it’s the conceptual basis that provides many illness memoirs with the rhetorical paradigms of voyage or quest – the ‘repeated themes’, as Anne Hunsaker Hawkins details extensively in *Reconstructing Illness*, of ‘the journey into a distant country’ where illness is a ‘strange otherworld’, an ‘unknown territory’, even an

‘exile’.⁹ To define ‘everyday life’ as wholly free of sickness then, as Cantor implies, is to equate viable selfhood with freedom from the compromising stain of unhealth and so to perpetuate a deeply entrenched reviling of illness and impairment as grotesque aberrations of normality. If we rush to the primary care office – even to the pharmacy – in a panic at the slightest twinge of discomfort, we are satisfying a cultural imperative not just towards health but towards *return*, to revalidate ourselves according to standard expectations of being and doing.

And yet, by Cantor’s own argument (repeated throughout *Phantom Illness*), ‘the bulk of the patient population’ is comprised of people who do not appear to have organic disease.¹⁰ The irony here is that the very numbers purporting to demonstrate the prevalence of hypochondria might also disprove what we think of as health in the first place. For if we’re flocking to doctors’ offices with actual but medically unexplained symptoms (regardless of how we interpret them) and then sent home again without a diagnosis of ‘sick’, is there not something awry in our definition of ‘healthy’? What does it mean to be ‘well’, after all, if a diagnosis of hypochondria – the very *epitome* of not-sick – includes headaches, dizziness, nausea, back pain, fatigue, indigestion and various lumps, itchy bumps and stabbing sensations that do not sound like wellness at all but also seem practically synonymous with simply being alive? Scrambling these apparently opposed categories, hypochondria exists in between, holding the not-quite/not-yet-sick person in a state of *potential* illness and keeping attention trained on the imminence of what is feared.

Hypochondria is thus defined by time in two distinct senses: one the anticipation of the impending worse-to-come; the other what we’re *not* doing when we’re doing hypochondria. In this liminal place, at once ontological and temporal, it’s almost as if hypochondria hastens what we think of, stereotypically, as *aging* – with its litany of gripes about the changes and betrayals of embodiment (I return to aging in [Chapter Six](#)). In this sense hypochondria simultaneously stymies life’s forward motion and forecasts our own future. Such an achronology reiterates the question I’m raising here about how we abide by normative frameworks of effort and scheduling and about how we define accepted forms of *doing* something. If hypochondria acknowledges the advance of time towards our eventual death and yet defies the goal-orientated, speed-driven nature of many forms

of work, is it properly referred to as an irrational ‘fixation’? Or might its suspension in the might-be and the not-quite-yet afford us other possibilities, other ideas about where we focus our psychic energies? When hypochondria is framed as ‘emotional *paralysis*’, as Baur does, what kinds of movement might we be missing in relation to its characteristically specialized bandwidth?¹¹ What are we ‘doing’ when we’re doing the nothing of worrying about health?

In this chapter I will propose an important distinction between stasis and stalling to suggest that hypochondria encourages – even forces – a reorganization of how we relate to expectations of result, completion, achievement and closure in a variety of social contexts. My point is not to discount the particular distress that might attend a person spending ‘every minute of every day . . . checking whether they’ve got this disease or not’, in the words of one researcher studying illness anxiety.¹² But I do want to employ the concept of what Elizabeth Freeman has called, in relation to queer experience, ‘chrononormativity’ – to explore what hypochondria, for all its apparent ‘stuckness’, reveals about attitudes towards mental work, towards conventional life milestones like marriage and childrearing, even towards history and the past. ‘Crip time’, as Freeman and Ellen Samuels write in their introduction to a special 2021 issue of the *South Atlantic Quarterly*, refers to the myriad ways in which disabled experiences are both crafted by and defiant of the strictures of normative time: as when chronic illness refutes notions of futurity, or when fatigue challenges the cycles of work by which a bodymind is impelled to push beyond its own limits, or when the frustrations of waiting often imposed upon people with disabilities in an inhospitable world might open towards new forms of creativity, connection or caretaking. So too might ‘hyp-time’ expose the tyrannies of normative social life, work achievement and gender performance in stalling our so-called progress – and, in establishing a *pause*, divert our attention to what else is possible.

THE LITERARY TRADITION of portraying hypochondriacal figures as the buffoonish object of pity or an obvious, exasperating schemer – in neither case legitimately concerned about health, and in both instances draining the patience and compassion of others – stages the health anxious as directly, if

not always deliberately, obstructing proper life narratives. In other words, they get in the way. And yet the frequency with which they appear on the page tells us not just that they are commonplace but that they have a more important role to play in our acts of making sense of the bombardments of experience than they're given credit for. In complicating the linear chronology, the causality, of storytelling, the atrabilious move at a different speed. The scandal they represent in this context has less to do with the putative narcissism of their fixations than their refusal to abide by accepted timeframes for getting things done.

Henry Woodhouse, father of Jane Austen's titular Emma, is a prime generator of what I'll refer to here as hyp-time. Mr Woodhouse is among classic literature's great hypochondriacs, introduced to readers as a benevolent but prematurely aged 'valetudinarian' whose fretting about foodstuffs or the miasmic air of London prompts the solicitude – and constrains the actions – of those around him. He is a 'nervous man, easily depressed', who 'hat[es] change', loves gruel and holds his physician Dr Perry in particularly high regard.¹³ It is easy to read Mr Woodhouse's health anxiety as a deflection of riskier emotions. He laments to his elder daughter Isabella that 'in London it is always a sickly season,' that 'Nobody is healthy in London' and so it's 'dreadful' she is 'forced' to live there; yet whatever else he fears about the noxious air of the capital, a different implication of his antipathy appears in a brief aside – 'so far off!' – that registers a more acute sadness about his daughter's absence from his daily life.¹⁴ Hypochondria enables what can't be more overtly said, but its dampening effect on sociability is also obvious, especially in relation to the deleterious effects of weather, as when Mr Weston conceals the fact of a snowstorm 'lest it should make Mr Woodhouse uncomfortable, and be an excuse for his hurrying away' from a party, or when Emma, the next morning, skips a journey through snow to church because 'Mr. Woodhouse would have been miserable had [she] attempted it,' though this deprives her not just of socializing but the exchange of ideas.¹⁵ Deflections, veerings, stasis: even when hypochondria is rendered in terms of affectionate, compassionate, protective tolerance, it seems primarily to hinder and distract.¹⁶

Mrs Churchill, Mr Woodhouse's companion figure, is the opposite type of hypochondriac, an overt fake; her claims to feeling unwell are

understood as calculating deployments of the power she wields as a woman of means but no class, used in particular to control her nephew Frank. She is introduced as a ‘capricious’ woman upon whose ‘ill-humour’ Frank’s movements – and thus practically the whole of Highbury’s social calendar – seem ‘to depend’.¹⁷ ‘That she was really ill was very certain,’ we are told in such a way that ensures we understand Mrs Churchill’s illness to be not certain at all – so that Frank’s refusal ‘to say that her complaints were merely imaginary’ despite ‘his father’s doubts’ is meant to both augment our impression of Frank’s integrity and redouble our disgust at the conniving aunt.¹⁸ Illness is synonymous with selfish impulse in the figure of despotic Mrs Churchill, her ‘disorder’ a not-very-thinly disguised metaphor for the perversion of social codes of civility and decency. The fabrication of illness in Mrs Churchill’s case ‘turns on a double instrumentalization of health’, as the critic D. A. Miller once put it – since, in showing Mrs Churchill to use her ailments as a screen for the abrogation of social norms, Austen reaffirms their importance to the fictional world she creates.¹⁹ To be a hypochondriac in this sense is to guarantee the power of social niceties in so blatantly flouting them.

When Mrs Churchill dies of a stroke, however, the novel suddenly calls into question the fun it’s been having at her expense and stops us in our presumptuous tracks, for her demise works to make Mrs Churchill’s prior complaints of illness more credible: perhaps she *had* been feeling something, a harbinger of worse to come that is obscured by interpersonal dynamics and the otherwise unpleasant manner of her dealings with social subordinates. How do we know *for sure*, after all, that Mrs Churchill’s seizure is ‘of a different nature from any thing foreboded by her general state’? Given Austen’s slippery narrative tactics – known as ‘free indirect speech’ – point of view is continually shifting from omniscient narrator into the perspectives of individual characters. So whose interpretation of her symptoms and condition, whose version of the truth of Mrs Churchill, have we been trusting? Put differently again, why does anyone require nothing less than death to ‘full justif[y]’ her as having once been ‘seriously ill’? In dying, Mrs Churchill reveals hypochondria’s perverse irony, that suffering a terrible obsession becomes justified only in the moment of one’s strongest fears coming true; this has the effect of authorizing myriad bodymind complaints as meaningful *in some way* and so signalling to everyone that symptoms, precisely because they are felt, are worthy of our attention.

In an environment so future-directed, where characters spend much of their lives in states of expectation ('how very happy a summer must be before her!'), the persistence of hypochondria, its ceaseless, incurable nature, seems an impediment not only to the characters' evolution but even to the novel itself.²⁰ In one way then, Mrs Churchill's death releases the plot from a kind of hostage so it can pursue its course towards romantic resolution. But that death also throws the ambiguity of interpretation, authority and meaning into high relief. Several characters in the novel, including and especially its heroine, engage in acts of deceit and manipulation. Emma's matchmaking is notoriously wrong-headed; her machinations vis-à-vis her friend Harriet as well as Frank are motivated, if not by explicit self-interest, then at least a propensity for 'tak[ing] up [an] idea . . . and [making] every thing bend to it'. So too does Emma, with her 'imaginist' mind, turn chance occurrence into narrative and then into a conviction that 'every thing united to promise the most interesting consequences' – a sort of romanticized fatefulness that is Emma's brand of foreclosing the unpredictable.²¹ Emma in this sense is no less a storyteller than her more worried father and not so different a manipulator than the detested Mrs Churchill. If we cannot be *sure* of whom to believe – whose symptoms indicate disease, whose account of what happened and what it might portend should hold sway – and if the story's protagonist, patently the hub of her social environment, wilfully though naively skews truths in pursuit of her own ends, then hypochondria is merely a vocabulary that manifests worries about unpredictability, attachment and the future expressed by the novel's cast as a whole, *all* of whom participate in the intricately choreographed management of social panic.

Because the nervous Mr Woodhouse balks at any alteration to his routines, every ministration to his comfort safeguards a version of patriarchal stasis. Mrs Churchill's insistence on keeping Frank close during her bouts of unwellness similarly frustrates the younger generation's social plans (which is to say, *coupling*), and readers are clearly manoeuvred to resent her efforts to thwart their forward motion. But by interrupting the romantic logic of the plot – the novel famously opens with Mr Woodhouse lamenting the happy marriage of Emma's governess, after all – hypochondria establishes an intriguing delay in which something *else* can be imagined, something other than the expected, and in this sense might open towards a distinct if temporary space of opposition to the intrigues,

especially oppressive for women, of love. To quote D. A. Miller once again, ‘the social order imagined by hypochondria . . . is so intractably oppressive that resistance can only take the form of an equally total and nonnegotiable negation.’²² *Emma* suggests, then, that we err in reading hypochondria as mere exaggeration or outright invention of problems that do not exist. Instead it posits an unusual solution to problems not exclusively the sufferer’s own.

Rather than insisting on hypochondria as a distraction from or frustration of the primary action, the novel’s hypochondriacs invite us to query what we think we know and whose word for it we’re passively taking, and to put some pressure on the ways we inhabit culturally legitimized ideologies and identity. Whether they’re lovingly indulged or placated and disdained, Mr Woodhouse and Mrs Churchill are in fact two of the novel’s *realists*: alert to the prevalence of greed, the fact of mortality and the workings of a system whose precarity they gloomily observe.²³ Dizzy spells, fear of picnics – these are but embodied vehicles writ small of Austen’s larger interrogation, her skewering, of societal customs, especially the challenges confronting women. The fact that Mr Woodhouse never overcomes his worry – that he exists outside of normative arcs of ‘getting better’ – might invite us to reconsider the quality of time, what we expect of the progress of our lives or how to handle an environment perceived to be full of risk. Perhaps we find new ways of thinking about family, adulthood or caregiving, or disrupt the influence, largely male-dominated, of medical authority as a primary guarantor of our well-being.

A final instance of such reversals arrives at the novel’s close, where Emma, in a lather of trepidation, frets about ‘how to break it to her father . . . that she and Mr Knightley meant to marry’ – and indeed, for all his ‘love [of] Mr. Knightley’, Mr Woodhouse does try ‘earnestly to dissuade her from it’. The answer to this seemingly intractable riddle, that Emma and Knightley will live at *her* house, Hartfield, rather than his, is offered as evidence of the superiority of Knightley’s character: ‘very few of those men in a rank of life to address Emma would have renounced their own home’ for their wife’s.²⁴ Knightley’s willingness also, and more significantly, upends the ‘tamely and placidly conservative’ aspect of Emma’s display of ‘filial piety’, as Claudia L. Johnson has argued, since Emma’s ‘solemn resolution of never quitting her father’ requires that Knightley relinquish a

degree of the power accorded him as a man of rank, and ‘plac[e] himself within [his wife’s] domain’.²⁵

What Johnson’s important reading of this ‘unexpected turn’ leaves out, of course, is the specific role of *hypochondria* in Emma’s (or *Emma*’s) rebuke to normative arcs of progress. We might ask whether hypochondria holds Emma in thrall to her father’s house, like a child unable to grow up, in a kind of patriarchal amber of immobility. Or – to the contrary and much more subversively – we can read hypochondria as the agent of a remarkable pivot, as Emma, in the very moment of entering the social economy of marriage, manoeuvres herself (and her fiancé) away from its expected spatial and temporal trajectory.²⁶ The implied endpoint of living at Hartfield rather than Knightley’s Donwell Abbey is of course Mr Woodhouse’s death, one the novel’s conclusion holds in abeyance. Nor does the novel end with Emma established as matron of her husband’s estate, surrounded by children. Instead its final reference, to a wedding ‘very much like other weddings’, suspends us in hyp-time, Emma’s past and present held in a radical, contrary tension, denying futurity.²⁷

OUR NARRATIVES OF ILLNESS are suffused with references to positivity, battle, instruction and control that imply a hopeful – or at least intentional – movement through time. Ann Jurecic writes that language ‘orders the disordered experience of illness, which makes it possible for readers to attend to the details of an otherwise unimaginable experience’.²⁸ That ordering is not neutral; from word choice to the structural patterns we choose, the forms that conventional illness stories take are liable to cost, to borrow G. Thomas Couser’s notion that how we narrate experiences of anomalous embodiment participates in deeply coded beliefs about normalcy as well as the moralized stain of impairment and sickness. From the most conversational of utterances (‘what happened?’ and ‘what’s wrong?’) to the ways in which we are called to account for our ill and impaired selves in clinical or insurance settings, the portrayals of disease and disability we encounter on screen and page, and our most private narrations of what it feels like to be the embodied humans we are, we take our cues from the paradigms of causation and responsibility we’ve grown up into.

The most readily available narrative templates in Western ableist societies for grappling with bodymindedness assume both linearity and culprit: frightened by the random chaos of the body, we stabilize in tales of etiology, origin, often blame; we take eradication of disease to be the only tolerable outcome, inevitably demonizing or pitying the unrecovered; we rally ourselves towards restoration of agency over our wayward flesh. By imposing recognizable narrative shapes whose ending is presumed – allegory, romantic quest, mystery, epic battle – we neutralize the future, make it less terrifying. To render cancer as an entity we fight, for example, as in the most common of metaphors, is to shape recovery as a matter of valour rather than physiology and to imply a denouement in which we emerge victorious. We make ourselves heroes and victims, students of lessons and recipients of gifts supposedly learned and earned from bodily experiences that would otherwise feel intolerably arbitrary or punishing.

All of this rhetorical shaping to cope with an experience of illness assumes the correlation of time and improvement: there is the moment of onset, the diagnosis, the fluctuations of symptoms, then abatement; relief, rehabilitation and recovery follow suffering. Chronic, persistent conditions that defy the expectation of getting ‘better’ also challenge what we think of as the ‘natural’ way to tell a story. This is why so many recent disability memoirs eschew paradigms of transformation and overcoming – even narrative linearity – in favour of more associative structures that accommodate text and/or graphics to the contours of atypical bodyminds, rather than wrenching experience into conventional forms. Of course, though we may exist in time as we age, no one actually thinks in linearly straightforward ways (so-called ‘*stream of consciousness*’ might be better figured as a web or something like a pinball machine), and memory, especially traumatic memory, by definition enfolds the past in the present. Ideas about health are similarly achronological in the sense that what we fear often exists in a timeframe other than the one our bodies occupy; *what if*, and even *what will*, cast us into a looming but as-yet-unreal future.

Hypochondria, a story stuck on repeat, destroys the timeline. The hypochondriac craves narrative resolution, and yet, in refusing the ending offered by diagnostic exam, by authoritative reassurance, perhaps even by the body’s own evidence, keeps going back to the beginning to tell the story again. Journalist Alexander Nazaryan describes hypochondria’s recursiveness as ‘self-reinforcing frustration’ and writes of his own

experience that the condition is ‘dull and time-consuming’, its ‘eternal watchfulness . . . drain[ing] both time and attention, turning you into a solipsistic bore . . . We simply cannot help it.’²⁹ This sense of narrative loop, of being unable to extricate oneself from a story whose culmination is at once wearily familiar and desperately unknowable, pertains also to practitioners for whom hypochondria’s unyielding obduracy translates to lost minutes in a busy day of appointments. Here again hypochondria represents a danger to livelihood and is rendered in terms of its negative effect on appropriate interpersonal relations. In telling an ultimately opaque story, the hypochondriac seems to mock the conventions of both medicine and literature; in failing to develop ‘trust’ that the body is doing what it’s supposed to do (until it doesn’t) the hypochondriac gets stuck in a childlike ‘state of perpetual bewilderment’, to quote Nazaryan again, never getting weaned from what Sarah Mann-O’Donnell calls the ‘closed circuit . . . of self-interpretation’.³⁰

To think of hypochondria’s distinctly temporal quality only in terms of lost productivity or stunted intimacy, however, obscures the complex dimensionality of what it means to feel either ill or compelled to investigate one’s health. For the English poet Samuel Taylor Coleridge (1772–1834), the biologist Charles Darwin (1809–1882) and diarist Alice James (1848–1892), sister to Henry and William James, protracted malaise disrupted chronology in ways that are sometimes taken to be avoidance techniques – ways of absenting oneself from what Henry, in a letter about his sister, called ‘the practical problem of life’.³¹ But ‘retreat’ is far from the only purpose or effect of hypochondria for these figures. Each of them suffered in particular from gastrointestinal distress among other diagnosed or indistinct ailments; all were ill for most of their adult lives; all recorded their travails in journals and letters, so that we have direct accounts of their symptoms; all regularly consulted medical texts and/or practitioners and were well informed about what was happening to them; and all, to a startling degree, allowed that however confusing, painful or embarrassing the states they were in, illness constituted them – was thoroughly imbricated in, rather than a hiatus from, their intellectual work. Their versions of hypochondria track its evolution from a medicalized mind–body syndrome towards the somatic expression of primarily psychological complaints. In their unique ways of occupying time they also reappraise professional success, creativity, domestic relations, ideological protest,

religion, the tyranny of ideas, the quality of memory and the nature of death.

COLERIDGE HAD A ‘boundless fascination’, as historians George Sebastian Rousseau and David Boyd Haycock call it, for his hypochondriacal body, a source of mystifying and chronic disturbance.³² Coleridge attributed his ailments variously: to rheumatism, nervous anxiety, cholera, dyspepsia, gout, the effects of damp English weather and most emphatically to his stomach and bowels. The low spirits of ‘hypochondriasis’ were diagnostically linked to gastrointestinal disorder for eighteenth-century physicians such as Thomas Trotter, who declared in *A View of the Nervous Temperament* that the ‘human stomach is . . . a *centre* of sympathy between our corporeal and mental parts’; Rousseau and Haycock note that in Coleridge’s search for an organic cause of his manifold and distressing complaints, ‘over and again it is his gut which is the culprit’.³³ The disastrous effects of prolonged use of opium on the digestive tract was an especially vexed factor since Coleridge began taking the drug – at least in part – as pain relief.³⁴ In making a case for hypochondria as ‘a permanent pillar’ of Coleridge’s ‘psychological and psychosomatic being’, Rousseau and Haycock enumerate other features characteristic of hypochondria in a modern sense to which the poet was prone, including ‘bodily preoccupation’, ‘recourse to medical practitioners’, ‘fascination with medical texts’, fear of serious disease, amplification of symptoms and chronicity.³⁵ Importantly, they argue, Coleridge ‘was not merely imagining himself in pain’, even if he also fashioned himself according to the excessively ‘sensible’ body that was becoming, in his moment, the mark of literary genius. He was, in their words, ‘both *malade vraiment* [and] *malade imaginaire*’.³⁶

Coleridge’s self-professed ‘hypochondriasis’ was entangled with another distemper, new to England in the eighteenth century: homesickness, or nostalgia (*homecoming* plus *pain*), classified by the early 1800s as a legitimate disease with discrete symptoms of lethargy, dizziness, loss of appetite and melancholy. During extended sojourns in Germany, Coleridge wrote often to his wife of the physical and emotional anguish of a homesickness he seemed incapable of curing (i.e., by simply returning to

England): ‘I do languish to be at home!’ he cried to her in January of 1799 and again in April, ‘how infinitely I languish to be in my native Country’, though he made no move to get there.³⁷ The poem ‘Home-Sick’, written in May, describes the ‘wasting pang’ of longing that ‘hourly[,] more and more’, sickens a speaker who would only be cured at ‘the Door of his own Home’ and yet never arrives. Nostalgic homesickness, mourning for where he once was and is no longer, suspends the poet in fretful torment; the future holds out promise of reunion, the past is sentimentally remembered and the present inflected – we might reasonably in Coleridge’s case say *infected* – with a sense of crushing failure. Nostalgia appears often in Coleridge’s papers alongside hypochondria as an explicit hindrance to the fulfilment of his artistic aspirations, thus orientating the poet towards a return he felt unable to make and a future he could not seem to realize. Coleridge’s particular anxieties had not the ‘sensation of an eternal present’, as Thaís Klein and Regina Herzog propose of hypochondria’s timescape, but instead pulled him in opposing, maddeningly out-of-reach, temporal directions.³⁸

Chronic pain, the daily mortifications of his bowels (constipation, flatulence, diarrhoea, vomiting), months of flagging energy, the boils and swellings of scrofula, a sense of constitutional ‘indolence’ – these figure Coleridge’s relationship to time as one of perpetual rebuff. ‘My ideas, wishes, & feelings are to a diseased degree’, he wrote to the philosopher William Godwin in 1802, ‘disconnected from *motion & action*.’³⁹ And yet Coleridge was hardly idle. George C. Grinnell warns in *The Age of Hypochondria* against the critical habit of ‘reinforc[ing] normative associations of illness with a disabling non-productivity’.⁴⁰ Scholars have tended not only to construe hypochondria as characteristic of Romantic aesthetic sensibility but to insist on its pathology to explain Coleridge’s ‘many unrealized ambitions and projects’, or ‘why he has not accomplished more than he has’ – especially in relation to his friend, collaborator and rival Wordsworth.⁴¹ Instead we might frame the connection between creative intelligence and acute attention to the workings of the body as a ‘productive and innovative theorization’ of what it is to be human, and so rethink Coleridge’s preoccupations in relation to time.⁴² ‘Virtue & Genius are Diseases of Hypochondriacal and Scrofulous Genus,’ Coleridge wrote, making the link explicit; he described himself in a letter to his brother

James in 1802 as having a ‘power & habit of continuous Thinking’ that, far from being a sort of compensatory capability to offset his bodily troubles, was thoroughly bound up in them as part and parcel of an impulse to penetrate into the meaning of things.⁴³ Thus Coleridge foreshortens time, pulling the future into a present moment defined by inquisitiveness; death is not feared but heralded as an opportunity to know. In an era ‘remarkable’ for its ‘attention to infirmity’, in Grinnell’s words,⁴⁴ Coleridge’s hypochondriacal self-scrutiny did not manifest illness, however unpleasant, as life’s aberration. It was simply the human condition.

The imagination of sickness was one of Coleridge’s most profound philosophical mechanisms, thickly bound up with his ideas about aesthetics, poetic inspiration, beauty and the quality of knowing. Coleridge was alert to the explanatory power of the rhetoric of disease, the way diagnosis could be instrumental in accounting not only for physical symptoms but for otherwise puzzling characterological tendencies. This is why scrofula and hypochondria appear in his work as a mode of both apology and consolidation, bringing Coleridge publicly into being as an author in the very moment of announcing that his accomplishments, had he not been quite so persistently ill, might have been ampler (I return to themes of consolidation in [Chapter Four](#)). This relationship doesn’t just work in a negative sense, however (‘were it not for’); *because* of a body in crisis, because of restless self-diagnosis, fascination for what could happen in a body, Coleridge connected to the great scientists of his day, such as the chemist Humphry Davy and physician Thomas Beddoes, who were experimenting with new therapies for pain. In the way that disease etiology demands methodical, studious interrogation, so too might dreams reveal the origin of poetic inspiration; the heuristic, ultimately intuitive process of analysing art and literature is akin to reading one’s material and yet indeterminate self, subject to the unpredictable incidence and course of illness. For the coiner of the term ‘psycho-somatic’, mind and body were thoroughly integrated. That Coleridge asked to be autopsied after death is the most radical instance of all this enthusiastic, if often desperate, curiosity. The request implies some finally authoritative scientific gaze that might stabilize him as a legible corporeal entity once its vexingly obscure, innermost regions are revealed. Of course the living author has only imaginative access to that eventual moment and can never do more than speculate about what might be discovered within himself. In this way

hypochondria folds in on time, extending the life of the body in the very moment of imagining its demise.

‘I WAS . . . TROUBLED WITH palpitations and pain about the heart,’ wrote Charles Darwin, many years after embarking in 1831 on the HMS *Beagle* voyage that would set in motion a philosophical sea-change, ‘and like many a young man . . . with a smattering of medical knowledge, was convinced that I had heart disease.’⁴⁵ Perhaps this single admission would prove Darwin’s hypochondria (sounding like ‘medical students’ disease’ and so familiar to any of us who’ve spent our late nights on Reddit hypochondria threads), to say nothing of many years of ambiguous symptoms, which included periodic vomiting, nausea, eczema, boils, anxiety and emotional malaise, lethargy, headaches and, above all, ‘flatulence’, referring to a host of gastrointestinal pains and disturbances. To these ailments Darwin would apply a range of treatments (he was especially drawn to hydrotherapeutic techniques), but his dominant strategy for relief seems to have been a daily regimen of work, meals, walks and rest that coalesced in what Brian Dillon describes as a ‘strict rationing’ of time.⁴⁶ It seems curiously ironic that this world-altering proponent of evolution – by definition an inexorable process of responding to environmental forces through time – was himself highly susceptible to the effects of his social surroundings, so attuned to the interplay of his internal rhythms and those of other people that any alteration of routine could produce violent bouts of ill health and exhaustion that sometimes interrupted his work for weeks.

In an article published in 2005 in the journal *Postgraduate Medicine*, Anthony K. Campbell and Stephanie B. Matthews, then of the Darwin Centre for Biology and Medicine in the UK, assert with confidence that there is no more ‘convincing explanation’ for the range, timing and duration of Darwin’s gut problems than lactose intolerance. Dismissing some thirty different organic and psychological causes proposed over a century of biographical research as insufficiently compelling (these include ulcer, depression, bereavement syndrome, Chilean fever, arsenic poisoning, hepatitis, basic hypochondria and a pigeon allergy), Campbell and Matthews mount such counter-evidence as Emma Darwin’s cream-filled recipe book and the onset of distress two hours after meals.⁴⁷ It is intriguing

to consider the course of Darwin's theorizing had he not taken ill quite so regularly – had he not, as he puts it, suffered such 'frequent complaints from ill-health and discomfort'.⁴⁸ A different question is whether discerning a single origin of trouble, such as Coleridge before him so fervently sought, would have helped or hindered Darwin's imaginative work. Beyond the relief of symptoms that a definitive diagnosis might have brought, would the sheer fact of *knowing* have provided some assuagement of the distress of ambiguous pain? And there's a related question: what does certitude about cause do for *us*? For I would argue that the impulse towards retroactive diagnosis betrays a concern to quell the disturbing image of the great thinker as a traumatized hypochondriac. If Darwin was merely lactose intolerant, then the decades of mysterious agony can be safely corralled as an involuntary corporeal reaction to his wife's rich cooking.

Darwin himself, unlike Coleridge, seems to have been more invested in controlling symptoms than in uncovering cause. Francis Darwin's *Life and Letters of Charles Darwin* explains the strict daily schedule whereby his father structured his hours, with set times for working, walking, dining, resting, reading, smoking and studying. These 'stated intervals of work and rest' were 'essential to his well-being', Francis tells us, such that any divergence from the timeline brought on 'pain and difficulty'.⁴⁹ Though Darwin is hardly unique in this respect, there's a tendency to style his fiercely held routine as primarily inhibitive, holding illness at bay (to say nothing of the entire Darwin household), and in turn to understand illness – at least its potentiality – as controlling the rhythms of time.⁵⁰ Dillon defines Darwin as a hypochondriac not because he was afraid of disease but because he 'used' the ailments he had, or perceived himself to have, instrumentalizing them in the form of 'retreat' – a word Susan Baur also deploys to describe Darwin's 'dealings' with a 'disheartening world'.⁵¹ But it's only in relation to some normative delineation of time and to supposedly normal modes of engagement with family life, sociability, work productivity and diurnal clocks that a person can be deemed to *retreat*, a word that connotes denial and failure as much as sanctuary or renewal. The language of *use* – along with preference, choice, allowance and permission, which suffuse both Dillon's and Baur's discussions of Darwin – relies upon our imagining a kind of un-ill doppelgänger Darwin who applies illness to himself as a functional aberration.

Darwin had a different way of framing the segmented quality of his hypertime. As he writes in *Autobiography*, ‘I know well that my head would have failed years ago had not my stomach saved me from a minute’s overwork.’⁵² Where Coleridge’s various ailments *were* the intellectual work, Darwin’s existed in a sympathetic partnership with his mental endeavours, as if mind and body naturally understood – in ways perhaps subsequent biographers and biologists have not – how much time, and what forms, work could most effectively take up. Dillon’s claim that ‘Charles Darwin did not live wholly in the world whose view of itself he altered for ever’ thus strikes an unduly pitying note – or a typically estranging ableist one that polices the boundaries between the isolated ‘sick’ room and the rest of the ‘world’.⁵³ My point here is not to discount the real consequences, the fatigue and consternation, of chronic pain. But Darwin’s comment above renders ‘stomach’, as synecdoche for full-body upset, the saviour rather than the villain; illness in his formulation takes part in a holistic system of which the cognitive work for which we celebrate Charles Darwin is but a part. Indeed it obviates a separation between cognition and digestion altogether, telling us that attending to the stomach’s ills is the mind’s way of protecting itself from overexertion – and setting itself up for further creativity. To persist in thinking of Darwin’s pervasive discomforts as a distraction from his more important endeavours, an excuse to withdraw from unpleasantness, even the unfortunate underside of scientific genius, misses the point that illness was not an interruption of progress; it was the modality of Darwin’s work.

Darwin’s anxiety, as he approached publication of *On the Origin of Species by Means of Natural Selection* in 1859, was intense. Some fourteen years prior, in January of 1844, he wrote to a friend, the botanist Joseph Dalton Hooker, that he was ‘almost convinced . . . that species are not (it is like confessing a murder) immutable’.⁵⁴ Darwin was fully cognizant of the censure and retribution he might incur as a result of his ideas, and the magnitude of his trepidation, his sense of moral culpability rather than intellectual amazement and delight, reveals itself in his startling simile. To feel oneself embroiled in work whose radical implications were no less than full-scale epistemological, theological and political rupture would trouble even the stoutest of viscera.

IN EARLY 1853 Alice James's father, Henry James Sr, published a defence of male superiority in the prestigious periodical *Putnam's Monthly*. Among other standard insults, 'Woman and the "Woman's Movement"' declared that women were 'disqualifie[d] . . . from all didactic dignity' by virtue of a 'practical sense, which leaves [them] indifferent to past and future alike'. Here we learn that a woman's 'strictly practical genius', 'her superiorly practical efficiency', her 'eminently practical' nature makes her unfit for the kind of visionary intellectual work of teaching, preaching or judging – but also shields her 'from *ennui* and dyspepsia, and every form of spiritual fidget'.⁵⁵ Ironically of course – or tragically – Henry's daughter was not at all protected from emotional and physical travail by her 'genius' for practicality. To the contrary, Alice James spent much of her adult life enduring a series of nervous collapses and a host of diverse symptoms often grouped together as hypochondria or hysteria: severe headaches, stomach upset, limb weakness, heart palpitations, neuralgia, fatigue. By all accounts, this youngest of the five James children – and only daughter – was every bit as wryly intelligent as her two most famous brothers (though hardly as well educated) and not unique in the family for being sick. Only Alice, however, would suffer the oppressive effects of Victorian patriarchy. She learned early to 'clothe [her]self in neutral tints' and to 'possess [her] soul in silence'; neither encouraged nor supported in any other direction, Alice James became what has been called a career invalid.⁵⁶

James descends through history as a pointed instance of neurotic somatization. Biographers have interpreted her chronic ailments as the manifestation of suppressed rage at the father who discounted her and the suffocating atmosphere of her circumstances – the 'turning inward' that supposedly results from lack of opportunity to engage in worthier pursuits. Charlotte Perkins Gilman's story 'The Yellow Wallpaper' (1892) famously depicts the damaging effects of the so-called rest cure, popularized by neurologist Silas Weir Mitchell (the era's leading 'expert' on women's mental health), which prescribed bed rest, overfeeding and a strict ban on mental work like reading and writing. In this context 'idleness' is not an individual flaw (such as James Boswell felt of himself) but a form of cultural tyranny imposed upon women deemed too fragile for the stimulations of modern life. At the same time 'the ubiquity of ill health' generally in the nineteenth century meant that chronic invalidism had a different kind of cultural cachet – it was at least 'something of a shared

experience', in Maria H. Frawley's words, that gave the lie to medical authority and dominated print culture (novels and periodical press) in ways that undercut stereotypes of stasis, overwrought emotion, social irrelevance, even female nature.⁵⁷ The political economist Harriet Martineau wrote of her six years in the 'sick-room' that 'Nothing is more impossible to represent in words . . . than what it is to lie on the verge of life and watch, with nothing to do but think, and learn from what we behold.'⁵⁸ Alice James's own experience of illness, then, was multifaceted. She was surely troubled by unresolved fury, for example – intrusive fantasies of 'knocking off the head of the benignant pater as he sat with his silver locks, writing at his table' – and the 'never-ending fight' of keeping it contained.⁵⁹ But her complex identification with her symptoms must also be situated more broadly within cultural attitudes towards illness as an intellectual means, a form of resistance, a mechanism of kindred attachment.

Henry James Sr's treatise, to return to that defensive parade of misogyny, also genders time. Man, James Sr claims, has the rational capacity to heeds warnings, to 'weigh consequences', to refrain from immoral action – all of these entailing a process of synthesis, bringing historical precedent to bear on possible future outcomes.⁶⁰ Woman, by contrast, enjoys but the gratifications of immediacy, unable to cast her awareness beyond what pleases her, childishly, in the present moment, or the pressing tasks of homemaking, being naturally inclined towards the caretaking of others. Often framed in terms of a moral immaturity that Freud, in *Three Essays on the Theory of Sexuality* (1905), would explain as the incomplete development of women's superego, this aspect of the gender binary temporally relegates women to an 'absolute present' not unlike the one often associated with hypochondria. As Klein and Herzog put it, 'When one situates themselves outside time, the refusal of motion ensues, which prevents things from happening'; in this suspended state of idleness, the psyche 'tr[ies] to recover and heal its own wounds', engaged in a complex process of reconsolidation. Caught between these explanatory paradigms, a woman like Alice James will seem doubly delayed, first sickened by the diminishing influence of her culture and then cultivating herself as sick; denied the rational capacity to organize herself according to the sweep of time, she descends into neurotic helplessness, unable to break free of melancholic repetition, to make the past the past.⁶¹

But there are different ways to approach what Talal Asad calls the ‘post hoc, recursive’ quality of hypochondria – its fallacious causal reasoning, or its faulty relationship to the timing of events – that might reframe what we understand James to have been ‘doing’ in the many years of her apparent invalidism. Asad proposes that hypochondria, as ‘a means of resolving problems’, can ‘secure spaces for living in a particular, productive way’ – that is, ‘a distinctive way of living and working through suffering’. What we think of as a problematic denial of social life, a stifled narcissism, might instead ‘be a claim to much desired privacy’ that can in turn ‘open up creative talents – by shutting out the world, by obsessive attentiveness’. (Just as Harriet Martineau felt herself to be in ‘no hurry’ in the reflective space of the sick-room.) For Asad the tension between privacy and publicity inherent in hypochondria (hypochondria displays the objects of its suffering yet maintains an exaggerated inwardness) reflects the ‘political contradiction’ of liberal society, which safeguards ‘private spaces of freedom’ and yet assumes the reach of state power. What makes hypochondria scandalous, in this argument, is its open declaration of a pain and fear that cannot be assuaged by the very entities to which it appeals – indeed, the fact that it asserts pain and fear as so much ‘a condition of life’ that they elude the protective function of authority, medical or otherwise, altogether.⁶²

The psychical as well as cultural work of hypochondria in this framework then relates to the good it might be doing as a reprieve from the effort of sustaining both socially approved identities and an illusory subjective integration. According to Asad, to accuse hypochondria of pointlessness – useless in relation to socially productive work – misses its psychological mechanics. In the intense perceptiveness that is hypochondriacal focus we encounter the strangeness of ourselves, perhaps acclimating to the assemblages we are, of unknowable and unpredictable parts. For hypochondria dismantles our pretence of self-integrity and continuity; it explodes our confidence in explanatory narratives as insufficient or naive, harkening back to some unified sphere of meaning before the dissensions and cynicism of modernity cracked things apart. Perhaps it exposes the rhythms of daily life, to which we submit unthinkingly as members of our social clans, as coercive, prejudicial, even destructive. And illness has its own story to tell. Here I refer not to the socially prescribed symbolics of particular diseases such as cancer or AIDS,

nor to the historical development of diseases in relation to vaccines, such as measles or polio, but rather to the narration of self-in-sickness whereby the body speaks its own language and subjectivity emerges by way of the alternative logic of unwellness. As a kind of renegade social identity, hypochondria announces its resistance to the penetrating gazes – and the normative temporal rhythms – of ableism, medicine, the world of work.

When Alice James then, constrained by the dictates of her ideological moment, cast herself into sickness's temporal arc, she ungendered time and complicated the idea that hypochondriacs languish in melancholic present-tense because they fail to divest the body's interior of its anxious fascination. Her symptomatic self was in fact in great flux: sensations altered in quality and intensity presaged further complications, called for future treatments; her situation worsened; it improved. As a hypochondriac, she studied, awaited, imagined; there was always an endpoint and a beginning-again. I emphasize such fluctuations to suggest that hypochondria is less a fixation than a dynamically variable state of being that the individual traces – nearly by definition – over time. As a diarist too James dedicated herself not to the conventional comforts of a household but to textualizing her psychological observations, particularly the intersubjective dynamics of care. Her accounts of her own doctors are renowned for being cleverly astute, and in this hypistemological way, feeling ill (whatever the cause) gave James a vocabulary with which to organize her relationships and to understand the psychological dispositions of those around her.

But I also want to press back against the tendency to invoke James's diaries as proof of a finally realized self, a writerly persona more worthy of our attention than – or somehow compensating for – the nervously ill woman who failed in marriage and motherhood and literary success alike. Our rejection of the gendered stereotype mustn't perpetuate an ableist one, in other words – the notion of illness as a waste – or coincide with denigrating health anxiety as evidence of tainted submission to pressure, as if hypochondria would simply disappear if James had had the wherewithal to speak up, ultimately accusing her of an inability to interrogate her own anguish. We have the idea that 'real' life happens in less recessive or recursive frames than hyp-time, that hypochondria is a screen for something else and that effort should lead to product rather than repetition. Of course it makes sense to map Alice James's ailing decades on the trope of the

Victorian female invalid, to wonder about the life she might have had away from a patriarch who publicly denied women the capability of intellectual work and proclaimed them contented in the narrow straits of home life. But we cannot claim with any certainty that she would thereby have been happier because less sick, or less sick because more mentally stimulated. And though it's possible that Alice James, like Darwin and Coleridge before her, had undiagnosable medical conditions, given the transitional progress of nineteenth-century medical knowledge, we might again interrogate the impulse to diagnose as the corrective to hypochondriacally expressed psychological problems. (Much is made of the fact that when James was diagnosed with breast cancer in 1891, she wrote in her diary, 'Ever since I have been ill, I have longed and longed for some palpable disease.')

⁶³ For hypochondria is also just itself – an amplified but authentic attendance to one's state of being that needs no justification as something *else*, the more 'real' thing it is calling attention to.

Taking our cues once again from the notion of crip time, we should reject the idea that Alice James was ossified in a paltry, unwell simulation of the world her brothers were depicting and distilling in their literature and philosophy. She was simply alive – and busy with a strategy of world-making that featured forms of care outside of heteronormativity and claimed time, focus and thinking for herself. The paradox of diagnosing the hypochondriac as 'preferring' physical to psychological problems that are too difficult to face – a common formulation – is that it manages to stigmatize both the hypochondria and the unresolved 'other' emotions as the person's fault. My point is not that Alice James wasn't in pain but that her illness and pain have a place in the properly employed social body.

I CLOSE THIS DISCUSSION of hyp-time with Charles Kauffman's 2008 film *Synecdoche, New York*, starring Philip Seymour Hoffman as theatre director Caden Cotard. The name is significant: Cotard's syndrome is a rare psychiatric condition that begins with hypochondria and develops into mood disorders and nihilistic delusions of bodily nonexistence, sometimes denying the functionality of internal organs. As the film begins Caden is directing young actors in a production of *Death of a Salesman*, a casting decision he anticipates will deepen the play's emotional effect by reminding

the audience that everyone, in time, becomes Willy Loman: crushed by circumstance and failure. At home, meanwhile, Caden's four-year-old daughter, Olive, fretting over her green faeces, wonders 'what's wrong' with her; wife Adele fantasizes about Caden dying; and Caden cuts his forehead, landing in A&E for stitches that within moments have become a scar. A year passes between scenes, but the play is just premiering. Caden thinks he has arthritis and then a skin rash (called 'sycosis' but pronounced 'psychosis'), which disappears. He tells his therapist, 'I'm afraid I'm going to die. I don't know what's wrong with me.' He seems to have a seizure; he has 'synaptic degradation'. Another year passes suddenly, but Caden believes 'it's been a week'. The film jumps ahead again, and Caden becomes a MacArthur Fellow – the so-called 'genius grant'. To make something monumental, he recreates people's apartments inside an enormous warehouse and hires actors to play real people in real time. 'I've been thinking a lot about dying lately,' he says. 'I will be dying . . . We're all hurtling towards death.' The actor playing Caden Cotard dies, but the scar on the 'real' Caden's forehead never fades. 'Death comes faster than you think,' Caden says. His play is still in production after seventeen years.

What is the meaning of this temporal flux in relation to bodily anomalies, to the sense that something is wrong? (There's even a close-up of Caden's therapist's feet, looking oddly swollen.) Klein and Herzog define the nonlinearity of the film as an 'eternal present', the 'presentification of time triggered by hypochondriac anxiety'. Reading Caden's health worries as a neurotic 'update of something that cannot be elaborated', they suggest that the film's achronicity enacts an unconscious process of traumatic repetition. As temporal structure collapses – and so too the distinction between real and representational experiences – each new symptom Caden brings to a doctor loses diagnostic credibility, and *status*, the sense of consequence and legitimacy in one's real life, becomes *stasis*. Unable to make progress or to ascertain what's fabricated, Caden loses 'egoic' unity altogether, ending the film as an actor playing a different character in his own autobiographical work. In this reading hypochondria manifests 'what cannot become past' – what hasn't been, or cannot be, resolved in the working-through of psychoanalytic interrogation. Anxiety Caden cannot articulate 'targets', again and again, his body.⁶⁴

But disorientating as it is to watch *Synecdoche, New York* (on first pass, the time jumps and character connotations are indeed surreal), I would

disagree that the film depicts Caden's hypochondria as helplessness or the never-ending development of his *opus* as a kind of anaesthetized artistry, a pointless simulacrum of 'real' life. Reviewers of *Synecdoche* enthused about Kauffman's 'extravagantly conceptual' appeal to viewers to 'be here now, alive in the world as it is'; gushed over the 'captivating, exasperating', 'hyperreal intensity' with which the film explores life's disappointments.⁶⁵ But Caden's life and the art he strives to make do not just disappear into the ever-receding distance of futility. I don't mean that the film we watch redeems the failure of the stage production within it. Rather, the *mise-en-abîme* effect – audience watching a film in which a director watches an actor playing him watch another actor cast as a different version of him (staged in a giant warehouse with no audience) – *does* get us somewhere, and it only does so because of the obsessive aspect of Caden's desire to penetrate, to be *real*. Assuming he fails in this quest perpetuates the correlation between authenticity and closure and disregards the virtues of process. Without hypochondria, *Synecdoche* intimates, a play would *only* be mimetic, mere representation; it could tell but one story. With hypochondria, possibilities, selves, iterations proliferate; hypochondria offers a multitude of opportunities to rediscover, reexamine. This is how 'Schenectady' becomes 'synecdoche', because wholes are made up of parts; the idea that Caden is 'so busy living in one world that he forgets to live in another' insists on a hierarchy.⁶⁶ But why *inevitably* is 'this' world better than 'that'?

Where it all ends is death, of course – but to fret about that, to consider such an ending unduly grim, betrays a deathdenying naivety. And Caden's death is not the only final act of *Synecdoche, New York*. Fellow actors may describe Caden as 'very lonely', 'a man already dead', 'turned to stone' (just as Brian Dillon claims of Alice James that hers 'was a kind of posthumous existence'), but existential bleakness is not the conclusion here, either for Caden or for Alice.⁶⁷ If no one is an 'extra', as the film tells us, if we are 'all leads in [our] own stories', then we all *matter*, whatever our bodily circumstances or the quality of our moods. No one is extraneous, and everyone is central.

MANY HAVE ARGUED that societies around the world prematurely declared the COVID-19 pandemic to be ‘over’, and that a sense of invulnerability to threat, especially in the global north, masks our shockingly poor preparedness for widespread health crises.⁶⁸ No one living in the long nineteenth century could retreat behind the felt guarantee of inoculation or take the relative stability of ‘health’ as a given, despite advances in germ theory and vaccine technology. To the contrary, theirs was hyp-time in high relief, marked by legitimate fear of both local and international contagion. Mary Shelley’s apocalyptic novel *The Last Man*, published in 1826 and set in 2071, levels some critique against the insufficiency of medical research as it imagines a dystopian future in which humanity is nearly annihilated by a global bubonic plague (I return to Shelley in [Chapter Four](#)). What might be the advantage of living with some measure of ‘threat belief’? Paul M. Salkovskis and Hilary M. C. Warwick explain in their chapter in *Hypochondriasis* that future-orientated fear, fear of what *could* happen as opposed to what *is* happening, makes some manifestations of hypochondria impervious to what they call threat ‘disconfirmation’, since the plausibility that something terrible might occur cannot readily be disproven.⁶⁹ When the World Health Organization (WHO) downgraded COVID-19 as a global public health emergency in May 2023, it did not also call an end to the pandemic. The incidence and death rates of COVID were still high, though official entities were no longer tracking that data, and healthcare systems around the world remain burdened. The rush towards *no* concern seems as much of a misinterpretation of evidence as believing that a headache presages glioblastoma.

In their introduction to ‘Crip Temporalities’, Samuels and Freeman write of the COVID moment that ‘it can be said that all of us now are living in crip time.’⁷⁰ They offer such examples as the time-consuming frustrations of the sudden pivot to online work (mirroring similar aggravations that face disabled people who make use of alternative technologies) and, in a different way, the newly discovered rhythms, pleasures and companionship that arose in the first phase of lockdown and demonstrated that we all, disabled or nondisabled, could survive the interruption of ‘rigid socioeconomic structures of time’. What if hyp-time similarly confounds the imperative to obediently move on and achieve something? Primes us to slow down, exhorts us to remember our radical precarity? What if, instead of rushing to quell hypochondria, we heard, in its plenitude, the real time of

what a body feels like and could become? What if hypochondria is not a wasted present but all the possible futures that we might, with gentle compassion, try to believe?

OceanofPDF.com

3

Hyp-istemology: Hypochondria Knows

‘I dared not contradict: doctors are so self-opinionated, so immovable in their dry, materialist views.’

CHARLOTTE BRONTE, *Villette*¹

On 17 February 1673 at the Palais Royal Theatre in Paris, the actor playing the title character of Molière’s last work, *Le malade imaginaire* (The Imaginary Invalid), collapsed onstage. That actor was Molière himself, and in an ironic twist worthy of the great playwright, he would die that very night: rushed off stage and carried home to the rue de Richelieu, where he succumbed at last to tuberculosis, with which he had struggled for most of his adult life. ‘Your Molière is a fine impertinent fellow’, says Argan, the titular hypochondriac, in a meta-moment that becomes even more reflexive considering what befell the writer who acted the role he had created for himself:

If I were a physician, I would be revenged of his impertinence, and when he falls ill, I would let him die without relief. In vain would he beg and pray. I would not prescribe for him the least little bleeding, the least little injection, and I would tell him, ‘Die, die, like a dog; it will teach you to laugh at us doctors’.²

Such a rant is typical of Argan, whose antipathy for the doctors whose ministrations he craves and idiocy he contemns is the play’s running joke.

Argan's brother Béralde offers a gentle rebuke – 'It is not the doctors themselves that he takes off', Béralde says of Molière, 'but the absurdity of medicine' – and Molière's comic genius lies in just this sort of rhythmic juxtaposition between excess and economy, between the fatuous and the reasonable.³ Setting pedants and proselytizers against the searchingly intelligent, Molière's work delighted in exposing the worst sort of greedy self-deluders in bourgeois life. Here Béralde's remark reveals him as the more incisive reader: not just of literature but of situations, people, symptoms. Argan, the prolix hypochondriac caught up in the particulars, fails to recognize a bigger picture – the structural problems of an institution like medicine (and, as the play goes on to dramatize, also patriarchy, class stratification, marriage, education and religion). There's a double irony at work too, because Argan himself mocks the doctors while idealizing medicine for what it isn't but *could be* doing for him.

Le malade imaginaire is often translated and adapted as *The Hypochondriac*. This distinction strikes me as significant, since Molière might have chosen *L'hypochondriaque* but didn't, and 'the imaginary patient' in English has a somewhat different connotation than a term that has come to encapsulate manipulation and foolishness. The more literal translation of the French retains an intriguing ambiguity of who is concocting what: if the patient is only imaginary, is he fabricating his ailments? Or has someone else fabricated *him*? The play clearly demonstrates that the 'invalid' and everyone around him, including family, apothecary and physicians, exist in mutually informing relationships, whereas 'hypochondriac', especially in a vernacular sense, situates the cause of imaginary illness – the blame – squarely in the individual. Indeed, what is 'imaginary' in Molière's title is not disease at all; it is the *patient*, which does not rule out the possibility that something is actually amiss with Argan's body. The satire of the play in this sense has more to do with the status of patient, the socially constructed behaviour of illness, than with the corrupt or ethically evacuated quality of healthcare. Though Argan experiences his several ailments as discrete afflictions that call for specialized intervention, the play as a whole distributes Argan's 'sickness' across various relational vectors, entangling the attitudes and opportunism that create him as a certain kind of patient and the suffering he actually feels.

One subplot of *Le malade imaginaire* involves Argan's attempt to marry off his daughter Angélique to a doctor. When this doesn't work – she's a youthful romantic who prefers the impoverished musician Cléante, in another of the play's critiques of social climbing – Argan is persuaded to put on a robe, mumble something that sounds like professional jargon and become a doctor himself: the joke, now enshrined on hypochondria message boards across the Internet, that the health-obsessed know more than the experts. Of course viewers will not take Argan seriously as a physician any more than the play encourages them to honour his ailments, in a farcical conflation that utterly explodes any secure locus of authority. If neither the supposedly learned expert nor the embodied person can be counted on to know what's happening in the physical self, whom do we trust? To whom do we turn for reliable information? Perhaps Molière presents the hypochondriac as a sort of inverse doctor, both preoccupied with bodily problems and fixated on the money they either spend or gain (the play opens with Argan tallying a bill).⁴ But the play doesn't just upend knowledge and authority to leave a vacuum; it invites an epistemological reframing of what and how we know. Even if *Le malade imaginaire* does close with absurd, congratulatory songs in a gibberish of mock-formal Latin, the collapse of patient and doctor in the person of Argan implies something more serious too: that what we need to understand in life (of which health is only one aspect) is *always* available as a matter of bodily wisdom. To doctor oneself is not inevitably a matter of arrogating knowledge that is but poorly or partially grasped.

It's an accident of history that the real-life Molière, a hypochondriac in his own right, wasn't acting when he had an acute attack of illness on stage. Yet the stunning coincidence of his final night nonetheless sets in relief the perennial challenge of hypochondria, to parse the performance and interrogate our assumptions about what's authentic. *Le malade imaginaire* quite obviously stages the social and interpretive dimensions of sickness, since the play's farce largely entails the clash between Argan's private and interiorized sensations of *having* an illness and what *being* ill means to him and to everyone around him – given the way in which illnesses are produced as such by medical consensus and by the attitudes of the historical moment. (Think of what 'cancer' has meant, particularly in the twentieth century: not simply as a collection of sensations or felt processes in a lived body but in a cultural sense, as an undifferentiated disease inspiring dread

that demands of us a certain kind of physical and emotional comportment.) When the always-about-to-be-ill Argan becomes his own doctor – his corporeality abuzz with symptoms *as well as* the promise (and parody) of authority – he ‘collapses together’, in Stephen Connor’s words, two dimensions of embodiment: the ‘*mineness* of my illness’ and how we are constituted by external knowledge or forces: the needs, fears, desires, expertise, authority and beliefs of the Other.⁵

While contemporaries in France and across the Channel were busily determining the location of hypochondriacal complaints in individual abdomens and began to decry hypochondria as an ailment of the over-educated and idle, Molière was staging it as a relational phenomenon entangled with power and money. *Le malade imaginaire* depicts a world in which most people are frauds in one way or another, whether by faking their learning, masking their betrayals or capitulating to romantic myths. Argan the hypochondriac sits literally at the centre of this world, receiving his enemas and publicizing what should be hidden: his bodily functions, his pain, his contingency. He repeatedly breaches decorum, and as the patriarch of the play’s family and the focal point of everyone’s attention – including the audience – he destabilizes the delicate workings of polite society. His hypochondria is ‘subversive’ in this sense, as Sneha Mantri argues, because Argan refuses to stay put in a culturally bound role of submissive patient: first by being both ‘victim and agent’ of his own corporeal self and later by becoming a ‘doctor’, exploding the doctor–patient hierarchy and ‘centraliz[ing]’ power within himself.⁶ The true scandal of Argan’s body is not the deceptiveness or hypocrisy it enables, but the lie it gives to notions of authority.

MOLIÈRE’S PLAY CALLS attention to hypochondria as ‘epistemopathic’, as Connor calls it – ‘a disease of knowledge’.⁷ That hypochondria has historically been diagnosed as resistant to reassurance puts the contest over expertise in sharp relief: the condition was *defined* by interpretive disagreement in both the American Psychological Association’s *Diagnostic and Statistical Manual of Mental Disorders* and the International Classification of Diseases until their most recent editions. The *DSM-IV* (1994–2013) states that the hypochondriac’s ‘preoccupation’ with a specific

disease ‘persists despite appropriate medical evaluation and reassurance’, and the ICD similarly lists ‘Persistent refusal to accept medical advice’ through its tenth edition (1994–2022). Such language precludes any confusion about who’s in charge of ascertaining health. ‘Medical evaluation’ and ‘medical advice’ are positioned as absolute in their epistemological accuracy, their trustworthiness, which means that the hypochondriac’s insistent obduracy can only be accounted for as pathological. Thus the patient who believes in a diagnosis ‘despite’ their doctor’s denial of it occupies an impossible paradox – simultaneously healthy (free of the dread disease) and hypochondriacal: psychiatrically ill. To believe in something that seems factually or demonstrably untrue is to engage in superstition – to be a fantasist, a paranoiac, even a child. This is just how Molière’s Argan is treated: as ‘crazy’, ‘infatuated’, ‘look[ing] at things with strange eyes’.⁸

Will Rees approaches hypochondria’s battle over certitude slightly differently, arguing that the hypochondriac ‘wants dense intransigent flesh to become fully and finally knowable’.⁹ In this first stage of an elaborated pursuit for a definitive but elusive truth of oneself, it is the self’s own status that remains frustratingly tenuous: how can we ever be sure? The hypochondriac then enlists the physician in the quest for control, and when the physician fails to comply (as, by definition in a hypochondriacal scenario, they will), the expert becomes the new adversary. The paradox here is that while the hypochondriac may feel convinced that they know what the bodymind is doing – how precisely it has gone ‘wrong’ – that certitude requires verification from the clinician, now become stubborn and non-compliant. If experiential knowing were sufficient, in other words, our independent, private assessments of ourselves might obviate the clinical visit. This isn’t just a question of assuming the need for treatment, I think, but rather the evasive quality of knowledge itself and the complex dynamics of confirmation that a desire for certitude sets in motion.

In a 1979 study of physicians’ ‘affective orientation’ towards patients, Carlton A. Hornung and Michael Massagli reported that it is ‘relief from the emotional disturbance and anxiety associated with the fear of being ill’ that really impels someone to the doctor’s office, as much as we may also seek medical answers.¹⁰ If patients are driven towards consultation in some measure by *emotion* – and certainly the hypochondriac experiences anxiety

to an intense degree – clinicians are trained to respond primarily through *information*.¹¹ This means that any clinical encounter has the capacity to become mired in competing expectations, and the conflict of hypochondria will turn on a fulcrum of antithetical strategies, approaches and needs. Any discounting of the symptoms a patient brings to a standard fifteen-minute office visit, of that patient's honest if worried report of the body's plethora of news, will be felt as a direct assault on the viability of self – a rejection not just of the legitimacy of suffering but of personhood itself, since in the raw vulnerability of our expressions of feeling and hurt we expose the material fact of ourselves. Catherine Belling writes that 'hypochondriacs are shunned because of the threat their doubt poses to the fragile certainties on which medicine – and our medicalized culture – have learned to depend,' agreeing with Susan Baur that the psychological dimension of hypochondria 'force[s] the doctor' to confront not only the inadequacy of medical omniscience but their own emotional fragility.¹²

In a renowned 1978 paper in the *New England Journal of Medicine* called 'Taking Care of the Hateful Patient', James E. Groves sorted hypochondriacs into four broad categories, including 'entitled *demanders*', 'dependent *clingers*', 'manipulative *help-rejecters*' and 'self-destructive *deniers*' – a taxonomy of overt derision that has retained some currency.¹³ Belling interprets the intensity of Groves's hostility as an expression of defensiveness against 'not only frustration and anger' but even the 'paranoia' that hypochondria might inspire in medical professionals because – annoyance aside – 'it tells a truth: it tells us that we cannot know for sure.'¹⁴ As a projection of doctors' own fears of a lacuna at the core of their professional identities, hypochondria represents an 'instability of knowledge that the authorities are supposed to have overcome' and so upsets the practitioner by repeatedly disproving, with its embarrassing unrelenting aches and pains, its rashes and lesions, its inexplicable eructations and obstipations, the illusion of medical omniscience.¹⁵ Who, after all, is the appropriate arbiter of what all this is, feels like or portends? We may agree that *this* scan does not reveal breast cancer, or does not reveal spinal stenosis *now*, but can such conditions be definitively ruled out for all time? What of the numerous medical mysteries a local GP, even a specialist, might be ignorant of? The dichotomy of emotion and information is a false one, finally, that elides the way in which 'experiential knowledge

is necessarily emotional knowledge,’ as Yvonne Wechuli argues of ‘self-determined knowledge production’ in the process of crippling norms of embodiment.¹⁶ What the hypochondriac brings to the clinical encounter may thus have less to do with anxiety than with the importantly lived knowledge of what it feels like – what it means – to be *this* body at this moment in time.

Hypochondria arises within a series of distressing gaps: between the individual’s ‘subjective experience of the body and its underlying organic reality’, in Rees’s words;¹⁷ between what the subject knows is being felt and what the cause could be; between one possible cause and another; and between the individual bodymind (throbbing, itching, sweating, clenching, forgetting) and medicine’s vast but somehow fallible storehouse of knowledge. Always the feared condition *could* be true – but also, horribly, might *not* be true. Because hypochondriacs tend to be well aware of their own catastrophizing, both the terrible organic pathology *and* the fantastical fixation may be equally distressing; we could arrive at an awful truth, or we might be frustrated by an ever-receding truth. This makes the condition of hypochondria an epistemological vanishing point.

THE STORY TOLD BY contemporary psychiatric definitions of health anxiety is of a serious and intrusive form of mental work that preoccupies a person’s awareness to the exclusion of other pursuits and substantially compromises well-being. The *DSM-5* (2013) replaced ‘hypochondriasis’ with two related conditions: illness anxiety disorder (IAD) and somatic symptom disorder (SSD).¹⁸ Both are characterized by a high state of anxiety about the state of one’s health; individuals with IAD tend to be unduly worried about having or getting a serious medical problem, while those with SSD focus on physical sensations to a disruptive degree. The change in nomenclature was by design: because ‘hypochondria’ is firmly installed in the vernacular as a dismissive – often explicitly pejorative – term, freighted with centuries of popular rather than strictly scientific representation, the new language aimed to eliminate belittling connotations.¹⁹ The new categories also emphasize presence over absence; where the older ‘hypochondriasis’, based on medically inexplicable symptoms, was an exclusionary diagnosis (if it isn’t anything else, it’s hypochondria), the criteria for IAD and SSD focus on

positive symptomology (anxiety and fixation are real and disabling). The current diagnoses, taking seriously the fact more than the content of a person's worry, are thought to generate more holistic possibilities for treatment.

Yet the new categories don't fully eliminate a troubling possibility that something is awry; nor do they resolve an implicit debate about whose version of reality takes precedence. The previous definition of hypochondria, which specifies 'preoccupation with fears of having, or the idea that one has, a serious disease based on the person's misinterpretation of bodily symptoms', is replaced in the *DSM-5* with something more succinct: 'Preoccupation with having or acquiring a serious illness'. So too is persistence 'despite medical evaluation and reassurance' eliminated in the newer edition, so that illness anxiety is now defined not in terms of a contestatory and interpretive dynamic between individual and practitioner – between ignorance and knowledge – but as a 'disproportionate' degree of uneasiness about symptoms. Some measure of credibility is restored to the knowing bodymind in this formulation; health anxiety may be 'maladaptive', but it is characterized as a problem of degree rather than a failure of rational thought. These 'softer' criteria seek a new tone of respect, and they refocus attention away from what medical practitioners know and on to the behaviours a person exhibits. In so doing, however, they manage to reopen the door – wide open, one might say – for worry. If IAD is not to be established in relation to reassurance of an absence, then it can never rule out a presence; in fact 'somatic symptoms' and other 'medical condition[s]' are named, even if 'mild', as contributing factors in the condition, such that hypochondria remains, if not an argument *with*, then still an argument *about*. Framing the problem as one of intensity rather than interpretation means that people prone to amplification of bodily sensation, diagnostic criteria of health anxiety aside, may find themselves in a struggle for verification with medicine – an institution that, because it 'lays claim to an impossible certainty', as Rees says, ends up 'perpetuating hypochondria by creating an epistemological expectation that no lived reality could satisfy'.²⁰

The question raised by a play like *Le malade imaginaire* is where else this crisis of knowing might take us, if not into a looping dispute over proof about the existence of disease. What knowledge might a suffering bodymind offer us about ourselves, or about systems of power, economic or

academic structures of success, inequity and bias? Two epistemological frameworks seem relevant here: embodied cognition, defined as ‘gaining knowledge and intelligent problem solving’ through corporeality, and the Disability Studies concept of cripistemology, which refers to how knowledge is produced and shaped by disabled experience.²¹ Both might offer a way of thinking about *hyp*-istemology as generating creative perceptiveness and imagination rather than caught up in a putatively wasteful repetition of thought. A typical figure of hypochondria is one who acts negatively (even selfishly) upon others – projecting worry outwards, demanding reaction – and yet is passively unable to combat or forestall the flood of meanings that arise from bodymind data. What I want to propose as the ‘hyp-istemological’ figure becomes a locus of knowledge that, in conventional systems and narratives of wellness, healthcare and social value, has difficulty finding outlet or expression aside from complaint.

Embodied cognition emerges from the philosophical tradition of phenomenology, particularly the work of Edmund Husserl, Martin Heidegger and Maurice Merleau-Ponty, for whom consciousness was grounded in bodily experience. Merleau-Ponty, for example, writes in *The Phenomenology of Perception* that consciousness is mediated through what he calls *Leiblichkeit*, or lived bodiliness, which is ‘our general medium for having a world’. ‘The subject that I am’, Merleau-Ponty declares, ‘is inseparable from this body and this world.’²² For later phenomenological theorists of cognition, as Komarine Romdenh-Romluc explains, the ‘cognizing subject’ apprehends the environment, processes thought and initiates action through the physical self. We don’t just think with our brains, in this rejection of Cartesian mind–body dualism, but through sensory information, including interoception (how we sense and understand signals from within the body) and proprioception (how we sense the location and movement of our bodies in space). Philosopher Mark Rowlands poses the question this way: ‘do cognitive processes occur purely in the brain . . . or do they, in part, extend into the bodies’ of ‘cognizing creatures’?²³ What it means to have ‘agency’ from this perspective is not that our actions are guided by our intentions – by our mental states – but that we are embodied; the distinction is a potent one, since we associate agency with having control over actions as well as outcomes in both innocuous and broadly political contexts.

Antonio R. Damasio's 1994 book *Descartes' Error: Emotion, Reason, and the Human Brain* offers a neuroscientific approach to the complex relationship between minds, bodies, cognition and selfhood. Damasio explains that we are 'far more aware of the overall state of the body than we usually admit'. A 'background body sense is continuous', in fact, and this 'ongoing, unintermittible representation of the body' is thoroughly integrated with how we articulate 'individual identity'. Disruptions in body-state mapping, whether through structural damage or psychiatric trauma, have a profound effect on people's awareness of the general state of their health and in turn, Damasio contends, on the integration of self. A person who is 'unable to plot current body signals on the ground reference of the body . . . cannot . . . reason effectively on the current personal and social state' and thus exists in a kind of ahistorical limbo, unable to construct exact mental theories-of-mind either for themselves or for others.²⁴ What then of people who might be said to *constantly* plot their bodily information? Anosognosia is defined as a cognitive inability to register bodily signals and so to maintain an updated sense of one's bodily condition or health. If such patients experience a flattening of self-awareness, might hypochondriacs, so sharply attuned to their body-state, be *more* intensely, more *fully*, themselves? Hypochondria in this respect would represent not a deviation from 'normal' selfhood at all but a more readily available wealth of bodily information than usually comes to consciousness.

If consciousness is thus embodied in these various ways, what is a 'crip' form of knowing? The term 'cripistemology' was introduced in a special issue of the *Journal of Literary and Cultural Disability Studies* in 2014 by Robert McRuer and Merri Lisa Johnson. In 'Cripistemologies', Johnson and McRuer argue for alternative ways of 'knowing disability' that challenge the 'monovocal singularit[ies]' of medical and rehabilitative models of disability and promote creativity and activism.²⁵ Fundamental to cripistemology is 'better knowledge *about* disability' (my italics) – resistance, for example, to ableist narratives of the horror of diminishment and the rush to fix what is deemed 'wrong', to restore problematic bodyminds to a dominant sameness in the service of neoliberal productivity. Also foundational to cripistemology is the knowledge that arises *from* experiences of impairment. What does a crip perspective have to tell us about pain, for example, that contemporary medical and advertising discourse typically elide? What might we discover about how academic

systems enforce normalcy, even (and especially) when attending to the ‘special’ needs of some students, or when deploying such slippery euphemisms as ‘quality of mind’ in hiring or promotion? What might we learn about the built environment, the normalizing attitudes that determine our demeanour, from a visually impaired colleague or from someone who identifies as neurodivergent? What is beauty for a wheelchair-using dancer, an artist with spina bifida? How might we rethink caretaking, kinship, intimacy, sex, with and through bodies that defy norms of motion or symmetry? Acts of ‘cripping’ what we (think we) know manifest the profound inequities of the environments we cohabit as well as possibilities for new forms of thinking, working, being human.

The challenge of hypochondria to the idea of cripistemological knowing is its apparent rootedness in unreality, a correlation so thoroughly popularized that it undergirds the most casual of jokes (‘The doctor said I’m a hypochondriac . . . but I’d like a fifteenth opinion’; ‘The hardest thing about being in Hypochondriacs Anonymous . . . is admitting you don’t have a problem’). As a condition of fear deeply bound up with the stabilizing parameters of medicine, hypochondria would not readily seem to organize around a shared political identity that mobilizes *resistance* to diagnosis and, in particular, to the norms of cognitive fitness – such as Mad Pride or the neurodiversity movement – of which Western psychiatry is a prime upholder. We understand hypochondria to be a condition one has, not a subject position whose public, social legitimacy one might choose to advocate for. But as I have argued thus far in this study, hypochondria reveals much more than a faulty response to stimuli. As a mode of attention, it gives the lie to ‘wellness’ as its own flip side and exposes limitations that metanarratives of knowledge like medicine work hard to obscure. (For we might say that the hypochondriac isn’t wrong as such but that everyone else is answering the wrong question.) Thinking of hypochondria only as a category of obsessiveness or phobic anxiety elides its legitimacy as a mechanism whereby we might figure things out that we need to know how to do – such as sustain trust, read the embodied world, endure both frailty and risk or contest the privileging of health that trains our focus on to the quality of our embodiment and faults us for not measuring up. To think of hypochondria only in terms of its fear of what isn’t, in other words, misses its profound attentiveness to what is: that ‘people are never entirely well’, as Frederic Neuman writes in *Worried Sick?*, even though our narratives,

expectations, technologies and institutions conspire to convince us that it is within our agentic means to make ourselves so.²⁶

It's one thing to say that we know – that we are conscious – through our bodies, and another to acknowledge the limits to what we can know *of* our bodies, even with ever more precise instrumentation. The enticement of medical technology that we *can* know masks a more insidious inducement that we *should* know and that we fail if we ignore what can be discovered. But of course bodies will always behave randomly, impairments and illnesses will happen, and much will remain mysteriously beyond our ken, as the hypochondriacal body makes plain. That body is at once the object of our thinking and the source of knowing – a form of cognition that establishes the subject as such. And it exists to remind us of what can never be conclusively guaranteed.

IT IS A WELL-KNOWN QUIRK of the history of hypochondria that Freud had relatively little to say on the subject, despite his own tendencies in that direction. 'One would naturally expect that Freud and his followers would have something pithy to say about a prevalent and ill-defined "condition" with a history spanning two thousand years,' writes Don R. Lipsitt, but in fact Freud investigated hypochondria less on its own terms than in relation to paranoid psychosis, obsessional neurosis and narcissism.²⁷ Indeed Freud seems baffled by how to comprehend the psychical mechanism of this form of anxiety, writing in a letter to Hungarian psychoanalyst Sándor Ferenczi in 1912 that he had 'always felt the obscurity in the question of hypochondria to be a disgraceful gap in our work'.²⁸ He associated hypochondria with neurasthenia and anxiety neuroses – so-called actual neuroses characterized by emotional and corporeal intensity but not by unconscious conflict – and concluded in *On Narcissism* in 1914 that 'It is not within the scope of a purely psychological inquiry to penetrate so far behind the frontiers of physiological research.'²⁹

The close of the nineteenth century marked hypochondria's uncertain reclassification as a mental problem rather than a malady with emotional, intellectual and corporeal features. For Freud hypochondria seemed to lack the antecedent of repressed psychical struggle that defined the symbolism of psychosomatic disorders like hysteria, wherein a physical symptom is

freighted with psychical meaning but relatively little affect (what is sometimes called hysteria's *belle indifférence*); arising from the body rather than unresolved unconscious trauma – from 'dammed up' libidinal energy attached to the organs of the body, creating a flood of nervous excitation – hypochondria was thus 'unanalyzable' ('insufficiently neurotic', as Baur puts it).³⁰ At the same time, medical doctors, swept up in the new germ theory, were increasingly focused on one-to-one correspondences between specific diseases and their causes, rather than the constellation of complaints, including emotional ones, that had tended to characterize hypochondria in the days of Boswell and Coleridge. Distastefully profligate in its range of physical manifestations but no longer etiologically confirmable as a disease of certain body parts, hypochondria 'became an orphan hybrid', as Belling says, 'as both and therefore neither somatic nor psychological and as such a condition that neither domain of health care was obliged to take on as its own'.³¹

What happens to hypochondria over the course of the century to follow reflects post-war jurisdictional disputes between psychiatry, psychoanalysis and clinical psychology. So thoroughly do we now associate psychiatry with the dispensing of psychopharmaceutical drugs that it may surprise readers to know that analytic treatment was once the 'dominant paradigm in outpatient psychiatry', in Steven Reidbord's words.³² Yet 'psychoanalytically inclined practitioners', as Andrew Scull explains, 'dominate[d]' the profession after the war, chairing departments of psychiatry and ushering in a new generation of training programmes. At the same time, what Scull calls the 'medical monopoly' of American psychoanalysis subordinated clinical psychologists, so that that group, in a 1949 countermove, codified their own training requirements and attached themselves to the burgeoning resources of academic departments equipped with labs and federal support.³³ This development began to disadvantage psychoanalysis as a practice philosophically disinclined towards large-scale research, as did widespread closure of state-run psychiatric institutions in the 1970s; the extended style of analysis was impractical for outpatient treatment, which tended towards short-term interventions like cognitive behavioural therapy.

It was the development of anti-psychotic and antidepressant medications in the 1950s and '60s, however, that initiated the most significant shift. Not

only did psychotropics like Thorazine reduce symptoms; drug companies, seeking regulatory approval for their new products, needed not individual sufferers with bad relationships or childhood trauma but ‘homogenous groups of patients on which to test their wares’.³⁴ The 1980 publication of the *DSM-III* – later described by American Psychiatric Association (APA) executive Melvin Sabshin as the triumph of ‘science over ideology’ – solidified the link between the pharmaceutical industry and psychiatry with a revised nosology of mental disorders that privileged symptoms rather than etiology and made diagnosis a matter of ‘meeting criteria’.³⁵ Drug companies could then correlate medication to lists of symptoms – thereby demonstrating improvement or even cure – and psychoanalysis and psychodynamic approaches in turn came to seem vague and unscientific. U.S. President George H. W. Bush next proclaimed the 1990s the ‘Decade of the Brain’, initiating a joint venture of the Library of Congress and the National Institute of Mental Health ‘to enhance public awareness of the benefits to be derived from brain research’.³⁶ These combined events, to quote Scull once again, ‘marked the decisive recapture of the profession by a biologically reductionist view of mental disorders’.³⁷

The field’s trajectory from a psychoanalytically inflected discipline to a medicalized one tells us that when the *DSM-5* replaced ‘hypochondriasis’ in 2013 with illness anxiety disorder and somatic symptom disorder, as I suggest above, it didn’t just remove an objectionable name or stipulate unreasonable worry over a focus on health and disease to mitigate the derogatory implications of hypochondriacal fixation. It was following a move away from a notion of human psychology as dynamic, experiential and idiosyncratic, and towards ever more ‘minutely mappable’ neurological phenomena whereby any conduct can come to seem pathological and thus available to pharmaceutical ‘management’.³⁸ Such is the criticism of this edition generally: that the process of revision was compromised by editors’ links to big pharma along with a tendency to medicalize behaviours not deemed particularly extreme.³⁹ In view of what he names ‘the rift between biological psychiatry and psychotherapy’, Reidbord has argued for a return to the ‘diffuse dissatisfactions, stormy relationships, unwitting self-sabotage, dissociative reactions and other misery that cannot readily be reduced to DSM diagnostic criteria’.⁴⁰ But what might this mean for our understanding of hypochondria, if it cannot exactly be ascribed to biology

and yet presents primarily at the level of bodily discomfort? What might lie between psychiatry's absolute faith in neuroscience and psychological interpretations that read the hypochondriac's bodily suffering as an expression of unresolved guilt, hostility and rage – a pathological and 'immature' deflection of what cannot be uttered aloud, always and only a referral to psychological phenomena that cannot be brought to consciousness? Or between a psychoanalytic session and the doctor's office? What can we know of, and through, hypochondria that *isn't* 'captured' by these competing paradigms, that might restore body and mind to a worthwhile integration?

In the Romantic age of 'high hypochondria', as we might call it, the German philosopher Novalis pronounced hypochondria 'an art', even an 'education', 'prepar[ing] the way to bodily self-knowledge' and 'self-vitalization'.⁴¹ Though psychodynamic and neurochemical explanations of health anxiety have overtaken that aesthetic type with their emphasis on causation and cure, might we restore something of its epistemological valence, a hypochondriacal style of self-awareness that reveals our attitudes towards expertise, vulnerability and what Steven Connor calls 'the swirling dread of insubstantiality'?⁴² This would refute a reductive sense of hypochondria's complaints as always and only erroneous as well as the idea that hypochondriacal fear arises from an adversarial relationship with the body – the body as source of 'thousands of tiny invasive events', in John Diamond's words, of which it might be 'accuse[d]'.⁴³ Instead the hypochondriac is the realist in the room, as I suggested in the last chapter, fully cognisant of the body's awesome and unpredictable complexity and not afraid to declare illness as the norm from which moments of relative health are the odd deviation. To believe in something that can't be proven through ordinary diagnostic means is also of course to have *faith*, which makes hypochondria, among its many other iterations, endeavours and explanations, a curious form of spiritual knowing.

THE FRETFUL HYPOCHONDRIAC Mr Woodhouse in Jane Austen's *Emma* buttresses his many certitudes about the health benefits of gruel or the dangers of sea air with reference to a physician, Perry, who never actually appears in the novel. In an early two-page scene, the signifier 'Perry'

instigates a disagreement between Mr Woodhouse and his son-in-law, John Knightley. 'You should have consulted Perry,' says the one, to which the other replies, 'Mr. Perry . . . would do as well to keep his opinion till it is asked for. Why does he make it any business of his, to wonder at what I do?'⁴⁴ As I discuss more fully in [Chapter Two](#), mention of health and hypochondria frequently 'stall' the forward motion of *Emma*'s plot, so what is the 'point' of this dispute, which seems similarly obstructive – or at the very least irrelevant? It does allow George Knightley, Emma's eventual betrothed, to intervene, thus displaying the gentlemanly tact and diplomacy for which he is prized throughout the text, as well as the 'soothing attentions' of Emma and her sister, equally necessary to the maintenance of the social milieu. But the interlude also has craftier designs that relate to a larger thematic of knowledge, independence and the power to make one's own decisions.

The narrator lets us know that 'Perry' serves as a proxy for Mr Woodhouse to 'unconsciously' articulate his 'own feelings and expressions'. John Knightley meanwhile asserts *his* own 'judgment' as equally worthy of respect. Both men claim the right to know best salubrious practice; one deploys the figure of scientific learnedness, the other experiential savvy, but each is essentially arguing from what might otherwise be called his *gut*. Nor is either of them demonstrably wrong. Any philosophy of how to survive the risks and challenges of life, these pages thus intimate, must entail counterpoint, intrusion, negotiation, 'interposition'. Mr Woodhouse's error here – if that isn't too accusatory a word – is not the concern he has about maintaining 'health perfectly well', but his recourse to a phantom doctor. Except that 'Perry' isn't just an empty signifier. At no point is Mr Woodhouse treated, medically or psychologically, for disordered mentation, nor is he excluded from his social world; to the contrary, he – with all his 'erroneous' fears – resides at its very hub. Is the centre thus 'diseased'? Or has Austen rearranged the margins, displacing 'normalcy'? It's important that Mr Perry has no physical presence in the story, that he exists only as a manner of verbalizing what Mr Woodhouse knows in his own bodily way; 'Perry' may be the singular referent, but what sustains Mr Woodhouse is a complex amalgamation of private, corporeal, familial, folk and medical knowledge that exceeds disciplinary boundaries.

Something similar can be said of the epistemological puzzle of Edgar Allan Poe's renowned story 'The Fall of the House of Usher' (1839), in which the narrator's 'conventional and unimaginative application of scientific and medical knowledge', as David W. Butler once argued, fails to explain the mysterious occurrences that befall the titular siblings.⁴⁵ When the narrator of the story first enters the house, he remarks that 'the eye . . . struggled in vain to reach the remoter angles of the chamber, or the recesses of the vaulted and fretted ceiling', an early signal that something will remain beyond his capacity to understand, and later, when both Roderick and Madeline Usher have died in circumstances that defy rational explanation, he '[flees] aghast', unable to account for what he witnessed, its 'anomalous species of terror'.⁴⁶ In the end, as Butler claims, there is no 'unquestionably valid and complete medical' or 'mystical' explanation for these events.⁴⁷

Though Roderick Usher is the only character in the story named a hypochondriac – in the Romantic sense of 'acute bodily illness' combined with 'mental disorder' in an artistic 'sensibility' – his sister Madeline clearly suffers from a related condition, perhaps neurasthenia or catalepsy, though (or because?) her 'disease . . . had long baffled the skill of her physicians'. She displays 'a settled apathy, a gradual wasting away'; she is 'entombed . . . in the maturity of youth' (recalling Brian Dillon's remark that Alice James, herself a hypochondriac, lived a 'posthumous existence').⁴⁸ Butler argues that the narrator of the tale represents the inadequacy of 'doctors' equation of the normal and the true', but I read him as the story's third hypochondriac and the one who most overtly embodies a principle of alternative knowing.⁴⁹ As the story progresses, the narrator feels increasingly 'infected' by his friend's condition – the stigmatizing fear that madness is contagious – but he has already admitted, within the very first lines, to 'an utter depression of soul', 'an iciness, a sinking, a sickening of the heart' that utterly 'unnerve[s]' him as he approaches the house of Usher and that he feels unable to define. He is 'forced to fall back', he says, on the 'unsatisfactory conclusion' that some 'combinations' of stimuli affect us in ways we cannot fathom – certainly sounding like a disappointed empiricist – but also that 'a mere different arrangement of the particulars' might lead to a different effect or understanding.⁵⁰ This comment is intriguing, as it suggests that the cause of misunderstanding is not a baffling

or imaginary affliction but rather an inadequate set of explanatory terms. If we ‘rearranged’ the signs and symptoms, perhaps according to some not-as-yet formulated rubric of meaning, they might align with perfect comprehensibility. Thus if the narrator is a diagnostician, frustrated by a plethora of intense symptoms and their unknown origin, he is also the hypochondriac whose body is his first indication of what *is*, fully psychosomatic, acutely aware of its environment and a source of understanding, if he can settle into the language of its depths.

Poe offers a form of hypistemology in the ‘phantasmagoric conceptions’ that Roderick ‘contrived to throw upon his canvas’. There is always a ‘rational’ explanation for the mysterious – as when the narrator attributes the eerily animated miasma that surrounds the house to ‘merely electrical phenomena’, or protests that his ‘excited fancy ha[s] deceived’ him. This seems akin to mapping hypochondriacal symptoms on a backdrop of unconscious psychological needs that confuse us because they are physically iterated but could be dispelled if only the hypochondriac would submit to the proper professional care. But a more complex story emerges in the ‘conflicting sensations’ that Poe’s narrator attempts, and fails, to disentangle.⁵¹ The more the boundaries between apparent reality and fantasy, living and dead, human and architectural collapse, the less distinct mind seems from body, psyche from soma, and so neither science nor imagination definitively prevails.

THE HYPOCHONDRIACAL DIMENSIONS of ‘The Fall of the House of Usher’ neatly encapsulate the dangers of siloed, highly specialized scientific and philosophical territoriality. The hypochondriac seeks explanation of what is wrong, but as Hilary M. C. Warwick and Paul M. Salkovskis point out, medical reassurance says only ‘what is *not* wrong’ and refers the patient to the next specialist.⁵² The typical scenario of repeated referral for ‘exclusion of relevant pathology’ may rule out a singular diagnosis but it provides no alternative, and it fails to respect continuities of mind and body, of self and environment. It makes sense to recommend, as Robert Kellner does in *Somatization and Hypochondriasis*, that practitioners slow down and listen more carefully to their patients before sending them off to the next diagnostic exam – but to what end is that listening instructive?⁵³ Salkovskis

and Warwick propose that reassurance *itself* can be misinterpreted. This too makes sense; hypochondria entails not just heightened sensitivity to what is felt but restlessness about the many dimensions of meaning: why *would* one denial of a single possibility suffice? (The older criteria of ‘resistance’ to medical reassurance seems less accurate than *impatience*.) Any one answer will have a way of hinting at all the further questions as yet unasked, which means that the act of reassurance can *perpetuate* hypochondriacal worry rather than eliminate it, as Salkovskis and Warwick suggest. A more effective strategy than denying what isn’t, they propose, is to ‘provide an alternative’ for what is – a tactic that, in breaching boundaries of mental and physical health expertise, recalls their mutual insufficiencies.⁵⁴

As a condition of exaggerated quest, of searching, hypochondria is unique in being defined as an act of proving its own inexistence. This is why, beyond the frustrations of the clinical setting and the epistemological crises that arise there, hypochondria has been so rich a subject for philosophers and literary and cultural critics interested in metaphysical questions of identity, difference, language and reading. Will Rees points out that ‘since antiquity, hypochondria has frequently been blamed on reading’ and goes on to explore hypochondria as a condition *of* reading: ‘not the best way to live’ but nonetheless an intriguing way to read, with its indefatigable refusal to accept ‘the impossibility’ of hermeneutic clarity.⁵⁵ For Rees the hypochondriac is a dogged interpreter, a kind of deconstructionist par excellence for whom symptoms indicate every conceivable disease, just as Post-Structuralist semiotics decrees that language pull its accumulated traces of signification through history in ever more freighted lodes of meaning. The French philosopher Gilles Deleuze beheld hypochondriacs somewhat differently, as ‘the Outcasts of reason’, by which he means that the ‘unreasonableness’ of hypochondria more accurately reflects life’s ‘indeterminate, obscure, indiscernible zones’ – those that ‘defy . . . clarification’ – than any supposedly rational system of law, thought or social organization. Specifically rebelling against paternal arrogance and authority, Deleuze argues, the hypochondriac embodies an ‘archipelago-perspectivism’ that exposes our pretensions to wholeness as false. Seeking the transcendental signified of original cause, the hypochondriac proliferates ambiguity – precisely the unsettling indeterminacy that makes the health worrier so inconvenient for doctors.⁵⁶

Deleuze's remarks appear in an essay that discusses the infamously unknowable Bartleby, the scrivener, titular protagonist of Herman Melville's 1853 short story. Bartleby is a notorious interpretive conundrum who sets in motion an agitated scramble to make sense of him and whose radical non-compliance – what the narrator calls Bartleby's 'strange wilfulness' – has been read by critics in equally multifarious ways: as passive resistance to convention and wilful eschewal of the doctrine of capitalism; as depression, schizophrenia, autism and suicidal ideation; as an autobiographical comment on Melville's own writerly concerns about originality; as the failure of homoerotic longing and/or the possibility of friendship, intimacy and care; and as a symbolic sacrifice to society's cruelty towards difference, especially the stigma of mental illness.⁵⁷ Understanding the *story* hypochondriacally takes stock of these proliferating 'causes' but shifts the focus away from diagnosing Bartleby himself and towards the dynamics of reading that occur both within the tale and between the tale and its audience.

Hired as a copyist in the narrator's Wall Street law firm, Bartleby is initially an industrious worker whose 'sedate' physical 'aspect', his 'motionless[ness]', balance the temperaments of the narrator's other two clerks, the 'flighty' Turkey and 'fiery' Nippers. (Nippers has a 'dyspeptic nervousness' about him, itself a symptom of hypochondria.) But when the narrator asks Bartleby to proofread some papers, the scrivener utters what has become one of literature's iconic lines, 'I would prefer not to': a non-refusing deferral that stymies but does not actually say 'no'.⁵⁸ 'I would prefer not to', 'I would prefer not', 'I *prefer* not' – these become Bartleby's steadfast responses to his employer's requests – whether for work tasks or personal information, or, inevitably, when Bartleby stops copying altogether – that he quit the office. Bartleby's behaviour grows increasingly curious: he stares at a wall, stands silent in the middle of the office, refuses to answer questions. And these non-actions have an ambivalent effect on the narrator, who simultaneously prides himself on doing what he can to 'rid' himself of an uncooperative – a *contagious* – employee and attends to Bartleby with a striking, if sometimes desperate, tenderness. Indeed he cycles through a range of emotional responses to the scrivener, careening from exasperation to 'nervous resentment' to 'squeamishness', recognizing perhaps only too late Bartleby's essential 'humanity'.⁵⁹ Indeed, Bartleby simply *is* throughout these fluctuations; it is the narrator who works himself

into a froth over this ‘unheard-of perplexity’ that has installed itself at the centre of his experience. ‘Nothing so aggravates an earnest person as a passive resistance,’ he says at first, later admitting that he ‘had got into the way of involuntarily using this word “prefer” . . . [and] trembled to think that [his] contact with the scrivener had already and seriously affected [him] in a mental way’. Whatever Bartleby ‘means’, then, it is not confined to him; it circulates, it exists in relationship, and it has far more to do with what others make of him than we can discern of the man himself.⁶⁰

Of Bartleby’s intractable speech – ‘I prefer to give no answer’, ‘I would prefer not to be a little reasonable’, ‘I would prefer not to make any change’, ‘not to make any change at all’ – Deleuze writes, ‘the formula is ravaging, devastating and leaves nothing standing in its wake.’⁶¹ In preferring a negation Bartleby frustrates linguistic logic; he does not choose, though he clearly evaluates; he has desire but expresses only what he doesn’t want. He reveals nothing of himself to the narrator, despite repeated appeals, aside from the *fact* of himself, his continuity (‘I like to be stationary’); he is the ‘cause of great tribulation’ to the narrator, but cannot be deciphered, excised, assuaged.⁶² And it is this startling, vexing, open-ended endurance that brings us to the core of Bartleby’s hypochondriacal ambiguity. Reading Deleuze (or any critical treatment of the story) reading the story’s narrator trying to read Bartleby enacts the scramble of hypochondriacal knowing: its opacity, the provocation of it, its refusal to be pinned down. Hypochondria says, over and again, that something is *there*, something that ‘cling[s]’ to us, but no one can determine precisely what it is. Bartleby is in this sense hypochondria itself, an irritation the narrator can’t ease, which fascinates him, ‘haunting’ him. Bartleby is a hypochondriac too – he is somehow ill, his eyes ‘dull and glazed’, his aspect ‘cadaverous’, yet cannot be ‘threat[ened] . . . into compliance’ – as is the narrator, who can’t stop obsessing over Bartleby, even as he is also Bartleby’s doctor, frantic to diagnose. And Bartleby is the very failure of medical reassurance, which says ‘No’, over and over, but has no language for what else might be, for *alternatives*.⁶³

A symptom without a story, ‘of whom nothing is ascertainable’, Bartleby is the narrator’s story, which moves from first encounter with something unexpected all the way to death; its trajectory is the hypochondriac’s worst fear, the one that propels the anxious attempt to *know*.⁶⁴ It is implied at the

end that Bartleby, jailed for vagrancy, starves himself, and the narrator later discovers that Bartleby once clerked for the Dead Letter Office. This disquieting brush with mortality elicits the narrator's final, pained outcry (as well known as Bartleby's weird 'prefers'): 'Ah Bartleby! Ah humanity!'⁶⁵ Whatever hypochondria is – if hypochondria is simply being alive, mortal, aware of our vague longings, not always sure how to actualize our preferences – we *all* have it; we are all susceptible to it, whether it arouses annoyance, disgust, contempt or compassion. I disagree with Deleuze's reading in that Bartleby's maddening articulation, though it certainly incites consternation, doesn't raze everything to the ground at all; in fact the narrator is profoundly moved by his relationship with Bartleby, whom he describes as 'a perfectly honest man, and greatly to be compassionated, however unaccountably eccentric', and Bartleby attempts, such as it is, also to make contact with him: 'I would prefer *not* to quit you.' When the narrator avows that he has 'at last . . . penetrate[d] to the predestinated purpose of [his] life', he *intends* to exorcize himself of 'the apparition in [his] room', the 'intolerable incubus'.⁶⁶ But instead he continues to reach for Bartleby, to care for him, feed him, grasp the import of their connectedness. Just as the narrator survives his encounter with hypochondria, so will he, like Bartleby, eventually come to death. *Knowing* Bartleby brings him to that ultimate ontological recognition.

THE NARRATOR OF 'Bartleby', like the narrator of 'Usher', uses the word 'aghast'.⁶⁷ The spectacle of hypochondria arouses horror, amazement, shock. But it can also inspire care – and a refined sense of what it means to be a member of a society, responsible to others and to the collective. Dr John Reid observed in his *Essays on Hypochondriacal and Other Nervous Affections* (1817) that 'sufferings which arise from no palpable or assignable cause, are not the less real or pitiable on that account,' and his repeated dictum to fellow physicians is to understand hypochondriacal complaints in relation to patients' somatic comprehension of their interior states of being along with the world around them.⁶⁸ It's a rallying call now resounding in communities of long-COVID sufferers.

The COVID-19 years have brought many of us unfamiliarly close to the mechanics of how medical knowledge is generated. The process can be at

once frustratingly slow and confusingly fast. Innovations in the treatment of cancers and HIV/AIDS, for example, have been considerable but also painstaking over the last forty years, while in the situation of the novel coronavirus, regular public communication at a broad scale was troubled by a lag between guidelines and the rapidly changing science. Lauren A. McCormack and others wrote in *Public Health Reports* in early 2021 that ‘in late February and early March [2020], the scientific understanding of COVID-19 and SARS-COV-2 was limited, including lack of clarity about how the virus is transmitted, the incubation period, peak infectiousness, risk factors, basic epidemiology, and effective transmission prevention.’⁶⁹ The speed with which both burgeoning statistics and laboratory research improved experts’ understanding of COVID meant that public health recommendations repeatedly changed; given that people were primed to be at once panicky about sickness and unsure about the veracity of what they were told about that sickness (especially in a politicized environment), changing or even conflicting information from such entities as the Centers for Disease Control and Prevention and the World Health Organization often translated, at least colloquially, as incompetence and ignorance. Thus the realization that the experts themselves weren’t sure if COVID-19 was transmitted by aerosol or surface contact often led to a more pervasive conviction of ‘how little [doctors] actually know about medicine’ – as posted some years ago in a different context on a hypochondria message board: ‘They have no understanding of risk factors, or root causes, or genetics . . . The damn doctors are fools.’⁷⁰

At the heart of such outcry lies an aggrieved frustration – one the writer in question wanted someone to validate even as the content of the post suggests an adamant refusal to acquiesce to the writer’s own inadequate *academic* knowledge. The desire is not to be corrected and so reinstated in the comforting arena of medical authority but rather to be ‘heard’: verified as a reasonable arbiter of one’s own condition. Herein (once again) lies the source of what is so infuriating about hypochondria for medical practitioners, because the patient seems nervously to *crave* and arrogantly to *deny* the physician’s expertise. But of course doctors do *not* always know, in any absolute sense; they may even be guilty of what is increasingly being derided as ‘medical gaslighting’. A study published in December 2022 by Larry Au et al. reports that a new form of ‘ontological politics’ has arisen over long-COVID, for example, a condition – like others

carrying a so-called ‘legitimacy deficit’ – that has frequently been discounted as unreal or ‘cryptically dismissed as ‘psychosomatic’.⁷¹ In the absence of definitive diagnostic criteria or treatment protocols, long-COVID can be shrugged off as hypochondriacal, returning our attention to the ‘privileging of biomedical expertise over lived experience’ and the position of medical practitioners as the ‘gatekeepers’ of the establishment, in a position to confer validity upon one’s symptoms. It is doctors, not patients themselves, who turn bodily events and sensations into ‘signs’ of recognized disease that then become meaningful as evidence. As Arthur Kleinman puts it in *The Illness Narratives*, doctors ‘recast’ and ‘reconfigure’ patients’ total illness experience – the interrelated psychological, environmental and physiological facets of being or feeling sick – as ‘narrow technical problems’. What the *patient* knows about what is happening gets translated, through established nosology, to ‘a new diagnostic entity – an “it” –’ that can be subjected to intervention.⁷² What’s lost in this process, Kleinman contends, are myriad points of information that inflect the course of an illness and that could contribute to its healing.

I address the question of credibility in relation to hypochondria in [Chapter Five](#). Here I want to emphasize the mysteriousness of medical conditions such as long-COVID and the effect of the ‘odysseys’ patients endure as they consult evasive practitioners and seek elusive treatments – because these too have the power to exacerbate our fears. The more the doctors seem not to know, all the while insisting that it’s the patient who fabricates the problem (just as public messaging puts the onus on us to take appropriate care of ourselves) – and the more this debate structures the quality of our relationship to both healthcare and our own bodies – the closer we get to certain fundamental instabilities: how diseases and disorders are defined, for instance, along with the problematic exclusion of subjective experience in the process of gaining medical knowledge. In her harrowing tale of undiagnosed endometriosis, novelist Hilary Mantel (also discussed at greater length in [Chapter Five](#)) includes a scene in which she describes to her partner an encounter with the doctor: “‘I said my legs ached and he said it was accounted for by no known disease. Except one called idiopathic something-something’ . . . I knew [that] meant disease about which we doctors have no bloody idea.’⁷³ The shifting meanings of ‘hypochondria’ over thousands of years exemplify the fact that illnesses do not simply come into being as biophysiological problems; and because

hypochondria is a condition that cannot be objectively verified through standard medical assessment, it epitomizes the inconsistent ways in which the *feeling* of illness gets incorporated into diagnostic and treatment paradigms. But as a phenomenon of thinking in itself and one that by definition points elsewhere, to what else is or could be, hypochondria *could* be valued for deepening the investigation rather than decried for peddling in false and immaterial knowledge.

It's a common trope in literature about hypochondria to repudiate, as the late John Diamond does, 'the error of our little hypochondriacal ways' when 'a proper illness' arrives. From his memoir *Because Cowards Get Cancer Too: A Hypochondriac Confronts His Nemesis* (1998), this comment typifies Diamond's sense of the relationship between his 'hypochondriacal imagination' and the cancer with which he is eventually – though by no means immediately upon presenting to his doctors with symptoms – diagnosed. Diamond opens the memoir with the claim that 'hypochondria has always seemed to [him] to be the only rational position to take on life,' but that respect evaporates upon confirmation of disease: cancer is serious; hypochondria a vestige of childish fears, trivializable and spectral.⁷⁴ My point here isn't to equate the two conditions in relation to an immediate threat of mortality but to query this habit of discounting the seriousness of hypochondria – both its content and the mental work it performs – in contrast to 'real' sickness. The belittling of hypochondria as *cowardly* is especially interesting, since Diamond later protests 'the whole battlefield vocabulary' that moralizes disease experience and acknowledges of his own situation that the 'slippery statistics' and 'unknowable' odds of cancer in effect *justify* the very hypochondria his title constructs as embarrassing and disgraceful.⁷⁵

In this post-COVID period, in the wake of catastrophic loss and the revelation of broad disparities in access to and quality of healthcare worldwide, that facile equation of hypochondria and cowardice, an irrational timidity (hardly limited to Diamond), strikes me as especially misguided. Surely we've learned that we are all at risk, now or eventually, in our exceedingly various states of being; that our bodyminds are vulnerable, despite our posturings of mastery and control; and that the social privileging of fitness, cognition and youth, as naturalized as it is, does more to coerce regulatory behaviours and increase suffering than to

produce nations of ‘well’ people. Hypochondriacs may be heeding a dictum to ‘know our bodies’ that comes *from* public health messaging and is uttered in complaints *against* ‘gaslighting’. But neither of these, I think, exactly encapsulates the knowledge of radical precarity that hypistemology cultivates, its manner of exploding ableist or ideological smugness even as it cries out for assistance in its faith that answers exist. There may be no way out of not really knowing what one craves to know. But hypochondria nonetheless credits the search.

OceanofPDF.com

4

Bodies of Matter: Hypochondria Consolidates

‘But for six straight hours I am someone else, calm, professional, in charge. I am well and whole.’

JENNIFER TRAIG, *Well Enough Alone*¹

In 1733 George Cheyne explained the title of his tome on melancholic disorders – *The English Malady: Or, a Treatise of Nervous Diseases of all Kinds* – as ‘a reproach universally thrown on this island by foreigners and all our neighbours on the continent, by whom Nervous Distemper, Spleen, Vapours and Lowness of spirits are in derision called the English Malady. And I wish there were not so good grounds for this reflexion!’² This defensive admission notwithstanding, the French appraisal of English hypochondria exceeded the terms of national rivalry. Eric Gidal explains in a trenchant essay that though French travellers in the eighteenth century ‘were by turns amused and appalled by the gloomy disposition of the island nation . . . they often viewed the English melancholy as inextricable from the very civic culture they so admired.’³ The perpetually discontent, prone to fretting over and examining *everything* (so Gallic thinking had it), were also temperamentally resistant to the seductions of tyranny; far from isolating the individual in a depressed remove from social engagement, melancholy restlessness fostered an independent frame of mind that in turn favoured constitutional government. From the humoral imbalance we’ve observed in English work of the previous century, melancholy became for French writers like Voltaire and Madame de Staël ‘both a symptom and cause of political freedom and national identity’ – a portrait of themselves

that, as it circulated among English readers, perhaps recalled for some the revolutionary spirit of their not-so-distant past and surely went a long way towards dispelling a prevailing notion of a country overwhelmed by morose indigestion.⁴

This abbreviated political picture helps us to understand how hypochondria transforms from a somewhat embarrassingly common malaise of Englishness to the intellectual ailment *de rigueur* in the British metropole by the time of Coleridge and the Shelleys – and later, in a less complimentary way, to one of the ‘Southern diseases’ that characterized the American South as the nation’s ‘colonial other’.⁵ It points to hypochondria’s role in the nationalizing of health discourse, a practice that has long participated in the creation and policing of borders both ideological and geographical. Even if England’s wet weather did have something to do with that country’s choleric populace, even if the psychology of hypochondria was ‘associated with the enervating climate’ of U.S. Southern states in the nineteenth century – the climatic theory of temperament traces back to Hippocrates – a different form of consolidation is at work in crafting national identity through the language of illness and distemper, however foregrounded the connection to intelligence and artistic sensitivity might in some cases have been.⁶ Illness, as I have been exploring throughout these chapters, always extends beyond the single person, no matter how private, solitary or lonely the experience of it is, and illness *fear* by definition establishes the self in relation to what is not-self, whether that otherness is an intrusion of pathogens or a people. This might mean, as it did for Plato (discussed in [Chapter One](#)), that the well-being of the polity demands an individual responsibility to keep *oneself* healthy. It might also mean that a nation attempts to protect itself from incursions of disease from elsewhere – the very threat that motivated Daniel Defoe’s *Journal of the Plague Year* (also in [Chapter One](#)): a retrospective account of plague intended to rouse readers to compliance with restricted trade regulations as a new disease ravaged parts of Europe. Fear of impending or imagined illness, then, can take part in solidifying both individual and group identities, precisely because getting sick is beheld as compromising our sovereignty.

In this chapter, I will approach hypochondria as an act or site of consolidation, recognizing both the problematic effects that it might have in

national, ethnic and racial contexts as well as the resistance it might represent to ableism's edict that sickness estranges us from our proper selves. It's precisely this conception of illness as *strange* that extends its reach away from medicine and towards a politicized rhetoric of otherness (as we will discover in relation to COVID). In some situations, hypochondria may function by projecting sickness on to the offending 'others' who must be feared for the contagion they represent; and in others by isolating and pathologizing an offending 'organ' so as to shore up a fragile sense of self. Hypochondria circulates in literary, medical or popular texts to impel – but also to interrogate – the character of a nation; defined as a personal problem, it penetrates to the core of subjectivity, raising questions of what and why we fear and what it means to the individual to be sick. Thus hypochondria may help to reconstitute us – even if around a presumably diseased body part or function – or fracture us further, revealing the rifts and fissures of our own egos or of the social fabric whose integrity we believe in – or are exhorted to accept. Hypochondria may serve exclusionary purposes, or it may push back on such practices, 'speak[ing] the unspeakable', as Anna Pehkoranta has suggested of hypochondria in Maxine Hong Kingston's memoir *The Woman Warrior*, to be discussed below.⁷

Steven Connor writes that the hypochondriac 'look[s] to the body for reassurance against the prospect that the self may be no more than . . . nothingness', and, in a devastating pun, declares that in fact, '*nothing* is wrong with hypochondriacs', just as 'they have been told all along'.⁸ If hypochondria can be said to instantiate some categorical insufficiency we fear in our beings, in a psychodynamic sense, it also performs the opposite, amalgamating a bodymind with lived, social and material reality. 'The hypochondriacal elaboration of physical symptoms', Don R. Lipsitt argues, 'represents a sacrifice of the part for the whole, a kind of offering to the gods for protection against destruction or annihilation of the entire body of self.'⁹ As we shall discover, that coalescence has varying social consequences, as singular hypochondriacal bodies negotiate friction with others and with national body politics. How do the questions of legitimacy, wholeness and knowledge posed by hypochondria become embroiled in broader dynamics of assimilation, for example? Or complex entanglements of ethnicity and mental illness? Conversely, how might the consolidating effect of hypochondria facilitate a crucial interrogation of the culturally

endorsed body – hale, fit, young – precisely in its insistence that something is unhealthy or awry? This post-pandemic era seems a prime cultural moment to reassess such dynamics, perhaps to reimagine what it means to deem ourselves – or anyone else – as insufficiently condensed.

THE NARRATOR OF Defoe's 1722 *Journal*, recounting his initial struggle to decide whether or not to flee London at the first signs of plague, admits that he could not be sure that his trust in God's protection was 'foolhardy' or not.¹⁰ Then he relates 'the mischievous consequences which attended the presumption of Turks and Mahometans in Asia' who would, 'presuming upon their professed predestinating notions . . . go unconcerned into infected places' and then die by the thousands, whereas 'Europeans or Christian merchants' kept to themselves and so 'escaped the contagion'.¹¹ This mini-morality tale explicitly aligns disease with religion and race, presenting it as a nearly inevitable consequence of risky behaviours that arise from subordinated bodies and their dubious beliefs. Bodily comportment, in turn, is associated with the borders not just of one's individual health but that of entire lands; the 'retired and reserved' aspect of European Christianity coalesces body, temperament, religion, race and nationality in a single, privileged and protected subject.¹² It seems obvious that such an anecdote insists to readers that by maintaining a stalwart defence against infection, they do their part in upholding the cultural values of 'the whole nation', and to be hypochondriacal in this circumstance is leveraged to foster rather than to undermine collective well-being.¹³

When real diseases are on the horizon, as for Defoe, metaphors of health and illness deployed in political discourse will take on a particular urgency. What George C. Grinnell calls, in the context of eighteenth-century British colonialism, a 'discourse of hypochondriacal nationalism' did not simply abstract the body in order to manifest psychological or political 'nervousness' about the consequences of an 'aggressive empire'; it gave a particular shape to the reality of contagion in an increasingly fraught and interconnected world. Commerce with other nations 'all but guaranteed the arrival of unwelcome disease', Grinnell says, and such risk provided a convenient, medicalized language with which to justify white paranoia not just about the fevers that might come from the West Indies or Africa but the

possibility of revolution, such as the one led by Toussaint L'Ouverture in San Domingo (modern day Haiti) in the late 1790s.¹⁴ The colonial enterprise depends upon racial others to establish and maintain the boundaries of its power, yet repeatedly, paradoxically exposes itself to infection. Geographical borders could prevent the transmission of neither people nor diseases, a fragile porousness that the hypochondriacal body at once epitomizes and warns us of, since – to quote Grinnell once again – ‘the hypochondriac lives the plague everyday’.¹⁵

These vectors of signification in which hypochondria structures national identity feature in such works as Mary Shelley's 1826 dystopia *The Last Man*, a novel sometimes read as indebted to Defoe's *Journal of the Plague Year* for the trenchancy of its portrayal of disease, yet with opposing philosophical intentions. The narrator is Lionel Verney, who begins his story as a lonely orphan and ends it as the last human being alive on earth, a sheepdog his only companion, in the wake of a global pandemic. (The novel is set in the late twenty-first century; reading *The Last Man* during COVID-19, as I did, was just slightly less distressing than *Plague Year*.) Between these solitary poles, and for most of the book, Lionel is ensconced in friendship, marriage, parenthood and politics. He befriends Adrian, son of the last king of England, who teaches him philanthropy and scholarship; he starts a family with Idris, Adrian's sister; he takes part in political intrigue on the Continent, fighting on behalf of the Greeks against the Turks alongside Lord Raymond (a stand-in figure for the poet Lord Byron); he endorses Adrian's republican ideals but is ultimately more motivated by familial commitments than ideological ones. Eventually plague sends the friends into exile in Italy and Switzerland, where all but Lionel die (whether from disease or other calamity). Lionel spends a year in Rome with no company apart from the dog and then resolves to travel through Africa and Asia, in search of human survivors.

What does it mean for a novel preoccupied with the viability of democratic principles and the damage wreaked by empire to stage the near extinction of humanity from disease? Some readers argue that *The Last Man* narrates the potential improvement of the *social* condition but insists on the essential isolation of the *individual*; had the plague not overwhelmed them, one by one, Lionel, Adrian and their band of forward-thinkers might have prevailed over more conservative, reactionary forces invested in

restoring the country to a rigid class hierarchy, even if each of them will inevitably die alone. This suggests that the ultimate arbiter is not human political systems or even philosophical ideals but nature itself; the loss of a political goal to a globally transmitted disease obviously suggests both an international problem of permeable borders (the vulnerability of any body politic to forces outside itself) and the human body's susceptibility to forces of biology. In this context, some scholars read *The Last Man* as an indictment of the very cautiousness that Shelley's first novel, *Frankenstein*, exhorted scientists and explorers *towards*; if Victor Frankenstein went too far in his zeal to overmaster nature, the cast of *The Last Man* aren't taking the dangers of illness, or the fragility of the human body, seriously enough.

But both novels finally tell a similar story about what happens when we disregard the hubris of control or indulge the folly of assuming that rhetoric and politics can conquer the facts of mortality. More than once, apparently progressive characters associate sickness with evil and with tyranny, exploiting the rousing discourse of a paradise restored to promise its eradication (along with poverty and servitude).¹⁶ But the arrogance of thinking that 'the little channel between our island and the rest of the earth was to preserve us' is soon dispelled by an illness that respects no boundaries of landmass, no vaunted privilege of religion, race or political leaning.¹⁷ Disease is soon everywhere; 'the vast cities of America, the fertile plains of Hindostan, the crowded abodes of the Chinese, are menaced with utter ruin'; and when Lionel 'spread[s] the whole earth out as a map before [him]', he sees that 'On no one spot of its surface could [he] put [his] finger and say, here is safety.' Belief, integrity, valour, 'determin[ation]', 'even good-humour and benevolence' – the qualities with which Adrian and his followers congratulate the English – these are meaningless against the 'enemy' of pestilence and contagion. 'There is no refuge on earth,' cries the anti-monarchical Ryland, Lord Protector of England, quaking with fear of the disease to which he is soon to succumb; 'Where can any of us go?'¹⁸

If Adrian epitomizes a certain forward-thinking political enterprise, he also embodies the malady that George Cheyne deemed quintessentially English. His intellect 'yield[s]' to the 'tyranny' of 'misery'; he is exhausted by writing romantic letters; he suffers an excess of sensibility; he has the habits of a 'recluse'. 'I am not a man fitted to govern nations,' he says, despite his pretensions to reformation; 'sufficient for me, if I keep in

wholesome rule the little kingdom of my own mortality.’ As a figure of fashionable hypochondriacal nervousness, Adrian cites ‘the delicacy of his health [as] a sufficient argument’ to ‘never . . . enter into public life’. This retreat, made possible by his privilege but which cannot forestall the infiltration of plague, suggests that capitulating to a rhetoric of *characterological* illness, an ‘English’ phenomenon, withdraws our awareness from the myriad depredations that affect real bodies in an increasingly ‘commercial’ environment: not just disease but poverty, hunger, gendered and ethnic violence, even shipwreck.¹⁹

The image of Lionel wandering the continents of Africa and Asia in search of humanity thus becomes highly ambivalent. Is he a British Adam, fated to repopulate the earth in his image, or a damned exile? Is he the figure of a renewed imperialism, spreading its ‘Homer and Shakespeare’ across the continents, along with its disease? Or does his solitary journey acknowledge that disease – or more fundamentally, embodiment in general, the fact of our humanity – must always level the ideological inflection of difference? Is his ‘tiny bark, freighted with [Lionel] Verney’, a mock Noah’s ark, or the ark of a new covenant, deliberately ironizing colonial naval movement around the globe? We navigate by our own lights, finally, and yet our individual perspectives are far from infallible. Can Lionel be sure, after all, that he *is* ‘the LAST MAN’? The novel is understood to be dystopian, but perhaps the final apocalyptic comment is not that Lionel is doomed to solitariness, but that he *presumes* to be the last man alive out of nationalistic arrogance.²⁰

A STORY CIRCULATES in the industrial management world that Franz Kafka, celebrated Czech author of ‘The Metamorphosis’, ‘The Burrow’ and the unfinished novel *The Trial*, invented the hard hat. From 1908 to 1922, Kafka handled insurance compensation claims for the Kingdom of Bohemia’s Workers’ Accident Insurance Institute, where he involved himself in efforts to improve workplace safety conditions. The hard hat anecdote, credited to the noted management theorist Peter Drucker, is almost certainly apocryphal, but it bears some relationship to Kafka’s obsessiveness about matters of the body and its vulnerabilities: not only on the job but simply by existing. For Kafka was a well-known (and amply

documented) hypochondriac. In a letter to his father from 1919 he reported that ‘there was nothing at all [he] was certain of’, that he ‘needed to be provided at every instant with a new confirmation of [his] existence’ – a feeling of immateriality that ‘open[ed the way] to every sort of hypochondria’.²¹ Ensuring that his own body was *there*, protecting it from harm, evaluating its relative functionality – such tendencies writ large became part and parcel of Kafka’s own on-the-job responsibilities.

For Kafka hypochondria served a project of maintaining bodily wholeness that had individual as well as corporate significance. He understood the body’s vulnerability to the inhumanity of certain forms of work, as well as the tendency in work environments to construe any complaint of illness as deliberate fakery. Thus Gregor Samsa, upon waking up as an insect in ‘The Metamorphosis’, seems initially less worried about his transformation than the fact that the ‘insurance doctor’ from his workplace (Gregor is a travelling salesman) ‘regarded all mankind as perfectly healthy malingerers’ – and indeed when the chief clerk arrives at the Samsa home to find out why Gregor isn’t in the office, he tells Gregor’s mother that ‘we men of business . . . have to ignore any slight indisposition, since business must be attended to.’²² Such scepticism about the truth-status of sickness is an inherent aspect of the experience of hypochondria; worrying persistently about what sensations could portend necessarily entails a concern about whether or not one’s account will be believed. In Gregor’s situation the drama of body integrity – the horrible scenario of a body behaving in unfathomable ways that defy rational diagnosis and that cannot be controlled – conflicts with commercial interests and the pressure to maintain the cohesion of the workplace. In the first pages of the story, Gregor’s failure to show up for work arouses more shock and outrage than the fact that he has turned into some sort of vermin.

The absurdity of that change – the alienation it represents, the debasement and isolation of modern life – also evokes a long-held association between hypochondriacal melancholy and hallucinatory identification with objects, animate or otherwise. We recall that Robert Burton’s *Anatomy of Melancholy* (discussed in [Chapter One](#)) describes a woman who believed she’d swallowed an eel and a man who thought he had frogs in his gut; Burton also references the so-called glass delusion, first recorded in the Middle Ages and surprisingly common in Europe

throughout the early modern period. By some accounts linked to ‘scholars’ melancholy’ as well as to the newfangled technology of transparent glass at the height of the Renaissance, the glass delusion was a psychiatric condition in which people believed that parts or all of their bodies were made of glass and so vulnerable to breakage.²³ Burton includes in ‘*Symptoms or Signs in the Mind*’ of melancholy that ‘Some are afraid that . . . they are all glass, and therefore they will suffer no man to come near them’, and again in ‘*Symptoms of Head-Melancholy*’ that some ‘think themselves glass . . . from the brain’s distempered heat’.²⁴ The theory that this phenomenon bespoke profound fears of emotional contact – that it was in effect a delusion of intimacy, even the sense that actual glass as it was increasingly used in domestic spaces exposed people to unaccustomed degrees of surveillance – reminds us that monitoring the self’s boundaries for evidence of having been breached by foreign entities is a characteristic of far less eccentric forms of nervous disorder. Gregor Samsa becomes a bug not simply because of hypochondria’s disordered apprehension of the body as alienated (as a dread disease expands to overwhelm the hypochondriac’s interoceptive sense of corporeality) but because the spaces *around* the body, the places we inhabit as interpersonal beings, can also feel dangerously inf(1)ected with the stuff of contagion – with ‘dirt’. Thus for Kafka, who described himself in one letter as ‘endlessly dirty’ and in another as ‘taciturn, unsociable, morose, selfish, a hypochondriac, and actually in poor health’, hypochondria manifested not simply as fear of illness but as a function of homeostatic management of the physical spaces he occupied.²⁵

In ‘The Burrow’, an unfinished story written just before his death, Kafka returns to the theme of interior space as simultaneously protective and precarious, at once an actual place of respite for one’s embodied self and a metaphor for the threats and pleasures of physicality. The story is narrated by an unspecified creature whose elaborate underground home safeguards it and yet is constantly vulnerable to attack; offers it warmth, food and comfort and yet cannot forestall the lurking suspicion that, unbeknownst to the creature, ‘the enemy may be burrowing his way slowly and stealthily straight toward me.’²⁶ That this predatory action of *burrowing* bears the same word as the creature’s laboriously constructed *burrow* tells us that danger is everywhere, all around as well as within us; the creature ‘can scarcely pass an hour in complete tranquility’ because its mode of escape

from an underground enemy is also an aboveground enemy's way in, so that it can never adequately barricade itself: the creature is, in effect, its own threat. This is why 'the burrow has so many unavoidable defects imposed by natural causes' – because our human bodyminds are vulnerable as if by design *and* because we exacerbate our suffering through our own 'laborious calculation': 'the sheer pleasure of the mind in its own keenness is often the sole reason why one keeps it up.'²⁷

When eventually a mysterious noise pervades the burrow ('louder' and 'nearer', 'everywhere and always at the same strength', 'dangerous beyond all one's powers of conception'), it's as if the hypochondriacal creature–narrator has just received an official diagnosis – which is just what happened to Kafka, who contracted tuberculosis when he was thirty – and so bemoans 'all the *petty* dangers' it has devoted so much time to 'brooding over' (my italics). Like Poe's tell-tale heart, this unceasing noise mirrors the narrator's own being – *is* the narrator, in fact; late in the story the creature says that he heard the noise long ago, when it seemed fainter and the creature was 'young', more inclined to be 'curious' and 'interested' than afraid. The point seems clear: it may be easier to disregard the body when we feel the fortitude of youth, but the body is there nonetheless – not just in a dangerous sense but ontologically *there*. Even a life spent devising 'labyrinthine' defences leads to 'maturity', the brute fact of our aging and a kind of stalemate: burrow all we like, we are embodied beings, and change and illness will come inexorably upon us, baring their 'claws and teeth'.²⁸

With its alarming, maddening sounds and a creature at once desperate for silence and unable to take its mind off the possibility of invasion, 'The Burrow' dramatizes Immanuel Kant's idea that hypochondria is like 'listening, in the quiet of the night, to a cricket chirping in the house, which disturbs our mental repose and so prevents us from sleeping'.²⁹ This is hypochondria as a form of sensory amplification, either a 'right reading of exaggerated bodily sensations' or a 'wrong reading of normal sensations'.³⁰ 'Human nature has the peculiar characteristic', Kant continues, 'that paying attention to certain *local impressions* makes us feel them more intensely or persistently.'³¹ Kafka's creature would presumably detect all sorts of underground sounds but latches on to the one that feels the most pressing, the direst. But 'The Burrow' also depicts hypochondria as a profoundly spatial experience. The tale is one of intense proprioceptive awareness, as

the creature presses itself into tunnels, firms the walls of hollows with its forehead, curls itself into a protective stillness. The burrow is itself remarkably corporeal, with its intestinal twists, orifices and vacuities; its stores of food; its rumblings and breaths. The creature recognizes that it cannot keep every part of that secure, aware that no body-burrow can ever be hermetically sealed against pathogen – that in fact absolute enclosure is beside the point, since the ‘beast’ is always just on the other side of a permeable membrane. Thus two things are true: all the while cognizant of the futility of its own vigilance – because ‘Anything might happen!’ – the hypochondriac’s watchfulness remains ‘unchanged’.³²

Gilles Deleuze writes of Melville’s *Bartleby* that the scrivener ‘can survive only by whirling in a suspense that keeps everyone at a distance’. *Bartleby*’s mind-bending assertions of negation – ‘I would prefer not to’ – produce ‘the vertiginous impression, each time, that everything is starting over again from zero’.³³ So too does Kafka’s creature keep *at it*, the burrowing and listening, even if these don’t lead to discovery. But while both of these characters’ actions might strike *us* as futile, the characters themselves are sustained by them, in fact constituted as themselves by them; the distress they incite is more significant for others – readers or other characters – than it is for the hypochondriacal figure itself. Even Gregor Samsa, in his newly ‘monstrous’ form, explores new vantage points and capabilities with ‘almost blissful absorption’.³⁴ If this is not quite *hope*, as Will Rees points out of ‘*The Metamorphosis*’, it does counter a more typical idea of hypochondria as relentless despair, and it suggests that we can reorganize ourselves differently after a profound, estranging experience – of fear, for example, or the fluctuations of illness or impairment. If we think of hypochondria solely as an act of ‘self-constitution that frames itself in relation to a prescribed totality of health’, as Sarah Mann-O’Donnell argues, then its self-scrutiny will only serve the norms of ableism. It is possible, however, that hypochondria is also a mode of being *through* the myriad realities of illness that refutes ‘a master-text of ideal health’ precisely because it demonstrates the inadequacies of that text, its gaps and inconsistencies.³⁵

This is how hypochondriacal references and figures serve to rupture continuities of identity: the bourgeois complacency of the Samsa family, the burrowing creature’s fantasies of security, the lawyer’s antic efforts to get

work done in ‘Bartleby’. To imagine that ‘health’ is the static quiescence from which we emerge as ourselves, illness as what erupts through the edges of consciousness to force us into contact with unfamiliar, offending body parts, is to perpetuate a dualistic fiction that separates *mind* from *body* and ignores the plethora of ways in which our attention is trained on our bodyminds even in the most ordinary of times. Robert Kellner opposes ‘unjustified conviction of bodily damage’ to ‘actual disease’, thus relegating a host of physical sensations to the realm of imitation; Kellner’s phrasing also entrenches a firm divide between bodily coherence and the disintegration of being marred: ‘damage’ connotes failure, mistake.³⁶ So much of hypochondria involves friction, betraying the fault lines that characterize embodiment, as between our desire for wholeness and the variable, fantastically complex beings that we are; or between us and everything else, from a world of risk to the practitioners who pit their impatience against our neediness. But as no form of life is seamless and uninterrupted, we can think of hypochondria as simply attending to the borderlands. We might put it this way, that hypochondriacal awareness brings our *contiguity* into being, the reality of self as an amalgamation rather than an undifferentiated continuity.

KAFKA’S BRAND OF hypochondriacal consolidation, with its anxious relationship to bodily boundaries – maintaining them, worrying their fragility, revelling in transgression – in conjunction with his emphasis on domestic interiors, recalls the condition’s anatomical, and inconsistently gendered, past. Long associated with the gut, sometimes styled as the male version of women’s hysterical wombs but not inevitably so, hypochondria takes on distinctly gendered as well as geographical and racial meanings at different moments in time. An examination of a late eighteenth-century medical compendium called *Reflexiones Historico Fisico Naturales Medico Quirurgicas*, written by the Spanish doctor Francisco Barrera y Domingo and published in Havana, Cuba in 1798, reveals an unusual category of hypochondriac defined specifically as a nervous, melancholic somatization of ‘unfulfilled dreams’ on the part of Catalanian men. As explained by José María Aguilera-Manzano, ‘Catalonian hypochondria’ primarily afflicted sea captains and merchants ‘prone to fits of uncontrollable fantasy’, including

the fabrication of ‘whole new syndromes’ or the amplification of minor discomforts into fatal though imaginary disease. For Barrera, ‘chronic hypochondria’ was the ‘steep price’ paid by ambitious men whose energies were not always fulfilled, either professionally or romantically, sexually. The resulting anxiety – in effect an inward-turning frustration over competition for various forms of commodity – took shape in a familiar litany of hypochondriacal complaints: digestive distress, flatulence, depression, chest pain.³⁷

An intriguing inversion of Catalanian hypochondria was a syndrome affecting young men that Barrera referred to as Creole dyspepsia: a ‘painful fatigue that the Creoles from Havana suffer as a result of the influence of relaxed lifestyles, libertinage, bad education, premature sensual appetites, obscene legends and songs’.³⁸ Here the ‘idleness’ and ‘indolence’ of hypochondria bear some relation to British iterations of a kind of melancholic laziness (we recall James Boswell exhorting himself to energetic bursts of activity to stave off his own hypochondriacal tendencies) but situated in the specific context of multicultural Havana, whose population in the 1790s would have included indigenous people, immigrant Spaniards and African slaves. The dyspepsia of ‘white’ Creole boys, so the argument went, arose from a combination of disdain for manual labour encouraged by the presence of slave labour, ‘frequent . . . exchanges with female slaves’, and a general atmosphere of lax ethical and moral guidance.³⁹ The ‘immature masculine freedoms’ of Creole dyspepsia were a kind of corollary to Catalanian hypochondria’s excessive male industry – but these were linked for Barrera as ‘pathological manifestation[s] of a troubled masculinity’.⁴⁰

Aguilera-Manzano suggests that part of what vexed masculinity in colonial Cuba were the ambiguous categories of race and nation in an environment of fluctuating demographics and shifting meanings of ‘whiteness’ or indigeneity. The influx of ‘energetic Spaniards’ alongside the undisguised reality of plantation conditions set bodies’ differences in high relief even as their boundaries were constantly flouted and ignored.⁴¹ Issues of self-image, social validation and expectations related to work thus primed certain constituencies to anxiety about bodily wholeness – anxieties that continue to resonate in a more contemporary American context, where fraught questions of ‘legitimacy and authenticity’, in Anne Anlin Cheng’s

words, link the ‘assimilating racial-ethnic body’ to that of the hypochondriac nervously assessing the relationship between ‘actual’ and imagined pathology.⁴² Hypochondria’s historical connection to melancholy, defined in the more psychoanalytical sense of an incomplete and self-directed process of mourning, takes on particular valence in relation to the losses of immigration or religious and ethnic difference. In *The Melancholy of Race*, Cheng argues that hypochondria is ‘a form of melancholic self-allergy’ that manifests the trauma of an inherently incompletionable assimilation.⁴³ What Cheng calls ‘the racial-ethnic hypochondriac’ becomes embodied as a process of internalizing racial otherness, experienced in much the way the hypochondriac senses that something is wrong in the bodymind, particularly as judged against consensus about what constitutes health. Acutely aware of one’s embodied self in relation to the dominant norm of American whiteness – as well as the open secret of its violence, ‘the haunting presence’ of victims of its imperial past – the ethnoracial subject becomes ‘hypochondriacally aware of and allergic to the abjection lodged within’.⁴⁴

As I have been suggesting throughout this study, the hypochondriacal body proves the instability of narratives of corporeal self-sufficiency and integrity, which means that it can paradoxically buttress the self against the specific kinds of threats inherent in discourses of racial or ethnic superiority. A kind of double-hypochondria activates, as the racial-ethnic hypochondriacal figure disrupts the very hypochondriacal narrative of ‘natural’ national boundaries, characterized by fear of invasion, that it has been deployed to concretize. Monika Hogan writes of Philip Roth’s hypochondriacal Zuckerman novels, for instance, that ‘Zuckerman’s mysterious illness . . . point[s] out the inaccessibility of American fictions of bodily “wholeness”,’ and ‘expose[s] the “American dream” as being reliant on a fantasy notion of bodily purity and wholeness’. The character of Zuckerman may be organized around a body ‘signifie[d]’ by illness and death, but it is ‘the fiction of American “whiteness”’ that ultimately implodes.⁴⁵ In a book like Defoe’s *Journal*, motivated to incite illness anxiety in order to protect national borders, vulnerability to disease becomes corporeal proof of cultural superiority even as it registers the sense of threat that might arrive with ethnic, racial and religious others from across the ocean (a point to which I will return below in the context of

COVID); it makes sense that the racial–ethnic subject, inheriting the hostility and antagonism of this brand of hypochondria, might experience embodiment as fraught with a kind of helpless treachery. But as we will discover through a text like Maxine Hong Kingston’s lyrical memoir *The Woman Warrior* (1975), hypochondria can enact a different drama, too, helping to galvanize acts of resistance to the strains of difference, even if sickness is inherently modulated by feelings of guilt.

The Woman Warrior exemplifies the vanguard of second-wave feminist life writing in the 1970s that overtly breached conventions long established in Western autobiography by men of letters, state and/or artistic renown. In particular, such work restores the body to subjectivity, frequently orientating the story of self around family, friendship and domestic concerns and speaking explicitly about life experience in relation to gender, race, class, sexuality and impairment. An important feature of many such memoirs is the rupturing of strictures of silence and politeness. *Woman Warrior*, for example, famously begins by breaking a promise never to reveal the story of American-born Kingston’s aunt, who, back in China, gave birth to an illegitimate child and then drowned herself and the newborn in the family well. The memoir’s first words – “‘You must not tell anyone,’ my mother said, ‘what I am about to tell you’” – at once claim the bravery of the radical act of revelation and indicate its potentially terrible cost to the narrator’s relationship with her mother, and even with words. The text thus opens with a declaration of the power *and* the dangers of speaking, which is subsequently aligned with complex tensions of family, female sexuality, ethnoracial difference and illness. For example, the narrator explains early on that making herself ‘American-feminine’ involved ‘speaking in an inaudible voice’ to accommodate ‘American tones’, even though ‘Chinese communication was loud’ and ‘only sick people had to whisper’. To ‘be’ American, then, is to be marked as ill, while ‘try[ing] to understand what things in you are Chinese’ involves the very self-assessment of hypochondria, its constant scrutiny – and the ambiguity of whether or not some feature of embodiment should be deemed unwell.⁴⁶

Choosing memoir, as the genre of personal identity, Kingston politicizes her ‘bad, small-person’s voice that makes no impact’ in direct confrontation with the ‘Chinese word for the female *I* – which is “slave.” Break the women with their own tongues!’⁴⁷ Her efforts to understand and define

herself within competing American and Chinese cultural expectations are consistently depicted in terms of bodily extremity, in scenes that cannot be fathomed apart from the memoir's emphasis on speech acts. There is the keeping and spilling of secrets, of course, and the deliberate obfuscation between myth and memory, fact and fiction, to mimic the immigrant's fear of being 'found out' and deported. There is the meaning of 'tongue' as both a language and a body part as well as the narrator's fraught relationship with a mother whose own deployment of story is at once a source of empowerment and a bewildering onslaught of admonishment and demand. To claim both a self and the control of words redeems the loss of No Name aunt as well as Kingston's own younger self, whom the narrator describes as having 'a terrible time talking'. Her 'broken voice come[s] skittering out into the open', 'a crippled animal running on broken legs', filling her with 'disgust' and 'shame'. To hold to silence, then, is both a refusal to communicate in a language of oppression and to succumb to that same pressure: silence has 'to do with being a Chinese girl' in a troubled liminal space. This is why Kingston's mouth becomes 'permanently crooked' with the effort of trying to accommodate the 'guttural peasant noise' of 'Chinese sounds' to 'American ears', to turn herself into a version of 'American-feminine' by 'whisper[ing] even more softly than the Americans'. 'At times', the adult narrator says, 'shaking my head no is more self-assertion than I can manage.'⁴⁸

And this is also why in the sixth grade, 'arrogant with talk', young Kingston unleashes her rage on the body of a girl in class who 'would not talk' at all. In the scene – which extends for several pages – she corners the girl in the school bathroom and tortures her to make her speak, pulling her hair and pinching her 'meat'-like cheeks:

'Talk', I said. 'Are you going to talk?' . . . 'You do have a tongue', I said. 'So use it' . . . 'O.K O.K. Don't talk. Just scream, and I'll let you go' . . . 'Talk! Talk! Talk!' . . . 'Why won't you talk?' I started to cry . . . 'Don't you dare tell anyone I've been bad to you. Talk. Please talk'. Her sobs and my sobs were bouncing wildly off the tile . . . 'Look. I'll give you something if you talk'.⁴⁹

The emotional swings of this encounter, from belligerence through guilt to pleading and cajoling, mimic and distort the narrator's personal experience of racist torment. Projected on to the mirror image of another girl ('in other ways we were the same') is the narrator's disgust for her own body. She taunts because she has been taunted; she is desperate to interrupt silence as she has longed to be able to speak; silence is at once a scandalous defiance of and a grotesque surrender to the coerciveness of 'tongue'. The scene exaggerates shame but also, I think, suggests a longing for symmetry, bodily continuity, not just with white America's power to compel a certain kind of utterance but with another Chinese American girl whose mouth might – but doesn't – form itself around the shape of communication.

Immediately following this scene, Kingston writes that she spent eighteen months in a 'rented hospital bed in the living room' like a 'Victorian reclus[e]', suffering a 'mysterious illness' with 'no pain and no symptoms', watching TV and using a bedpan.⁵⁰ She presents this as cosmic justice for her misbehaviour, but I disagree that the illness episode represents pure somatizing, arising *only* 'from a deep sense of guilt', as Pehkoranta puts it.⁵¹ Kingston's body is emphatically *there* – taking up space in the middle of the family home – but it also disappears in a sensory state that seems out of sync with what happens to it, a simultaneous loss and density of embodiment that coheres around an unnameable sickness. If this episode was in fact a reaction to what Kingston did in the school bathroom, it reads more as a retreat into family than a 'hysterical' manifestation of unspeakable emotion. Most intriguingly, the family seems unperturbed by the disruption; in the telling of it, at least, Kingston's 'illness' is simply absorbed into the household's rhythms. Does this mean that the core of the family has been infected by the sickness of Americanness, by the failure of any Chinese American girl's attempts to manoeuvre the requirements of assimilation? Or is it also that the family will always carry on caring for itself, allowing the unreadable body to demand what it needs, adjusting to the altered routines of illnesses no one can diagnose?

Kingston writes of this period that it was 'the best year and a half of [her] life': hardly a punishment (indeed 'pure rapture', as Cheng puts it) – more a relief from the intolerable pressures of trying to belong.⁵² To the extent that she exhibits the characteristics of nineteenth-century neurasthenia, often understood as a way of managing untenable demands of

patriarchal society, what Kingston's asymptomatic body announces is less guilt for cruelty and more a dramatic refusal to accept the dictum that 'if you don't talk, you can't have a personality', to abide by the rules of the world beyond her family of origin. From a desperate sense of fracture and dismantling, young Maxine puts herself back together through a hypochondriacal sickness that her family unquestioningly nurtures. *Something* is happening, but in an environment whose dominant edict is 'Don't tell. "Never tell"' – where telling the aunt's story, bringing her identity and her sexual body out of shame is also 'telling *on*' her (my italics) – no one, including the body itself, can or will say what.⁵³

The 'mysterious illness' endures in Kingston's story as an aspect of her character that, like her tangled hair, 'dirty hands' and general 'mess[iness]', contributes to a worry that she might be the 'crazy girl' of her household. The narrator reports that she 'did not want to be our crazy one', yet she 'was getting stranger every day'. She 'affect[s] a limp'. And she wonders if 'the mysterious disease [she] had had might have been dormant and contagious'.⁵⁴ Two things seem important here: one is the ongoing concern that some undefinable part of herself deep within is hateful, even reprehensible, but the other is that all of this corporeal 'strangeness' is deployed to assert herself in opposition to her parents, and what Kingston perceives to be an impetus to behave properly, to attract a husband. In effect anomalousness is *amplified* via these bodily symptoms that do not accord with traditions in either of Kingston's cultural milieus, yet in so doing actualize an identity that strives to define itself *for* itself. We should remember too that the potentially 'dormant' illness once served a protective function, enacted a challenge – just as headaches and pneumonia manifest the adult Kingston's efforts to manage the trauma of her childhood by pulling her away from her mother. 'I've found some places in this country that are ghost-free,' she tells Brave Orchid. 'And I think I belong there, where I don't catch colds.'⁵⁵

Brave Orchid had been a doctor in China who 'could cure the most spectacular diseases'. Kingston describes her exorcizing a fearsome ghost from her medical school dormitory as if it were 'disease', 'bacteria', and attending to her sister Moon Orchid's fear as if it were "'an illness'". She might '[pry] open [Kingston's] head and [her] fists and cra[m] into them responsibility for time, responsibility for intervening oceans'.⁵⁶ But she

does not solve the problem of her daughter's unexplained illness. It endures – something in Kingston that arises from the girl herself, a quality or a phenomenon that needs no fixing, that consolidates her, establishes her difference, her assertiveness, her corporeal persistence. It is allowed to speak for itself.

CONTRARY TO HIS countryman Cheyne's assertion of a uniquely English malady, James Boswell wrote in *The Hypochondriak* in November 1777 that '*Hypochondria . . . is not peculiar to Britain. We may trace it in all countries in one shape or other.*'⁵⁷ What might Boswell have thought, not quite 250 years later, when U.S. President Donald J. Trump referred to COVID-19, the official WHO designation for the new disease, as 'the Chinese virus'? A year after Trump's offending tweet, a University of California San Francisco study found that anti-Asian sentiment increased 'precipitously' on Twitter in the week following 16 March 2020, with the hashtag #chinesevirus more likely to appear with 'overtly racist hashtags' than #covid19.⁵⁸ In blatant disregard for the science of viral development and transmission, the study reports, Trump defended the phrase as an 'accurate' rebuttal to purported Chinese claims that the coronavirus had been introduced to Wuhan by the U.S. military (then-Secretary of State Mike Pompeo referred to COVID as the 'Wuhan virus' no less than six times in a single news conference on 17 March 2020). Such an explanation, specious as it will sound, obviously doesn't account for 'kung-flu', another racist epithet that began to circulate, even among White House administrators and Congressional leaders, in the first weeks of the pandemic.⁵⁹ And Trump made the nationalist purpose of such language explicit, declaring that 'We will be stronger than ever before!' Meanwhile the Associated Press reported just days after the initial presidential tweet that 'virus-related discrimination' and retaliation against Asian Americans were on a sharp increase.⁶⁰ Illness becomes *synonymous* with ethnic identity in these slurs, displacing sensible recognition of one with insidious alarm at the other.

Nor do the American circumstances fomenting such terminology – and the feelings of hate and resentment these terms encourage – encapsulate the extent of anti-migrant sentiment that intensified, as in Italy, through the summer of 2020. As reported by Human Rights Watch in July of that year,

for example, migrants testing positive for COVID who landed in Calabria were resoundingly abjured by several municipal officials, including ‘the central government in Rome [which] did not challenge the views of those who depicted people arriving by sea as spreaders of the disease’.⁶¹ Such responses to disease in relation to racial and national difference – official statements that stoke not legitimate caution about a spreading virus but rather unfounded fears of other people rooted in bias – recall Defoe’s ‘Europeans [and] Christian merchants’ who held off plague to the extent that they held away from Turks and Muslims. In this way illness fears are heightened in direct correlation with the hierarchizing of identity positions as well as a sense of vulnerability to geographic exposure, as if the racialized rhetoric of distinction could counteract the absolute penetrability of oceans and air. In effect the meaning of a disease corresponds to its social and cultural emplacement and the narratives that circulate to define, characterize and contain it, and these in turn will inevitably take on the prejudicial qualities of the historical moments in which they arise. To be afraid of a disease is always to fear it in a certain social milieu.⁶²

Grinnell suggests in *The Age of Hypochondria*, published many years before COVID, that ‘the circumstances of a pandemic render everyone into hypochondriacs who search themselves and others for signs of deterioration.’⁶³ Much of the psychological research on hypochondria emphasizes either archaic events – childhood illness or imitative behaviour – or neurochemical/structural causes, linking the condition to others such as functional neurological disorder (FND) or obsessive-compulsive disorder (OCD).⁶⁴ But another, perhaps more straightforward, way to grasp its currency is simply that as long as there is disease in the surround, some will worry inordinately about their susceptibility to it. As I said at the outset of this project, the threshold of ‘excessive’ worry becomes especially hard to determine with a novel contagion in the atmosphere, when ‘worst-case scenarios’ are unfolding all around us, and public health messaging *prompts* us to be alert for symptoms. At the same time, of course, anxiety itself can produce sensations both physical and psychological that mimic, and so can be interpreted as, the symptoms of organic illness (as Freud writes in *On Narcissism*, ‘hypochondria must be right: organic changes must be supposed to be present’), thus creating a kind of feedback loop in which harmless bodily activity takes on sinister import.⁶⁵ The explosion of COVID

cases worldwide in the early stages of the pandemic, combined with doubt about the accuracy of tests and the ambiguity of both symptoms and outcomes, made for a hypochondriacal perfect storm. But the phenomenon is not simply individualized and internal. I would argue that the encouragement of prudence about COVID, priming us to be aware of our traversable corporeal boundaries, though resisted by some in a *medical* sense, was simultaneously exploited in a *political* context to stoke racist and xenophobic bias. Hypochondria in our own pandemic is thus liable to a quasi-eugenic rhetoric of insularity such as that which provoked Defoe, in the *Journal of the Plague Year*, to advocate for the closing of his nation's borders.

Here I want to return briefly to Shelley as a possible counterpoint to the rigidity of disease discourse that pits a normative, immunized – that is, purified – community against threats of otherness, both racial and biological. Many scholars point to a scene late in *The Last Man* as evidence of the novel's 'embrace of alterity', in Fuson Wang's words, but Wang argues further that Shelley, well versed in the contemporary science of inoculation, also mounts a cunning argument in favour of 'medical experiment and cultural exchange'.⁶⁶ In the scene, Lionel Verney describes how 'with mixed horror and impatience [he] strove to disengage' himself from a 'negro half clad' who 'wound his naked festering arms round [Verney], his face close'. The man's breath, Verney goes on,

death-laden, entered my vitals. For a moment I was overcome, my head was bowed by aching nausea; till, reflection returning, I sprung up, threw the wretch from me, and darting up the staircase, entered the chamber usually inhabited by my family.⁶⁷

As Grinnell notes, for many readers this scene links disease 'with a foreign and abject body' by way of insisting on the detriments of colonial intrusion.⁶⁸ For Wang the encounter works in the opposite way (if to similar ends), not only enacting Shelley's commitment to interracial and cultural contact but representing the history of vaccination, which begins with Edward Jenner's 1790s experimentation with cowpox virus – whereby immunity to smallpox was produced by a cross-species process of

incorporation. Jenner's innovation was to use milder virus particulate from cows to vaccinate (from *vacca*, 'cow') rather than live smallpox virus from humans (a procedure called 'variolation').⁶⁹ As Verney takes in the breath of an infected other he is inoculated, and he survives.

What follows this remarkable inhalation, though, is that rather than finding some way of conversing with alterity for the 'regenerative potential' of collaboration, Verney repudiates the possibility of what Wang calls 'cosmopolitan community', pushing 'the wretch' away from him and struggling to 'disengage' from his clasp.⁷⁰ What is then apocalyptic about *The Last Man* is not that it dramatizes the risks or the inadequacy of vaccination but rather that its world's population fails to invite a robust intercultural, interspecies form of 'experimental enthusiasm' (we remember Verney's final fellow traveller is a lonely dog – a companion, but not one who helps Verney to repopulate earth). Describing Romantic medicine as a discipline of 'radical inclusiveness', Wang suggests that Shelley's text is best read as an encouragement towards experimentation – even the 'monstrous' forms of hybridity that might be enacted by vaccines, human–canine friendships or interracial embrace.⁷¹ To repel such opportunities for growth and knowledge is to annihilate humanity.

It's worth noting the nearly instant appearance of physical symptoms as Verney feels the touch of the dying man – the swooning nausea as well as the quick retreat away from the crowd and back to his family's private quarters. Verney reacts, that is, *as if* he has plague, though he doesn't; his body manifests symptoms out of fear, not contagion; contagion is feasible, of course, but gets attached in this scene to the figure of racial difference and to an exaggerated idea about the speed and mechanics of transmission. When Verney tells his wife, Idris, 'It is true that I am sick,' that he is 'changed', what should be definitive – plague or not plague – becomes suddenly ambiguous, unreliable; *something* has happened to him, but is it infection, inoculation, internalization or rejection of difference?⁷² What of the word, the *concept*, of 'sick'? Verney's hypochondriacal experience, as with many such brief encounters during COVID, destabilizes any confidence in 'English health' in opposition to foreign disease, according to Grinnell, just as it simultaneously exposes and denies our fundamental perviousness.⁷³ Shelley thus stages this interchange as one of failed opportunity in which survival requires an exploitative 'inhalation' of the

other that subordinates the body of the ‘negro half clad’ to the presumed betterment of whiteness. The ‘alterity’ by which Verney is clutched is a dying one; Verney survives the plague at the expense of the other, but – and – his future is bleak in the extreme. The better outcome of hypochondria, *The Last Man* implies, is not to guard national or racial identity at any cost, but to hold fast to the quest for understanding that it inevitably sets in motion.

THE SOCIOLOGIST David Mechanic wrote in 1968 that people are ‘less likely to seek care’ when ‘a symptom is familiar’. It is the ‘unusual, strange, threatening [or] unpredictable’ sensations that send us to the clinic, A&E, the doctor’s office – perhaps in part, as many experts on hypochondria have suggested, to establish the kind of nurturing familiarity that the offending symptom seems to violate.⁷⁴ This commonsense depiction of hypochondriacal behaviour reminds us that categories of wellness, fitness, health and ability are formulated in relation to what Michel Foucault refers to as ‘irregularity’, often in contexts that have nothing whatsoever to do with disease.⁷⁵ The hale and healthy body, secure in its boundaries, championed as autonomous and self-sufficient, is the normative body of ableism – or, to state this somewhat differently, if ‘the putatively disembodied mind’ is ‘the irreducible marker of the sovereign subject’, as Margrit Shildrik puts it, then the acceptable body is one that recedes out of view, ‘relatively unimportant, so long as it falls within normative parameters that pose no hindrance to the implementation of self-determination’.⁷⁶ To bring the body emphatically into conversation without at least the redemptive justification of ‘real’ disease is to fracture the happy fantasy that we can will our fleshly selves into submission. Hypochondriacs are surely so reviled in part because they bring to the surface the very strangeness of ourselves that our cultural habits and narratives are organized to deny. To reprise a comment once made by the philosopher Susan Wendell about chronic fatigue syndrome, if we tell people that something feels different, or hurts, or alarms our awareness, then we remind them that such catastrophes do occur.

But when we take seriously our existence as both relational and embodied beings, the intensely psychosomatic quality of hypochondria

appears somewhat less pathological than originary. This is how D. W. Winnicott describes ‘*psychosomatic illness*’ in ‘Mind and Its Relation to the Psyche-Soma’: as ‘draw[ing] the psyche from the mind to the original intimate association with the soma’ in a process that has ‘*positive value*’ precisely because the bodily ‘disturbance . . . counteract[s] a “seduction” of the psyche into the mind’ (italics in original).⁷⁷ Where elsewhere Winnicott accounts for hypochondriacal anxiety as a problematic rechannelling of inexpressible feeling into body states that consume focus, here he presents a phenomenon of organicism and repair that, even in appealing to another for reassurance, facilitates important forms of reconnection. From this perspective, the intense fear of hypochondria might be managed not by minimizing it, along with the very presence of body parts that suddenly emerge into consciousness, but rather by encouraging a kind of literal incorporation of hypochondriacal sensations: that is, by encouraging *psyche* to recognize and tolerate *soma*. The point would be to address fear through reintegration rather than dismissing as fantastical the claims the body rightly makes, in health as in illness, on our existence.

Considering hypochondria as a protracted engagement with unfamiliarity invokes the many troubled tactics humanity has deployed to cope with life’s chaotic nature. We seek to stabilize, to categorize, to secure distinctions, and our efforts to manage the profusions of existence tend to the violent and suppressive, even against ourselves. Typical strategies for dealing with hypochondria impose an adversarial quality on embodiment; it might not be that something is *medically* wrong (at least not currently diagnosable), but the system is nonetheless dysfunctional – whether in mind, emotion, interpretation, even family or relationships. We recall hypochondria’s mind–body variousness, as detailed so thoroughly in Burton’s *Anatomy*: that hypochondria was once a condition defined by a plethora of psychosomatic symptoms, including shortness of breath, gastrointestinal distress, fatigue, headache and back pain and anxiety, of which preoccupation with or fear of illness was only one part. Today many conditions of this sort with unknown etiology have been recognized as discrete syndromes with more or less accepted treatment protocols. Chronic pain, chronic fatigue syndrome (myalgic encephalomyelitis), irritable bowel syndrome – these have been articulated as discrete if vague maladies and are treated, and respected, as such. It’s intriguing to consider what would change in our apprehension of how our bodyminds function if we returned

to a rubric of ‘hypochondriasis’ that might coalesce a range of phenomena, if we once again beheld hypochondria as itself the organizing syndrome reuniting mind and body and thereby signalling the still-mysterious ways in which our corporeal selves behave and interact. What if the answer to the question of hypochondria were not repeatedly ‘You do *not* have . . .’ but rather ‘You *are*’: normally embodied, understandably afraid, a complete unit of parts whose communication is often frustratingly obscure.

For we contain worlds within us, as the poet John Donne was famous for saying, which sometimes inspired his exploratory impulses and sometimes filled him with terror; for Donne the newfound availability of anatomical texts and such momentous epistemological shifts as William Harvey’s discovery of the circulation of blood displayed the workings of the human subject in bold new ways, both stoking a fascination for his own complexity and revealing just how out of our conscious control the microcosm truly is. A comic approach to this hyperacute sense of how much is ‘in’ us – how much we *are*, so to speak, and so how much eludes our ken – appears in Jerome K. Jerome’s 1889 novel *Three Men in a Boat (To Say Nothing of the Dog)*, loosely based on a real-life boating excursion on the Thames that Jerome took with friends George Wingrave and Carl Hentschel. At the start of the tale the narrator J. – who has a ‘liver that was out of order, because [he] had just been reading a patent liver-pill circular, in which were detailed the various symptoms by which a man could tell when his liver was out of order’ – recalls a day spent reading a medical text at the British Library. As he encounters the symptoms of typhoid fever, St Vitus’s Dance, Bright’s disease, cholera and more, he immediately has them all – indeed realizes he *has* had them, was even *born* with them. ‘The only malady’ he hasn’t got, he says, is ‘housemaid’s knee’:

I thought what an interesting case I must be from a medical point of view, what an acquisition I should be to a class! Students would have no need to ‘walk the hospitals’, if they had me. *I was a hospital in myself*. All they need do would be to walk round me, and, after that, take their diploma.⁷⁸

The point is gullibility, and the hypochondriac’s faulty interpretative practices (or the inaccurate correlation that’s made between direct bodily

sensation and abstracted information). But Jerome also captures hypochondria's unique sense of completeness, its unexpected self-containment. Perhaps this is the true antidote to ableism's fierce denial of contingency, even to the fictions of wholeness and unity: that we hold within ourselves all the possible ailments that could befall us, from asthma to 'zymosis', and live to tell the joke.

OceanofPDF.com

Illness on the Doorstep: Hypochondria Precedes

‘Still, you must agree to this, that doctors know more than others.’

MOLIÈRE, *The Imaginary Invalid*¹

The tension between scorning, pitying and doubting hypochondriacs is thickly embedded in the history of health anxiety. Dismissive mockery was being levelled against sufferers of ‘spleen and vapours’ at least by the end of the seventeenth century, when essayist Jeremy Collier wrote in his 1695 *Miscellanies upon Moral Subjects* that ‘spleen’ is ‘a handsom Cover for many Imperfections’, establishing the link between melancholy and falsehood.² In the late eighteenth century, Immanuel Kant would compound that insult in *Anthropology from a Pragmatic Point of View*, stating that ‘the hypochondriac is a crank’ who ‘refuses to be talked out of his imaginings’ and must be ‘treat[ed] . . . like a child’ – an infantilizing habit that runs through Susan Baur’s 1988 *Hypochondria: Woeful Imaginings*, as when she claims that ‘the less secure a person is, the more he tends to rely’ on ‘substituting body language for direct verbal communication’, just as ‘a very young child often cannot say precisely what is wrong’.³ Robert Hutchison in 1934 also referred to the hypochondriacal ‘crank’, one who follows health fads to a fault, revealing a different aspect of what makes the figure so loathsome: that ‘almost every one of these “cranks” can claim some medical authority for what he does’, suggesting that a little knowledge is a dangerous thing.⁴ Current therapeutic approaches have obviously dispensed with handling the hypochondriac as ‘an immature,

demanding child’, as Beverly Mead confidently prescribed in 1965, but the credibility dispute remains: patients are ridiculed for what they think they know and stigmatized for what they don’t, while the array of medical, psychiatric and psychological authorities carry on debating how hypochondria presents, where it comes from and whose problem it is to resolve.⁵

It should be obvious that the quality of stubbornness imputed to hypochondria is not the only factor in power struggles that arise between practitioners and patients. In this chapter I want to explore the believability conundrum as it attaches to people who have historically been too easily dismissed in clinical encounters: women, the working class, the elderly, the disabled and BIPOC and LGBTQ+ communities, who are more likely than those in dominant social groups ‘to be perceived by healthcare providers as malingering’.⁶ Meghan O’Rourke argues in her study of chronic illness that ‘if a female patient fails to get better, the problem lies with *her* . . . Indeed, the notion that the female body is often expressing psychological distress still powerfully animates medical thinking.’ There is even an acronym for this phenomenon: SDBLVP, for ‘somatization disorder besets ladies and vexes physicians’.⁷ Arthur Kleinman makes a similar point about the treatment of Black patients in the United States, where individuals may be ‘reduced’ to their conditions and blamed for ‘noncompliance’ rather than supported or adequately treated. Too often, Kleinman contends, diagnoses amount to ‘a systematic distortion’ of what is actually communicated in a clinical interview, as ‘social sources’, ‘life problems’ and ‘cultural issues are allowed to slip by . . . in a way that would be regarded as sheer clinical incompetence if the issues were biological.’⁸ In still a different context, a discussion of cultural metanarratives of OCD, Angela J. Kim sums up the situation succinctly: ‘I inevitably fear that I will not be taken seriously.’⁹

What such remarks tell us is that ‘hypochondria’ can mask prejudice and be used as a potent tool to deflect from what medical providers either don’t understand or aren’t inclined to spend time navigating. Maya Dusenberry cites a 1965 editorial by British psychiatrist Eliot Slater claiming that diagnoses such as hysteria and hypochondria are ‘a disguise for ignorance and a fertile source of clinical error’. And the stigma of hypochondria can be a powerful obstacle to seeking medical care when it might in fact be clinically warranted; Dusenberry writes of women, for example, that

delaying visits to health professionals is often caused by ‘fear of being [deemed] a hypochondriac’, calling this situation a ‘double bind’: ‘either you’re hysterical or there’s nothing wrong.’¹⁰ In these circumstances the accusation of hypochondria serves as a professional gatekeeper, whether because patients are turned away too soon or without necessary interventions, or because the shame associated with the condition promotes self-policing. What *is* hypochondria, then, if it can be at once trivialized as a manifestation of childish fantasy *and* utilized as a powerfully suppressive tool of social ordering? And what is the antidote – not for disease, but for patients distrusting their own bodily signals, what they know of how something ought to or typically feels? Or for providers discounting the legitimacy of a complaint even before a person has crossed the threshold of clinical space?

We are returned here to the historical ‘omniscience of medicine’, as Annemarie Jutel names it, and the fact that such diagnoses as ‘medically unexplained symptoms’ (at best) and hypochondria or hysteria (at worst) assume the physician’s ‘infallibility’.¹¹ The physician serves as the arbiter not just of disease but of reality itself when a designation of what cannot be *medically* explained (at least not now) suffices to annul a person’s experience of pain or discomfort and to dismiss that individual from care – or to subordinate bodily distress to psychological problems implicitly stigmatized as failings of character. The determination of ‘stress’ may feel more reassuring because it applies to most of us and can be blamed on ‘real’-life hardships, but it is a convenient blanket term for symptoms that might not be easily explained. What, then, does it mean that somatoform disorders are more likely to be reported among women, people of colour or people with less income or education, if not that such patients are simply less likely to be believed when they complain of medical symptoms? Consider Baur’s shocking assertion that ‘the poor, minorities, women and both the young and very old are more likely to retreat from a realistic appraisal of their problems and substitute physical failures in their stead.’¹² One wonders on what basis such a sweeping condemnation could be made – not only of the populations Baur indicts but even of hypochondria as a bodymind phenomenon seemingly unworthy of academic study and respect.

All of these issues – problematics of knowledge and authority; appeals to be taken seriously; attitudes about cultural value; and closely guarded

border control – become activated in relation to so-called medical gaslighting and the still-new syndrome of long-COVID. Some three years after the first reports of unusual and lingering symptoms following a bout with the virus, studies began to suggest that many patients are engaged in a protracted ‘tug of war with the medical profession’ to ‘overcome the “legitimacy deficit”’ and obtain appropriate treatment.¹³ Disputes about the accuracy of the term ‘gaslighting’ notwithstanding (I will return to this below), long-COVID has become our current moment’s fibromyalgia or endometriosis, two other conditions whose ‘objective’ medical veracity was long contested (and perhaps, by some, still is). Larry Au and his co-authors write that ‘gaslighting . . . is rooted in social inequalities such as race, gender and class, which reflect unequal power relations between patients and physicians.’¹⁴ Whether the hypochondriacal complaint is one of persistent dread of an illness that can’t be confirmed (the brain tumour, the multiple sclerosis) or a set of diffuse sensations that defy categorization by recognized nosology, inequity is a through-line in the dynamics of care, and it seems clear that the aggressiveness inherent in that differential between patient and provider worsens if the putatively hypochondriacal individual occupies a socially marginalized category. What is being disputed, after all, isn’t just the presence of disease, and not only a kind of social capital, but ontological status: who gets to determine what can be claimed as real?

Complicating these dynamics is the fact that while patient advocacy instructs us not to succumb to self-doubt – to trust the evidence of our own bodies – we are also culturally encouraged towards absolute faith in medical knowledge. The paradox involved in many contested illnesses is that, even as sufferers might complain that physicians prioritize diagnostic protocols over subjective evidence, patients themselves are often engaged in attempts to demonstrate physiological proof of disease, to achieve the status of ‘real’ through the very mechanisms they might simultaneously resist the dominance of. This accounts for the push for more tests, the repeated doctor’s visits, the hours online researching symptoms and etiology, alongside the embittered posts to social media about physician neglect. It’s also the case that some conditions can take years to diagnose, a situation that long preceded the pandemic and that has even taken on something of the cachet of mystery narrative, as in such forums as Dr Lisa Sanders’s ‘Diagnosis’ feature in the *Sunday New York Times Magazine*. As long as we’re aware, in some generalized sense, that diagnoses can be

elusive and might require both a long time and a battery of specialists to ascertain – if we think of ‘hypochondria’ as referring to an illness that doesn’t exist *yet* rather than one that has been fabricated outright by an over-anxious imagination – then a continued search for evidence isn’t inevitably unwarranted; this is what justifies more effort, more scans and blood draws, in the drive towards biomedical legitimacy.

As a psychiatric condition so designated by its relationship to medical disease, in fact, modern hypochondria comprises the ‘not-yet-sick’ along with the ‘worried well’. The latter controversial phrase (discussed in the Introduction) asserts physiological wellness as the determining feature of reality, to which, somewhat paternalistically, worry is attached as a diminishing adjunct. But ‘worried well’ is ‘problematically post hoc’, writes Judy Z. Segal, quoting a private email from physician Eric Cassell: ‘Here are two people with persistent headache who come to a doctor; both are worried because they never had such a headache before . . . One turns out to have a brain tumor. The other has “nothing” . . . Which of these two, while they were in the waiting room with their new severe headaches, was the worried well?’¹⁵ And which of them was the not-yet-sick, given that neither of them at that moment knew – not just their current circumstances but what their state of wellness would be six months on? To quote Catherine Belling, ‘is “previvor” not just a heroic and respectable synonym for “hypochondriac”?’¹⁶

‘Worried well’ is not just a thought experiment, of course; nor is it a new term. A more controversial use of the concept arose at the height of the AIDS epidemic in the 1980s. In a study of the ‘profound psychological ramifications’ of the epidemic, which the authors contended in 1984 was not being sufficiently addressed, a kind of hypochondriacal excess is attributed to ‘gay men . . . without reason to be particularly fearful’, men who ‘interpret every cough as PCP [*pneumocystis carinii* pneumonia], look for KS [Kaposi’s sarcoma] lesions on their bodies several times a day, and dread minor infections that they are sure will turn into a life-threatening illness . . . Those who fall into this group are often referred to as the “worried well”.’ Understanding that ‘actually dying of AIDS’ could have been definitively ruled out for some patients, by what measure is anxiety in the midst of horrific loss and widespread social stigma – about the disease *and* the identities of the people who contracted it – to be characterized as

obsessional or pathological?¹⁷ Given the authors' investment in raising awareness about the need for swifter and more subtle attention to people's emotional response to the epidemic, the language of 'develop[ing] psychological symptoms' seems unhelpfully additive (even pointlessly obvious, though that is with the benefit of some hindsight) – that is, to impose diagnostic labels on individuals already burdened with feelings of shame, guilt and dread. My point is not to deny that suffering occurred, or that anxiety could have a serious disruptive impact on daily life. Receiving a negative test for a condition like AIDS could of course be enormously reassuring. But being deemed 'hypochondriacal' in the sense of fabricating a reality – in the context of an illness decimating one's community – surely compounds an already volatile set of circumstances. A 1994 article in the *British Medical Journal* reported (not surprisingly) that 'there was homophobia in the medical profession' and that doctors, 'who are supposed to be guided by an ethical code', are no less influenced 'by ideology and the values of their culture' than the layperson.¹⁸ What the author describes as some physicians' 'blatant' homophobia extended even to the denial of treatment of patients with AIDS.

The refutation of a condition we believe we have is likely to feel discrediting in a characterological more than just an intellectual sense. Dismissiveness, after all, doesn't just pertain to an idea but exerts control; 'gaslighting' refers to the *maddening* effects – I use the term deliberately – of having one's perception distorted. Whether or not the term is now being used too casually, the point remains: patients accused of hypochondria are denied a sense of reality in ways that frequently amplify a pre-existing cultural subordination. 'Hypochondria' retains this efficacy as a categorical annulment of that validity precisely because it relies on a mind-body dichotomy wherein problems of 'mind' – understood as *self*, as personhood – are so much more disgracing than those of body. Of course, if we want providers never to be wrong and always to know (or more precisely always to confirm what we suspect), they will inevitably fail. So what is being defended in a judgement of 'hypochondria', or sought by the individuals so described? Belling makes the intriguing point that hypochondriacs exist in a discomfiting middle ground between the outright deceit of a 'wilful malingerer' and the authentic sickness of someone 'visibly diseased'. In this liminal space the hypochondriac embodies an 'unstable distinction between the serious and the trivial'. Belling suggests that while it's common to

assume we can easily tell the difference between imagined or exaggerated seriousness and real illness, the motivation to ‘deflat[e] that seriousness’ is driven not by logic but by emotions that are less readily acknowledged; hypochondriacs are mocked because what they represent – all the doubt one might have about whether one’s education and training measure up, all the fear we harbour about pain or loss – makes the rest of us too vulnerable.¹⁹ To put it this way renders hypochondria a kind of broad social mirror that enfolds illness discourse and the dynamics of medical power with other markers of subject position, ones embedded in the history of social injustice.

There are uglier realities at stake in hypochondria than even a pervasive derision or ‘disqualification’ of certain patients’ narratives.²⁰ The fact that clinical research has historically taken the cisgender, white, male body as its representative object of study means that not enough is known about how certain conditions affect women and people of colour; when members of underrepresented groups present with confusing symptoms, the process of aligning those symptoms with diagnoses can be especially protracted if front-line practitioners and academic researchers alike are not alert to what’s possible in bodies beyond the norm. For instance, a December 2021 article in *STAT* called ‘How Medicine Erased Black Women from a “White Man’s Disease”’ recounts the years’-long search on the part of several women for an explanation for symptoms ultimately diagnosed as ankylosing spondylitis (AS), an inflammatory disease. AS is rare among Black women, who are virtually absent from scientific research on the condition.²¹ A 2020 chapter by Kerri K. Morris on gender and bladder cancer reports similarly that women have a higher mortality rate from this type of cancer not because they contract it more frequently but because they are more often misdiagnosed with urinary tract infections or other more benign conditions; Morris writes that when she was finally diagnosed with cancer herself, she was shown a model of a male urinary tract.²² The literature on disparities in heart disease and care is also extensive; it is by now widely advertised that the signs of cardiac arrest differ in women than in men and so are frequently misinterpreted.

And still more insidious prejudices compromise treatment. To take pain as just one arena in which racial and ethnic disparities in healthcare have been amply documented, multiple studies of the treatment of chronic pain

reveal that people of colour are consistently rated by physicians as being more tolerant of pain than white patients, a bias derived from the same false biological beliefs that were deployed to justify slavery and its signature cruelty.²³ The authors of a 2016 paper contend that ‘Black Americans are systematically undertreated for pain relative to white Americans’ and that ‘physicians are more likely to underestimate the pain of Black patients’, even as Black patients themselves tend to rate their pain as higher than white counterparts; a similar study from 2007 found that physicians are ‘more likely to overestimate pain in nonblack patients’.²⁴ A 2012 cumulative review of twenty years of data reported such widespread differences in the treatment of pain as to ‘raise not only normative but quality and safety concerns’. This study, which reports that Hispanic/Latinx patients are 22 per cent less likely to be prescribed opioids than white patients and Blacks/African Americans 22 per cent less likely to receive *any* analgesic treatment at all, argues that in addition to misconceptions about the demographics of opioid abuse (which is consistently higher among whites), implicit racial bias is exacerbating a crisis in the United States of ‘unrelieved pain’.²⁵ As recently as 2021, a study of pain management in children with appendicitis and limb fracture – severe conditions – corroborates these trends, finding that Black and Hispanic children were less likely to receive opioid pain relief than white children.²⁶

When we recall that diffuse bodily pain has long been a defining characteristic of hypochondria, ‘underestimation of pain’ by doctors implies a synonymously hypochondriacal *exaggeration* of pain on the part of patients for reasons other than ‘objective’ medical circumstances. This means that patients of colour may be suspected of hypochondriacal hypersensitivity and of greater tolerance for discomfort *at the same time*, a diagnostic incoherence that is linked to a high level of distrust, particularly among Black Americans, of ‘medical research’ and ‘the health care system’.²⁷ Consider that Black women are far less likely than white women to seek therapy or psychiatric treatment, which some studies ascribe *not* to the stigma of mental health issues but rather to prior experiences with healthcare and the awareness that non-white patients are more likely than white patients to receive severe diagnoses with worse outcomes, even when presenting with similar symptoms.²⁸ Melba Wilson noted in 2001 that in Britain, for instance, ‘ill-informed ideas about black communities inevitably

leads to poorly designed and inappropriate service delivery.’²⁹ Tina K. Sacks puts it this way: ‘Black women are not allowed to be sick and tired even though the doctor’s office is ostensibly the place one should be able to let one’s guard down and fully explain one’s condition to the provider.’ Respondents in Sacks’s study of middle-class Black women’s experiences in American healthcare ‘spoke of having to mask their internal feelings and present a calm and composed demeanour no matter how badly they felt physically or psychically’.³⁰

Again the conundrum is often a prejudicial imputation of ‘craziness’ entangled with a contradictory minimizing of such distress as depression or anxiety. When individuals are deemed unreliable witnesses to their own health circumstances, providers lose the evidence they could offer to help refine a course of treatment, to say nothing of perpetuating specious beliefs about the biology of race and gender. Both popular and academic accounts of hypochondria can be guilty of casually reinforcing ethnic stereotypes, as when Carla Cantor tells us that ‘Jews and Italians are more apt to *fixate* on their bodies and *complain* about pain than Anglo-Saxon or Irish-Catholics’ (my italics) – an apparently uncited reference to even more damning assertions made in a 1977 Australian study on ethnicity and illness behaviour that condemns ‘patients of Jewish or Italian background’ for being ‘*more demanding and dependent*’ and for ‘*publicly* [seeking] sympathy for their pain’, as compared to those of ‘American or Irish origin [who] had an accepting, matter-of-fact attitude’ (my italics).³¹

The predetermined hypochondriac arrives at a medical visit at the confluence of attribution error and confirmation bias. Attribution error is a process of minimizing the annoyance and frustration of imprecise symptoms by simply interpreting them as a function of the patient’s already unpalatable subject position. Confirmation bias is the selection and interpretation of evidence that confirms a preconceived hypothesis. In a brief account of the dangers of diagnostic delay, psychologist and bioethicist Ron Berghmans argued in 2011 for a wholesale ‘transformation in the mindset of [general practitioners]’ who tend to ‘look for support for their [own] convictions’ rather than ‘alternative explanations for the complaints of patients’ – ‘complaints that appear to be clear, obvious, undeniable and easy to explain, but are not’. The collision of these two forms of cognitive error at a healthcare visit can have ‘serious if not fatal

consequences’.³² Jerome Groopman explains that physicians are trained to make decisions according to preset clinical algorithms that proceed linearly along axes of association and disambiguation, from symptom (for example, sore throat) to diagnosis and therapy (strep throat; antibiotics). A typical clinician, he suggests, begins formulating ‘hypotheses about the diagnosis . . . even before a word of the medical history is spoken’ and then forms an idea of ‘what is wrong with the patient within twenty seconds’. Often this process of diagnostic ‘framing’ works, but ‘accepting the frame as *given* can be a serious error’ (my italics) – as it nearly was in one case study Groopman recounts, a woman whose deterioration from undiagnosed coeliac disease was persistently miscast as the emotional travails of neurosis. Even ‘self-aware’ physicians can be insensitive to the operations of their emotional state – the feelings that certain kinds of patients arouse in them – on their judgement about what somebody might have in relation to who they are. ‘A wealth of research’, as Groopman sums it up, ‘shows that patients thought to have a psychological disorder get short shrift’ – with the operative word being *thought*.³³

HYPOCHONDRIA’S IMPLICATION IN racism – as well as its function as an act of resistance – can be traced to a phenomenon observed in the late eighteenth century among enslaved African people in the Americas. Francisco Barrera y Domingo’s 1798 *Reflexiones Historico Fisico Naturales Medico Quirurgicas* (discussed in the previous chapter) was the first medical treatise to name ‘slave nostalgia’ as a form of psychosomatic distress affecting slaves recently arrived in Cuba. Characterized by lethargy, loss of appetite, intense sadness and a refusal to work, slave nostalgia bore a perverse kinship with the nostalgic homesickness that affected European intellectuals such as Samuel Taylor Coleridge (discussed in [Chapter Three](#)). As Barrera described it, a similar combination of climatic and lifestyle change (including unfamiliar foods and local customs), hostility and personal insult, and obsessive thoughts of home would produce the very disease that had first been designated as such by Swiss physician Johannes Hofer in 1688 in relation to Swiss nationals abroad. But for Barrera a century later, pathologizing homesickness as a form of disordered melancholy served a larger purpose of condemning the practice of chattel

slavery and the wrongdoing of the ‘English, French, Americans, Dutch, and some Spaniards’.³⁴ To diagnose captives with a disease of emotion was to presuppose their subjectivity – in effect to convert ‘abstract value’, in Ramesh Mallipeddi’s words, back into ‘human life’.³⁵ Barrera wrote in the *Reflexiones* that this iteration of nostalgia was an inwardly expressed consequence of frustrated rage: ‘because . . . they can not take revenge on the injurer, they take revenge on themselves for the hate they feel toward the white man.’³⁶ From a contemporary vantage point, Mallipeddi argues more forcefully that nostalgia ‘functioned as a weapon of the oppressed, seized by the enslaved to influence the outcome of a social contest’. Even if painful, Mallipeddi says, nostalgia allowed for a ‘measure of self-possession’ because it signified desire, longing and protest.³⁷

Isabela Fraga offers a somewhat different reading of melancholic nostalgia in the context of slavery, contending that the personhood emerging from a narrative of suffering and affront is problematically coincident with sickness. The horrors of slavery might have been the key triggers of slave nostalgia, as Barrera claimed, but the classification of a disorder in those terms thereby theorizes Black feeling as a pathology, with lasting and largely negative implications. Where Coleridge’s hypochondriacal nostalgia was an ingredient of his creative momentum, providing a useful language with which the poet could memorialize the very home he was wilfully avoiding returning to, a romanticized rhetoric of the sensitive captive, even as co-opted for abolitionist purposes, played directly into a discourse of race that found justification for oppression in biology. We can read slaves’ melancholic nostalgia as a mode of political opposition, then, but it also gets inscribed in the pages of medical texts as a disease of identity, ushering forward an association between Blackness and a kind of madness: wilful somatizing as a screen for violent emotions and a recalcitrant obstruction of both knowledge and commerce.³⁸ As I’ve been exploring throughout these chapters, the constellation of conditions grouped as hypochondriacal can serve to reject oppressive norms of sociability or productivity in provocative, even liberatory ways. But when melancholy is deployed to illegitimize a sense of reality in the service of buttressing power relations, it evokes a tortured history of synonymy between bodies and incredible – I mean that literally, in the sense of not-to-be-believed – disease.

Modern epidemiology has rewritten slave nostalgia, sometimes also called ‘fixed melancholy’, as a direct consequence of dysentery. Kenneth F. Kiple and Brian T. Higgins surmised in 1989 that the ‘mysterious disease reported aboard the slave ships’ was extreme dehydration resulting from amoebic dysentery (known in the eighteenth century as ‘bloody flux’), whose symptoms include apathy, ‘sunken eyes’, muscle cramping and weakness – symptoms also long associated with melancholic hypochondria.³⁹ As we’ve observed in previous chapters, early modern medical theory understood melancholy generally and hypochondria specifically as thoroughly psychosomatic; just as one iteration of melancholy becomes a bodily affliction according to the evolution of medical research and thinking, so does hypochondria’s confusing etiology land it, if uneasily, in the purview of psychiatry. I pause over this chiasmic development because it highlights the mind–body distinction that would come to locate hypochondria as a dysfunction of mental process and emotion, even when its ‘content’ is corporeal, and because the circumstances of nostalgia during the slave trade make the emphatic point that no disease is ever ‘simply medicalized’, as Mallipeddi puts it, but is rather ‘a deeply historical phenomenon’.⁴⁰ Physical symptoms exist always and importantly in tandem with where we are situated and with who we are, have been made to be, and are becoming.

SO WHAT IS MEDICAL gaslighting, and why has the phrase come under some fire for inaccuracy or even potential damage? ‘Gaslighting’ was Merriam-Webster’s 2022 word of the year; some in the United States attributed the prevalence of the term to the first Trump presidency and a sense that statements issuing from the White House bore little relation to demonstrable facts. Readers familiar with the 1944 Ingrid Bergman noir classic *Gaslight* will know that to be ‘gaslit’ in the way the term came to be used in scholarly circles is to have one’s sense of reality denied, confused and undermined. (The abusive husband in the film actively seeks to convince his wife, and everyone around her, that she is hysterical and a kleptomaniac – losing her mind, colloquially – so that he can have her committed, and then take control of a fortune in jewels he believes is hidden in the house. In this deliberate sowing of distortion, he refutes her memory of events,

redefines and narrates both her past and her future through the scrim of madness, and blames the nightly fluctuations in the gas lamps on her imagination, rather than his own antics in the attic.) Some protest that to accuse a person of gaslighting when what we really mean is something like pushy superciliousness is to dilute the very real consequences of emotional abuse. To be disagreed with on a point of fact is not to be ‘gaslit’ in the word’s clinical sense, though casual use of it might suggest as much.

Is the diagnosis of hypochondria, then, a form of emotional manipulation on the part of physicians? A *New York Times* article from March 2022, ‘Women Are Calling Out Medical “Gaslighting”’, reports that in addition to such inequities as less aggressive treatments, longer waits for diagnosis and more frequent misdiagnosis, women are more likely than men to have their symptoms ‘inappropriately dismissed as minor or primarily psychological’. Melinda Wenner Moyer writes that women ‘say doctors frequently blame their health problems on their mental health’ and that women are ‘twice as likely as men to be diagnosed with a mental illness when their symptoms are consistent with heart disease’.⁴¹ Among the several thousands of comments appended to the online edition of this piece are quite a few that insist ‘gaslighting’ is being inappropriately used as a kind of counterattack to ‘hypochondria’ simply because patients resent being corrected by professionals with greater expertise. Yet the struggle to be taken seriously is well documented – among women, people of colour, people with disabilities, people considered overweight. The article quotes cardiologist Jennifer Mieres as saying that when people “‘show up with symptoms that don’t fit into the algorithm we’re taught in medical school,” . . . they get “gaslit and ignored”’.

Yet we are back to a curious circularity wherein the solution to being *accused* of hypochondria is in effect to *become* a hypochondriac. Many online newsbriefs about ‘the pains and dangers of medical gaslighting’ (Hannah Yasharoff) or doctors ‘ignoring [the] symptoms’ of long-COVID (Katie Camero) recommend precisely that form of proactivity that, as I’ve explored in previous chapters, summons the acute self-examination of hypochondria. For example, the ‘Tips’ section of Yasharoff’s *USA Today* piece called “‘They Don’t Believe Me’: The Pain and Dangers of Medical Gaslighting’ (May 2023) counsels readers to ‘Get to know your body’, ‘Take your well-being into your own hands’ and ‘Be your own advocate’. Camero’s BuzzFeed piece from the previous spring makes the same

suggestion. Quoting Diana Berrent, founder of Survivor Corps, an organization devoted to education and networking resources for COVID patients, Camero writes that ‘when asked how often the 200,000 members of the Survivor Corps felt like medical professionals didn’t believe them or weren’t taking their symptoms seriously, Berrent said it’s the one theme shared by “every single” person.’ ‘Multiple doctors told me I was a hypochondriac,’ reports one patient, which made him feel like he ‘was going insane’, and another that she ‘truly felt like no one believed what [she] was experiencing’. In the face of such disbelief, says Berrent, ‘Patients need to be their own best advocates . . . You cannot depend on any doctor to do that for you.’⁴² Such recommendations will sound unremarkable – in fact empowering and self-actualizing – and necessary, given ample evidence that the healthcare system has been indifferent to post-COVID misery. At the same time, they play into an exhortation of vigilance that has historically shifted responsibility for wellness, and therefore often blame for its failure, on to individuals rather than systems. As Danielle Kohfeldt and Gregory Mather suggest, it’s a slippery slope from earnest messages of self-care to the more insidious idea ‘that *you* are the cause of *your* own suffering’ (italics in original).⁴³

Is the point that such advice problematically creates hypochondriacs *per se*? Or that we might profitably rethink hypochondria as a measure of self-awareness crucial in situations where disbelief precedes the clinical encounter? Because hypochondria is fundamentally an epistemological conflict, having one’s health experience disbelieved, especially in the context of other forms of social bias, constitutes what Miranda Fricker has named ‘epistemic injustice’ – to be wronged in one’s capacity as a knower.⁴⁴ Fricker defines two forms of such injustice. *Testimonial injustice* occurs when a person is denied credibility because of prejudice – when we are disbelieved because of some feature of identity (gender expression, disability, weight, race and so on). *Hermeneutic injustice* refers to a kind of interpretive unintelligibility – when we struggle to understand experience if there is no concept or language with which to articulate and make sense of what is happening to us or to others. (Fricker gives the example of *sexual harassment*, a 1970s phrase that helped to give shape to a pre-existing phenomenon.) Any encounter between the healthcare system and patients disadvantaged by ideology will be vulnerable to both of these forms of epistemological injustice in the form of accusations of hypochondria. The

terminology, the ideas and information, the categories of possibility that obtain in a clinical setting, are typically not shared by both parties – or are strenuously disagreed about – and some patients will be socially devalued as knowers well before the office visit. What philosopher Vivian M. May says of the ‘interpretive bias’ sounds very much like the dynamics of hypochondria in a medical office: when a ‘pathologizing gaze . . . frames subjugated knowers/marginalized groups as “problems”’, attention is ‘divert[ed] . . . away from structures of dominance and systems of privilege’.⁴⁵ While both patient and doctor might struggle to comprehend the specifics and the import of what is communicated in this interaction, they are not equally at risk of being harmed.

Countervailing the concept of epistemic injustice in healthcare is that of epistemic humility, which counsels the practitioner to recognize what Abraham Schwab calls the ‘fickle companion’ of all evidence-based medicine: uncertainty. Schwab argues for humility as a feature of *claims* rather than persons, the latter being liable to idiosyncrasy and inconsistencies of temperament. Instead, healthcare delivery must rely structurally on a calibration of ‘reliabilism’ with a ‘taxonomy of uncertainty’ such that bias and overconfidence might be tempered. Schwab is not alone in advocating for greater attention to ‘the social side of medical practice’ – in effect who people *are* and not just what they *have*.⁴⁶ Larry Au and co-authors take this notion of humility in a more affective direction. In their discussion of long-COVID patients and the ‘distorting effect of experts’ gatekeeping power’, they describe epistemic humility not only as ‘the will to acknowledge the limits of medical expertise amid deep uncertainty’ but as a means of establishing trust, which provides emotional if not always physiological relief.⁴⁷ Groopman makes the same point in *How Doctors Think*, that ‘taking uncertainty into account can enhance a physician’s therapeutic effectiveness’, because it demonstrates ‘honesty’ and a ‘willingness to be more engaged’ with patients.⁴⁸ The respondents to the study by Au and others repeatedly cite the emotional anguish incited by providers’ reactions to their symptoms, from ‘lack of empathy’ and ‘cruel and dismissive’ behaviour to treating patients ‘like an anxious child’ or ‘unreliable witness’.⁴⁹ The issue goes beyond soothing frazzled nerves. Consider Groopman’s claim that ‘how a doctor asks questions’ and ‘responds to [their] patient’s emotion are both key’ to eliciting information

vital for understanding the ‘medical enigma’ before them: thus ‘freedom of patient speech is necessary’.⁵⁰ When providers engage with information that exceeds the supposedly objective reality of medical testing, they aren’t simply admitting that current medical research into a disease might be limited. To invite a patient’s perception into the contested, challenging space of developing treatment protocols for unfamiliar conditions is to solicit alternative truths.

TOWARDS THE BEGINNING of her memoir *Well Enough Alone: A Cultural History of My Hypochondria*, Jennifer Traig declares that hypochondria is ‘a performance’ not unlike donning a costume at ‘Halloween, a chance to wear a different identity for a while’. ‘It’s a pageant,’ says Traig, ‘and for the duration, the spotlight is on you . . . It gets you attention.’⁵¹ It’s interesting to note that the cover design of Traig’s 2008 book is a woman’s face and hair composed of differently coloured pills and capsules, as if to demonstrate that the author’s identity is made up entirely of pharmaceutical attempts to heal or at least assuage some symptom, without which she would evaporate into the blank white of the cardstock cover (though an intriguing ambiguity arises: does the author ‘need’ the treatment, or the symptom itself?). But beyond the image’s fanciful, tongue-in-cheek nod to hypochondria’s dubious status as a medical problem, the language of attention-seeking performativity seems unfortunate. Trying to make hypochondria funny as a direct riposte to its reputation as a dour and pessimistic affliction of the chronically depressed, Traig has played directly into stereotypes of fakery that make it challenging, especially for the communities I invoke throughout this chapter, to be taken seriously.

That challenge is precisely what drives the most harrowing sections of Hilary Mantel’s 2004 memoir *Giving Up the Ghost*, in which Mantel’s struggle for a diagnosis for ‘a pain [that] sliced through [her]’ devolves into a grotesque encounter with medicine’s most paternalistic tendencies. The pain is eventually discovered to be caused by endometriosis, but for ten years doctors characterize it as ‘psychosomatic’: ‘It was in the nature of educated young women’, as Mantel sardonically comments, ‘to be hysterical, neurotic, difficult and out of control, and the object was to get them back under control.’ Thus while Mantel is ‘knotted with pain’, no one

can manage to ‘sort’ analgesics for her, and she feels repeatedly ‘ignored, invalidated, and humiliated’. Worse perhaps is that ‘complaining gets you . . . in the madhouse’, if only because that course of action perpetuates a caricature of the female hysteric while delaying discovery of the actual corporeal ailment. Mantel is put on antipsychotic medication, becomes profoundly ashamed, believes pain to be her lot in life (‘the burden of womanhood’) and learns to shield herself because she knows well that, in hospital, ‘staff were inclined to treat the patients as malingerers’. The tale is one of alienation from embodiment, and Mantel comes to mourn various losses – not just of motherhood, for instance, or writing time, but of intimacy with, and respect for, herself.⁵²

Mantel’s experience puts the problem of attribution error in high relief. Consider that while ‘autoimmune diseases are increasingly common’, as O’Rourke tell us, the fact that ‘women are the overwhelming sufferers’ of such conditions casts doubt on their reality – not because they are ‘notoriously hard to identify in blood work’ but because women themselves are apt to be disbelieved.⁵³ In Mantel’s case the diagnosis she finally receives prompts her to reassess ‘what was happening inside [her]’, a kind of recalibration of body and mind: the ‘spasm’ and ‘ache’ and ‘washes of nausea’ she now knows are not ‘evidence of a neurotic personality, or of ambivalence about [her] gender’, or ‘brought on by “nerves,” or by fear of failure in a man’s world’. The account of endometriosis begins late in the text, but it is only one in a sequence of experiences that teach Mantel not to ‘trust herself, [her] perceptions of things, the evidence of [her] senses’.⁵⁴ Finally being treated for her condition is not a matter of being returned to narrative coherence by the confirming approbation of medical authority, but in effect a complex and retroactive process of ungaslighting herself, ensuring herself as the arbiter of her own actuality.

It’s a curious but unsurprising phenomenon that while ‘the stereotypical hypochondriac tends to be male’, according to Belling – think Felix Unger from *The Odd Couple*, George Costanza on *Seinfeld*, Reginald Barclay on *Star Trek: The Next Generation* – women fare worse in practice, often being named hypochondriacal when an existing medical problem simply hasn’t yet been identified.⁵⁵ One way to understand the contrast between popular conceptions of the type and women’s lived experience is that hypochondria upends normatively masculine behaviours. In its emphatic focus on the

body, its nervousness and need for external reassurance, its supposed self-absorption, hypochondria infantilizes and emasculates: indeed it *feminizes*. Characters like George and Reg are provocative because they defy expectations of maleness, of rational, practical command; the comedy depends on recognition and acceptance of that reversal. At the same time, audiences will on some level understand the precarity of that version of masculinity – that it is ideological rather than essential – so that the broad humour of caricature also allows anxiety to be discharged: laughter is a screen for, if also an eruption of, fears of weakness, vulnerability and mortality that get occluded by the normative male role. A female hypochondriac isn't funny, as the detestable Mrs Churchill in Jane Austen's *Emma* makes quite clear, but in some sense simply a woman; when 'Dr. G' says to Hilary Mantel in the 1980s, 'I don't want you writing,' she might have been a nineteenth-century neurasthenic suffering her male doctor's terror of the female imagination.⁵⁶

A 2023 SERIAL PRODUCTIONS/*New York Times* podcast called 'The Retrievals' details a scandal that unfolded at the Yale University Fertility Center. Over several months in 2020, some two hundred women underwent egg retrieval at the clinic – without the benefit of pain management. Unbeknownst to the patients, the clinic staff, and most of the attending physicians and nurses, one nurse was stealing fentanyl and substituting saline. As reported by Susan Burton, the women involved experienced excruciating pain during and after their procedures, an agony compounded by the fact that, because no one could account for this extreme but anomalous discomfort, it was frequently minimized, even denied. 'Nobody believed me,' says one of Burton's interviewees, as she tried to explain during a typically straightforward and amply medicated procedure that she could 'feel everything'. Another, immobilized with pain in the days after her retrieval, 'got the sense [she] was annoying' the on-call doctor to whom she spoke; when her primary physician told her that he was 'not alarmed, but perplexed and surprised by [her] experience', she began to doubt her 'ability to assess [her] situation rationally'. 'I felt *crazy*,' she says, and comments that the doctor's words '[rang] pretty hollow', given the 'pattern' already emerging of one after another woman reporting unbearable pain.

‘You start thinking about your whole life,’ says this interviewee, as the discounting of her distress throws everything into question. She wonders, as so many have been accused in reputedly hypochondriacal encounters, if she is ‘being difficult’.⁵⁷

Burton frames the podcast in terms of how women’s pain is narrated, interpreted and treated. ‘In the absence of information about the true cause of the pain’, she explains of what happened at Yale, multiple and competing explanations arose. The patients themselves, pleading ‘again and again’ that something was ‘wrong’, concluded that they were ‘not sensitive to certain opiates’ or leaned into trust for the institution – medicine and Yale alike – or tried to focus on the hoped-for outcome: the baby. At least one did surmise that the drug was being swapped for something less effective from the very first. Others resigned themselves to the torment of the procedure, thinking – as did Hilary Mantel – that ‘this is what women go through’. ‘There’s a balance you have to strike as a woman patient,’ says Burton. ‘You have to complain just the right amount to be taken seriously, but not so much that you seem shrill. Still, your pain is more likely to be underestimated than men’s. Black women are more likely to have their pain ignored no matter what they do.’ An additional feature of the case not remarked upon in the podcast is that every doctor mentioned is male; surely those gendered dynamics mattered to what happened, as when one physician barks at a patient, ‘If you don’t stop yelling, I’m just going to leave’; ‘I can’t do this if you don’t stop moving.’ Pain gets ‘normalized’, says Burton, and yet the repeated articulation of *severe* and unaccountable pain in itself wasn’t enough to trigger concern about the drug supply.

And the egg retrievals were just one part of an ordeal that unfolded into 2021. When the ‘drug diversion’ was discovered, Yale sent letters to patients that for many simply added insult to the injury they’d already endured. ‘There’s no reason to believe that this event has had any negative effect on your health,’ this letter read, ‘or the outcome of the care that you received’ – a statement so tonally nonchalant, so defensive and so apparently out of touch with what it actually meant for women to undergo the retrieval procedure without pain relief that a sense of ‘vindication’ was mixed with ‘fury’. ‘They ignored my pleas for help,’ says one of Burton’s interviewees. ‘I *know* what I . . . went through.’ The accused nurse’s trial, in the spring of 2021, became a third site of contestation about women’s pain – not just whether it is respected in an aggregate sense but whose pain

matters most in a legal context: that of the patients who were harmed, that of the beleaguered mother-of-three with a nasty ex-husband for whom fentanyl was a reprieve of sorts or, implicitly, that of the many women of colour historically disadvantaged by a system now seeming to prioritize a single white woman's suffering and to forgive her wrongdoing. Burton's respondents express outrage that the judge in the case 'accommodated' the nurse, who avoided significant incarceration in being sentenced to a month of house arrest and alternate weekends in jail. 'Was motherhood the decisive alliance?' asks Burton of what she calls 'bespoke sentencing', citing the fact that both the judge and defence attorney are mothers. One of her interviewees wonders, more sardonically, if 'this is whiteness in America.'

There's a singularly horrific crime at the heart of 'The Retrievals'. But much of the language we hear in the podcast should sound disappointingly familiar: women being doubted as 'unreliable narrators' of their own bodies; beheld as hysterical 'from the second you walk in there'; accused of 'over-dramatizing' their experience; belittled as angry, 'bitchy', anxious, 'hormonal'.⁵⁸ Burton is careful, of course, to cite and quote clinic staffers who were worried for their patients and did what they could to respond to their discomfort swiftly and adequately. That these events took place in a *fertility* clinic is part of what makes them so egregious, because the treatments happening there are by definition charged with strong emotion, bound up with how patients feel as women, with deeply intimate parts of themselves and with complex cultural narratives about the meaning and value of motherhood. 'We are unwilling to take the meanings of pain as seriously as we take its biology,' writes Arthur Kleinman, and it's clear from the patients Burton interviews that one individual nurse's disregard for their physical discomfort is only a part of their torment, which extends beyond the clinic or healthcare more generally to much broader patterns of misogyny and racism.⁵⁹ They feel a betrayal that comes from encountering the burden of proof in an environment they assumed was trustworthy – that indeed seemed devoted to *their* care. Pain is notoriously subject to disbelief because it cannot be seen or directly measured and thus crystallizes hypochondria's problematic of credibility. The women of 'The Retrievals' were literally crying out to have their pain validated and assuaged. Theirs was obviously a unique and rare occurrence of serious distress being misunderstood because it seemed to defy logic. But how many other

instances of ‘real’ pain or illness does hypochondria represent, if only the story of it were believed?

PAIN, LONG-COVID, HEART trouble – or fibromyalgia, endometriosis, lupus, dyslexia, chronic fatigue, even neurological disease and brain tumours – many such conditions elude standard diagnostics and are difficult to pin down in the body. To some extent they exist as narrative: a story of feeling, of strain, of difficulty, of weariness, of confusion, passed from an individual bodymind, one embedded in layered ideologies of value and propriety, to the socially empowered arbiters of meaning, soothing and cure. I’ve said elsewhere in this study that hypochondria is a condition of reading, a fundamentally interpretive gesture that embattles us in contests of hermeneutic control; hypochondriacs are ‘extreme narratophiles’, as Belling puts it, ‘experiencing themselves as always already part of a narrative’.⁶⁰ Hypochondria is an utterance, a rhetorical situation that acts on both tellers and listeners, on sufferers and healers. The subject of scepticism and dismissal, it is also an opportunity for understanding and knowledge. I think here of a moment at the very end of trans disability activist Eli Clare’s memoir *Exile and Pride*, in which Clare imagines ‘sit[ting] on the wide, flat top of [a] wall’ with ‘people of many varieties and trad[ing] stories long into the night’: ‘sad stories’ and ‘enraging stories’; ‘devastating’, ‘untold’, queer stories; ‘bold, brash stories about reclaiming our bodies and changing the world’.⁶¹ The stories Clare envisions and the people telling them challenge norms of gender and ability; they might have been labelled as hypochondriacal for their insistence on forms of malaise not always recognizable by mainstream medical testing. The hypochondriac’s story belongs in that eclectic mix, as part of the traumatized, brave, defiant – and too often, to calamitous effect, unheeded – tales of who we are and what we feel.

6

The Wisdom of Worry: Hypochondria Ages

‘In the end, every hypochondriac is his own prophet.’

ROBERT LOWELL, ‘Obit’^{[1](#)}

Among our many contradictory and mostly negative stereotypes about aging is an obsession with illness – the old person’s fixated griping about bodily travails that sends the rest of us scrambling for cover at family reunions. The hypochondriacal octogenarian is a stock butt of jokes as well as befuddled consternation. On Quora.com, for instance (a user-generated website that ‘focuses on high quality questions and answers’), readers post perplexed questions: ‘Why do some older people talk about their health so much?’, ‘Why do old people constantly talk about their illnesses?’, ‘What do really old people like to talk about (besides their health)?’ So distressing is this phenomenon, so ‘boring’ and irrelevant must health concerns be, that one blogger has declared her ‘private rule: no more than 5 minutes per meeting per person may be spent discussing health problems, whether they be bunions or cancer’ (one wonders what it would be like to be the ailing friend of such a dogmatic personality).^{[2](#)} The perception revealed by such comments is of a straitened frame of reference, an idea that the elderly concentrate on their aches and rumblings because their lives have become radically narrowed: without jobs, romance or apparently any sustaining interests, what else is there to speak of but how bad we feel? Of course, illness, aging and death – the markers of our existential fleshliness – terrify us, and derision and humour have always been sure methods of regaining

some mastery over threat. Have you heard the one about the hypochondriac's gravestone? It reads, 'I told you I was sick.'

It seems reasonable that the normal if often startling physical and cognitive changes associated with getting older might make us apt to announce them; we return to the subjects that preoccupy us, about which we have something to figure out, perhaps to reconcile ourselves to them. The adjustments of aging are many, to be sure, not least the increasing proximity of an event our culture, and likely our individual imaginations, train us to avoid thinking about by keeping us so fully invested in youth and fitness. But as members of our cohort begin to die, as we become naturally more vulnerable to health risks, as the condition of our bodymind takes on the status of portent rather than background noise or momentary discomfort and annoyance, we may become differently, more urgently – and rightfully – preoccupied with the contours of being sick. The U.S. National Council on Aging reported in March 2023 that among adults over 65, 'Nearly 95 per cent have at least one chronic condition, and nearly 80 per cent have two or more.'³ It's hardly surprising in such circumstances that one would pay heightened attention to the quality of one's health – and especially to the possibility, indeed the *probability*, of its worsening. The image given to us by geriatric psychiatrist Elliott M. Stein of 'nutriceuticals' and a whole 'polypharmacy' 'accumulat[ing] on every available surface in [the] bathrooms' of the elderly amuses precisely because it strikes a familiar chord.⁴

And yet academic research tells us that hypochondria is not in fact correlated to aging at all, dismantling this derision of the elderly as incessant health pessimists. Study after study suggests that *youth* is the greater risk factor for hypochondria, showing that older adults, whatever the popular conception of them might be, experience more emotional well-being than other age cohorts and are not statistically more prone to disruptive worry about illness than younger people; those who do experience such anxiety likely felt it earlier in life as well.⁵ In 'The Relation Between Hypochondriasis and Age', Arthur J. Barsky and others write that older patients score lower in both illness conviction and fear, even if they are objectively sicker, than younger patients, and a later report states explicitly that the 'narcissistic, self-indulgent, selfish and impulsive' stereotype of the hypochondriac is '*less pronounced in older individuals*'

than in younger (my italics).⁶ In fact fear of aging correlates to hypochondria more than *being* aged.⁷ Stein thus asks, ‘what do we call an older patient who exhibits [the] behaviors, concerns, and preoccupations’ that are associated with hypochondria, ‘and how should we respond?’, proposing that ‘somatic preoccupations’ among the aging should be considered *adaptive* rather than excessive.⁸ Gerontologists Paul T. Costa and Robert R. McCrae, both of the National Institute on Aging, wrote in a 1985 *American Psychologist* article that behaviours indicating ‘hypochondriasis in a 30-year-old may be evidence of accurate self-assessment for an 80-year-old.’⁹ What this means is that hypochondria can be understood as a *functional* response to the demands of growing older. Not an interruptive condition dismantling a purposeful life, illness consciousness simply *is* one’s later years.

In dispensing with the idea that the elderly are by an inherent quality of aging prone to hypochondria, however, some work manages to reiterate ageist, ableist biases towards those attributes and behaviours typically associated with youth. For instance, Carla Cantor argues in *Phantom Illness* that while it is ‘natural for health expectations to decline with age’, one factor ‘mitigat[ing] hypochondria in the aged [may be] a conscious attempt to work against the stereotype of the doddering senior who collects symptoms like postage stamps and drives doctors and children crazy with insufferable complaints’.¹⁰ Susan Baur tells us that when ‘physical disabilities . . . deprive an old person of the walking, gardening, swimming or puttering that consoled him in earlier years’, preoccupation with the body is inevitable.¹¹ What’s significant in such comments isn’t the unobjectionable notion of resisting stereotypes – nor the idea that aging is associated with losses or loneliness that may be felt acutely – but rather the intimation that we refute the insult of ‘doddering senior’ *only* insofar as we mimic being young: by defying the very conditions that define us as naturally aging, proclaiming such triumphant markers of ableism as athleticism and independence and above all not breaching decorum by announcing our aches and pains. When renowned feminist activist Gloria Steinem announced that ‘fifty is what 40 used to be’ at a *Ms.* magazine celebration of her birthday, the emancipated declaration, seeming to reject the fear and cultural disgust associated with the inexorable fact of advancing years (especially for women), simply reinforced it.

Our reactions to the changes of aging (like our attitudes towards pain, impairment and injury generally) take their cues from narratives associating ‘fitness’ with being ontologically *appropriate* – not just healthy, strong and toned but suitable to requirements: of work, of legal representation, of family life, of education and finally of subjectivity. To be deemed unfit is not just to disappoint some physical ideal of musculature or expectations for a particular bodily or mental task but to fail cultural standards of *personhood* itself. A society in which ‘worth’ is correlated strictly to economic contribution (as discussed in [Chapter Two](#)) will incite the elderly, especially upon retirement, to replace work with activity that signals legible ‘doing’: hobbying, crafting, games and sports, family – normative forms of maintaining connection to the social fabric. In the absence of such enterprising busyness, identity becomes destabilized; and because illness, ‘real’ or imagined, is understood to interrupt the forward motion associated with progress and success, the aging individual with myriad health complaints becomes by definition estranged from networks that guarantee a self. This is not to discount acute feelings of loneliness and disengagement that can arise in the aftermath of retirement, or when children move away and friend groups alter. It is to argue that the story of aging teeters on a narrowly conceived binary opposition between decrepitude and the mimicry of youth – and in turn to suggest that the relational, generative and consolidating functions of hypochondria explored elsewhere in this book, challenging enduring cultural mythologies of health and youth, might also help us to reimagine late-life experience as meaningful even when it stimulates anxious cogitation upon the state of ourselves. I will return to this idea of hypochondriacal wisdom in the specific context of senescence as a provocative collision of stereotypes, in which the elderly emerge as our truest knowers of the human condition.

The correlation between hypochondria and aging in our cultural imagination surfaces in the horror of losing control. The elderly hypochondriac must be impugned for failing at so-called positive aging, one side of a neat conceptual opposition in which to be rescued from older “‘decline and loss’ stereotypes”, as Clara W. Berridge and Marty Martinson explain, demands ‘the avoidance of disease and disability’ and ‘the maintenance of physical and cognitive function’. ‘Successful’ aging, in this model, is thus incoherently defined as ‘not be[ing] old at all’, whereas the tainted obverse of unsucess blames the individual for experiencing,

perhaps intensely, the normal shifts and losses of age.¹² The encouraging terms of successful aging may have been intended to resist the cultural neglect of the elderly as imminently senile and already irrelevant, but beneath them lies the old ideology of ableism, with its preference for functionality, autonomy and intelligence. But what if ‘later-life floundering’, as in the title of a piece by Scott Herring, were defined *as* success rather than in contradistinction to it? What if the ‘mistake-prone and mistake-laden’ habits of some aging minds weren’t deemed a pathological deviation from selfregulated vitality, but instead an alternative cognitive modality and a potent source of knowledge and artistry?¹³

Herring, following other recent scholars of senescence, proposes that we reconceptualize ‘unwisdom and obliquity’ as ‘ways of flourishing [that] refute the perfectibility of knowledge’, ‘do not culminate in utmost insight’ and ‘wind up right back at square one’. Such ‘nonexpert, nonstandardized ways of growing older’, he argues, ‘shirk wise mastery’ in favour of ‘err[ing], goof[ing], [and] stumbl[ing]’, thus posing a significant challenge to a cultural imperative to age our way straight back to youth – and to define viable personhood through privileged forms of cognition. Though Herring doesn’t name them as such, ‘those who keep at it and keep getting it wrong’ sound very much like hypochondriacs, studiously fretting and erroneously diagnosing; his invitation, in turn, to revel in ignorance and ambivalence and forego disciplining the aging mind – yoking ourselves to a myth of perfectible wisdom – might mitigate the panic and humiliation that attach to hypochondria as a condition defined within the crucible of medical authority.¹⁴ Herring’s concept of ‘*geroresilience*’, moreover (italics in original), finds strength rather than fragility in the very kinds of mental eccentricities, including undignified because ‘unreasonable’ fears, that tend to deprive the elderly of respect.¹⁵ What if we decide to ‘carry on error-ridden until [we] cannot’?¹⁶ Such a framework might dissolve the double whammy of aging hypochondria wherein sensations and symptoms disregarded as foolish compound the expectation that the elderly will quietly withdraw from social engagement as preparation for death. We might become ‘wise old fools’ instead, as Herring has it, blundering, thriving and unpenalized.

But to tolerate ‘floundering’ as ‘an art form’, in Herring’s words, let alone ‘revel in what gerontology sees as the vices of senescence’, is to court

the perilous disintegration of dementia, with its specific and terrible loss of self-continuity.¹⁷ The spectre of Alzheimer's looms so large in our cultural imaginary as the collapse of personhood that even the slightest failures of retrieval can feel like portents of the utter dissolution of 'what makes [us] recognizable' to ourselves, as Helen Small puts it – a kind of elaborated Jekyll and Hyde, a narrative to which I will also return.¹⁸ Indeed, so fearsome is the transformation represented by senility that hypochondriacal anxiety about cognitive function may set in well before official senescence. The question posed by Bob Woods and Carolien Lamers at the start of a chapter on the 'psychological problems' of the elderly – 'How can cognitive changes associated with a dementia be distinguished from normal ageing?' – draws a somewhat surprisingly stark distinction, as if even a more severe alteration in cognitive function were not part of 'normal aging' for a considerable population.¹⁹ Hypochondria in this context is intimately bound up with an entrenched conceptual synonymy of self and mind: we are because we think we are. To worry that something is wrong with the brain, affecting memory, mood or thought process, is not just to approach the threshold of mortality but to come into contact with the fragility of how we define our very subjectivity, however old we are. The fact that 'cognitive decline with age is not a uniform or a universal phenomenon', as Woods and Lamers report, may thus be the kind of logical datum (like the comparatively low crash rate of airplanes) that fails as an antidote to phobia.²⁰ We are prone to be hypersensitive to intimations of our collapse.

German E. Berrios and others suggest that some patients fearing the onset of dementia are 'neglected' by memory clinics if their experiences "do not fit" into any ongoing definition of "disorder of memory". 'Memory complainers' who present with no neuropsychological deficit might be written off as hypochondriacal, in other words, when perhaps they 'suffer from a still unknown form of memory disorder'.²¹ This suggests that older patients are apt to have their complaints discounted for reasons of ageism in addition to the impatience inspired by symptoms that don't fit established nosological categories. Hypochondria's temporal dimension emerges here as a form of scientific suspension: hypochondria anticipates what has yet to be discovered, delineated, defined. Of course the effort to retrieve patients from the slur of hypochondria by suggesting that they suffer an as-yet-unidentified but 'real' organic neurological problem risks

playing into, even legitimizing, fear of psychiatric illness. Thus another possibility arises: that concern about cognitive decline *is* taken more seriously, at least in casual settings, because we resist so acutely any hazard to sense of ‘self’ – and because a condition like Alzheimer’s does not lend itself so easily to regimes of healthy living or to narratives of self-determined longevity and wellness. Perhaps it is easier to discount a friend’s seemingly random pang as cancer or heart disease since these don’t violate so thoroughly our ideas of who we *are*, even if they do entail the risk of death. But forgetfulness, disorientation, emotional irascibility, an inability to fend for oneself – these scramble selfhood, undo ego, confront us with an awful truth about the limitations of mentality as a guarantor of independence and individuality.

We can surely discern this fear of age as the loss of rational capability in Dr Jekyll’s repeated morphing into monstrosity and the helpless fascination with which we behold the Mr Hydes of our own aging, grotesque inversions coded as eventuality rather than possibility, resembling us and yet behaving in unaccountably bizarre ways. And so perhaps the flip side of Hyde isn’t the respectably aging Jekyll at all but the ridiculous figure of the hypochondriac, ostracized and deranged by worry about an aspect of mortality the rest of us are whistling furiously to ignore.

NEARLY AT THE END of her journal *At Eighty-Two* – her last work, published after her death in 1995 – the poet and serial autobiographer May Sarton writes of starting Prozac that ‘Of course [she does] not want to die, although death seems to be the only solution to [her] problem at present.’²² She frets in this brief paragraph less about depression than about the constipation caused by the drug but then asks, ‘Why talk about this?’ She goes on: ‘But I say also, “Why not?” I seem to be totally absorbed now in my body and what it is doing, and this is miserable.’ Typical of Sarton is the refracting subjectivity, from the writing self to the narrating ‘I’ to the baffling, recalcitrant brain and bowels that refuse to bend to conscious will. Yet this is not the note the journal ends on, nor is it the prevailing mood of the book’s three-hundred-plus pages, which, to the contrary, amalgamate an aging self that variously fears, recoils from and grows accustomed to change and loss.²³

The appeal of Sarton's journals – she wrote six on the subjects of aging and illness, among several others – has much to do with their narrator's unconcealed vulnerability, her willingness to describe the loneliness and panic of sickness and aging, especially as a single woman (Sarton was unpartnered in later life and did not have children). Self-absorption may be recorded here as boring the author, but the obvious presumption across the journals is that it will *not* tire the reader, whose interest in the minutiae of body and mind is taken for granted. In fact I would argue that Sarton's diaries have had an enduring popularity precisely because their moment-to-moment observations of experience memorializes being alive however much it hurts or worries us. The very act of writing defies the notion that illness interrupts 'normal' life or that the mundanity of aging is inherently dull, even as the content of the writing crafts Sarton's persona from a narrative of complaint that is at once legitimizing and literary. The stereotype of age is that its cranky, hypochondriacal nattering speaks no truths and contributes nothing to a viable existence. Sarton's autobiographical work suggests otherwise and exuberantly so, even as it indulges in some bemoaning of unrealized writerly aspirations, petty interpersonal aggravations and an array of corporeal travails. All of this, quite literally, becomes a body of material, a life's communicative achievement.

These tensions are especially prevalent at the end of *At Eighty-Two*, where Sarton describes pain throughout her body that might (or might not) be a recurrence of cancer. She writes of the 'drastic step' of insisting on answers – and a painkiller – from her doctor after 'ten days of acute pain in [her] muscles and bones' and a series of inconclusive tests. 'I have really had it,' declares the narrator, and the next day, 'it is so strange to have so much pain in all the muscles and bones.' Recalling her mastectomy of 'eight or nine years ago', she wonders if she is experiencing nerve pain from radiation; remembering a quasi-diagnosis of 'probably pneumonia' of the previous autumn, she hopes that 'this time something definite will happen' so that she 'can start to live again'. 'Sometimes I have this crazy dream,' Sarton had written the week before, 'that someday I will feel well. It would be so extraordinary to feel well . . . but it is rather like being dead.' The mysteriousness of discomfort, the instances of professional uncertainty, the rehearsal of past and probable causes, the sense that pain is likely one thing but might be anything and the need to *tell* all of this murky suffering –

these are the hallmarks of hypochondria. The sense that the opposite of ‘feel[ing] well’ is ‘being dead’ rather than being *sick*, however, seems more related in these entries to Sarton’s sense of herself in ‘a not very easy old age’, one in which everything that besets her, both psychologically and physically, is contoured by ‘how awful’ it is to be elderly: a time of life when, by more or less explicit social agreement, it is ‘all finished forever’.²⁴

For the last months of this journal – some forty pages – Sarton repeatedly complains of feeling ‘terribly ill’, ‘sicker and sicker’, an inchoate distress in which serious depression is entangled with physical pain that only gets ‘partially diagnosed’ in the last pages of the text as ‘inflammation of the bones and muscles’.²⁵ As is common in Sarton’s later-life journals, the anguish can feel relentless. When she says, just a few weeks before the text ends, that she ‘want[s] to recover a few things other than feeling ill and [her] despair at the condition [she is] in’, it is the ‘patient’ who speaks: a version of Sarton constructed through the culling of details specific to her ailment(s). But alongside that Sarton is the one ‘happy’ to be interviewed on tape for the instructional purpose of ‘helping . . . medical students to understand . . . what old age is like’ – to think of herself, if temporarily, not as a passive casualty of ‘infinite frustrations’ but as an agent of narration who interprets her experience for auditors structurally apt, even authorized, to dismiss her.²⁶ It may be unsurprising to critics impatient with hypochondria generally that Sarton would report some contentment in the rehearsal of her complaints to a captive audience of medical professionals. But this moment also strikes me as exemplary of what hypochondria’s concentration means for aging, whether that entails coming to terms with being-in-death or understanding our older years as inevitably, but not hopelessly, concerned with ‘what is wrong with you’. ‘It will be good for me’, Sarton remarks with sanguine understatement, ‘to talk about old age, find out where I should bear down more heavily on myself and where I should let things go.’²⁷

Here the pleasure of talking about herself is explicitly stated as a useful reckoning – inwardness can be productive – that avoids the twin accusations of hypochondria as self-indulgent health obsession or manipulative and emotionally stunted carping. Sarton would die a year after these entries; there is still in the last moments a hopefulness about getting

‘better’.²⁸ Yet she also speaks with careful resilience – geroresilience, as Herring would say – about what can be learned by attending with some dispassionate clarity to what it is like to be oneself. We can be in charge of ourselves, she implies, even as we are ‘letting go’, can come into contact with sensations and symptoms that portend worse to come and yet keep writing, keep working, keep being. Perhaps it is the very definition of hypochondria to long ‘to feel well’ while persisting in the daily notation of feeling ‘disintegrated’, which is a way of saying that we *do* persist, that we exist.²⁹ Who ‘is’ May Sarton, as the amalgamation of months of such fluctuating contentment and (some might protest unearned) misery? The ‘social discourses of ageing’, as Jeannette King writes, can ‘easily produce a sense of unstable identity’, especially for the aging woman.³⁰ Yet Sarton declares this ‘old woman living alone’ as her prized literary persona, one who ‘writes poems, cooks up a supper of asparagus on toast with hard-boiled eggs . . . who is often depressed but has learned how minds change and how to handle the down in weeks or months’.³¹

Notwithstanding some controversy over the veracity of Sarton’s journals (it’s well known that she stylized herself as a single woman loving solitude without fully disclosing the thickets of her friendships and romantic entanglements), I find them compelling in large part for their insistence on the public significance of the aging woman – one who openly announces where and how often she hurts.³² This chapter began with contemporary quotes from bloggers and posters about the annoyance of that very subject, the older person’s obnoxious refusal to obey social decorum and skirt the subject of bodily problems. Sarton does not apologize for the thing she is compelled to do – to ‘talk about old age’ – as if to do so would be a terrible breach. The casual laughter, the *here-we-go-again* sheepishness as we reveal we can’t take our minds off our nausea, headaches, bowels and fatigue: these may disguise deeper fears engendered by what our discomfort could possibly signify (a basic anxiety about death), but they’re also determined by social agreement about what constitutes appropriate adult discourse.³³ The irony of the stereotype about the aging hypochondriac lies in this return to the body’s infantile helplessness, so that even a serial autobiographer like Sarton confesses her bodily alterations and occurrences, of which she is obviously not in control, with some embarrassment. At the same time – and frankly regardless of the putative accuracy of her

remembrances – she issues forth a figure of ‘the old woman’ whose daily confrontation with bad feeling, often ill defined, is a source of creative knowledge.

THIS NOTION OF HYPOCHONDRIA in a generative rather than negating guise returns me to Franz Kafka’s frantically anxious narrator of ‘The Burrow’ (introduced in [Chapter Four](#)), whose underground warren has kept him occupied, if not exactly at ease, over an apparently long life. The creature lets us know from the start that he is old, organizing the tale of his burrow across a distance of years – which is to say, his enduring terror, the intractable quality of his conviction that something is amiss, the ‘incomprehensible’ fact that he feels encroached upon by invisible threat despite his best efforts to shield himself from harm. The age of the creature is not incidental, since it proves the point that obsessive effort to protect the self may be sustaining; indeed vigilance doesn’t just protect the creature: it effectively *is* the creature, who has no real existence beyond his activity within the limits of his underground den, where even at the end the ‘old architect . . . frets and gnaws’ and carries on seeking a ‘solution’ to his problem. That problem is easily defined as hypochondria, dramatized as a maddeningly elusive ‘beast’ whose scritchings the narrator can hear through the walls of his burrow but also as the burrow itself, an elaborated body beset by ‘so many unavoidable defects’ that it cannot be relied upon to guard against predators. To the contrary, the burrow is beset by internal structural problems so that predation comes from within too, in the form of innumerable ‘small fry’ (like germs, like pathogens) that ‘swarm’ through the dirt.³⁴

The creature is fully aware of the discrepancy between ‘the enormous labor’ he devotes to the upkeep of this ineffectual burrow and the ‘actual security’ it provides him. Yet for all his attempts to ward off vulnerability, it is *pleasure* that shapes his embodied relationship to his surroundings: ‘infinite pleasure and reassur[ance]’ in the process of ‘keeping vigilant guard over’ oneself, even if ‘little creatures’ and ‘almost inaudible whistling[s]’ – bodily noise, the obscure symptom – do not resolve, produce only ‘hypothesis’, never diagnosis or cure. ‘If only I had carried out the best of the grand plans I thought out in my youth and early manhood,’ he reflects

towards the end, sounding very like a stereotypically regretful septuagenarian ruing the loss of time and initiative. A bit later he admits that he ‘would be quite content if [he] could only still the conflict going on within [him]’ (my italics) – rather like May Sarton figuring out ‘where to bear down more heavily on [her]self and where [to] let things go’ – seeming to relocate the source of his distress from an inscrutably dangerous world to a state of anxious and uncontrolled ‘imagination’.³⁵

But later still the creature says that ‘the sole plan [he has] left that [he] can carry out’ is to ‘*enjoy [his] store of food as fully as possible*’ (my italics): as bold an avowal of contentment with the current state of one’s affairs as can be imagined for an inveterate hypochondriac. ‘Nothing has altered’ after all; the creature has not solved the various threats of his burrow, where ‘all remains unchanged.’ Indeed all that can be said to have truly evolved over the course of the tale is that he has aged – and it is only now, in his ‘maturity’, no longer young, that he makes his most audacious claim of all: that he ‘no longer wish[es] to have certainty’.³⁶ We understand that hypochondria turns on this very conundrum, the forward press (on both sides of the clinical encounter) towards certitude of diagnosis and thus prognosis, the need to project oneself into one’s own future, the impulse to *know*. For the creature to eschew a desire for certitude is to hold him, paradoxically but pleasurably, in an enterprise that presumes and yet has no conclusion, as hypochondria also demands and resists an answer simultaneously. To regard that activity as inherently futile is to perpetuate what Kafka himself might have argued against as the damaging expectation of productivity. The wisdom of the creature’s old age derives not from *stopping* his hypochondriacal efforts but rather intentionally *enjoying* them.

I have been arguing throughout these chapters that for all the disruptive misery it can cause, hypochondria also affords us a range of alternatives – forms of self-appraisal that resist the obviousness of health or attitudes towards time, national and racial boundaries and epistemological authority. If hypochondria is, to invoke Catherine Belling’s title, a ‘condition of doubt’, it is also one that inheres in the impulse to consult, compare, console; it is a powerfully relational condition as it is also a narrative one, prompting story. Kafka’s creature longs for ‘someone [he] could trust’, perhaps ‘someone of [his] own kind, a connoisseur and prizer of burrows’, and at one point ventures an intriguing assertion of universality: ‘For who,

even if he were merely on his wanderings, would not change his itinerary and his plans for the future on catching sight of the burrow?’ A hypochondriacal, aged self; a body if also a grave; a metaphor of gradual inoculation whereby we are strengthened through contact with harm – the creature takes something for granted about how his auditors will react to his ‘observations’ about his burrow: not with contempt, impatience or ridicule but rather interest, even envy.³⁷ Given a choice, he implies, we would all trade aimless ‘wanderings’, our carefree but undirected existence, for the security and comfort of the burrow where we can pass our elder years in furious scrutiny, listening intensively to ourselves and safe – at least – in the knowledge that just on the other side of the wall, someone else is doing the very same thing.

IN A 2003 STUDY, John Monopoli and Frank Vaccaro posit that ‘hypochondriasis may take a more subtle, more inwardly focused form in the elderly than in younger populations.’ The pith of this inwardness, according to the authors, is *self-reliance*, a specifically age-won relinquishing of control over external circumstances – what Monopoli and Vaccaro refer to as ‘reality bending’.³⁸ Their language is intriguing, since it positions the elder hypochondriac as less beholden to fanciful notions of sickness, and less manipulative of anyone else’s perceptions, than stereotypes of either old age or health anxiety would have us suspect. Instead hypochondriacs in their later years emerge as refreshingly grounded, calmly (if somewhat neurotically) musing upon the vagaries of embodied experience. ‘Subtle’, too, suggests an unexpected refinement to the work of hypochondria, not a blunt, insistent phobia but an intricate, even sophisticated form of bodymind investigation borne of years of practice. To describe hypochondria this way, as a form of self-actualized maturity, is not just to protest its characterization as a kind of arrested infantile illness behaviour but to go so far as considering its rigorous analyses a foundational component of how we grow up, into ourselves.

Hypochondria as an indication of ‘healthy’ aging – to signal the irony of such a formulation – gives the lie to the ableism of ageism, as Kathleen Woodward reminds us: ‘the able body is the norm; [and] so too is the healthy youthful and middle-aged body.’ We work hard to conceal the

‘spectacle’ of age, Woodward argues, even as ‘visibly marked aging bodies are typically considered so ordinary that they recede from view, becoming invisible.’³⁹ This is the puzzle of difference, the simultaneous hyper/in-visibility whereby the marked body cannot be ignored and yet is persistently so, relegated to a spot-lit periphery. That we all inevitably become disabled in one way or another, given normal aging – a fact often invoked to demystify the identity category of disability, to naturalize it – can have the unintended effect of retrenching our fears of aging itself by equating age with a dreaded frailty and diminishment. And at the same time we merely reinscribe ‘damaging and dominant stereotype[s]’, as I argue at the start of this chapter, when we replace the vision of aging as ‘solely an experience of decline and deterioration’, in Woodward’s terms, with a differently problematic one of ‘grandmother or grandfather’, ‘vigorous, spirited, and glad to be alive’, signing up for ‘aerobics class[es]’ – in the words of Rutgers University sociologist Stephen Crystal.⁴⁰ We seem philosophically incapable of tolerating a fuller range of human experience between these poles, wherein hypochondria might arrive in old age as a form of honest reckoning with loss and transformation.

Hypochondria might also controvert Western philosophy’s long-standing association between aging and wisdom in ways that could afford us with less constrictive profiles of later-life flourishing than seem possible within contemporary dichotomies of youth and age, health and sickness, productive social value and disengaged nostalgia in the guise of ‘experience’. Helen Small details the foundations of this connection in *The Long Life* (2007), where she examines thinkers for whom advancing years were beheld as a time of subdued emotional volatility (or sublimated erotic desire) in which life experience could burnish the intellect towards philosophical acuity. ‘Almost without exception those who philosophize well in the Platonic writings are old,’ Small explains, because the ‘understanding and self-control’ that come with age grant the ‘free[dom] to devote [oneself] to the pursuit of wisdom’. Thus ‘what Platonic philosophy does for old age is . . . to establish thinking about the “declining” years of life as the best possible position . . . from which to do philosophy.’⁴¹ Cicero’s later essay *De senectute* (‘On Old Age’), composed in 44 BCE, makes a similar point, assuring its readers that there is ‘no need of bodily strength’ if there be ‘suavity of utterance – i.e, wise intellect’.⁴² In our

contemporary moment this notion of ‘wisdom as a desired outcome of aging well’ becomes the basis of a range of later-life accounts, from self-help books to the positive psychology movement to literary anthologies of ‘flourishing’.⁴³ In *Being Mortal*, for example – a deeply compassionate study of the harm done by medicine in end-of-life situations – Atul Gawande writes confidently that ‘the calm and wisdom of old age are achieved over time.’⁴⁴

But not all scholars of senescence endorse this picture of wise old age. Woodward, for instance, calls for a ‘moratorium on wisdom’ and vehemently rejects what she calls ‘the cultural reflex’ of layering wisdom on to age. In her own work, including *Aging and Its Discontents* (1991) and a more recent chapter in *Statistical Panic* (2009) called ‘Against Wisdom: Anger and Aging’, Woodward contends that in ‘graciously bestow[ing] a social role’ of wise elder upon a population that is ‘in reality . . . ignored or dismissed as useless’, we simply ‘reproduc[e] a stereotype that doesn’t carry real meaning’.⁴⁵ Instead, Woodward tells us, it’s time to restore all the unseemly affect of adolescents – angst, anger, uncertainty, arousal, shame and also playfulness, curiosity and wit – to their older counterparts.

In what follows, then, I want to consider what happens if we disentangle aging from conventionally articulated quiescent wisdom by rethinking it through hypochondriacal knowing – what I’ve coined in this study as hypistemology. Two classic works of literature dramatize how hypochondria might encourage new models of aging that do not subordinate roily (and obviously ongoing) emotion beneath a sedate – Woodward might say defanged – wisdom: Robert Louis Stevenson’s 1886 *Strange Case of Dr Jekyll and Mr Hyde* and Oscar Wilde’s 1891 *The Picture of Dorian Gray*, which dramatize the perils of denying an anxiously attentive, hypochondriacal maturation in the effort to quell unruliness, to split off aspects of self experienced as intolerable. In Wilde’s novel, Dorian sells his soul to retain his youth and extraordinary beauty; only his portrait ages, recording Dorian’s debauched, hedonistic behaviour in ever more grotesque brushstrokes and colours. Locked in a room for eighteen years, the painting absorbs Dorian’s every unfettered urge, its hideousness manifesting all that Dorian refuses of himself: selfishness, cruelty, deception, hypocrisy – and of course mortality, aligned with a debility he deems antithetical to art. When he ultimately decides to destroy the only material evidence of his

guilt, Dorian slashes the painting with the very knife he had used to kill its artist, believing that this synchrony will ‘kill the past’ and free him from his own temporality; instead the final act of vanity kills Dorian himself. When he is discovered by his servants, he is unrecognizable: ‘a dead man, in evening dress, with a knife in his heart. He was withered, wrinkled and loathsome of visage.’ It is *age* here that marks the body of a degraded spirit, the visibly aged body that provides the shock of moral ugliness – for Dorian is only 38.⁴⁶

But at the same time, the painting ‘had been like conscience to him’, which means that even (or especially) in its most dramatically detestable visual state, the aged painting is somehow effectively *wholesome* – or, to put this another way, it manifests a natural chaos, an unpleasantness, about life that Dorian strives to subdue in his own imagination.⁴⁷ In this sense the portrait operates as Dorian’s hypochondriacal elderly Other, immersed in – suffused with – awareness of contingency, vulnerability, flaw. When Dorian bemoans that he ‘shall grow old, and horrid, and dreadful’, he denies contact with such features of his own humanity; foregoing hypochondria’s alert scrupulosity, he loses himself altogether, hastening the very event he sought to delay.⁴⁸ So too does Stevenson’s Dr Jekyll vie with the inscrutable aspects of self that seem to defy explanation even as they instigate ‘nausea’, ‘gloom’ and fascination. This story famously pits good against evil, but in his first-person account of events that closes the tale, Jekyll admits that since his ‘knowledge does not pass beyond’ that familiar dichotomy, he can only ‘hazard the guess that man will be ultimately known for a mere polity of multifarious, incongruous, and independent denizens’. And ‘if each’, he goes on, ‘could but be housed in separate identities, life would be relieved of all that was unbearable.’⁴⁹ Beyond the inescapable reality that death coincides with life, that age and ailment are attendant upon the bloom of youth, then, it is *mixture* that appals these men, the fact of our suspected but ultimately unknowable innermost reaches and the awesome persistence of sensations and ‘symptoms’ we might wish to jettison from a sense of self.

Curious amalgamations of identity and the dangerous consequences of trying to disentangle ourselves would have been especially meaningful subjects of inquiry for Wilde and Stevenson as nonnormative men, the former imprisoned for ‘indecentcy’ in 1895, the latter a lifelong sufferer of

respiratory problems. As a boy, Stevenson had symptoms consistent with tuberculosis (though now diagnosed by some as a rarer condition like bronchiectasis), and it was pulmonary distress that propelled him away from the dankness of his native Edinburgh to the south of France, to California and a restorative cottage at upstate New York's Saranac Lake, and ultimately to the South Seas, where he settled in Samoa in 1890. Illness frequently kept the young Stevenson away from school, convalescing under the care of his beloved nurse Alison Cunningham, and he was prone to night terrors as a child – bad dreams and sweats.⁵⁰ The writer would later attribute something of his storytelling talents to the workings of a childhood imagination grappling with states of sickness and recuperation, and an intriguing detail of the composition of *Jekyll and Hyde* is that the story came to its ailing, feverish author in a dream. Stevenson grew up to be notoriously thin and seemingly frail (the historian Henry Adams described him as 'emaciated', 'a bundle of sticks in a bag'), and he died very young, at 44, from a stroke.⁵¹ He was eccentric in dress, craved an operatic life and early on espoused progressive ideas, eschewing convention and declaring himself an atheist. He studied engineering and law but did not practise either professionally; as a writer he was enormously prolific, penning poems, short stories, over a dozen novels, travel and political essays, fantasy, romance and science fiction. He loved to travel, though his excursions were often necessitated by, or the cause of, serious illness. Stevenson was, in a sense, both the kindly and learned Dr Jekyll and the daring, rambunctious Mr Hyde, the young in the old, the healthy and the sick – corporeally the object of curiosity, perhaps derision, certainly professional ministrations, as a popular, well-regarded author whose work even in his lifetime garnered great success. A set of contradictions, in other words, entangled by blood and nerves.

Thus it makes sense that Mr Hyde would be wreathed in hypochondriacal nervousness about mystery and proliferation – the embodied self as always on the brink of exceeding its socially and medically approved boundaries, tenacious and precarious at once. Hyde's strangeness and obscurity positions readers and other characters alike in the deciphering of profoundly exaggerated psychosomatic shifts. Hyde is 'deformed somewhere' that cannot be specified. He arouses hypochondria's epistemological compulsiveness, as when the lawyer Utterson feels 'a singularly strong, almost an inordinate, curiosity to behold the features of

the real Mr. Hyde', to 'set eyes on him', as if through such ocular examination 'the mystery would lighten and perhaps roll altogether away'. Yet exactly like a pain or aggrieved body part that eludes diagnosis while causing the worst sensations of panic and sadness, Hyde is an interpretive riddle (his name is an obvious pun): 'the few who could describe him differed widely . . . Only on one point, were they agreed; and that was the haunting sense of unexpressed deformity.'⁵² This is literal: Mr Hyde's shrunken stature relative to Dr Jekyll's is repeatedly mentioned. But as a convenient metaphor, 'deformity' is less pertinent to Hyde's moral characteristics than to onlookers' discomfited awareness that something has altered in an unexpected and elusive fashion; it is the 'unexpressed' quality of Hyde's being that particularly haunts them. Hyde is thus the projection of a generalized suspicion about human embodiment: that we are mutable, prone to shapeshifting in ways we can only pretend to a form of control over. That Jekyll's drug ceases to work – that he comes to experience transformation seemingly against his conscious will – testifies to this fact, but importantly it's one that Jekyll himself refers to as 'natural and human'.⁵³ What is *unnatural*, the story implies, is actually not Hyde at all but rather Jekyll's exaggerated impulse to partition all that he deems impure or corrupt in himself, to excise such aspects of self only because they frighten or disgust – or worse, because they appeal, and in both these senses offend social niceties.

Stevenson was well versed in both psychological and neurological theories of personality, and the *Strange Case* provides ample evidence for numerous compelling interpretations of its depiction of consciousness. A psychoanalytic reading of Oedipal rage and repression makes sense (Hyde murders the fatherly, white-haired Sir Danvers Carew, for instance), as does the contention made by Elaine Showalter and others that the story manifests late Victorian homosexual panic, with Hyde embodying bourgeois fascination with the eroticized figure of the working man. A particular brand of Victorian hypocrisy is often invoked to account for the dualities of such characters as Hyde's 'evil' maid with her 'excellent' manners, or Hyde himself, a 'gentleman' who kills.⁵⁴ Or is Hyde 'original' man? The Victorian obsession with degeneration, what H. G. Wells called 'zoological retrogression' in an 1891 paper, reads Darwinian evolution backwards, fretting that Britain's cultural eminence was in the process of stalling,

civilization on the brink of collapse.⁵⁵ In this context Hyde is an atavistic throwback, emerging from Jekyll like an embarrassingly disobedient genetic trait defying scientific progress.

Yet another reading holds that while Stevenson deliberately refused to explicate his tale with reference to any single point of origin, its portrayal of split personality is neatly explained by the so-called dual-brain. In this theory, the hemispheres of the brain operate independently; they are in effect ‘two perfectly formed brains’, as Anne Stiles explains it – the left side the ‘logical seat’ of rationality, civilization and white masculinity, the right side the ‘feminine [and racialized] seat of emotions, instincts, and the unconscious’. According to some Victorian scientists, mental disorders like ‘multiplex personalities’ were caused by an imbalance of the hemispheres’ cooperation, and some further believed that exertion of the lesser-used brain, stimulating blood flow, could redress the problem in question. Stiles thus argues that Jekyll has ‘initially relie[d] too heavily on his left hemisphere, and then shifts the balance too sharply towards the right’.⁵⁶ This accounts for Jekyll’s remark that he became aware of ‘a more generous tide of blood’ as Hyde ‘[grew] in stature’, indicating an overuse of the right brain he worried he should curtail, lest ‘the balance of [his] nature’ become ‘permanently overthrown’.⁵⁷ Jekyll’s difficulty in this scenario is not twoness per se but rather the lack of equivalence or synergy in the dynamic interaction between his two halves. Indeed, to quote Stiles once more, in performing his ‘fictional corpus callosotomy’, Stevenson ‘explores the potentially heretical possibility that human beings are inherently double even in a healthy state’.⁵⁸ Humanity, as Jekyll says, is naturally ‘composite’.⁵⁹

I describe dual-brain theory because certain details that indicate it as the basis for Jekyll and Hyde’s circumstances (issues of circulation, a suspicion of imbalance in the brain and thus in one’s ‘nature’) also evoke hypochondria’s unsettling conviction of disease, its attachment to offending body parts – and perhaps more specifically the ‘concerns about “not doing well intellectually”’ that were the hallmark of another nineteenth-century psychosomatic ailment, neurasthenia, which included a specific fear of cognitive failure.⁶⁰ Often grouped with melancholy and hypochondria, neurasthenia ‘was associated with a heterogeneous symptom cluster’ including fatigue, muscle weakness, anxiety and depression.⁶¹ Today

neurasthenia has been revived as a disorder akin to syndromes like myalgic encephalomyelitis and chronic fatigue. There is no definitive test, no known cause for ME or CFS – these are differential diagnoses, though sometimes ascribed to viral and bacterial infection or immune imbalance – and that lack of clear pathophysiology results in a persistent disagreement about whether the conditions are psychiatric or physical in origin. The *Strange Case* is an obvious case study, but of what? Cerebral dysfunction that causes personality change, or psychological distress that manifests as ‘a grinding in the bones’, a ‘sickness’ from which an individual comes to, somehow altered and continuous at once?⁶² Or perhaps of hypochondria’s nothingness, its not-yet-discovered something, its shapeshifting protagonist, its story of abiding in multiplicity. From organic malfunction through the ambiguous psychosoma of neurasthenia, we thus return to hypochondria’s defiant status as a mode more than a condition, hypochondria as being-towards-death more than a pathology of rational thinking.

In this context I invoke both the legal and medical denotations of the word ‘case’, since this *Strange Case* entails both doctors and lawyers in the question of what constitutes humanity. The titular characters are bound by biology and legality alike: their bodies are startling versions of each other (connected, at least at first, through a mystified quasi-medication), and the livelihood of one is protected by the will of the other, as Jekyll drafts legal documents to protect Hyde in the event of his absence, thus anticipating his own demise with the same gesture that guarantees Hyde’s continuation. The two figures careen towards death within the frameworks of these institutions, both their promises and their hubris – that we have absolute control of our bodyminds, that we can wield a form of control even beyond death. Jekyll claims in his closing narrative that he is ‘growing toward the elderly man’, that he ‘*prefer[s]* the elderly and discontented doctor’ (my italics) in himself though he is drawn like a kind of addict to Mr Hyde’s ‘comparative youth’, Hyde’s reckless indulgence of pleasure and his *totality*.⁶³ Yet in his grandiose attempt to fashion a version of himself that repudiates his supposedly baser emotional impulses, the doctor demonstrates the monstrous consequences of also repudiating what he calls ‘the trembling immateriality’, ‘the mere aura and effulgence’, of his body – that is, of denying his corporeal self its equal role in establishing his personhood.⁶⁴ So too does Jekyll’s attempt to safeguard Hyde (which is to

say, himself) in the form of legal protections also fail, since Hyde – made maximally *criminal* by virtue of Jekyll's experimentation – cannot, independently or corporeally, survive.

Victorian fears of decadent ontological backsliding tap into a sense that we are individually responsible for upholding the wellness of the species, and it's interesting to consider Mr Hyde as a manifestation of hypochondria's deeply inward sensing in relation to pressure to manage one's health as a matter of moral imperative. But there is another notable feature of the *Strange Case* that problematizes a reading of the story as in any way acquiescent to prevailing public health trends: its foregrounding of narrative pastiche. The story begins in third person, told primarily from the perspective of the lawyer Utterson; it reproduces a letter from Dr Lanyon that describes the horror of witnessing Jekyll's transformation; and it culminates with Jekyll's statement, narrated by an 'I' who refers to both Jekyll and Hyde by name. Who has interpretive authority in these shifting accounts? When Jekyll admits he 'was slowly losing hold', he isn't just referring to his mental state; he points to the story's overt critique of dogmatic medical and scientific privilege, especially over 'right-brain' patients.⁶⁵ Legal and medical manipulation of our apprehension of events at the beginning of the tale gradually gives way to subjective experience, as Jekyll the objective medical practitioner cedes interpretive authority and a different way of knowing takes over – fractured, unruly, resistant, transgressive. If this looks like 'madness', it also resembles the hypochondriacal contestation between physician and patient who struggle for hermeneutic primacy; it even gestures at the determined insistence of the aged to be recognized by a world that fetishizes youth. Jekyll had asserted his prerogative as a man of science to wield absolute control over the state of his body no less than his soul. That his enterprise fails so catastrophically works to undermine the philosophical premises of the institution within which he is made socially legible. But the structure of the story he features in offers an alternative to assuage whatever panic might be inspired by that loss of reliable epistemological certainty, reminding us that we are 'healthiest' in our contingent multiplicity.

Dorian Gray wanted never to be old; Henry Jekyll could not be content with an aging version of himself troubled by what felt like incompatible impulses. The doppelgänger effect undergirds both stories: the creation of externalized resemblances that seem to guarantee our immunity to

inevitable human messiness but mock their originals with the probability that uncoupling can only ever be partial, impermanent and, what's worse, profoundly damaging. The solution for such tortured self-loathing (or reckless effort towards whole(some)ness and 'fitness') is not to disavow our inherently disorganized natures ever more drastically – not to claim a wisdom for old age born of placid reconciliation to impending inconsequence – but to be more hypochondriacal in the cultivation of that dexterous inwardness John Monopoli and Frank Vaccaro claim as selfreliance. To *insist*, in other words, on the continuity, the material *thereness*, of our embodiment, in all its noisy randomness. We focus – but no more so than the young might also do – on the fact of mortality, and on the possibility of damage and disintegration, as ineradicably installed within us. For the aged hypochondriac, far from a cliché, a redundancy or an object lesson in lonely narcissism, navigates the very splits that drive Jekyll and Gray towards self-destruction in the first place.

MANY OF THIS study's hypochondriacs, from *Emma's* Mr Woodhouse and Mrs Churchill to Melville's lawyers; from Kafka's burrowing creature to Maxine Hong Kingston and Charles Darwin; from Caden Cotard in *Synecdoche, New York* to May Sarton and Alice James, epitomize the aging hypochondriac as surprisingly contented with states of anxiety and discombobulation. A certain pride attaches to arriving at old age, care-worn and perhaps actually scarred, with knowledge to impart to generations in one's wake: 'How marvelous to be old', as May Sarton declares in her journal *Recovering*, written in her mid-sixties in the aftermath of breast cancer, 'and to give courage to those younger than oneself!'⁶⁶ That sense of hard-won experience is part of what counteracts the obscure malaise Sarton feels, some twenty years later, as she worries about a recurrence of illness, and it's important to notice that Sarton's 'wisdom' is inextricable from her ongoing sensitivity to bodily unpredictability: hers is hypistemological courage. Even Kingston's mother, who levels conventional threats at her daughter – 'I'm getting so old . . . I'm dropping dead any day now' – also remarks that she's 'amazed [Kingston has] lived to have white hair' of her own, a taunt that works to *liken* rather than disaffect these lifelong embattled women in the shared experience of aging's relentless (or at least

perennial and potential) aches and pains.⁶⁷ Gerontological research demonstrating the relative life satisfaction of later years, taking into account the proportionally higher rate of illness among the elderly, directly challenges popular social messaging that impels older people to perpetuate youth, defined in terms of robust mobility, strength and energy. Contentment, contrary to a deeply entrenched ideology of ability, does not inherently correlate to health.

I acknowledge here those aspects of the story of hypochondria and aging that highlight the deleterious effect of long-term chronic stress on our bodies and minds. A 2018 study links hypochondria in the elderly with loneliness, arguing that ‘lonelier older adults may have an activated threat system which prompts greater intolerance of uncertainty and anxiety.’⁶⁸ This is hypochondria’s inwardness at its least functional, stoking the image of agitated, trepidatious seniors fixated on, because unable to adequately manage, their aches and itches. But what comes first – loneliness, heightening a person’s sense of vulnerability, or an anxious temperament, making one wary of the unfamiliar, reluctant to engage in the messiness of relationship? Hypochondria, stimulating fears of a world full of noxious bodies? Or isolation, sharpening the edges of suspicion, of paranoia? In the authors’ brief account of their study, the salient issue seems to be less health anxiety per se than the absence of interpersonal networks through which to express, assess and calibrate sensation, and again to frame it this way makes hypochondria a secondary function of routine life changes, some of them inherently challenging, rather than a psychogenic pathology.

A grimmer study, led by David Mataix-Cols and reported in *JAMA Psychiatry* in December 2023, found that patients diagnosed with severe hypochondria ‘died at an earlier . . . age’ than people without hypochondria,⁶⁹ with ‘an 84 per cent higher risk of all-cause mortality’⁷⁰ and a ‘4-fold higher risk of death by suicide’.⁷¹ Importantly the authors of this large-scale Swedish project – the first of its kind to investigate the cause of death among clinically diagnosed hypochondriacs with reference to ‘the largest cohort of individuals with a formal diagnosis of hypochondriasis ever studied’⁷² – note that stigma has as much of a deleterious effect on people’s well-being as does sustained anxiety about illness. Arguing that ‘most deaths could be classified as potentially preventable’,⁷³ they attribute hypochondriacal mortality to a system hostile

to apparently unjustified health anxiety, both because practitioners impatient with hypochondria are often less apt to treat its physical consequences and because patients so diagnosed may be reluctant to seek treatment from professionals they fear will ignore or minimize their suffering. ‘Dismissing . . . somatic symptoms as imaginary may have dire consequences’,⁷⁴ the authors warn, since ‘not being properly recognized or taken seriously by health professionals’ potentially exposes the hypochondriac to a cascade of worsening problems.⁷⁵

What interests me in particular about this study is that its warning is directed more at the consequences of stigma than at hypochondria itself; the authors seek to reverse conditions in which the hypochondriac is *unprotected*, rather than the prevalence of hypochondria as a disqualifying condition. What are the implications of such a paradigm for how we might think about hypochondria in relation to longevity? Though the authors don’t name the social model as a construct within which hypochondria is defined as such – they take its diagnosis as a psychiatric disorder as given – they do call attention to a network of social, institutional attitudes and behaviours that, in exacerbating its symptoms, effectively create hypochondria as a life-threatening condition, which implies in turn that what needs to change is not the individual hypochondriac but the barriers that impede access, whether to care or acceptance. To be taken seriously, after all, is not necessarily synonymous with the elimination of symptoms, but to be legitimized in announcing them. What prevents such articulation, if not ageism and cultural resistance to vulnerability, unhealth and a supposedly indecorous self-regard? That an open, perhaps richly emotional declaration of symptoms might also require an uncomfortable degree of self-assertion derives from aging’s contrasted stereotypes – ‘peevis[h] and cantankerou[s]’, on the one hand, to quote Woodward, and on the other serenely heading towards death with ‘coolness of reason and evenness of judgment’, with ‘balanced reflection and judgment’.⁷⁶ Where does a disposition wisely nervous about illness fit between these extremes? And what might change if we recast the culprit of our demise as shame rather than hypochondria, not the fear of illness but the fear of naming a fear?

To answer such questions is to turn our attention once again from individual bodyminds to broadly elaborated networks of meaning that encode aging as the inexorable declension of capabilities, enjoyment and

status, and hypochondria as an embarrassing eruption – perhaps into youth itself – of death’s own opening act. In his poem ‘The Old Fools’, written when he was fifty, the notorious curmudgeon Philip Larkin demands to know of the elderly – with their ‘toad hands’ and ‘prune face[s] dried into lines’, with their ‘air of baffled absence’ – ‘why aren’t they screaming?’ (ll. 23, 12). Surely the old recognize the ‘change’ (l. 8) and recoil in horror from it. That they *don’t* seems nearly as awful as the alterations of age themselves: that some of these ‘old fools’ (l. 1) actually *don’t seem to care*. The poem goes on to inform us that ‘At death, you break up: the bits that were you/ Start speeding away from each other for ever/ With no one to see’ (ll. 13–15).⁷⁷ Helen Small suggests of this moment that ‘what terrifies the speaker about death is our absence from it: we are not just dispersed but unable to witness a dispersion which does not belong to us, because it is *of* us.’⁷⁸ I want to propose here that what might serve to bridge those qualities of aging the speaker mocks as ‘absence’ and the anticipated dispersal of ourselves in the process of dying is hypochondria as a specific form of material *thereness*.

For even as the speaker condemns the elderly as irrevocably stuck in the past – or worse, in a grotesque limbo, their bodies *here* but their minds fully occupied in the ‘lighted rooms/ Inside [their] head[s]’ (ll. 25–6) – he imagines them busy with a deeply interiorized inventory of what they have ‘known’: a word that recurs in quick succession (ll. 28, 30) as if to offset claims elsewhere that age is synonymous with losing just that sort of skilful apprehension of oneself. If it is true that the elderly don’t ‘live’ in the ‘here and now’ (ll. 34–5), then let’s call the time they’re in hyp-time: just how long it takes to consolidate the ‘bits’ of ourselves as we approach the moment of an ultimate dissolution; and let’s recall hypistemological wisdom as late-life knowing, guiding us towards the mysterious potentialities of our embodied lives even as they change and wane. The last lines of Larkin’s poem fret that the aged can ‘never tell’ us (l. 44) what it is that ‘keeps them quiet’ (l. 42), but this is wrong; it is only the apparently hale and healthy onlooker who can’t fathom what so preoccupies the seemingly disengaged, in part because he himself is swaggering in aggressive pitilessness, defended against his own inevitable aging. That posture can’t last, though, and in the end he arrives at something like appreciation – if couched in a shrug – of aging’s hypochondriacal inwardness as a conduit towards knowledge and back outwards again

towards a kind of inclusivity: 'Well', he says of what happens at the last, 'We shall find out' (ll. 47–8).

OceanofPDF.com

Afterword:

Hypochondria Cares

In the Introduction, I explain that I began to think critically about hypochondria as a phenomenon, one that takes its cues from environmental as much as internal factors, at the start of the COVID-19 pandemic. A seven-minute public service video, posted to YouTube by Peacock Presents in 2020, capitalizes on the idea that hypochondria is but degrees of separation from precautionary measures that make sense in the context of communicable disease. Titled ‘Monk in Quarantine’, this short piece features the principals from USA TV’s 2002–9 series *Monk*, about a famously phobic and obsessive detective, played by Tony Shaloub.¹ Here we watch Monk cleaning his fruit in the dishwasher, handling mail with tongs and microwaving it, putting on socks wearing surgical gloves on his hands *and* feet, and washing his hands to the tune of ‘100 bottles of beer on the wall’. These actions look just slightly different from his typical behaviours throughout the series, and the joke is clever: COVID may be Monk’s nightmare come true, but the pandemic comes as no surprise to him, and *he* at least is well prepared. And so he should seem just slightly more ‘obsessive’ than many folks in the spring of that year, when we were taught to wash our hands to two rounds of ‘Happy Birthday’ and touching anything might have felt perilous unless doused in sanitizer. On a video call with former assistant Natalie (Traylor Howard) and police chief Randy Disher (Jason Gray-Stanford), Monk’s colleague Captain Stottlemeyer (Ted Levine) makes the explicit point, ‘Monk, I hate to admit it,’ he says, ‘but you were right,’ and Disher corroborates: ‘We’re all Monk now.’

As I mention in [Chapter Two](#), the pivot required by social distancing measures – to online work and commerce, as a key example – introduced nondisabled people to technologies and accommodations already thoroughly familiar to the disability community. The ‘Monk’ PSA suggests the same of hypochondria. ‘We really are “all Monk now”,’ Shaloub himself

says to the camera after the skit ends. But I'd argue that the effect of 'now' is to subtly confirm that we're *not* in fact stuck in hyp-time – reassuring viewers that, to the contrary, at some point we will *no longer* be Monk, that we will in fact be relieved from the anxiety and repetitiveness, the stultifying loneliness and isolation, that is Monk sheltering-in-place: we may feel 'worried about' Monk (to quote Stottlemeyer again); we may be *imitating* Monk – but we are not finally Monk. And that is the arc our narratives encourage us to have faith in: that we will emerge from this place of fear because health will prevail, and because we are not so 'tightly wound' as *that*.

Still a possibility remains of braiding hypochondria into our understanding of our embodied lives even when the most severe disquiet of the pandemic blurs into memory. Shaloub, gently stroking his dog beside him on the couch, tells us that he and his wife, the actress Brooke Adams, had COVID in late April 2020 – 'a rough few weeks', he calls it. So he assures us that disease will happen – randomly and without regard for status. It is a position from which to speak; Shaloub's seriousness (but also the comforting dog) appeals to us to pay attention, to honour the fact of risk. Shaloub 'is' Monk, to his fans, even fifteen years later. Thus the person speaking to us of both caution and compassion is at once the admired actor and the exaggerated character he portrays: he has disease, he is a hypochondriac; he survived the one, he asks all of us to accept the wisdom, maybe the necessity, of the latter. The palimpsestic effect of all this layers 'relatable' quirks on to the gravitas of fame, the reality of sickness on the ephemera of fear; the singular actor on his couch merges back into the imagined video call, where the actor-characters in their respective squares on a screen ask after each other's well-being. These are the qualities of hypochondria I've tried to bring forth: its knowingness and communicative force; its coexistence with intimacy, work and aging; its habits of caretaking. To say we are all Monk is to say we are all embodied, which is to say we may all, at some point, feel our clangorous, contingent selves on the precipice of *something* and be glad of company.

It hasn't surprised me that when I tell people about working on a hypochondria book, many have assumed the project is a memoir (recall the yellow fever story from my Introduction). It's also not surprising that the news elicits comments like 'Oh, have I got material for you – my [insert relative] is the worst!', or 'Nothing like [insert health event] to give you

hypochondria’. Intrinsic to such reactions are the clichés as well as the truths and the beauty of hypochondria – the fact that it is often triggered by a different serious condition, the presumably shared derision it inspires and the way it resides in *story*, in the impulse to talk and so in the audience, the community, it’s able to create. My hope throughout these pages has been to avoid rehearsing both the joke of hypochondria and its strictly diagnostic trajectory, so as to elucidate its instructive and revelatory potential. I’ve read hypochondria not as a pathology to be subdued or transcended but rather understood for how it shapes our relationship to the extraordinary oddness of ourselves – the way we keep being other than what we expect or want, or are told to fashion. The way we keep *being*, even as we age our way towards an ultimate end.

Towards the start of his ambitious *Cambridge History of Medicine*, Roy Porter states that among the many ‘challenges’ of ‘understanding the history of medicine’ is the fact that ‘illness may be regarded differently by patients and practitioners.’² Perhaps this is not meant as laconically as it sounds, since Porter intends neutrally to distinguish the phenomenological *experience* of sickness from the objective *study* of disease, its causes and cures. But from the vantage point of hypochondria, that difference is slippery in the extreme. Gatekeepers flanking one side, unreliable witnesses the other – or embodied cognition pitted against epistemic haughtiness – such is the teetering see-saw of hypochondria’s disciplinary dispute, one that refuses to uncouple subjective feeling from knowledge, to disentangle experience from expertise. As a condition that spotlights what we dread as well as what we accord value, hypochondria is suspended in the possibility that our uncertainties, left unresolved, might have information to impart.

Another student of illness, the early nineteenth-century physician John Reid, urges fellow medical experts to stop trying to talk their patients out of their suffering. In *Essays on Hypochondriacal and Other Affections*, he puts it this way:

To command, or to advise a person labouring under nervous depression to be cheerful and alert, is no less idle and absurd, than it would be to command or advise a person, under the direct and most intense influence of the sun's rays, to shiver with cold . . . The practice of laughing at, or scolding a patient of this class is equally cruel and ineffectual. No one was ever laughed or scolded out of hypochondriasis. It is scarcely likely that we should elevate a person's spirits by insulting his understanding. The malady of the nerves is in general of too obstinate a nature to yield to a sarcasm or a sneer.³

Given how much professional discussion of the 'hateful patient' makes no attempt to hide its exasperation, I'm struck by the patience, indeed the forgiveness, in this paragraph extended towards the hypochondriac by the doctor. And more, Reid takes for granted both that hypochondriacal patients have a mature intelligence about their circumstances – that patients *know* – and that hypochondria exists somewhere in the body, beyond conscious control (so that to try to *will* it into abeyance is 'absurd'). Hypochondria appears here as a fully psychosomatic circumstance (mind and body, rather than 'all in the head') deserving of inquiry and respect. The fact that we no longer consider hypochondria to be a disorder of 'nerves' (though it is in some quarters fully lodged in the brain) might reveal more about how medicine, psychiatry and neuroscience demarcate and defend their terrain than it does about what a modern hypochondriac feels or needs, or reveals in their own way about the complexity of being embodied in the various social, attitudinal and political spaces I've examined throughout these chapters.

Not that long after starting to work in earnest on this book, I contracted COVID-19, and though I tested negative for the virus after a dozen or so days, I remained 'sick' for a full seven weeks. The crushing fatigue and mental fog – what some online sources explained as common after-effects of the Delta variant – dominated a summer, preventing me from getting much research (or indeed much of anything) done. These symptoms brought home to me the distinct anguish of hypochondriacal ailment. I no longer had COVID in an active sense, yet I was not at all 'well'; I could not organize my thoughts or walk around the block but also couldn't prove these to anyone as legitimate consequences of the virus, let alone qualities

of an ongoing illness. I had always feared the worst; was I simply anxious? I will not forget the hint of boredom that crept into my physician's voice as she offered, over the phone, paltry modes of relief (antihistamines, for example, for the feeling of a head as heavy as a pumpkin) that I recognized neither of us believed would do much good. I wondered what she thought was 'really' going on. Was I exaggerating for attention? Was I expressing an unscientific theory of irreparable damage? Maybe I just represented a problem she could not solve, and so, in a tone of thinly masked impatience, she foisted me off to an ear, nose and throat specialist, who scanned my sinuses and pronounced them unremarkably fine.

In that summer of malaise, I experienced every thematic of hypochondria I've tracked through these pages, from the embodied certitude of something unfamiliar at stake to the sense of inhabiting an aged version of myself; from a reorientation towards the passage of time to an appeal to others for tolerance if not commiseration. I scanned myself daily for signs of change; I tried to bear witness to the *occurrence* of it all. I did my best to calm a fear of never getting 'better'; I texted and emailed and finally started writing my way into narrative – stories that aimed at a truth, a sense-making, hoping for solace. I considered this version of self as the one I might (in fact I did) carry into a new school year, reforging a work identity; I sought, calling on my learned if not corporeal resources, to adjust my expectations, not just for how to feel but for who to be. At some point, I think, phenomena attributable to COVID (like a degree of aphasia) were indistinguishable from panic and synonymous with heading towards sixty. And in the end what mattered was no longer whether I 'had' something diagnosable but how I managed a complicated interaction, set in motion long before I caught the virus, between my mind and body, the communities I inhabit and value, the professional knowledge I do and do not have, my sense of the world's precarity, a lifelong habit of catastrophizing, and recognition of mortality, towards which illness brings us all just a little bit closer.

REFERENCES

Introduction: Hypochondria Exists

- ¹ D. W. Winnicott, 'The Psycho-Somatic Field', in *The Collected Works of D. W. Winnicott*, vol. XI: *Human Nature and the Piggie*, ed. Lesley Caldwell and Helen Taylor Robinson (London, 2017), p. 55.
- ² Catherine Belling, *A Condition of Doubt: The Meanings of Hypochondria* (Oxford, 2012), p. 131.
- ³ For example, the entry on Hypochondriasis in the 1833 *Penny Cyclopaedia of the Society for the Diffusion of Useful Knowledge* tells us that 'there is a class of students who, from the nature of their studies, are frequently affected with it – we allude to students in medicine.'
- ⁴ 'Web search engines have the potential to escalate medical concerns,' say Ryen W. White and Eric Horvitz, authors of a Microsoft Research study on the subject. 'Innocuous symptoms', the authors argue, 'can escalate into the review of content on serious, rare conditions' (Ryen White and Eric Horvitz, 'Cyberchondria: Studies of the Escalation of Medical Concerns in Web Search', *ACM Transactions on Information Systems*, XXVII/4 (November 2009), www.researchgate.net.) The term 'cyberchondria' was coined by the BBC in a 2001 World News piece called 'Cyberchondria Hits Web Users', BBC News, <http://news.bbc.co.uk>, 13 April 2001. See also Vladan Starcevic, 'Cyberchondria: Challenges of Problematic Online Searches for Health-Related Information', *Psychotherapy and Psychosomatics*, LXXXVI/3 (May 2017), pp. 129–33.
- ⁵ Judy Z. Segal, *Health and the Rhetoric of Medicine* (Carbondale, IL, 2005), p. 82.
- ⁶ The annoyance represented by hypochondriacs has given rise to a number of scurrilous acronyms and epithets, including GOMER (get out of my emergency room), crock, goldbrick, groupie and twitch. For more on the slang of hospital care, see David Paul Gordon, 'Hospital Slang for Patients: Crocks, Gomers, Gorks, and Others', *Language in Society*, XII/2 (June 1983), pp. 173–85; and Victoria George and Alan Dundes, 'The Gomer: A Figure of American Hospital Folk Speech', *Journal of American Folklore*, XCI/359 (January–March 1978), pp. 568–81.
- ⁷ Carlton A. Hornung and Michael Massagli, 'Primary-Care Physicians' Affective Orientation toward Their Patients', *Journal of Health and Social Behavior*, XX/1 (March 1979), p. 72.
- ⁸ Margaret Price first used the term 'bodymind' to signal the inseparability of body and mind processes in how people conceive of themselves and interact with their environments. The term has become a central concept in materialist disability studies, interrupting a tendency in the Western philosophical tradition to divorce mental and physical aspects of human experience. Margaret Price, 'The Bodymind Problem and the Possibilities of Pain', *Hypatia*, XXX/1 (2015), pp. 268–84.
- ⁹ Harriet Martineau, *Life in the Sick-Room* [1844], ed. Maria Frawley (Peterborough, ON, 2003), p. 89.
- ¹⁰ Maria Frawley, *Invalidism and Identity in Nineteenth-Century Britain* (Chicago, IL, 2004), p. 222.
- ¹¹ Martineau, *Sick-Room*, p. 118.
- ¹² *Ibid.*, pp. 88–9, 126.
- ¹³ *Ibid.*, p. 11.
- ¹⁴ Harriet Martineau, *How to Observe Morals and Manners* (London, 1838), p. 166, quoted in Ellen Annandale, 'Assembling Harriet Martineau's Gender and Health Jigsaw', *Women's Studies International Forum*, XXX/4 (July–August 2007), p. 357.

- ¹⁵ Harriet Martineau, *Health, Husbandry, and Handicraft* (London, 1861), pp. 22–3, quoted in Annandale, ‘Assembling’, p. 359.
- ¹⁶ Annandale, ‘Assembling’, pp. 360, 361.
- ¹⁷ See Martineau’s impassioned *Letter to the Deaf*, included in Frawley’s edition of *Sick-Room*.
- ¹⁸ Annandale, ‘Assembling’, pp. 362–3.
- ¹⁹ *Ibid.*, p. 360.
- ²⁰ Frawley, *Invalidism*, p. 234.
- ²¹ See Jessica Bright, MD, writing in *Current Psychiatry*, XXV/11 (November 2003), p. 2, https://cdn-uat.mdedge.com/files/s3fs-public/Document/September-2017/0211_Letters2.pdf; ‘Worried about Your Health? You May Have Health Anxiety’, *UCLA Health*, www.uclahealth.org, 16 December 2021; ‘How to Tell if Your Health Concerns Are Normal’, *Time*, <http://time.com>, 20 September 2022; *Psychology Today* has reported a much wider range, from 1.3 to a full 10 per cent of people, ‘Illness Anxiety Disorder’, www.psychologytoday.com.
- ²² John R. Walker and Patricia Furer, ‘Health Anxiety: Hypochondriasis and Somatization’, in *The Handbook of Adult Clinical Psychology*, ed. Alan Carr and Muireann McNulty (New York, 2006), pp. 593, 620.
- ²³ See Todd M. Edwards et al., ‘The Treatment of Patients with Medically Unexplained Symptoms in Primary Care: A Review of the Literature’, *Mental Health Family Medicine*, VII/4 (December 2010), pp. 209–21.
- ²⁴ Denis Pereira Gray, Molly Dineen and Kate Sidaway-Lee, ‘The Worried Well’, *British Journal of General Practice* (February 2020), p. 84.
- ²⁵ Carla Cantor, *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996), p. 8.
- ²⁶ See Giovanni A. Fava and Nicoletta Sonino, ‘The Biopsychosocial Model Thirty Years Later’, *Psychotherapy and Psychosomatics*, LXXVII/1–2 (January 2008), pp. 1–2.
- ²⁷ Richard Klein, ‘What Is Health and How Do You Get It?’ in *Against Health: How Health Became the New Morality*, ed. Jonathan M. Metz and Anna Kirkland (New York, 2010), pp. 16–25, p. 18.
- ²⁸ Michel Foucault, *The Birth of the Clinic: An Archeology of Medical Perception*, trans. A. M. Sheridan Smith (New York, 1994), pp. 114, 115.
- ²⁹ Belling, *Condition of Doubt*, p. 100.
- ³⁰ Klein, ‘What Is Health’, p. 22.
- ³¹ Lauren Berlant, ‘Risky Bigness: On Obesity, Eating, and the Ambiguity of “Health”’, in *Against Health*, ed. Metz and Kirkland, pp. 26–39, p. 28.
- ³² Scott Herring, ‘Wise Old Fools: Positive Geropsychology and the Poetics of Later-Life Floundering’, in *Literary Studies and Human Flourishing*, ed. James F. English and Heather Love (Oxford, 2023), pp. 164–82, p. 164.
- ³³ Steven Connor, ‘Beghosted Bodyhood: Hypochondria and the Arts of Illness’, essay based on talks given at the North-East Network for Medicine and the Arts, St Chad’s College, Durham, 16 June 2008, and Tate Britain, 18 September 2009, p. 2, available at <http://stevenconnor.com>.

1 Pleading the Case: Hypochondria Speaks

- ¹ Emily Paige Wilson, *Hypochondria, Least Powerful of the Greek Gods* (Toledo, OH, 2020), p. 18.
- ² Daniel Defoe, *A Journal of the Plague Year* [1722] (New York, 1960), p. 98.
- ³ *Ibid.*, pp. 29, 42, 44, 83, 91, 93, 108, 155.
- ⁴ *Ibid.*, pp. 30, 31, 63.

- ⁵ Ibid., pp. 29, 38, 237.
- ⁶ Catherine Belling, *A Condition of Doubt: The Meanings of Hypochondria* (Oxford, 2012), p. 10.
- ⁷ Margery Bailey, ed., *Boswell's Column: Being His Seventy Contributions to The London Magazine under the Pseudonym The Hypochondriack from 1777 to 1783* (London, 1951), p. 106.
- ⁸ Robert Burton, *The Anatomy of Melancholy* [1621], ed. Floyd Dell (New York, 1941), p. 351.
- ⁹ Ibid., p. 324.
- ¹⁰ Ibid., pp. 351, 352.
- ¹¹ George Cheyne, *The English Malady: Or, a Treatise of Nervous Diseases of all Kinds* [1733] (New York, 1995), pp. 2–3.
- ¹² German E. Berrios, 'Hypochondriasis: History of the Concept', in *Hypochondriasis: Modern Perspectives on an Ancient Malady*, ed. Vladan Starcevic and Don R. Lipsitt (Oxford, 2001), p. 15.
- ¹³ Susan Baur, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 215, n. 9.
- ¹⁴ Quoted in John M. S. Pearce, 'Sydenham on Hysteria', *European Neurology*, LXXVI/3–4 (October 2016), p. 176.
- ¹⁵ Burton, *Anatomy*, pp. 602, 603.
- ¹⁶ Quoted in Pearce, 'Sydenham', p. 177.
- ¹⁷ In fact, locating the problem in the digestive tract, Burton pre-dates current theories of the role of the gut microbiome in psychological as well as physiological disease. As James Kinross says of the gut microbiome, quoted in an article in *The Guardian* in 2021, scientists have 'discovered it – or rediscovered it – in the age of genetic sequencing', reminding us that older medical paradigms were based on theories about this vital part of the body. Current thinking attributes cancer, diabetes and autism spectrum disorders to disorders of the gut. Rebecca Seal, 'Unlocking the Gut Microbiome and Its Massive Significance to Our Health', *The Guardian*, www.theguardian.com, 11 July 2021.
- ¹⁸ Burton, *Anatomy*, p. 330.
- ¹⁹ Floyd Dell, from his introduction to Burton, *Anatomy*, p. ix.
- ²⁰ Arthur J. Barsky, 'Somatosensory Amplification and Hypochondriasis', in *Hypochondriasis*, ed. Starcevic and Lipsitt, p. 244.
- ²¹ Theodore Dalrymple, 'Hypochondria as Control', *British Medical Journal*, CCCXLI/7777 (16 October 2010), p. 837.
- ²² John Wain, ed., *The Journals of James Boswell 1762–1795* (New Haven, CT, 1991), p. 83.
- ²³ Ibid., pp. 83, 84.
- ²⁴ Ibid., p. 79.
- ²⁵ Frederick A. Pottle, *James Boswell: The Earlier Years, 1740–1769* (New York, 1985), p. 178.
- ²⁶ Wain, *Journals*, p. 92.
- ²⁷ Joseph W. Reed, ed., *Boswell, Laird of Auchinleck, 1778–1782* (New York, 1977), pp. 155, 159.
- ²⁸ Allan Beveridge, "'Teetering on the Verge of Complete Sanity": Boswell's Life of Boswell', *Journal of the Royal Society of Medicine*, XCIII/8 (August 2000), p. 434.
- ²⁹ Jess Keiser, *Nervous Fictions: Literary Form and the Enlightenment Origins of Neuroscience* (Charlottesville, VA, 2020), pp. 219, 224, 225.
- ³⁰ As when Boswell writes in his journal of 22 July 1763 that he 'complained to Mr Johnson that [he] was much afflicted with melancholy, which was hereditary in [his] family' (Pottle, *James Boswell*, p. 319).
- ³¹ Wain, *Journals*, pp. 84, 93.
- ³² Ibid., pp. 96–7.
- ³³ Ibid., p. 97.
- ³⁴ Ibid., p. 96.

- [35](#) Ibid., p. 110.
- [36](#) Reed, *Boswell*, p. 276.
- [37](#) Ibid., p. 279.
- [38](#) Quoted in Baur, *Hypochondria*, p. 17.
- [39](#) Carla Cantor, *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996), pp. 112, 113, 126.
- [40](#) Robert Hutchison, 'Hypochondriasis: Individual, Various, and Communal', *British Medical Journal*, I/3817 (3 March 1934), p. 367.
- [41](#) George C. Grinnell, 'Thomas Beddoes and the Physiology of Romantic Medicine', *Studies in Romanticism*, XLV/2 (Summer 2006), p. 227.
- [42](#) Oswald Doughty, 'The English Malady of the Eighteenth Century', *Review of English Studies*, II/7 (July 1926), p. 258; Blackmore's 1725 *Treatise of the Spleen and Vapours* and A Critical Dissertation on the Spleen are cited in Doughty, 'The English Malady', p. 259.
- [43](#) David Heyd, 'The Medicalization of Health: Plato's Warning', *Revue Internationale de Philosophie*, XLIX/193 (September 1995), pp. 377–8; Plato, *Republic*, quoted in Heyd, pp. 376–7.
- [44](#) Heyd, 'The Medicalization of Health', p. 390.
- [45](#) Burton, *Anatomy*, p. 188. In 1987 Pat Robinson notoriously opposed the expenditure of 'government money' on what he deemed an 'aberrant lifestyle'. A few years later, in 1993, he even more explicitly moralized AIDS, pronouncing in a speech that 'a new plague is stalking our land, because the people have cast aside the sexual morals of the Holy Bible.' See Miranda Blue, 'Flashback: When Pat Robertson Opposed AIDS Research to Help Those With "Aberrant Lifestyle"', Right Wing Watch, www.rightwingwatch.org, 27 August 2013.
- [46](#) Judy Z. Segal, *Health and the Rhetoric of Medicine* (Carbondale, IL, 2005), p. 83.
- [47](#) Ibid., p. 88.
- [48](#) Ibid., p. 89.
- [49](#) Ibid., pp. 89, 83.
- [50](#) Ibid., p. 79.
- [51](#) Belling, *Condition of Doubt*, p. 158.
- [52](#) Cantor, *Phantom Illness*, p. 13.
- [53](#) Baur, *Hypochondria*, p. 79.
- [54](#) Ibid., pp. 30, 56, 72–4, 91.
- [55](#) Cantor makes similar claims: 'Hypochondriacs learn that pain brings love, but the behavior is also an aggressive means of punishment. Hypochondria, in a psychoanalytic context, is a way of displacing anger' and other 'complicated feelings' of 'dependence, hostility, and guilt' (*Phantom Illness*, pp. 90–91). Theodore Dalrymple also takes for granted the 'use of symptoms to control someone' as a feature of hypochondria and writes of an Aldous Huxley story, 'Chawdron', that 'This is a story to stir the fear of every doctor who has ever been irritated by a hypochondriac – that is to say, every doctor' ('Hypochondria as Control', p. 837).
- [56](#) Segal, *Health*, p. 75.
- [57](#) James Johnson, *An Essay on Morbid Sensitivity of the Stomach and Bowels as the Proximate Cause, or Characteristic Condition of Indigestion, Nervous Irritability, Mental Despondency, Hypochondriasis* &. & (London, 1827); Ralph Fletcher, *Sketches from the Case Book, to Illustrate the Influence of the Mind on the Body with the Treatment of Some of the More Important Brain & Nervous Disturbances which Arise from this Influence* (London, 1833).
- [58](#) Thomas King Chambers, MD, 'Lecture on Hypochondriasis', delivered at St Mary's Hospital, Wales, 25 June 1873, printed in the *British Medical Journal*, II/653 (5 July 1873), p. 6.
- [59](#) Charlotte Brontë, *Villette* [1853] (London, 2004), p. 303.

- [60](#) Ibid., pp. 39, 40, 52, 73.
- [61](#) Ibid., pp. 205, 209, 303.
- [62](#) Rachel Ablow, 'Hypochondria and the Failure of Relationship', *Nineteenth-Century Gender Studies*, XI/3 (Winter 2015), paras 8 and 9, www.ncgsjournal.com.
- [63](#) In Daniel 4.5–7, Nebuchadnezzar 'saw a dream which made me afraid, and the thoughts upon my bed and the visions of my head troubled me. Therefore made I a decree to bring in all the wise men of Babylon before me, that they might make known unto me the interpretation of the dream. Then came in the magicians, the astrologers, the Chaldeans, and the soothsayers: and I told the dream before them; but they did not make known unto me the interpretation thereof.'
- [64](#) Brontë, *Villette*, p. 238.
- [65](#) D. A. Miller, 'The Late Jane Austen', *Raritan*, X/1 (Summer 1990), p. 66.
- [66](#) From an 1853 letter to Elizabeth Gaskell in *The Letters of Charlotte Brontë*, vol. III: 1852–1855, ed. Margaret Smith (Oxford, 2004), p. 182.
- [67](#) Chambers, 'Lecture', p. 8.
- [68](#) Bernard Cooper, *Maps to Anywhere* (Athens, GA, 1990), pp. 54, 56.
- [69](#) Ibid., p. 56.
- [70](#) Jane Austen, *Emma* [1815] (New York, 2012), p. 15.
- [71](#) Belling, *Condition of Doubt*, p. 52. Hutchison's other two categories are individual hypochondria and vicarious (fear for the health of a loved one).
- [72](#) The World Health Organization estimated nearly 7 million deaths from COVID-19 as of April 2023.
- [73](#) Sarah Mann-O'Donnell, 'From Hypochondria to Convalescence: Health as Chronic Critique in Nietzsche, Deleuze and Guattari', *Deleuze Studies*, IV/2 (July 2010, special issue on Deleuze and the Symptom), p. 174.
- [74](#) Wilson, *Hypochondria*, p. 16.
- [75](#) Ibid., p. 18.
- [76](#) Ibid., pp. 21, 25.
- [77](#) Ibid., p. 26.
- [78](#) Ibid., p. 38.

2 Bodies in Time: Hypochondria Stalls

- [1](#) William Greenway, 'Hypochondria', *Poetry*, CLIX/1 (October 1991), p. 14.
- [2](#) John Reid, *Essays on Hypochondriacal and Other Nervous Affections* (London, 1817), p. 413, available at Internet Archive, <http://archive.org>.
- [3](#) Thomas King Chambers, MD, 'Lecture on Hypochondriasis', delivered at St Mary's Hospital, Wales, 25 June 1873, printed in the *British Medical Journal*, II/653 (5 July 1873), p. 8. John Wain, ed., *The Journals of James Boswell 1762–1795* (New Haven, CT, 1991), p. 97.
- [4](#) An especially lively rant, printed in *The Guardian* in 2013, dubbed the health-anxious 'pest-patients' and 'professional timewasters' and claimed that no reliable data can be gathered on just how *much* time they take up 'because hypochondriacs are shameless liars'. Barbara Ellen, 'The Perfect Pill for the "Worried Well" – The Placebo', *The Guardian*, www.theguardian.com, 23 March 2013.
- [5](#) Susan Baur, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 46.
- [6](#) Jennifer Traig, *Well Enough Alone: A Cultural History of My Hypochondria* (New York, 2008), pp. 78, 83.

- ⁷ Scholarship on disability and work is extensive. See, as one brief example, Sarah F. Rose's entry on 'Work' in *Keywords for Disability Studies*, which explains that disability 'has often been equated with the inability to do productive work', even as many forms of impairment have historically been caused by work (Sarah F. Rose, 'Work', in *Keywords for Disability Studies*, ed. Rachel Adams, Benjamin Reiss and David Serlin (New York, 2015), p. 187).
- ⁸ Carla Cantor, *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996), p. 63.
- ⁹ Anne Hunsaker Hawkins, *Reconstructing Illness: Studies in Pathography* (West Lafayette, IN, 1998), pp. 27, 78, 79.
- ¹⁰ Cantor, *Phantom*, p. 30.
- ¹¹ Baur, *Hypochondria*, p. 115, my italics.
- ¹² Helen Tyrer, senior clinical research fellow at Imperial College London, quoted in David Robson, 'Hypochondriacs: Why It's Not Always Just in the Mind', BBC News, www.bbc.com, 15 February 2022.
- ¹³ Jane Austen, *Emma* [1815] (New York, 2012), p. 7.
- ¹⁴ Ibid., p. 74.
- ¹⁵ Ibid., pp. 90, 98.
- ¹⁶ Ibid., pp. 7, 74, 90, 98.
- ¹⁷ Ibid., pp. 13, 87.
- ¹⁸ Ibid., pp. 217–18.
- ¹⁹ D. A. Miller, 'The Late Jane Austen', *Raritan*, X/1 (Summer 1990), p. 67.
- ²⁰ Austen, *Emma*, p. 229.
- ²¹ Ibid., pp. 95, 230, 231.
- ²² Miller, 'Late Jane Austen', p. 66.
- ²³ The authors of a study of hypochondriacs' assessment of health risks conclude that it is *nonhypochondriacal* patients who 'manifes[t] a cognitive distortion, since they are more inappropriately and unrealistically optimistic and dismissive of the medical risks they face'. Arthur J. Barsky et al., 'Hypochondriacal Patients' Appraisal of Health and Physical Risks', *American Journal of Psychiatry*, CXLVIII/7 (July 1991), p. 787.
- ²⁴ Austen, *Emma*, pp. 320–22.
- ²⁵ Claudia L. Johnson, 'Emma: Woman, Lovely Woman Reigns Alone', excerpted *ibid.*, p. 427; Austen, *Emma*, p. 299.
- ²⁶ Emma tells her betrothed, 'I never can call you any thing but "Mr. Knightley"': Austen, *Emma*, p. 318. Knightley has addressed Emma by first name throughout the novel, but is there a sly hint here, that 'Miss Woodhouse' might also carry on after marriage?
- ²⁷ Austen, *Emma*, p. 333.
- ²⁸ Ann Jurecic, *Illness as Narrative* (Pittsburgh, PA, 2012), p. 60.
- ²⁹ Alexander Nazaryan, 'Internal Affairs: On Hypochondria', *New Yorker* (2 August 2012).
- ³⁰ Sarah Mann-O'Donnell, 'From Hypochondria to Convalescence: Health as Chronic Critique in Nietzsche, Deleuze and Guattari', *Deleuze Studies*, IV/2 (July 2010, special issue on Deleuze and the Symptom), p. 175.
- ³¹ Quoted in Brian Dillon, *The Hypochondriacs: Nine Tormented Lives* (New York, 2009), p. 127.
- ³² George Sebastian Rousseau and David Boyd Haycock, 'Framing Samuel Taylor Coleridge's Gut: Genius, Digestion, Hypochondria', in George Sebastian Rousseau et al., *Framing and Imagining Disease in Cultural History* (London, 2003), p. 231.

- ³³ Thomas Trotter, *A View of the Nervous Temperament; Being a Practical Enquiry into the Increasing Prevalence, Prevention, and Treatment of Those Diseases Commonly Called Nervous, Bilious, Stomach, and Liver Complaints; Indigestion; Low Spirits; Gout, &c.* (2nd edn, London, 1807), p. 205; Rousseau and Haycock, 'Coleridge's Gut', p. 238. Robert James's 1743–5 *Medical Dictionary* defines hypochondriasis as 'a spasmodic-flatulent Disorder of . . . the Stomach and Intestines, arising from an Inversion or Perversion of their peristaltic Motion . . . throwing the whole nervous System into irregular Motions' (quoted in Trotter, *A View of the Nervous Temperament*, p. 240).
- ³⁴ In a letter to Josiah Wade of June 1814, Coleridge acknowledged that he had 'been attempting to beat off pain, by a constant recurrence to the vice that reproduces it'. In Samuel Taylor Coleridge, *Collected Letters of Samuel Taylor Coleridge*, ed. E. L. Griggs, vol. III (Oxford, 1956–71), p. 511.
- ³⁵ Rousseau and Haycock, 'Coleridge's Gut', pp. 244, 250.
- ³⁶ Ibid., pp. 234, 252.
- ³⁷ Coleridge, *Collected Letters*, vol. I, pp. 460, 484.
- ³⁸ Thais Klein and Regina Herzog, 'Hypochondriacal Anxiety and the Eternal Recurrence of the Present: Considerations about the Film *Synecdoche, New York*', *Psicologia USP*, XVIII/2 (May 2017), p. 250.
- ³⁹ Coleridge, *Collected Letters*, vol. II, p. 782.
- ⁴⁰ George C. Grinnell, *The Age of Hypochondria: Interpreting Romantic Health and Illness* (London, 2010), p. 59.
- ⁴¹ Rousseau and Haycock, 'Coleridge's Gut', p. 249.
- ⁴² Grinnell, *Age of Hypochondria*, p. 60.
- ⁴³ Coleridge, *Collected Letters*, vol. II, pp. 897, 902.
- ⁴⁴ Grinnell, *Age of Hypochondria*, p. 62.
- ⁴⁵ Charles Darwin, *The Autobiography of Charles Darwin, 1809–1882*, ed. Nora Barlow (London, 1958), pp. 79–80.
- ⁴⁶ Dillon, *The Hypochondriacs*, p. 85.
- ⁴⁷ Anthony K. Campbell and Stephanie B. Matthews, 'Darwin's Illness Revealed', *Postgraduate Medicine*, LXXXI/954 (April 2005), p. 249. One detail seems especially spurious: 'examination of the timing of Darwin's episodes of ill health show that they do not correlate with dates that coincide with memories of his mother' – as if memories could be documented with such precision, and as if bereavement or trauma only works on us consciously.
- ⁴⁸ Quoted in Gavin de Beer, ed., *Charles Darwin, Thomas Henry Huxley: Autobiographies* (Oxford, 1974), p. 56.
- ⁴⁹ Francis Darwin, ed., *The Life and Letters of Charles Darwin*, vol. I (London, 1897), p. 105.
- ⁵⁰ Mason Currey's survey of famous 'daily rituals' features 162 creative types: musicians, philosophers, artists, novelists, playwrights, poets, psychologists, anthropologists, choreographers, cartoonists: *Daily Rituals: How Artists Work* (New York, 2013).
- ⁵¹ Dillon, *The Hypochondriacs*, p. 97; Baur, *Hypochondria*, p. 184.
- ⁵² Darwin, *Autobiography*, p. 70.
- ⁵³ Dillon, *The Hypochondriacs*, p. 78.
- ⁵⁴ Letter from C. R. Darwin to J. D. Hooker [11 January 1844] (MS DAR 114: 3), <https://cudl.lib.cam.ac.uk>.
- ⁵⁵ Henry James Sr, 'Woman and the "Woman's Movement"', *Putnam's Monthly* (1 March 1853), pp. 279, 281, 286.
- ⁵⁶ Diary entry of February 1890, in Leon Edel, ed., *The Diary of Alice James* (New York, 1964), p. 95.

- 57 Maria Frawley, 'Chronic Pain and Illness', in *A Cultural History of Disability*, vol. V: *A Cultural History of Disability in the Long Nineteenth Century*, ed. Joyce L. Huff and Martha Stoddard Holmes (London, 2020), p. 61.
- 58 Harriet Martineau, *Life in the Sick-Room* [1844], ed. Maria Frawley (Peterborough, ON, 2003), p. 78.
- 59 Diary entry of 26 October 1890, in Edel, *Diary of Alice James*, p. 149.
- 60 James Sr, 'Woman', p. 282.
- 61 Klein and Herzog, 'Hypochondriacal Anxiety', pp. 250, 252.
- 62 Talal Asad, 'Thinking about the Secular Body, Pain, and Liberal Politics', *Cultural Anthropology*, XXVI/4 (November 2011), pp. 658, 659, 666, 669, 672; Martineau, *Sick-Room*, p. 43.
- 63 Edel, *Diary of Alice James*, p. 206.
- 64 Klein and Herzog, 'Hypochondriacal Anxiety', pp. 248–50.
- 65 Manohla Dargis, 'Dreamer, Live in the Here and Now' (film review), *New York Times*, www.nytimes.com, 23 October 2008; Peter Bradshaw, 'Synecdoche, New York' (film review), *The Guardian*, www.theguardian.com, 14 May 2009.
- 66 Dargis, 'Dreamer'.
- 67 Dillon, *The Hypochondriacs*, p. 144.
- 68 See, for example, 'No, the COVID-19 Pandemic Isn't Over', Cleveland Clinic, <http://health.clevelandclinic.org>, 25 May 2023, and Molly Nebiolo, 'The Risks of Declaring the Pandemic Over Too Soon', *Washington Post*, www.washingtonpost.com, 23 September 2022.
- 69 Paul M. Salkovskis and Hilary M. C. Warwick, 'Meaning, Misinterpretations, and Medicine: A Cognitive-Behavioral Approach to Understanding Health Anxiety and Hypochondriasis', in *Hypochondriasis: Modern Perspectives on an Ancient Malady*, ed. Vladan Starcevic and Don R. Lipsitt (Oxford, 2001), pp. 218–19.
- 70 Elizabeth Freeman and Ellen Samuels, *Crip Temporalities*, special issue of *South Atlantic Quarterly*, CXX/2 (April 2021), pp. 246, 249.

3 Hyp-istemology: Hypochondria Knows

- 1 Charlotte Brontë, *Villette* [1853] (London, 2004).
- 2 Molière, *The Imaginary Invalid* [1673], trans. Charles Heron Wall (2003), www.gutenberg.org, p. 67.
- 3 Ibid.
- 4 Béalde suggests as much, telling Argan that the best course of action when we're ill is to do 'Nothing': 'Only rest. Nature, when we leave her free, will herself gently recover from the disorder into which she has fallen. It is our anxiety, our impatience, which does the mischief, and most men die of their remedies, and not of their diseases' (Act III, scene 3): *ibid.*, p. 65.
- 5 Steven Connor, 'Beghosted Bodyhood: Hypochondria and the Arts of Illness', essay based on talks given at the North-East Network for Medicine and the Arts, St Chad's College, Durham, 16 June 2008, and Tate Britain, 18 September 2009, p. 5, available at <http://stevenconnor.com>.
- 6 Sneha Mantri, 'Hypochondria and the Struggle for Control', *Synapsis: A Health Humanities Journal*, <https://medicalhealthhumanities.com>, 10 November 2017.
- 7 Connor, 'Beghosted', p. 18.
- 8 Molière, *Imaginary Invalid*, pp. 73–4.
- 9 Will Rees, 'Hypochondriacal Reading: Phantom Illness and Literature', *Oxford Literary Review*, XLIII/2 (December 2021), p. 296.

- [10](#) Carlton A. Hornung and Michael Massagli, 'Primary-Care Physicians' Affective Orientation toward Their Patients', *Journal of Health and Social Behavior*, XX/1 (March 1979), pp. 61–76.
- [11](#) This seems true even if practitioners have encountered the precepts of Narrative Medicine or 'whole person' approaches to healthcare at some stage in their education. See Rita Charon, who first used the term 'narrative medicine' in *JAMA* in 2001 (CCLXXXVI/15, pp. 1897–1902) and now serves as Chief of the Division of Narrative Medicine in the Columbia University Department of Medical Humanities and Ethics.
- [12](#) Catherine Belling, *A Condition of Doubt: The Meanings of Hypochondria* (Oxford, 2012), p. 96; Susan Baur, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 100. Jerome Groopman tells us in *How Doctors Think* (Boston, MA, 2008) that 'few of us realize how strongly a physician's mood and temperament influence his medical judgment', even though evidence-based medicine trains doctors to 'base all treatment decisions strictly on statistically proven data' rather than 'thinking independently and creatively' (pp. 5, 8).
- [13](#) James E. Groves, 'Taking Care of the Hateful Patient', *New England Journal of Medicine*, CCXCVIII/16 (20 April 1978), pp. 883–7. The ongoing currency of Groves's taxonomy may be indicated by the frequency with which the article continues, even quite recently, to be referred to, if also revised. See for example Richard B. Gunderman and Peter R. Gunderman, 'Forty Years Since "Taking Care of the Hateful Patient"', *AMA Journal of Ethics*, XIX/4 (April 2017), pp. 369–73.
- [14](#) Belling, *Condition of Doubt*, pp. 95–6.
- [15](#) *Ibid.*, p. 91.
- [16](#) Yvonne Wechuli, 'Between Crippling and Reclaiming: Epistemological Implications of Disability Studies' Feeling Strategies', *Emotions and Society*, IV/2 (April 2022), p. 150.
- [17](#) Rees, 'Hypochondriacal Reading', p. 296.
- [18](#) Somatic symptom disorder also subsumed the prior diagnosis of somatization disorder and no longer requires a specific number of symptoms for an official diagnosis.
- [19](#) Belling calls attention to the 'Columbia University and New York State Psychiatric Institute's Program for the study and treatment of Heightened Illness Concern, Health Anxiety, and Medically Unexplained Symptoms', no longer in operation, as an example of the field's concerted effort to avoid the stigma of 'hypochondria'. Belling, *Condition of Doubt*.
- [20](#) Rees, 'Hypochondriacal Reading', p. 298.
- [21](#) Komarine Romdenh-Romluc, 'Agency and Embodied Cognition', *Proceedings of the Aristotelian Society*, CXI/1, Part 1 (April 2011), p. 80.
- [22](#) Maurice Merleau-Ponty, *The Phenomenology of Perception*, trans. Colin Smith (London, 1962), pp. 146, 408.
- [23](#) Mark Rowlands, 'Intentionality and Embodied Cognition', *Philosophical Topics*, XXXIX/1 (Spring 2011), p. 82.
- [24](#) Antonio R. Damasio, *Descartes' Error: Emotion, Reason, and the Human Brain* (New York, 1994), pp. 152, 154–5, 233.
- [25](#) Robert McRuer and Merri Lisa Johnson, eds, Special 'Cripistemologies' issues of the *Journal of Literary and Cultural Disability Studies*, VIII/1–2 (May 2014), p. 137.
- [26](#) Frederic Neuman, *Worried Sick? The Exaggerated Fear of Physical Illness* (Hollywood, FL, 2016), p. 65.
- [27](#) Don R. Lipsitt, 'Psychodynamic Perspectives on Hypochondriasis', in *Hypochondriasis: Modern Perspectives on an Ancient Malady*, ed. Vladan Starcevic and Don R. Lipsitt (Oxford, 2001), p. 184.
- [28](#) Ernest Jones, ed., *Sigmund Freud Life and Work*, vol. II: *Years of Maturity, 1901–1919* (London, 1955), p. 453.

- [29](#) Sigmund Freud, 'On Narcissism: An Introduction', in *The Standard Edition of the Complete Psychological Works*, ed. James Strachey, vol. XIV (London, 1957), p. 84.
- [30](#) Baur, *Hypochondria*, p. 29.
- [31](#) Belling, *Condition of Doubt*, p. 48.
- [32](#) Steven Reidbord, 'A Brief History of Psychiatry', *Psychology Today*, www.psychologytoday.com, 20 October 2014.
- [33](#) Andrew Scull, 'Contested Jurisdictions: Psychiatry, Psychoanalysis, and Clinical Psychology in the United States, 1940–2010', *Medical History*, LV/3 (July 2011), pp. 402, 403.
- [34](#) *Ibid.*, p. 405.
- [35](#) Melvin Sabshin, 'Turning Points in Twentieth-Century American Psychiatry', *American Journal of Psychiatry*, CXLVII/10 (October 1990), pp. 1267–74.
- [36](#) Quoted in Edward G. Jones and Lorne M. Mendell, 'Assessing the Decade of the Brain', *Science*, CCLXXXIV/5415 (30 April 1999), p. 739.
- [37](#) Scull, 'New Psychiatry', p. 405.
- [38](#) Belling, *Condition of Doubt*, p. 51.
- [39](#) A summary of the 'Controversy Over DSM-5' can be found at *Nursing Times*, www.nursingtimes.net, 24 August 2013. Particularly vociferous was Allen Frances, who chaired the APA task force responsible for the 1994 fourth edition; Frances is an outspoken critic of the U.S. mental health system for its exclusionary practices and perpetuation of stigma.
- [40](#) Reidbord, 'Brief History'.
- [41](#) Quoted in Connor, 'Beghosted', p. 12.
- [42](#) *Ibid.*, p. 23.
- [43](#) John Diamond, *Because Cowards Get Cancer Too: A Hypochondriac Confronts His Nemesis* (London, 1998), pp. 8, 12.
- [44](#) Jane Austen, *Emma* [1815] (New York, 2012), pp. 76, 77. Subsequent quotes in this paragraph are from these pages.
- [45](#) David W. Butler, 'Usher's Hypochondriasis: Mental Alienation and Romantic Idealism in Poe's Gothic Tales', *American Literature*, XLVIII/1 (March 1976), pp. 1–12.
- [46](#) Edgar Allan Poe, *The Fall of the House of Usher, and Other Tales* (New York, 1960), pp. 116, 118, 131.
- [47](#) Butler, 'Usher's Hypochondriasis', p. 11.
- [48](#) Poe, *Usher*, pp. 119, 125; Brian Dillon, *The Hypochondriacs: Nine Tormented Lives* (New York, 2009), p. 144.
- [49](#) Butler, 'Usher's Hypochondriasis', p. 11.
- [50](#) Poe, *Usher*, pp. 113, 114, 125.
- [51](#) *Ibid.*, pp. 120, 127, 129.
- [52](#) Hilary M. C. Warwick and Paul M. Salkovskis, 'Cognitive-Behavioral Treatment of Hypochondriasis', in *Hypochondriasis*, ed. Starcevic and Lipsitt, p. 317.
- [53](#) Robert Kellner, *Somatization and Hypochondriasis* (New York, 1986).
- [54](#) Paul M. Salkovskis and Hilary M. C. Warwick, 'Meaning, Misinterpretations, and Medicine: A Cognitive-Behavioral Approach to Understanding Health Anxiety and Hypochondriasis', in *Hypochondriasis*, ed. Starcevic and Lipsitt, p. 204.
- [55](#) Rees, 'Hypochondriacal Reading', pp. 291, 312.
- [56](#) Gilles Deleuze, 'Bartleby; or, The Formula', in *Essays Critical and Clinical*, trans. Daniel W. Smith and Michael A. Greco (Minneapolis, MN, 1997), pp. 68–90. Available in *On Curating*, 40 (September 2018), www.on-curating.org, pp. 49–64.
- [57](#) Herman Melville, 'Bartleby, the Scrivener: A Story of Wall-Street', in *The Piazza Tales* (New York, 1856), p. 10.

- [58](#) Ibid., pp. 6, 7, 9.
- [59](#) Ibid., pp. 19, 21, 23, 24, 29.
- [60](#) Ibid., pp. 10, 16, 20.
- [61](#) Ibid., pp. 15, 16, 25, 26; Deleuze, 'Bartleby'.
- [62](#) Melville, 'Bartleby the Scrivener', p. 25.
- [63](#) Ibid., pp. 17, 20, 23, 24, 26.
- [64](#) Ibid., p. 1.
- [65](#) Ibid., p. 29.
- [66](#) Ibid., pp. 20, 22, 23, 27.
- [67](#) Ibid., p. 25.
- [68](#) John Reid, *Essays on Hypochondriacal and Other Nervous Affections* (London, 1817), p. 11.
- [69](#) Lauren A. McCormack, et al., 'Gaps in Knowledge About COVID-19 Among U.S. Residents Early in the Outbreak', *Public Health Reports*, CXXXVI/1 (January/February 2021), pp. 107–16.
- [70](#) Quoted in Rahul Parikh, 'The Real Reason Hypochondriacs Drive Doctors Crazy', *Salon*, www.salon.com, 7 August 2011. In a 2022 *LA Times* interview, Dr Anthony Fauci, former Director of the National Institute of Allergy and Infectious Diseases, said of this phenomenon during the early phase of the pandemic that 'people sometimes said, "You're flip-flopping". It has nothing to do with flip-flopping! It was a case of continuing to make decisions based on the latest and most accurate data you have. If you stick with the science, you're going to have to be prepared to change as the facts evolve': Melissa Healy, 'Fauci's Warning to America', *LA Times*, 20 December 2022.
- [71](#) Larry Au et al., 'Long Covid and Medical Gaslighting: Dismissal, Delayed Diagnosis, and Deferred Treatment', *SSM – Qualitative Research in Health*, II (December 2022), pp. 1–11. Other such conditions include chronic fatigue syndrome, fibromyalgia and complications from Lyme disease. I take up the credibility question in [Chapter Five](#).
- [72](#) Arthur Kleinman, *The Illness Narratives: Suffering, Healing, and the Human Condition* (New York, 1988), pp. 5–6.
- [73](#) Hilary Mantel, *Giving Up the Ghost* (New York, 2004), p. 160.
- [74](#) Diamond, *Cowards*, pp. 3, 8, 9, 67.
- [75](#) Ibid., pp. 52, 68. Susan Sontag's now-classic *Illness as Metaphor* set the bar for interrogating the damaging effect of martial imagery in discussions and representations of cancer.

4 Bodies of Matter: Hypochondria Consolidates

- [1](#) Jennifer Traig, *Well Enough Alone: A Cultural History of My Hypochondria* (New York, 2008).
- [2](#) George Cheyne, *The English Malady: Or, a Treatise of Nervous Diseases of all Kinds* [1733] (New York, 1995), preface.
- [3](#) Eric Gidal, 'Civic Melancholy: English Gloom and French Enlightenment', *Eighteenth-Century Studies*, XXXVII/1 (Fall 2003), p. 24.
- [4](#) Ibid., p. 26.
- [5](#) Justine S. Murison, 'Hypochondria and Racial Interiority in Robert Montgomery Bird's *Sheppard Lee*', *Arizona Quarterly*, LXIV/1 (Spring 2008), p. 6.

- ⁶ ‘Certain accidents of the weather’, says Lucy Snowe, narrator of Charlotte Brontë’s *Villette*, ‘were almost dreaded by me, because they woke the being I was always lulling, and stirred up a craving cry I could not satisfy’ (London, 2004, p. 121); Murison, ‘Racial Interiority’, p. 7. It should be pointed out that not all histories of hypochondria consider it to have been a disease of the upper classes in the British eighteenth and nineteenth centuries. As Susan Baur notes, ‘by the end of the eighteenth century . . . hypochondria and its dozen namesakes were among the commonest of disorders and were believed to afflict every kind of person’ (*Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 27).
- ⁷ Anna Pehkoranta, ‘Surviving the Loss: Abjection and Hypochondria in Maxine Hong Kingston’s *The Woman Warrior* and Fae Myenne Ng’s *Bone*’, in *On the Legacy of Maxine Hong Kingston: The Mulhouse Book*, ed. Sami Ludwig and Nicoleta Alexoae-Zagni (Berlin, 2014), p. 273.
- ⁸ Steven Connor, ‘Beghosted Bodyhood: Hypochondria and the Arts of Illness’, essay based on talks given at the North-East Network for Medicine and the Arts, St Chad’s College, Durham, 16 June 2008, and Tate Britain, 18 September 2009, p. 23, available at <http://stevenconnor.com>.
- ⁹ Don R. Lipsitt, ‘Psychodynamic Perspectives on Hypochondriasis’, in *Hypochondriasis: Modern Perspectives on an Ancient Malady*, ed. Vladan Starcevic and Don R. Lipsitt (Oxford, 2001), p. 193.
- ¹⁰ Daniel Defoe, *A Journal of the Plague Year* [1722] (New York, 1960), p. 20.
- ¹¹ *Ibid.*, pp. 20–21.
- ¹² *Ibid.*, p. 21.
- ¹³ *Ibid.*, p. 24.
- ¹⁴ George C. Grinnell, *The Age of Hypochondria: Interpreting Romantic Health and Illness* (London, 2010), pp. 95, 101, 106.
- ¹⁵ *Ibid.*, p. 102.
- ¹⁶ Mary Shelley, *The Last Man* [1826], ed. Hugh J. Luke Jr (Lincoln, NE, 1965), p. 159.
- ¹⁷ *Ibid.*, p. 179.
- ¹⁸ *Ibid.*, pp. 169, 175, 178, 188.
- ¹⁹ *Ibid.*, pp. 32, 68, 69, 167.
- ²⁰ *Ibid.*, p. 342.
- ²¹ Franz Kafka, *Letter to His Father* (New York, 1966), pp. 89–91.
- ²² In her 2014 essay ‘On Translating Kafka’s “The Metamorphosis”’, Susan Bernofsky explains that though Kafka ‘used the word “bug” (*Wanze*) when referring casually to the story’, the original German describing Gregor Samsa’s transformation is far more ambiguous. ‘The epithet *ungeheueres Ungeziefer*’, she writes, ‘poses one of the greatest challenges to the translator. Both the adjective . . . and the noun . . . are negations – virtual nonentities – prefixed by un.’ One means ‘monstrous’ or ‘huge’, the other a kind of ‘unclean animal unfit for sacrifice’. ‘I think Kafka wanted us to see Gregor’s new body and condition’, Bernofsky concludes, ‘with the same hazy focus with which Gregor himself discovers them.’ The linguistic ambiguity obviously mirrors that of hypochondria itself: *something* is amiss with Gregor’s bodily form, but neither he nor we, as his ‘interpreters’, can say exactly what it is. Susan Bernofsky, ‘On Translating Kafka’s “The Metamorphosis”’, *New Yorker* (14 January 2014); Franz Kafka, ‘The Metamorphosis’, in *Franz Kafka: The Complete Stories*, ed. Nahum N. Glatzer (New York, 1971), pp. 91, 96.
- ²³ See also Miguel de Cervantes’s story ‘The Glass Graduate’ from 1613. Princess Alexandra of Bavaria (1826–75) believed she had swallowed a glass piano – whole – as a child and moved through space with great care so as not to shatter it. Charles VI of France (1368–1422) believed his whole body to be made of glass and swaddled himself in woollen blankets to avoid breaking into pieces. The condition is said to have disappeared by the eighteenth century, though isolated reports of authentic cases do exist.

- [24](#) Robert Burton, *The Anatomy of Melancholy* [1621], ed. Floyd Dell (New York, 1941), pp. 328, 350.
- [25](#) Given something of an obsession with cleanliness, it makes sense that Kafka was also the kind of hypochondriac who the physician Robert Hutchison once deemed ‘The Physical Prig’. Writing in the *British Medical Journal* in 1934, Hutchison excoriated these ‘mostly young men with an exaggerated standard of health and well-being’ who ‘become addicts’ to fitness. Many historians of hypochondria have noted that an exaggerated focus on exercise and diet is really just hypochondria with different habits, ones that are approbated by a health-conscious society in which being ‘in shape’, even fanatically, is inherently preferable to an undue fascination with sickness – often perceived as a desire to *be sick*. Kafka was an avid devotee of the Müller method, a system of free-body movement developed at the turn of the century by the Danish gymnast Jørgen Peter Müller. Robert Hutchison, ‘Hypochondriasis: Individual, Various, and Communal’, *British Medical Journal*, I/3817 (3 March 1934), pp. 365–7.
- [26](#) Frank Kafka, ‘The Burrow’, in *Kafka: The Complete Stories*, ed. Glatzer, p. 326.
- [27](#) *Ibid.*, pp. 325, 332.
- [28](#) *Ibid.*, pp. 331, 353, 355, 356, 357, 358.
- [29](#) Immanuel Kant, *Anthropology from a Pragmatic Point of View*, ed. Mary J. Gregor (The Hague, 1974), p. 82.
- [30](#) German E. Berrios, ‘Hypochondriasis: History of the Concept’, in *Hypochondriasis*, ed. Starcevic and Lipsitt, p. 17.
- [31](#) Kant, *Anthropology*, p. 82.
- [32](#) Kafka, ‘The Burrow’, pp. 326, 359.
- [33](#) Gilles Deleuze, ‘Bartleby; or, The Formula’, in *Essays Critical and Clinical*, trans. Daniel W. Smith and Michael A. Greco (Minneapolis, MN, 1997), pp. 68–90. Available in *On Curating*, 40 (September 2018), www.on-curating.org, pp. 49–64.
- [34](#) Kafka, ‘The Metamorphosis’, p. 115.
- [35](#) Sarah Mann-O’Donnell, ‘From Hypochondria to Convalescence: Health as Chronic Critique in Nietzsche, Deleuze and Guattari’, *Deleuze Studies*, IV/2 (July 2010, special issue on Deleuze and the Symptom), p. 175.
- [36](#) Robert Kellner, *Somatization and Hypochondriasis* (New York, 1986), pp. 18, 19.
- [37](#) José María Aguilera-Manzano, ‘Slavery and Medicine in the Caribbean at the End of the “Ancien Régime”’, *Social History*, XXXIII/4 (November 2008), pp. 383–401.
- [38](#) *Reflexiones*, quoted *ibid.*, p. 397.
- [39](#) *Ibid.*, p. 398.
- [40](#) Aguilera-Manzano, ‘Slavery and Medicine’, p. 398.
- [41](#) *Ibid.*, p. 399.
- [42](#) Anne Anlin Cheng, *The Melancholy of Race* (New York, 2000), p. 72.
- [43](#) *Ibid.*, p. 65.
- [44](#) Monika Hogan, “‘Something So Visceral in with the Rhetorical’: Race, Hypochondria, and the Un-Assimilated Body in *American Pastoral*”, *Studies in American Jewish Literature* (1981–), XXIII (2004), p. 4; Cheng, *Melancholy*, p. 65.
- [45](#) Hogan, ‘Un-Assimilated Body’, pp. 2, 3, 6, 13.
- [46](#) Maxine Hong Kingston, *The Woman Warrior: Memoirs of a Girlhood Among Ghosts* (New York, 1989), pp. 3, 5, 11.
- [47](#) *Ibid.*, pp. 48, 47.
- [48](#) *Ibid.*, pp. 165, 169, 165, 166, 171–2.
- [49](#) *Ibid.*, pp. 172, 173, 176–81.
- [50](#) *Ibid.*, p. 182.

- [51](#) Pehkoranta, 'Surviving the Loss', p. 270.
- [52](#) Cheng, *Melancholy*, p. 66.
- [53](#) Kingston, *Woman Warrior*, pp. 16, 180, 184.
- [54](#) *Ibid.*, pp. 189, 190.
- [55](#) *Ibid.*, p. 108.
- [56](#) *Ibid.*, pp. 74, 82, 108, 155.
- [57](#) Margery Bailey, ed., *Boswell's Column: Being His Seventy Contributions to The London Magazine under the Pseudonym The Hypochondriack from 1777 to 1783* (London, 1951), p. 107.
- [58](#) Laura Kurtzman, 'Trump's "Chinese Virus" Tweet Linked to Rise of Anti-Asian Hashtags on Twitter', www.ucsf.edu, 18 March 2021.
- [59](#) House minority leader Kevin McCarthy (R-Calif.) referred to COVID-19 as the 'Chinese virus' on Twitter, and Rep. Paul A. Gosar (R-Ariz.) repeated the 'Wuhan Virus' epithet on social media. California Democratic representatives Ted Lieu and Katie Porter and Florida Democratic representative Lois Frankel were among those denouncing such messaging as xenophobic and racially provocative.
- [60](#) Deb Riechmann and Terry Tang, 'Trump Dubs COVID-19 "Chinese Virus" Despite Hate Crime Risks', *Associated Press*, <http://apnews.com>, 18 March 2020.
- [61](#) Fatima Burhan, 'Covid Creates Flashpoint for Anti-Migrant Sentiment in Italy', *Human Rights Watch*, www.hrw.org, 22 July 2020.
- [62](#) On health anxiety experienced in relation to migration and inequities of access to healthcare, see two articles lead-authored by Rieke Barbek: Rieke Barbek et al., 'Ethnic and Migration-Related Inequalities in Health Anxiety: A Systematic Review and Meta-Analysis', *Frontiers in Psychology*, XIII (August 2022), pp. 1–11; and Rieke Barbek, Anna Christin Makowski and Olaf von dem Knesebeck, 'Social Inequalities in Health Anxiety: A Systematic Review and Meta-Analysis', *Journal of Psychosomatic Research*, CLIII (February 2022), pp. 1–11.
- [63](#) Grinnell, *Age of Hypochondria*, p. 101.
- [64](#) FND is a controversial and complex diagnosis with movement, speech and sensory symptoms whose suggested causes range from sexual abuse to 'dislike of job' – and, not surprisingly, which some believe to be feigned. See the National Institute of Neurological Disorders and Stroke webpage on FND; and Xenos L. Mason, 'Challenges to the Diagnosis of Functional Neurological Disorder: Feigning, Intentionality, and Responsibility', *Neuroethics*, XVI/1 (April 2023), www.ncbi.nlm.nih.gov.
- [65](#) Sigmund Freud, 'On Narcissism: An Introduction', in *The Standard Edition of the Complete Psychological Works*, ed. James Strachey, vol. XIV (London, 1957), pp. 69–102.
- [66](#) Fuson Wang, 'Romantic Disease Discourse: Disability, Immunity, and Literature', *Nineteenth-Century Contexts*, XXXIII/5 (December 2011), pp. 472, 474.
- [67](#) Shelley, *The Last Man*, p. 245.
- [68](#) Grinnell, *Age of Hypochondria*, p. 112.
- [69](#) The modern history of vaccination begins with attempts to inoculate against smallpox by exposing healthy individuals to live virus – typically pulverized scabs. This practice, called variolation (from *la variole*, the French term for smallpox), was introduced to the West in 1721 by the poet and essayist Lady Mary Wortley Montagu, who had observed its use in Turkey, where her husband was a diplomat to the Ottoman Empire. Jenner had observed that milkmaids exposed to the bovine virus cowpox seemed naturally immune to smallpox; in 1796 he tested his theory on a young boy who fully recovered from a mild illness. Two months later Jenner inoculated the same boy with matter from a human smallpox sore; this time the boy remained completely well. James Phipps thus became the first person to be vaccinated against disease, via experimentation that crossed national, ethnic and religious borders.
- [70](#) Wang, 'Romantic Disease', p. 474.

- [71](#) Ibid., p. 478.
- [72](#) Shelley, *The Last Man*, p. 248.
- [73](#) Grinnell, *Age of Hypochondria*, p. 114.
- [74](#) David Mechanic, *Medical Sociology: A Selective View* (New York, 1968), quoted in Kellner, *Somatization*, p. 106.
- [75](#) Michel Foucault, *Abnormal: Lectures of the Collège de France, 1974–75*, ed. Valerio Marchetti and Antonella Salomoni, trans. Graham Burchell (London, 2004), p. 56.
- [76](#) Margrit Shildrick, *Dangerous Discourses of Disability, Subjectivity, and Sexuality* (New York, 2012), p. 19.
- [77](#) D. W. Winnicott, 'Mind and Its Relation to the Psyche-Soma', *British Journal of Medical Psychology*, vol. XXVII (1954), p. 208.
- [78](#) Jerome K. Jerome, *Three Men in a Boat (To Say Nothing of the Dog)* (London, 1889), p. 6; my italics.

5 Illness on the Doorstep: Hypochondria Precedes

- [1](#) Molière, *The Imaginary Invalid* [1673], trans. Charles Heron Wall (2003), www.gutenberg.org.
- [2](#) Jeremy Collier, *Miscellanies upon Moral Subjects* (London, 1695), *Early English Books Online*, University of Michigan Library Digital Collections, <http://quod.lib.umich.edu>.
- [3](#) Immanuel Kant, *Anthropology from a Pragmatic Point of View*, ed. Mary J. Gregor (The Hague, 1974), p. 83; Susan Baur, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 48.
- [4](#) Robert Hutchison, 'Hypochondriasis: Individual, Various, and Communal', *British Medical Journal*, I/3817 (3 March 1934), p. 365.
- [5](#) Beverley T. Mead, 'Management of Hypochondriacal Patients', *JAMA*, CLCII/1 (5 April 1965), p. 33.
- [6](#) Danielle Kohfeldt and Gregory Mather, 'The Metanarrative of Chronic Pain', in *Metanarratives of Disability: Culture, Assumed Authority, and the Normative Social Order*, ed. David Bolt (London, 2021), p. 149. 'Malingering' is defined in the *DSM–5* as a conscious and 'intentional production of false or grossly exaggerated physical or psychological problems' and is understood to be motivated by some presumption of external gain. It is this notion of 'gain' that differentiates malingering from 'factitious disorder', also an intentional fabrication of symptoms.
- [7](#) Meghan O'Rourke, *The Invisible Kingdom: Reimagining Chronic Illness* (New York, 2023), p. 111.
- [8](#) Arthur Kleinman, *The Illness Narratives: Suffering, Healing, and the Human Condition* (New York, 1988), pp. 134–5.
- [9](#) Angela J. Kim, 'The Metanarrative of OCD', in *Metanarratives of Disability*, ed. Bolt, p. 73.
- [10](#) Maya Dusenbery, *Doing Harm: The Truth about How Bad Medicine and Lazy Science Leave Women Dismissed, Misdiagnosed, and Sick* (New York, 2019), pp. 96, 97. Eliot Slater is quoted on p. 81.
- [11](#) Annemarie Goldstein Jutel, *Putting a Name to It: Diagnosis in Contemporary Society* (Baltimore, MD, 2011), p. 80.
- [12](#) Baur, *Hypochondria*, p. 159.
- [13](#) Larry Au et al., 'Long Covid and Medical Gaslighting: Dismissal, Delayed Diagnosis, and Deferred Treatment', *SSM – Qualitative Research in Health*, II (December 2022), p. 9.
- [14](#) Ibid.
- [15](#) Judy Z. Segal, *Health and the Rhetoric of Medicine* (Carbondale, IL, 2005), p. 170.
- [16](#) Catherine Belling, *A Condition of Doubt: The Meanings of Hypochondria* (Oxford, 2012), p. 174.

- [17](#) Stephen F. Morin and Walter F. Batchelor, 'Responding to the Psychological Crisis of AIDS', *Public Health Reports* (1974–), XCIX/1 (January–February 1984), pp. 7, 8.
- [18](#) Lynn Rose, 'Homophobia Among Doctors', *British Medical Journal*, CCCVIII/6928 (26 February 1994), p. 587.
- [19](#) Belling, *Condition of Doubt*, p. 98.
- [20](#) Au et al., 'Long Covid', p. 6.
- [21](#) Eric Boodman, 'How Medicine Erased Black Women From a "White Man's Disease"', www.statnews.com, 21 December 2021.
- [22](#) Kerri K. Morris, 'Women and Bladder Cancer: Listening Rhetorically to Healthcare Disparities', in *Interrogating Gendered Pathologies*, ed. Erin A. Frost and Michelle F. Eble (Louisville, CO, 2020), p. 158.
- [23](#) Fallacious notions about racial biology are implicated in more than one notorious instance of Black people's bodies being exploited in the name of medical research. J. Marion Sims, a nineteenth-century surgeon, gynaecologist and one-time president of the American Medical Association, is now infamous for having developed a novel surgical repair for vesicovaginal fistula by operating, without anaesthesia, on enslaved Black women. Another such occurrence is the Tuskegee Syphilis Experiment, a scandalous forty-year study under the auspices of the U.S. Public Health Service and Centers for Disease Control and Prevention in which some four hundred African American men with latent syphilis were observed without benefit of treatment, even after penicillin became the standard and widely available antibiotic medication for the disease. Yet another is the case of Henrietta Lacks, the woman whose cervical cancer cells were in 1951 extracted without her knowledge or consent to become the first human immortalized cell line used for biomedical research. Now known as the HeLa immortal cell line, Lacks's original cells were cloned in 1955 and have since been used for research in cancer, AIDS, polio and gene mapping. Lacks died in 1951; her family were not informed of the use of her cells until 1975. For more on the 'invulnerability' of Black people to pain, see Anna Mollow, 'Unvictimized: Toward a Fat Black Disability Studies', *African American Review*, L/2 (Summer 2017), special issue, Blackness and Disability, pp. 105–21.
- [24](#) Lisa J. Staton et al., 'When Race Matters: Disagreement in Pain Perception between Patients and Their Physicians in Primary Care', *Journal of the National Medical Association*, XCIX/5 (May 2007), p. 535; Kelly M. Hoffman et al., 'Racial Bias in Pain Assessment and Treatment Recommendations, and False Beliefs about Biological Differences between Blacks and Whites', *Proceedings of the National Academy of Sciences of the United States of America*, CXIII/6 (19 April 2016), p. 4296.
- [25](#) Salimah H. Meghani, Eeseung Byun and Rollin M. Gallagher, 'Time to Take Stock: A Meta-Analysis and Systematic Review of Analgesic Treatment Disparities for Pain in the United States', *Pain Medicine*, XIII/2 (February 2012), pp. 150, 156.
- [26](#) Romain Guedj et al., 'Racial and Ethnic Disparities in Pain Management of Children with Limb Fractures or Suspected Appendicitis: A Retrospective Cross-Sectional Study', *Frontiers in Pediatrics*, IX (3 August 2021).
- [27](#) Armstrong and her co-authors report that Black and Hispanic Americans demonstrate higher levels of distrust of healthcare providers than white Americans, but also that patterns of distrust are less uniform than expected, varying geographically as well as individually. Katrina Armstrong et al., 'Racial/Ethnic Differences in Physician Distrust in the United States', *American Journal of Public Health*, XCVII/8 (July 2007), p. 1287.
- [28](#) See Erica Richards, 'The State of Mental Health of Black Women: Clinical Considerations', *Psychiatric Times*, XXXVIII/9 (23 September 2021).
- [29](#) Melba Wilson, 'Black Women and Mental Health: Working Towards Inclusive Mental Health Services', *Feminist Review*, LXVIII/1, Women and Mental Health (Summer 2001), p. 42.

- [30](#) Tina K. Sacks, *Invisible Visits: Black Middle-Class Women in the American Healthcare System* (London, 2019), pp. 18–19.
- [31](#) Carla Cantor, *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996), p. 108. I. Pilowsky and N. D. Spence, ‘Ethnicity and Illness Behavior’, *Psychological Medicine*, VII/3 (August 1977), p. 447.
- [32](#) Ron Berghmans, ‘Sir Karl Popper, Swans, and the General Practitioner’, *British Medical Journal*, CCCXLIII/5469 (3 October 2011), p. 2. Berghmans, who lived in the Netherlands and died in 2013, wrote from personal experience. At his first consultation for low-back pain, his GP determined he had ‘work stress’ without any physical examination; then, even as Berghmans’s symptoms worsened, she was resistant to ordering scans or referring her patient to a specialist. Berghmans finally insisted on being examined by the neurology department and was diagnosed, ten months after that first appointment, with non-Hodgkin’s lymphoma. ‘At no point during the process was my GP willing to reconsider her initial diagnosis,’ Berghmans contends, ‘although my situation was progressively worsening and becoming unbearable. She was, and stayed, convinced that her truth was the truth.’
- [33](#) Jerome Groopman, *How Doctors Think* (Boston, MA, 2008), pp. 12, 22, 34, 39.
- [34](#) Quoted in José María Aguilera-Manzano, ‘Slavery and Medicine in the Caribbean at the End of the “Ancien Régime”’, *Social History*, XXXIII/4 (November 2008), p. 389.
- [35](#) Ramesh Mallipeddi, ‘“A Fixed Melancholy”: Migration, Memory, and the Middle Passage’, *Eighteenth Century*, LV/2–3 (Summer/Fall 2014), p. 237.
- [36](#) Aguilera-Manzano, ‘Slavery and Medicine’, p. 389.
- [37](#) Mallipeddi, ‘“Fixed Melancholy”’, p. 238.
- [38](#) Isabela Fraga, ‘Sick Minds, Unproductive Bodies: Nostalgia, Slavery, and Feeling in Late-Eighteenth-Century Cuba’, *Slavery and Abolition: A Journal of Slave and Post-Slave Studies*, XLIII/1 (2022), pp. 68–90. The damaging legacy of drapetomania, a psychological disorder invented by Louisiana physician (and pro-slavery advocate) Samuel Adolphus Cartwright in 1851 to ‘explain’ enslaved Blacks’ attempts to escape, cannot be overstated. A runaway slave was by definition ‘crazy’, and the ‘cure’ for the condition was hard work, beatings, even amputation. As Therí Alyce Pickens argues of the persistence of such pseudoscience, ‘In an ideological construct of white supremacy, Blackness is considered synonymous with madness or the prerequisite for creating madness’ (Therí Alyce Pickens, *Black Madness :: Mad Blackness* (Durham, NC, 2019), p. 4).
- [39](#) Kenneth F. Kiple and Brian T. Higgins, ‘Mortality Caused by Dehydration during the Middle Passage’, *Social Science History*, XIII/4 (Winter 1989), p. 427.
- [40](#) Mallipeddi, ‘“Fixed Melancholy”’, p. 246.
- [41](#) Melinda Wenner Moyer, ‘Women Are Calling Out “Medical Gaslighting”’, *New York Times* (28 March 2022).
- [42](#) Hannah Yasharoff, ‘“They Don’t Believe Me”: The Pain and Dangers of Medical Gaslighting’, *USA Today* (19 May 2023); Katie Camero, ‘Long COVID Patients Say Doctors Are Ignoring Their Symptoms’, BuzzFeed News, www.buzzfeednews.com, 29 April 2022.
- [43](#) Kohfeldt and Mather, ‘Chronic Pain’, p. 147.
- [44](#) Miranda Fricker, *Epistemic Injustice: Power and the Ethics of Knowing* (Oxford, 2007). Kohfeldt and Mather go further: in their discussion of narratives of pain they describe the ‘act of downgrading the authenticity of chronic pain . . . as a form of epistemic violence’, a ‘reduction in credibility [that] effectively silences the person with chronic pain when their testimony is not taken seriously’ (Kohfeldt and Mather, ‘Chronic Pain’, p. 149; my italics).
- [45](#) Vivian M. May, ‘“Speaking into the Void?” Intersectionality Critiques and Epistemic Backlash’, *Hypatia*, XXIX/1 (Winter 2014), p. 100.

- [46](#) Abraham Schwab, 'Epistemic Humility and Medical Practice: Translating Epistemic Categories into Ethical Obligations', *Journal of Medicine and Philosophy*, XXXVII/1 (February 2012), pp. 28, 31, 37, 44.
- [47](#) Au et al., 'Long Covid', pp. 1, 8.
- [48](#) Groopman, *How Doctors Think*, p. 155.
- [49](#) Au et al., 'Long Covid', p. 6.
- [50](#) Groopman, *How Doctors Think*, p. 17.
- [51](#) Jennifer Traig, *Well Enough Alone: A Cultural History of My Hypochondria* (New York, 2008), pp. 33–4.
- [52](#) Hilary Mantel, *Giving Up the Ghost* (New York, 2004), pp. 163, 166, 175, 177, 178, 190.
- [53](#) O'Rourke, *Invisible Kingdom*, p. 104.
- [54](#) Mantel, *Ghost*, pp. 203, 61.
- [55](#) Belling, *Condition of Doubt*, p. 101.
- [56](#) Mantel, *Ghost*, p. 168.
- [57](#) Susan Burton, 'The Retrievals', Serial Productions/*New York Times* podcast, 2023.
- [58](#) On the relationship between hysteria and hormones, see Amy Koerber, *From Hysteria to Hormones: A Rhetorical History* (University Park, PA, 2018), which traces shifting medical theories about women's bodies and health and their rhetorical significance.
- [59](#) Kleinman, *Illness Narratives*, p. 73.
- [60](#) Belling, *Condition of Doubt*, p. 205.
- [61](#) Eli Clare, *Exile and Pride: Disability, Queerness, and Liberation* (Cambridge, MA, 1999), p. 138.

6 The Wisdom of Worry: Hypochondria Ages

- [1](#) Robert Lowell, 'Obit', *Selected Poems: Expanded Edition* (New York, 2007).
- [2](#) Rachel McAlpine, 'Why Older People Talk About Their Ailments', Write into Life, <http://writeintolife.com>, 2 January 2017.
- [3](#) U.S. National Council on Aging, 'Get the Facts on Healthy Aging', www.ncoa.org, 16 August 2024.
- [4](#) Elliott M. Stein, MD, 'When Is Hypochondriasis Not Hypochondriasis? Emotional and Behavioral Adaptations to Aging and Illness', *Geriatrics*, LVIII/9 (September 2003), p. 42.

- 5 See Birgit aan de Stegge et al., 'Death Anxiety and Its Association with Hypochondriasis and Medically Unexplained Symptoms: A Systematic Review', *Journal of Psychosomatic Research*, CXV (December 2018), pp. 58–65. Russell Noyes Jr cites a 1989 study that 'found no relationship to age' in hypochondriacs aged sixty and older (Russell Noyes Jr, 'Epidemiology of Hypochondriasis', in *Hypochondriasis: Modern Perspectives on an Ancient Malady*, ed. Vladan Starcevic and Don R. Lipsitt (Oxford, 2001), p. 140). Robert Kellner writes in *Somatization and Hypochondriasis* (New York, 1986) that 'the findings on . . . hypochondriasis and age are inconsistent', in part because of the simple fact that in 'older age groups, minor physical disease' could account for 'reported symptoms' (p. 53). Carla Cantor in *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996) cites a study that 'found that those who felt sickest in old age were those who at age thirty fretted and complained most about their health' (pp. 181–2). Bob Woods and Carolien Lamers write that 'gerontological studies suggest that typically older people maintain life satisfaction and well-being' and 'a stronger sense of control, mastery or resilience' (Woods and Lamers, 'Psychological Problems of Older People', in *The Handbook of Adult Clinical Psychology*, 2nd edn, ed. Alan Carr and Muireann McNulty (New York, 2016), p. 947). Even one study that starts from the premise that 'older adults are particularly prone to health anxiety' goes on to acknowledge that 'when controlling for medical morbidity, increasing age is no longer related to hypochondriasis' (R. El-Gabalawy et al., 'Health Anxiety Disorders in Older Adults: Conceptualizing Complex Conditions in Late Life', *Clinical Psychology Review*, XXXIII/8 (December 2013), pp. 1097, 1099).
- 6 Arthur J. Barsky et al., 'The Relation between Hypochondriasis and Age', *American Journal of Psychiatry*, CXLVIII/7 (July 1991), pp. 923–8; John Monopoli and Frank Vaccaro, 'The Relationship of Hypochondriasis Measures to Correlates of Personality in the Elderly', *Clinical Gerontologist*, XXVI/3–4 (November 2003), p. 135.
- 7 See Barsky, 'Hypochondriasis and Age', and aan de Stegge et al., 'Death Anxiety', p. 60, and n. 27.
- 8 Stein, 'Aging and Illness', p. 42.
- 9 Paul T. Costa and Robert R. McCrae, 'Hypochondriasis, Neuroticism, and Aging: When Are Somatic Complaints Unfounded?' *American Psychologist*, XL/1 (January 1985), p. 24.
- 10 Cantor, *Phantom Illness*, p. 184.
- 11 Susan Baur, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988), p. 118.
- 12 Clara W. Berridge and Marty Martinson, 'Valuing Old Age without Leveraging Ableism', *Generations: Journal of the American Society on Aging*, XLI/4 (Winter 2017–18), Gender and Age: A Focus on Women, pp. 89, 84, 83.
- 13 Scott Herring, 'Wise Old Fools: Positive Geropsychology and the Poetics of Later-Life Floundering', in *Literary Studies and Human Flourishing*, ed. James F. English and Heather Love (Oxford, 2023), p. 173.
- 14 Ibid., pp. 172, 173–4.
- 15 Ibid., p. 179.
- 16 Ibid.
- 17 Ibid., pp. 175, 178.
- 18 Helen Small, *The Long Life* (Oxford, 2007), p. 255.
- 19 Woods and Lamers, 'Psychological Problems', p. 941. Woods and Lamers published their chapter in 2016. Columbia University reported in late 2022 that a full 22 per cent of adults over 65 in the United States have mild cognitive impairment.
- 20 Ibid., p. 942.
- 21 German E. Berrios, Ivana S. Marková and Nestor Giralá, 'Functional Memory Complaints: Hypochondria and Disorganization', in *Memory Disorders in Psychiatric Practice*, ed. German E. Berrios and John R. Hodges (Cambridge, 2000), pp. 384, 385.

- [22](#) May Sarton, *At Eighty-Two: A Journal* (New York, 1996), p. 333.
- [23](#) I write more fully about Sarton's autobiographical work in my chapter on the journals in *Unruly Bodies: Life Writing by Women with Disabilities* (Chapel Hill, NC, 2007).
- [24](#) Sarton, *At Eighty-Two*, pp. 334, 335, 336, 337, 341, 342.
- [25](#) *Ibid.*, pp. 306, 343.
- [26](#) *Ibid.*, pp. 338, 341, 342.
- [27](#) *Ibid.*, pp. 342, 343.
- [28](#) *Ibid.*, pp. 343, 345.
- [29](#) *Ibid.*, pp. 334, 342.
- [30](#) Jeannette King, *Discourses of Ageing in Fiction and Feminism: The Invisible Woman* (London, 2013), p. 103.
- [31](#) Sarton, *At Eighty-Two*, p. 345.
- [32](#) Margot Peters's authorized biography of Sarton, published two years after Sarton's death in 1997, holds a notoriously unflattering mirror to Sarton's temperament as well as her literary output. Margot Peters, *May Sarton: A Biography* (New York, 1997). Angeline Goreau's 6 April 1997 review of Peters in the *New York Times* ('Late Bloomer') and Regina Marler's 30 June 2002 piece in the *Los Angeles Times* ('Just How Good Was She?') both offer especially cogent summations of the debate between Sarton's hypercritical biographer and her far more forgiving, often adoring, fans.
- [33](#) On the association between hypochondria, medically unexplained symptoms and death anxiety, see aan de Stegge et al., 'Death Anxiety'.
- [34](#) Franz Kafka, 'The Burrow', in *Franz Kafka: The Complete Stories*, ed. Nahum N. Glatzer (New York, 1971), pp. 332, 345, 348, 357.
- [35](#) *Ibid.*, pp. 334, 339, 343, 345, 346, 347, 352, 353.
- [36](#) *Ibid.*, pp. 357, 358, 359.
- [37](#) *Ibid.*, pp. 336, 337, 358.
- [38](#) Monopoli and Vaccaro, 'Relationship of Hypochondriasis Measures', pp. 123, 135.
- [39](#) Kathleen Woodward, 'Aging', in *Keywords for Disability Studies*, ed. Rachel Adams, Benjamin Reiss and David Serlin (New York, 2015), p. 33.
- [40](#) *Ibid.*; quoted in Cantor, *Phantom Illness*, p. 184.
- [41](#) Small, *Long Life*, pp. 26, 29, 35.
- [42](#) Andrew P. Peabody, trans., *Ethical Writings of Cicero* (Boston, MA, 1887), p. 23.
- [43](#) Herring, 'Wise Old Fool', p. 165. Herring cites two anthologies edited by James O. Pawelski and D. J. Moores: *The Eudaimonic Turn: Well-Being in Literary Studies* (Plymouth, MA, 2013) and *On Human Flourishing: A Poetry Anthology* (Jefferson, NC, 2015).
- [44](#) Atul Gawande, *Being Mortal: Medicine and What Matters in the End* (New York, 2014), p. 95.
- [45](#) Kathleen Woodward, *Statistical Panic: Cultural Politics and Poetics of the Emotions* (Durham, NC, 2009), p. 74.
- [46](#) Oscar Wilde, *The Picture of Dorian Gray* [1890] (London, 2000), pp. 212, 213.
- [47](#) *Ibid.*
- [48](#) *Ibid.*, pp. 27–8.
- [49](#) Robert Louis Stevenson, *Strange Case of Dr Jekyll and Mr Hyde, and Other Tales* (Oxford, 2006), pp. 16, 53.
- [50](#) In the dedication to Cunningham in his collection *A Child's Garden of Verses*, Stevenson (in)famously refers to her as 'My second mother, my first wife. The angel of my infant life – From the sick child, now well and old, Take, nurse, the little book you hold!' The autobiographical poem 'The Land of Counterpane' therein describes a boy 'sick and lay-a-bed' on 'two pillows', with 'all [his] toys beside [him]' to 'keep [him] happy all the day'.

- [51](#) Quoted in Danny Heitman, 'Treasure Island Author Robert Louis Stevenson Was a Sickly Man with a Robust Imagination', *HUMANITIES*, XXXVI/4 (July/August 2015), www.neh.gov.
- [52](#) Stevenson, *Strange Case*, pp. 9, 13, 23.
- [53](#) *Ibid.*, p. 55.
- [54](#) *Ibid.*, pp. 7, 22.
- [55](#) H. G. Wells, 'Zoological Retrogression', in *The Fin de Siècle: A Reader in Cultural History 1880–1900*, ed. Sally Ledger and Roger Luckhurst (Oxford, 2000), pp. 5–12.
- [56](#) Anne Stiles, 'Robert Louis Stevenson's *Jekyll and Hyde* and the Double Brain', *SEL*, XLVI/4 (Autumn 2006), pp. 884–5, 887.
- [57](#) Stevenson, *Strange Case*, p. 59.
- [58](#) Stiles, 'Double Brain', p. 885.
- [59](#) Stevenson, *Strange Case*, p. 59.
- [60](#) Berrios et al., 'Functional Memory Complaints', p. 385.
- [61](#) Ruth E. Taylor, 'Death of Neurasthenia and Its Psychological Reincarnation', *British Journal of Psychiatry*, CLXXIX/6 (December 2001), p. 550.
- [62](#) Stevenson, *Strange Case*, p. 54.
- [63](#) *Ibid.*, p. 60.
- [64](#) *Ibid.*, p. 53.
- [65](#) *Ibid.*, p. 59.
- [66](#) May Sarton, *Recovering*, reissue edn (New York, 1997), p. 157.
- [67](#) Maxine Hong Kingston, *The Woman Warrior: Memoirs of a Girlhood Among Ghosts* (New York, 1989), p. 102.
- [68](#) Jenna M. Moore et al., 'Loneliness and Hypochondriasis Among Older Adults: The Mediating Role of Intolerance of Uncertainty and Anxiety', *Innovation in Aging*, III/Suppl. 1 (2019), pp. 531–2.
- [69](#) David Mataix-Cols et al., 'All-Cause and Cause-Specific Mortality Among Individuals with Hypochondria', *JAMA Psychiatry* online, <http://jamanetwork.com>, 13 December 2023, p. E4.
- [70](#) *Ibid.*, p. E5.
- [71](#) *Ibid.*, p. E6.
- [72](#) *Ibid.*, p. E7.
- [73](#) *Ibid.*
- [74](#) *Ibid.*
- [75](#) *Ibid.*, p. E2.
- [76](#) Woodward, *Statistical Panic*, pp. 59, 65, 67.
- [77](#) Philip Larkin, 'The Old Fools', in *Collected Poems*, ed. Anthony Thwaite (New York, 1989).
- [78](#) Small, *Long Life*, pp. 137–8.

Afterword: Hypochondria Cares

- [1](#) 'Monk in Quarantine', Peacock Presents, 2020, www.youtube.com.
- [2](#) Roy Porter, *The Cambridge History of Medicine* (Cambridge, 2006), p. 71.
- [3](#) John Reid, *Essays on Hypochondriacal and Other Nervous Affections* (London, 1817), pp. 8–9, available at Internet Archive, <http://archive.org>.

SELECT BIBLIOGRAPHY

- Austen, Jane, *Emma* [1815] (New York, 2012)
- Bailey, Margery, ed., *Boswell's Column: Being His Seventy Contributions to The London Magazine under the Pseudonym The Hypochondriack from 1777 to 1783* (London, 1951)
- Barsky, Arthur J., et al., 'The Relation between Hypochondriasis and Age', *American Journal of Psychiatry*, CXLVIII/7 (July 1991), pp. 923–98
- Baur, Susan, *Hypochondria: Woeful Imaginings* (Berkeley, CA, 1988)
- Belling, Catherine, *A Condition of Doubt: The Meanings of Hypochondria* (Oxford, 2012)
- Brontë, Charlotte, *Villette* [1853] (London, 2004)
- Burton, Robert, *The Anatomy of Melancholy* [1621], ed. Floyd Dell (New York, 1941)
- Burton, Susan, *The Retrievals*, Serial Productions/*New York Times* podcast, 2023
- Cantor, Carla, *Phantom Illness: Recognizing, Understanding, and Overcoming Hypochondria* (Boston, MA, 1996)
- Cheng, Anne Anlin, *The Melancholy of Race* (New York, 2000)
- Cheyne, George, *The English Malady: Or, a Treatise of Nervous Diseases of All Kinds* [1733] (New York, 1995)
- Coleridge, Samuel Taylor, *Collected Letters of Samuel Taylor Coleridge*, ed. E. L. Griggs, 6 vols (Oxford, 1956–71)
- Connor, Steven, 'Beghosted Bodyhood: Hypochondria and the Arts of Illness', essay based on talks given at the North-East Network for Medicine and the Arts, St Chad's College, Durham, 16 June 2008, and Tate Britain, 18 September 2009, <http://stevenconnor.com>
- Costa, Paul T., and Robert R. McCrae, 'Hypochondriasis, Neuroticism, and Aging: When Are Somatic Complaints Unfounded?' *American Psychologist*, XL/1 (January 1985), pp. 19–28
- Dalrymple, Theodore, 'Hypochondria as Control', *British Medical Journal*, CCCXLI/7777 (16 October 2010), p. 837
- Damasio, Antonio R., *Descartes' Error: Emotion, Reason, and the Human Brain* (New York, 1994)
- Darwin, Charles, *The Autobiography of Charles Darwin, 1809–1882*, ed. Nora Barlow (London, 1958)
- Darwin, Francis, ed., *The Life and Letters of Charles Darwin*, vol. I (London, 1897)
- Defoe, Daniel, *A Journal of the Plague Year* [1722] (New York, 1960)
- Dillon, Brian, *The Hypochondriacs: Nine Tormented Lives* (New York, 2009)
- Edel, Leon, ed., *The Diary of Alice James* (New York, 1964)
- Frawley, Maria, *Invalidism and Identity in Nineteenth-Century Britain* (Chicago, IL, 2004)
- Greenway, William, 'Hypochondria', *Poetry*, CLIX/1 (October 1991), p. 14
- Grinnell, George C., *The Age of Hypochondria: Interpreting Romantic Health and Illness* (London, 2010)
- Groopman, Jerome, *How Doctors Think* (Boston, MA, 2008)
- Herring, Scott, 'Wise Old Fools: Positive Geropsychology and the Poetics of Later-Life Floundering', in *Literary Studies and Human Flourishing*, ed. James F. English and Heather Love (Oxford, 2023), pp. 164–82
- Hutchison, Robert, 'Hypochondriasis: Individual, Various, and Communal', *British Medical Journal*, I/3817 (3 March 1934), pp. 365–7
- Jurecic, Ann, *Illness as Narrative* (Pittsburgh, PA, 2012)

- Jutel, Annemarie Goldstein, *Putting a Name to It: Diagnosis in Contemporary Society* (Baltimore, MD, 2011)
- Kafka, Franz, 'The Metaphormosis', in *Franz Kafka: The Complete Stories*, ed. Nahum N. Glatzer (New York, 1971), pp. 89–139
- , 'The Burrow', in *Franz Kafka: The Complete Stories*, ed. Nahum N. Glatzer (New York, 1971), pp. 325–59
- , *Letter to His Father* (New York, 1966)
- Kellner, Robert, *Somatization and Hypochondriasis* (New York, 1986)
- Kingston, Maxine Hong, *The Woman Warrior: Memoirs of a Girlhood Among Ghosts* (New York, 1989)
- Klein, Richard, 'What Is Health and How Do You Get It?' in *Against Health: How Health Became the New Morality*, ed. Jonathan M. Metzl and Anna Kirkland (New York, 2010), pp. 16–25
- Kleinman, Arthur, *The Illness Narratives: Suffering, Healing, and the Human Condition* (New York, 1988)
- Koerber, Amy, *From Hysteria to Hormones: A Rhetorical History* (University Park, PA, 2018)
- McRuer, Robert, and Merri Lisa Johnson, eds, Special 'Cripistemologies' issues of the *Journal of Literary and Cultural Disability Studies*, VIII/1–2 (2014)
- Mallipeddi, Ramesh, "'A Fixed Melancholy": Migration, Memory, and the Middle Passage', *Eighteenth Century*, LV/2–3 (Summer/Fall 2014), pp. 235–53
- Mann-O'Donnell, Sarah, 'From Hypochondria to Convalescence: Health as Chronic Critique in Nietzsche, Deleuze and Guattari', *Deleuze Studies*, IV/2 (2010), pp. 161–82
- Mantel, Hilary, *Giving Up the Ghost* (New York, 2004)
- Martineau, Harriet, *Life in the Sick-Room* [1844], ed. Maria Frawley (Peterborough, ON, 2003)
- Melville, Herman, 'Bartleby, the Scrivener: A Story of Wall-Street', in *The Piazza Tales* (New York, 1856), pp. 1–29
- Merleau-Ponty, Maurice, *The Phenomenon of Perception*, trans. Colin Smith (London, 1962)
- Molière, *The Imaginary Invalid* [1673], trans. Charles Heron Wall (2003), www.gutenberg.org
- Neuman, Frederic, *Worried Sick? The Exaggerated Fear of Physical Illness* (Hollywood, FL, 2016)
- O'Rourke, Meghan, *The Invisible Kingdom: Reimagining Chronic Illness* (New York, 2023)
- Parikh, Rahul, 'The Real Reason Hypochondriacs Drive Doctors Crazy', *Salon*, www.salon.com, 7 August 2011
- Pearce, John M. S., 'Sydenham on Hysteria', *European Neurology*, LXXVI/3–4 (October 2016), pp. 175–81
- Poe, Edgar Allan, *The Fall of the House of Usher, and Other Tales* (New York, 1960)
- Porter, Roy, *The Cambridge History of Medicine* (Cambridge, 2006)
- Pottle, Frederick A., *James Boswell: The Earlier Years 1740–1769* (New York, 1985)
- Reed, Joseph W., ed., *Boswell, Laird of Auchinleck 1778–1782* (New York, 1977)
- Rees, Will, 'Hypochondriacal Reading: Phantom Illness and Literature', *Oxford Literary Review*, XLIII/2 (December 2021), pp. 290–312
- Reid, John, *Essays on Hypochondriacal and Other Nervous Affections* (London, 1817)
- Reidbord, Steven, 'A Brief History of Psychiatry', *Psychology Today*, www.psychologytoday.com, 20 October 2014
- Rousseau, George Sebastian, and David Boyd Haycock, *Framing and Imagining Disease in Cultural History* (London, 2003)
- Sabshin, Melvin, 'Turning Points in Twentieth-Century American Psychiatry', *American Journal of Psychiatry*, CXLVII/10 (October 1990), pp. 1267–74
- Sacks, Tina K., *Invisible Visits: Black Middle-Class Women in the American Healthcare System* (London, 2019)

Sarton, May, *At Eighty-Two: A Journal* (New York, 1996)
Segal, Judy Z., *Health and the Rhetoric of Medicine* (Carbondale, IL, 2005)
Shelley, Mary, *The Last Man* [1826], ed. Hugh J. Luke Jr (Lincoln, NE, 1965)
Small, Helen, *The Long Life* (Oxford, 2007)
Starcevic, Vladan, and Don R. Lipsitt, eds, *Hypochondriasis: Modern Perspectives on an Ancient Malady* (Oxford, 2001)
Stevenson, Robert Louis, *Strange Case of Dr Jekyll and Mr Hyde, and Other Tales* (Oxford, 2006)
Traig, Jennifer, *Well Enough Alone: A Cultural History of My Hypochondria* (New York, 2008)
Wain, John, ed., *The Journals of James Boswell, 1762–1795* (New Haven, CT, 1991)
Wilde, Oscar, *The Picture of Dorian Gray* [1890] (London, 2000)
Woodward, Kathleen, *Statistical Panic: Cultural Politics and Poetics of the Emotions* (Durham, NC, 2009)

OceanofPDF.com

ACKNOWLEDGEMENTS

Thanks to Reaktion editor Vivian Constantinopoulos for her early support of this project and for graciously extending me more time to finish it. Gratitude also goes to my research assistants, Rhea Fitzpatrick and Liza Schnauk, for their enthusiastic resourcefulness; to them both and to their compatriots in my autumn 2023 ‘Hypochondria’ class, an especially warm thanks for such an extraordinary semester of reading and learning together. I have benefitted greatly from the wise counsel of many friends who conversed with me about hypochondria, who read portions of this project in early stages and who helped me untangle some interpretive knots, including Barbara Black, G. Thomas Couser, Nick Junkerman, Michael Vincent Miller, Sarah Day-O’Connell, Jamie Parra, John Peabody, Lisa Schebetta-Jackson, Linda Simon, Mason Stokes, Susan Walzer and Martha Wiseman; and from audiences at two spring 2023 lectures at Skidmore College whose attentive presence aided in the process of refining my angle of approach. To my family – the ones who share my trepidations, the ones who cure and care, the ones who ache and struggle and endure: boundless love for all that you do to sustain me. This book, with its hopeful reach towards equanimity, is for you.

OceanofPDF.com

INDEX

- AIDS 183–4
Alzheimer's disorder 215
Asad, Talal 97–8
attribution error 189
Austen, Jane, *Emma* 66–7, 76–82, 127–9
- Barsky, Arthur J. 47, 209
Baur, Susan, *Hypochondria* 59–61, 75, 114, 123, 177, 180, 210
Beddoes, Thomas 54, 89–90
Belling, Catherine, *A Condition of Doubt* 8, 23, 41, 67, 114, 183, 205
Berlant, Lauren 27
Berrios, German E. 214
Boswell, James 42, 48–3, 167
Brontë, Charlotte, *Villette* 61–5
Burton, Robert, *The Anatomy of Melancholy* 42–5, 46, 153
- Cantor, Carla, *Phantom Illness* 19, 53–4, 59, 73–4, 188, 210
Catalonian hypochondria 158–9
Chambers, Thomas King 71
Cheyne, George 44, 55, 142
chrononormativity 75
Cicero, *De senectute* 226
Clare, Eli 205–6
Coleridge, Samuel Taylor 85–90, 191
confirmation bias 189
Connor, Steven 35, 127, 145
Cooper, Bernard 65–6
Couser, G. Thomas 83
COVID-19 68–9, 104–5, 137–9, 167–8, 181, 242
Creole dyspepsia 158–9
crip time 75–6, 105
cripistemology 118, 120–22
cyberchondria 251 n. 4
cytokines 47
- Damasio, Antonio R., *Descartes' Error* 119
Darwin, Charles 90–94
Davy, Humphry 89–90

Defoe, Daniel, *Journal of the Plague Year* 37–41, 146–7
 Deleuze, Gilles 132–3
 Diamond, John, *Because Cowards Get Cancer Too* 140–41
Diagnostic and Statistical Manual of Mental Disorders (DSM) 112, 115–16, 125
 dual-brain theory 232–3
 Dusenbery, Maya 179

 embodied cognition 117–19
 epistemic injustice 196–8

 Foucault, Michel, *The Birth of the Clinic* 21–2
 Freud, Sigmund 122–3, 169

 gaslighting, medical 181, 184–5, 193–8
 Gawande, Atul 226
 gerontology 208–9
 Gilman, Charlotte Perkins, ‘The Yellow Wallpaper’ 95
 glass delusion, 153
 Grinnell, George C. 54, 88–9, 147, 170
 Groopman, Jerome 189–90

 Hawkins, Harriet Hunsaker, *Reconstructing Illness* 73
 Herring, Scott 212–14
 Hutchinson, Robert 54, 67–8
 hypochondria
 and aging 33–5, 207–41
 as communication 24–6, 37–70
 and consolidation 30–32, 143–76
 and gender and racial bias 32–3, 159–60, 167–8, 177–206, 274 n. 23
 and ‘hateful patient’ 114, 246, 251 n. 6
 and health anxiety 17
 and knowledge 28–30, 107–41
 and pain 186–7, 201–5
 and psychiatry and psychoanalysis 124–6
 and time 26–8, 71–106
 hysteria 44, 123, 277 n. 58

 illness anxiety disorder 116–17
 International Classification of Diseases 112

 James, Alice 94
 James, Henry Sr 94–5, 96
 Jerome, Jerome K., *Three Men in a Boat* 175–6
 Johnson, Merri Lisa 120

Johnson, Samuel 48, 50, 255 n. 30

Jurecic, Ann 82

Jutel, Annemarie 180

Kafka, Franz 151–6

‘The Metamorphosis’ 152–3, 156

‘The Burrow’ 154–6, 221–4

Kant, Immanuel 177

Kingston, Maxine Hong, *The Woman Warrior* 161–7

Klein, Richard 21, 25

Kleinman, Arthur 139, 179, 204

Larkin, Philip, ‘The Old Fools’ 240

Lipsitt, Don R. 145

McRuer, Robert 120

Mantel, Hilary 140, 199–201

Martineau, Harriet 13–17, 96

Melville, Herman, ‘Bartleby, the Scrivener’ 133–6, 156

Merleau-Ponty, Maurice 118–19

Miller, D. A. 65, 78, 80

Mitchell, Silas Weir 95–6

Molière, *Le malade imaginaire* 107–11

Monopoli, John, and Frank Vaccaro 224

neurasthenia 13, 123, 165, 233

nostalgia 87–8

‘slave nostalgia’ 190–93

O’Rourke, Meghan 178, 199–200

pain 32, 47, 72, 87, 90, 92, 93, 98, 101, 111, 114, 120, 158, 174–5, 180, 185–9, 199, 201–5, 217–18

Plato 55–7

Poe, Edgar Allan, ‘The Fall of the House of Usher’ 129–31

positive aging movement 212–13, 225

psychiatry 121, 124–6, 192, 246

psychoanalysis 103, 123–5, 160, 231, 256 n. 55

racial-ethnic hypochondria 160

Rees, Will 112–13, 115, 132

Reid, John 71, 137, 245–6

‘Retrievals, The’ (Serial Productions) 201–5

Roth, Philip 160–61

Sarton, May, *At Eighty-Two* 216–20

Segal, Judy Z. 9, 57–8, 61, 183
Shelley, Mary, *The Last Man* 105, 147–51, 170–72
Small, Helen 213–14, 225–6
somatic symptom disorder 116–17
 somatization disorder 264 n. 18
Stein, Elliott M. 208–9
Steinem, Gloria 210
Stevenson, Robert Louis, *Strange Case of Dr Jekyll and Mr Hyde* 215, 227–36
Sydenham, Thomas 44–5
Synecdoche, New York 101–4

Traig, Jennifer, *Well Enough Alone* 72, 198

vaccination 99, 170–71

Wang, Fuson 170–71
Wilde, Oscar 227–8, 236
Wilson, Emily Paige, *Hypochondria, Least Powerful of the Greek Gods* 69–70
Winnicott, D. W. 173–4
Woodward, Kathleen 224–7
worried well 18, 181–2